

Auszug aus der UAW-Datenbank des BfArM

Stand der Datenbank: 07.03.2016

Einzelfallausdrucke

Ibuprofen: Inland, ab 1992, UAW schwere Hautreaktionen

Hinweise zur Interpretation der Daten:

Es handelt sich bei den Berichten sowohl um Spontanmeldungen als auch um Berichte aus systematisierten Untersuchungen, aus Projekten oder aus der Literatur. Die Meldungen sind dem Bundesinstitut für Arzneimittel und Medizinprodukte durch pharmazeutische Unternehmer, durch die Arzneimittelkommissionen der Heilberufe, direkt durch niedergelassene oder in Krankenhäusern tätige Ärzte sowie durch sonstige Meldequellen, z. B. von Patienten, berichtet worden. Es muss beachtet werden, dass es sich um Verdachtsfälle unerwünschter Arzneimittelwirkungen handelt, dass also ein Kausalzusammenhang im Einzelfall nicht sicher belegt ist. Ferner lassen sich auf der Grundlage von Spontanberichten keine Aussagen darüber machen, wie häufig eine bestimmte unerwünschte Wirkung unter der Anwendung eines Arzneimittels vorkommt, und auch vergleichende Angaben darüber, wie viel mal häufiger eine bestimmte unerwünschte Wirkung bei einem Arzneimittel im Verhältnis zu einem anderen Medikament auftritt, sind anhand solcher Berichte kaum möglich.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92900172

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 60 Year	weight (kg): 85	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme	19920303		3 Week		recovered/resolved
2 Erythema exudativum	19920303		3 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acerbon lisinopril	Oral	020 Mg milligram(s)/Day	-	Essential hypertension, unspecified
2 anco ibuprofen	Oral	002 Df dosage form/Day	19920221-19920303	Other unspecified back disorders

Drugs (concomitant)

3 munobal felodipine	Oral	001 Df dosage form/Day	19920221-	Essential hypertension, unspecified
4 nephral hydrochlorothiazide; triamterene	Oral	001 Df dosage form/Day	-	Essential hypertension, unspecified

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92006495

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 44 Year weight (kg): 84 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bullous eruption	19910702		3 Day		recovered/resolved with sequel
2 Thrombocytopenia	19910702				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Oral	600 Mg milligram(s)/	19910702- -	Cephalgia

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92006059

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 41 Year weight (kg): 85 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	19920714		4 Hour		recovered/resolved
2 Urticaria	19920714		4 Hour		recovered/resolved
3 Face oedema	19920714		4 Hour		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolgit 400 ibuprofen	Oral	400 Mg milligram(s)/	-	Neck pain
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified adverse effect of drug, medicament and biologi			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92008034

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 56 Year	weight (kg): 81	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19911003				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 asasantin acetylsalicylic acid; dipyridamole	Oral	002 Df dosage form/Day	-19911005	-
2 corvaton molsidomine	Oral	001 Df dosage form/Day	-19911005	-
3 dolo-dolgit ibuprofen	Oral	600 Mg milligram(s)/Day	19910930-19911002	-
4 gelonida na acetylsalicylic acid; codeine phosphate; paracetamol	Oral	001 Df dosage form/Day	19910928-19911001	-
5 isoptin mite verapamil hydrochloride	Oral	002 Df dosage form/Day	-19911005	-
6 uripurinol-300 allopurinol	Oral	002 Df dosage form/Day	-19911005	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hyperuricaemia			k.A.	concomitant disease
Alcoholic liver damage, unspecified			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93011112

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 12 Year weight (kg): 57 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin nodule	199307				recovered/resolved
2 Conjunctivitis	199306				recovered/resolved
3 Vasculitis					Unknown
4 Eye pain	199301				recovered/resolved
5 Lymphadenopathy	199307				recovered/resolved
6 Neoplasm NOS	199306				recovered/resolved
7 Skin discolouration	199306				recovered/resolved
8 Skin necrosis	199301				Unknown
9 Blepharitis					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 urem ibuprofen	Oral	/	- 1 -	Menses painful

Drugs (concomitant)

2 chloramphenicol chloramphenicol	Topical	/	199301- -	Other acne
3 ciprobay ciprofloxacin hydrochloride	Unknown	/	199301- -	
4 rulid roxithromycin	Unknown	/	199301- -	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93011742

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes						

Patient

Initials:	Sex: Female	Birth: privacy	Age: 42 Year	weight (kg): 63	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	19930728		21 Day		recovered/resolved
2 Enanthema	19930728		21 Day		recovered/resolved
3 Erythema multiforme	19930728		21 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dismenol n	Unknown	1 Df dosage form/	-	Dysmenorrhoea
ibuprofen			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93011100

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes No

Patient

Initials: Sex: Male Birth: privacy Age: 56 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19920918		1 Month		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin acetylsalicylic acid	Oral	/	-	Unspecified circulatory system disorder
2 cytotec misoprostol	Oral	/	19920902-	Unspecified functional disorder of stomach
3 defluina peri buflomedil hydrochloride	Oral	/	-	Unspecified circulatory system disorder
4 diclac diclofenac sodium	Oral	/	19920826-19920903	Unspecified arthropathy
5 ibuprofen ibuprofen	Oral	/	19920908-19920914	Unspecified arthropathy
6 indometacin indometacin	Oral	/	19920914-19920918	Unspecified arthropathy
7 methotrexat methotrexate	Oral	/	19920914-19920921	Unspecified arthropathy
8 penicillin benzylpenicillin sodium	Unknown	/	19920917-19920917	Erysipelas
9 rantudil acemetacin	Oral	/	19920903-19920907	Unspecified arthropathy

Drugs (concomitant)

10 acc 200 acetylcysteine	Oral	/	19920918-	Cough
11 agarol paraffin, liquid; phenolphthalein	Oral	/	19920918-	
12 augmentan amoxicillin trihydrate; clavulanic acid	Intravenous (not otherwis	/	19920918-19920921	Erysipelas
13 heparin heparin	Subcutaneous	/	19920918-	Thrombosis
14 valoron tilidine hydrochloride	Oral	/	-19920920	
15 voltaren diclofenac sodium	Intramuscular	/	-19920920	Unspecified arthropathy

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Other psoriasis			k.A.	concomitant disease
Diabetes mellitus without mention of complication			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95002417

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 doxycycline doxycycline	Unknown	/	-	-
2 erythromycin erythromycin	Unknown	/	-	-
3 tantum verde benzydamine hydrochloride	Buccal	/	-	-
4 urem forte ibuprofen	Unknown	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96016517

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 67 Year	weight (kg): 58	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19951023		14 Day		recovered/resolved
2 Stomatitis necrotizing	19951023		14 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 calcium calcium	Intravenous (not otherwis	001 Df dosage form/Day	19951020-19951022	Unspecified pruritic disorder
2 ibutop ibuprofen	Topical	/Day	19951019-19951021	Other general symptoms
3 muskel trancopal chlormezanone	Oral	002 Df dosage form/Day	19951019-19951021	Other general symptoms

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Fracture of metacarpal bones, closed			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97001281

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 39 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19940930				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ampho-moronal amphotericin b	Oral	/	-	-
2 ampho-moronal amphotericin b	Oral	/	-	-
3 aureomycin chlortetracycline	Ophthalmic	/	-	-
4 ben-u-ron paracetamol	Unknown	/	-	-
5 biklin amikacin sulfate	Intravenous (not otherwis	/	-	-
6 calcium brausetabletten calcium	Oral	/	-	-
7 diflucan 200 fluconazole	Oral	/	-	-
8 fortum ceftazidime	Intravenous (not otherwis	/	-	-
9 foscavir foscarnet sodium	Intravenous (not otherwis	/	-	-
10 gastrosil metoclopramide hydrochloride	Oral	/	-	-
11 ibuprofen 400 mg ibuprofen	Oral	/	-	-
12 kalinor-brausetabletten potassium carbonate; potassium citrate	Oral	/	-	-
13 klacid clarithromycin	Oral	/	-	-
14 musaril tetrazepam	Oral	/	-	-
15 novalgin tropfen metamizole sodium	Oral	/	-	-
16 sobelin soluble 900 clindamycin phosphate	Intravenous (not otherwis	/	-	-
17 tavegil clemastine	Intravenous (not otherwis	/	-	-
18 vancomycin vancomycin	Intravenous (not otherwis	/	-	-
19 vergentan alozapride hydrochloride	Intravenous (not otherwis	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97900143

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 76 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19960725				Unknown
2 Genital eruption female	19960710				Unknown
3 Erythema	19960710				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bromhexine hydrochloride	Oral	/	-	
bromhexine hydrochloride			-	
2 cephoral filmtabletten	Oral	/	-	
cefixime trihydrate			-	
3 imbun	Oral	/	-	
ibuprofen lysinate			-	
4 maninil	Oral	/	-	
glibenclamide			-	
5 muskel trancopal	Oral	/	-	
chlormezanone			-	
6 tethexal	Oral	/	-	
tetrazepam			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97002134

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 26 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19960726				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Unknown	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97002141

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth: privacy	Age: 84 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19961004				Unknown
2 Toxic epidermal necrolysis	19961004				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylcysteine	Intravenous (not otherwis /		-	
acetylcysteine			-	
2 bromuc	Oral	/	-	
acetylcysteine			-	
3 dipidolor	Intravenous (not otherwis /		-	
piritramide			-	
4 enantone monats-depot	Intravenous (not otherwis /		-	
leuporelin acetate			-	
5 gastrozepin	Oral	/	-	
pirenzepine dihydrochloride			-	
6 heparin	Subcutaneous	/	-	
heparin			-	
7 ibuprofen	Oral	/	-	
ibuprofen			-	
8 lasix	Intravenous (not otherwis /		-	
furosemide			-	
9 novalgin	Oral	/	-	
metamizole			-	
10 novodigal	Intravenous (not otherwis /		-	
beta-acetyldigoxin			-	
11 sterofundin	Intravenous (not otherwis /		-	
electrolytes nos			-	
12 tramal tropfen	Oral	/	-	
tramadol hydrochloride			-	
13 unacid	Intravenous (not otherwis /		-	
ampicillin sodium; sulbactam sodium			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97002158

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 28 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19930425				Unknown
2 Stevens Johnson syndrome	19930425				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin	Oral	/	-	
acetylsalicylic acid			-	
2 dismenol n 200	Oral	/	-	
ibuprofen			-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97006282

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 26 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin erythema	19960220				Unknown
2 Macular rash	19960220				Unknown
3 Stevens Johnson syndrome	19960220				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ampho-moronal amphotericin b	Oral	/	-	-
2 candio-hermal nystatin	Oral	/	-	-
3 emovate creme clobetasone butyrate	Topical	/	-	-
4 isocillin 1,2 mega phenoxymethylpenicillin potassium	Oral	/	-	-
5 otrin xylometazoline hydrochloride	Nasal	/	-	-
6 paracetamol paracetamol	Oral	/	-	-
7 penicillin benzylpenicillin sodium	Intravenous (not otherwis	/	-	-
8 tabalon ibuprofen	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97006355

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth: privacy	Age: 44 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell syndrome	19950610				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 agarol paraffin, liquid; phenolphthalein	Intravenous (not otherwis /		-	
2 aspisol acetylsalicylate lysine	Intravenous (not otherwis /		-	
3 bisacodyl bisacodyl	Unknown /		-	
4 ciprobay ciprofloxacin hydrochloride	Intravenous (not otherwis /		-	
5 cotrim forte sulfamethoxazole; trimethoprim	Oral /		-	
6 esprenit ibuprofen	Oral /		-	
7 fenistil dimetindene maleate	Intravenous (not otherwis /		-	
8 fortum ceftazidime	Intravenous (not otherwis /		-	
9 furorese injekt furosemide sodium	Intravenous (not otherwis /		-	
10 konakion phytomenadione	Intravenous (not otherwis /		-	
11 Kortison cortisone acetate	Oral /		-	
12 lasix furosemide	Intravenous (not otherwis /		-	
13 nepresol dihydralazine sulfate	Intravenous (not otherwis /		-	
14 novalgin metamizole	Intravenous (not otherwis /		-	
15 paracetamol paracetamol	Oral /		-	
16 pipril piperacillin sodium	Intravenous (not otherwis /		-	
17 reparil escin	Intravenous (not otherwis /		-	
18 rivotril clonazepam	Oral /		-	
19 theralene alimemazine tartrate	Oral /		-	
20 vancomycin vancomycin	Intravenous (not otherwis /		-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97007208

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug eruption					Unknown
2 Pyrexia abnormal					Unknown
3 White blood cell decreased					Unknown
4 Headache					Unknown
5 Thrombopenia					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Unknown	/	-	
ibuprofen			-	
2 paracetamol	Unknown	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98003482

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 29 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bronchospasm	19980210				Unknown
2 Conjunctivitis	19980202				Unknown
3 Wheals	19980210				Unknown
4 Flushing	19980203				Unknown
5 Rash urticarial	19980211				Unknown
6 Respiratory distress	19980203				Unknown
7 Circulatory disorder peripheral					Unknown
8 Throat tightness	19980203				Unknown
9 Facial flushing	19980210				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Acetylsalizylsaeure acetylsalicylic acid	Unknown	/	-	-
2 ibuprofen ibuprofen	Oral	/	-	-
3 novalgin metamizole	Unknown	/	-	-
4 piroxicam piroxicam	Unknown	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98003738

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 58 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19960401				Unknown
2 Toxic epidermal necrolysis	19960401				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuhexal 600	Oral	/	-	
ibuprofen			-	
2 Kodeinphosphat	Oral	/	-	
codeine phosphate			-	
3 paracetamol	Oral	/	-	
paracetamol			-	
4 thomapyrin	Oral	/	-	
acetylsalicylic acid; caffeine; paracetamol			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006114

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 24 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19971110				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolormin ibuprofen lysinate	Oral	/	-	
2 grippostad caffeine; diphenhydramine hydrochloride; hexethylamine citrate; propyphenazone; salicylamide	Oral	/	-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008085

Serious Yes	Death	Lifethr. No	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 28 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19980726				Unknown
2 Fever	19980726				Unknown
3 Exanthema	19980726				Unknown
4 Weight loss	19980726				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 avonex interferon beta-1a	Intramuscular	030 Mg milligram(s)/Day	199806-199807	Multiple sclerosis
2 imbun ibuprofen lysinate	Oral	/	-	Other general symptoms
3 planum temazepam	Oral	/	-	Sleep disturbances
4 tegretal 400 retard carbamazepine	Oral	003 Df dosage form/Day	199807-199808	Other trigeminal neuralgia
5 Urbason methylprednisolone	Oral	004 Mg milligram(s)/Day	-	Multiple sclerosis

Drugs (concomitant)

6 ergenyl 500 valproate sodium	Oral	001 Df dosage form/Day	-	Generalized nonconvulsive epilepsy
7 heparin heparin	Subcutaneous	/	-	Thrombosis
8 maaloxan aluminium hydroxide; aluminium hydroxide gel; magnesium hydroxide	Oral	080 ml millilitre(s)/Day	-	Chronic ulcer of skin, unspecified site
9 oviol desogestrel; ethinylestradiol	Oral	21 Df dosage form/	-	
10 paspertin metoclopramide hydrochloride	Oral	/	-	Nausea and vomiting
11 ranitidin 300 ranitidine hydrochloride	Oral	002 Df dosage form/Day	199806-199808	Unspecified gastritis and gastroduodenitis
12 tramal tramadol hydrochloride	Intravenous (not otherwis /		-	Other general symptoms
13 Urbason methylprednisolone	Intravenous (not otherwis	001 G gram(s)/Day	-	Multiple sclerosis
14 vitamin b complex calcium pantothenate; nicotinamide; pyridoxine hydrochloride; riboflavin; thiamine hydrochloride	Oral	004 Df dosage form/Day	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont. k.A.	Comment
Multiple sclerosis				concomitant disease

Primary source

Qualification:	Physician	observestudytype:
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Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008091

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 73 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19960721				Unknown
2 Stevens Johnson syndrome	19960721				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol 100 allopurinol	Oral	/	-	-
2 anco 400 ibuprofen	Oral	/	-	-
3 furosemid furosemide	Oral	/	-	-
4 maninil 1,75 glibenclamide	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008092

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 75 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Mucosal inflammation	19960727				Unknown
2 Exanthema	19960727				Unknown
3 Stevens Johnson syndrome	19960727				Unknown
4 Lymphocytopenia	19960807				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass 100 acetylsalicylic acid	Oral	/	-	-
2 esidrix hydrochlorothiazide	Oral	/	-	-
3 euglucon glibenclamide	Oral	/	-	-
4 ibuprofen ibuprofen	Oral	/	-	-
5 zyloric allopurinol	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008337

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Male Birth: privacy Age: 52 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19970317				Unknown
2 Otitis externa	19970307				Unknown
3 Facial swelling	19970308				Unknown
4 Discoloration skin	19970308				Unknown
5 Itching	19970314				Unknown
6 Skin disorder	19970317				Unknown
7 Necrolysis epidermal	19970317				Unknown
8 Erythroderma	19970317				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aniflazym serrapeptase	Oral	/	-	-
2 baycuten azidamfenicol; clotrimazole; dexamethasone acetate	Topical	/	-	-
3 clindamycin clindamycin	Oral	/	-	-
4 jellin-neomycin salbe fluocinolone acetonide; neomycin sulfate	Topical	/	-	-
5 novalgin tropfen metamizole sodium	Oral	/	-	-
6 optalidon special noc dragees caffeine; dihydroergotamine mesilate; propyphenazone	Oral	/	-	-
7 phenhydan phenytoin	Oral	/	-	-
8 tabalon ibuprofen	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008656

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 58 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19980228				Unknown
2 Fever	19980228				Unknown
3 Sore throat	19980228				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolo-puren granulat	Oral	/	-	
ibuprofen			-	
2 migralave n	Oral	/	-	
bucizine hydrochloride; paracetamol			-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001183

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 69 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19951026				Unknown
2 Leukocytosis	19951102				Unknown
3 Gamma GT increased	19951102				Unknown
4 SGPT increased	19951102				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 imbun	Oral	/	-	
ibuprofen lysinate			-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99900432

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 50 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	19980828				Unknown
2 Leucopenia	19980901				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 decortin-h prednisolone	Oral	/	-	
2 quensyl hydroxychloroquine sulfate	Oral	/	-	
3 togonal n gegen kopfschmerzen ibuprofen	Oral	/	-	

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002520

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 17 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	19980926				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bromhexine hydrochloride bromhexine hydrochloride	Oral	/	-	-
2 ibuprofen ibuprofen	Unknown	/	-	-
3 paspertin metoclopramide hydrochloride	Oral	/	-	-
4 vitaferro kapseln ascorbic acid; ferrous sulfate	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002554

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 60 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leucopenia	19990224				Unknown
2 Stevens Johnson syndrome	19990220				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibu 400 mg abz	Oral	/	-	
ibuprofen			-	
2 pirobeta 20	Oral	/	-	
piroxicam			-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99003325

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 10 Month weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Exanthema	19990805				Unknown
2 Blistering	19990805				Unknown
3 Discomposure marked	19990805				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 nurofen fuer kinder fiebersaft	Oral	/	-	
ibuprofen			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00000146

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 44 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Urticaria					Unknown
2 Drug eruption	19991207				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	/	-	
ibuprofen			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00006291

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Creatinine blood increased	1997				Unknown
2 Erythema multiforme	1997				Unknown
3 Erythema exudativum	1997				Unknown
4 BUN increased	1997				Unknown
5 Leukocytosis	1997				Unknown
6 Haematuria	1997				Unknown
7 Proteinuria	1997				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolormin	Oral	/	-	
ibuprofen lysinate			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00007105

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Necrosis skin					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprof von ct amp. ibuprofen	Intramuscular	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00007669

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	200007				Unknown
2 Erythema multiforme	200007				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 imbun	Oral	/	-	
ibuprofen lysinate			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01008529

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 44 Year	weight (kg): 85	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20000928	deathcause: Shock without mention of trauma
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000913		16 Day		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	ampho-moronal amphotericin b	Unknown	/	-	Other and unspecified mycoses
2	antra 40 mg omeprazole	Unknown	/	-	Chronic ulcer of skin, unspecified site
3	aredia pamidronate disodium	Intravenous (not otherwis	/	20000909-20000909	Hyperkalaemia
4	arterenol norepinephrine hydrochloride	Unknown	/	-	
5	beloc zok metoprolol succinate	Unknown	/	-	Cardiac dysrhythmia, unspecified
6	ben-u-ron paracetamol	Unknown	/	-	Other general symptoms
7	combactam 1 g sulbactam sodium	Unknown	/	-	Pneumonia, organism unspecified
8	dexamethasone dexamethasone	Unknown	/	-	Malignant neoplasm of liver, specified as secondary
9	diflucan 100 fluconazole	Unknown	/	-	Other and unspecified mycoses
10	fentanyl fentanyl	Unknown	/	-	
11	fragmin dalteparin sodium	Unknown	/	-	Embolism and thrombosis of unspecified site
12	heparin-natrium heparin sodium	Unknown	/	-	Embolism and thrombosis of unspecified site
13	ibuprofen ibuprofen	Unknown	/	-	Other general symptoms
14	kalinor-brausetabletten potassium carbonate; potassium citrate	Oral	/	-	Hypokalaemia
15	lasix injektionsloesung furosemide sodium	Unknown	/	-	Oedema
16	meronem meropenem	Unknown	/	-	Other specified septicaemias
17	methotrexat methotrexate	Unknown	/	-	Secondary malignant neoplasm of kidney
18	noctamid lormetazepam	Unknown	/	-	Sleep disturbances
19	noctazepam oxazepam	Unknown	/	-	
20	novalgin tropfen metamizole sodium	Oral	/	-	Pyrexia of unknown origin
21	pipril 4 g piperacillin sodium	Intravenous (not otherwis	004 G gram(s)/Day	20000909-20000911	Pneumonia, organism unspecified
22	tazobac piperacillin sodium; tazobactam sodium	Unknown	/	-	Diarrhoea
23	udicil cytarabine	Unknown	/	-	Other malignant lymphomas
24	valdispert valerian extract	Unknown	/	-	
25	zyloric 300 allopurinol	Oral	/	-	Hyperuricaemia

Drugs (concomitant)

26	nutriflex	Unknown	/	-
	amino acids nos; electrolytes nos; glucose			-
27	soluvit	Unknown	/	-
	vitamins nos			-
28	tavanic 500	Unknown	/	-
	levofloxacin			-
29	voltaren emulgel	Topical	/	-
	diclofenac diethylamine			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Renal failure, unspecified			k.A.	concomitant disease
Colitis, enteritis, and gastroenteritis of presumed infectious			k.A.	concomitant disease
Atrial fibrillation and flutter			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01009052

Serious Yes	Death	Lifethr. No	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 39 Year	weight (kg): 79	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell syndrome	20010729		21 Day		recovered/resolved
2 Rheumatism	20010729		21 Day		recovered/resolved
3 Eczema	20010729		21 Day		recovered/resolved
4 Elevated sedimentation rate	20010729		21 Day		recovered/resolved
5 Leukocytosis	20010729		21 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 felden piroxicam	Intramuscular	20 Mg milligram(s)/	20010728-20010728	Lumbago
2 ibuprofen ibuprofen	Unknown	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01010453

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 28 Year	weight (kg): 70	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20010602				recovered/resolved
2 Cramp abdominal	20010527				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolormin	Oral	/	-	Other general symptoms
ibuprofen lysinate			-	
2 doxy-wolff 200	Oral	/	-	Acute tonsillitis
doxycycline hyclate			-	
3 karex-wolff 650	Oral	/	-	Acute tonsillitis
erythromycin stinoprate			-	
4 paracodin	Oral	/	-	
dihydrocodeine bitartrate			-	
5 penicillin von ct	Oral	/	-	Acute tonsillitis
phenoxymethylpenicillin			-	
6 zithromax 250 mg filtabletten	Oral	/	-	Acute tonsillitis
azithromycin			-	

Drugs (concomitant)

7 Multivitamine	Oral	/	-
ascorbic acid; ergocalciferol; folic acid; nicotinamide; panthenol; retinol; riboflavin; thiamine hydrochloride			

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Allergic rhinitis, cause unspecified			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01010459

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 51 Year	weight (kg): 83	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000413		19 Day		recovered/resolved
2 Oedema NOS					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron paracetamol	Rectal	/	-	Other general symptoms
2 cephalexin cefalexin	Oral	/	-	Unspecified otitis media
3 ibuprofen 400 ibuprofen	Oral	/	-	Other general symptoms

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02004241

Serious Yes	Death	Lifethr. Yes	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 44 Year	weight (kg): 76	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 CRP positive	20020305				not recovered/not resolved
2 Stevens Johnson syndrome	20020305				not recovered/not resolved
3 Fever	20020305				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ciprofloxacin	Oral	/	-	Bronchitis, not specified as acute or chronic
ciprofloxacin			-	
2 ibuprofen	Oral	/	-	Bronchitis, not specified as acute or chronic
ibuprofen			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02004587

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 39 Year	weight (kg): 78	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Oedema legs	20020226				not recovered/not resolved
2 Skin vasculitis NOS	20020226				not recovered/not resolved
3 SGPT increased	20020226				not recovered/not resolved
4 Vascular purpura	20020226				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuflam 600 lichtenstein ibuprofen	Oral	600 Mg milligram(s)/Day	20020224-20020225	Arthralgia
			-	

Drugs (concomitant)

2 mtx hexal methotrexate sodium	Oral	13 Mg milligram(s)/Week	-	Psoriatic arthropathy
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Atopy			k.A.	concomitant disease
Allergic rhinitis due to pollen			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02005477

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 51 Year	weight (kg): 97	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000706		19 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen 600 mg ibuprofen	Oral	/	-	
2 musaril tetrazepam	Oral	/	-	Low back pain
3 valoron n naloxone hydrochloride; tilidine hydrochloride	Oral	/	-	Enthesopathy of elbow region

Drugs (concomitant)

4 concor 5 bisoprolol fumarate	Oral	/	-	Hypertension arterial
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Displacement of intervertebral disc, site unspecified, without			k.A.	concomitant disease
Presbyopia			k.A.	concomitant disease
Dermatophytosis of groin and perianal area			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-02005489

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Initials: Sex: Male Birth: privacy Age: 64 Year weight (kg): 69 BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	20000830		20 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ambene aluminium glycinate; cyanocobalamin; dexamethasone; phenylbutazone; thiamine hydrochloride	Subcutaneous	/	- -	Low back pain
2 ibutad 800 retard ibuprofen	Oral	/	- -	Low back pain

MedDRA-term	Begin	End	Cont.	Comment
Behcet's syndrome			k.A.	concomitant disease
Acute nasopharyngitis (common cold)			k.A.	concomitant disease

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02005504

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 51 Year	weight (kg): 72	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000624		18 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc 200 akut acetylcysteine	Oral	/	-	Cough
2 ibuprofen ibuprofen	Oral	/	-	
3 Kodeinphosphat codeine phosphate	Oral	/	-	
4 timonil carbamazepine	Oral	/	-	Epilepsy, unspecified
5 vioxx rofecoxib	Oral	/	-	Other general symptoms
6 zithromax azithromycin	Oral	/	-	Acute bronchitis

Drugs (concomitant)

7 ampho-moronal suspension amphotericin b	Oral	/	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Malignant neoplasm of brain, unspecified			k.A.	concomitant disease
Hypertension arterial			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02007741

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 29 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug eruption	20020531				recovered/resolved
2 Headache	20020531				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolormin ibuprofen lysinate	Oral	001 Df dosage form/Day	20020529- -	Headache

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03000983

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	Yes	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 21 Year	weight (kg): 80	BMI_int: 26.73	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell syndrome	20030113				recovered/resolved
2 Erythroderma	20030111				recovered/resolved
3 Stevens Johnson syndrome	2003			154 Day	recovered/resolved with sequel

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 grippostad caffeine; diphenhydramine hydrochloride; hexethylamine citrate; propyphenazone; salicylamide	Oral	001 G gram(s)/Day	20030112-20030113 2 Day	Upper respiratory infection
2 IBUPROFEN ibuprofen	Oral	800 Mg milligram(s)/Day	20030112-20030113 2 Day	Upper respiratory infection
3 TAMIFLU oseltamivir	Oral	001 Df dosage form/Day Capsule	20030112-20030113 2 Day	Upper respiratory infection
4 umckaloabo pelargonium sidoides root	Unknown	/	20030111-20030112 2 Day	Fever

Drugs (concomitant)

5 ASS acetylsalicylic acid	Unknown	/	- -	Influenza
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pneumonia			Unknown	
Erythroderma			Unknown	
Respiratory infection			Unknown	
Pulmonary infiltration			Unknown	
Fever			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Lawyer	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

ROCHE, 20.01.2006 329349

Sender diagnosis: not reported

Narrative

THIS SPONTANEOUS CASE WAS REPORTED BY 3 PHYSICIANS AND THE ARZNEIMITTELKOMMISSION DER DEUTSCHEN AERZTESCHAFT (GERMANY) VIA THE BUNDESINSTITUT FÜR ARZNEIMITTEL UND MEDIZINPRODUKTE (GERMANY) AS WELL AS A LAWYER, AND CONCERNS A 21-YEAR-OLD FEMALE PATIENT WHO DEVELOPED 'STEVENS-JOHNSON DISEASE', WHILE RECEIVING OSELTAMIVIR (TAMIFLU), IBUPROFEN AND PARACETAMOL (GRIPPOSTAD) FOR RESPIRATORY INFECTION AND GERANIUM (UMCKALOABO) FOR FEVER AND GRIPPAL INFECTION.

11 JAN 2003: THE PATIENT STARTED GERANIUM.

12 JAN 2003: THE PATIENT COMMENCED OSELTAMIVIR (75 MG, DAILY, ORALLY), IBUPROFEN (800 MG, DAILY, ORALLY) AND PARACETAMOL (1 GM, DAILY, ORALLY). PARACETAMOL WAS TAKEN AS A COMPOUND MEDICATION WITH CAFFEINE (COFFEIN) AND

CHLORPHENAMINE (CHLORPHENAMIN). AFTER TAKING THE SECOND CAPSULE OF OSELTAMIVIR, THE PATIENT DEVELOPED AN EXANTHEMA, WHICH COVERED MORE THAN 50% OF THE SKIN. SHE HAD MORE THAN 30 DISSEMINATED LESIONS, WHICH INCLUDED MUCOSAL AND NAIL/HAIR LESIONS. HER SYMPTOMS INCLUDED BURNING OR PAIN, FEVER, INFECTION AND HYPOTENSION. PHOTSENSITIVITY WAS NOT SUSPECTED. UPON ADMISSION THE PATIENT WAS IN ACUTE REDUCED GENERAL CONDITION, RESTLESS AND EXPERIENCED SMALL MACULAR EXANTHEMA OF THE FACE, NECK AND UPPER THORAX, CHIMOSIS OF EYES, RED TONGUE AND TACHYCARDIA. THE PATIENT WAS TREATED WITH PREDNISOLONE (SOLUDECORTIN) IV, CLEMASTINE (TAVEGYL), IV, 1 AMPULE AS SHORT INFUSION, ONE TAKEN IN THE MORNING AND ONE TAKEN AT NIGHT AND STEROFUNDIN (ELECTROLYTES), IV, 4 X500ML. THE EXANTHEMA IMPROVED. HOWEVER, IT APPEARED AFTER AN HOUR AGAIN. IN ADDITION, THE PATIENT DEVELOPED PAINFUL STOMATITIS AS WELL AS PAIN IN THE URETHRA. IT WAS REPORTED THAT HAEMOGRAM, COAGULATION PARAMETERS, ELECTROLYTES, CREATININE AND UREA WERE UNSUSPICIOUS. BLOOD CULTURE SHOWED ANTINUCLEAR ANTIBODY AND ANTINEUTROPHIL CYTOPLASMIC ANTIBODY TO BE NEGATIVE. URINE CULTURE SHOWED PROTEIN MIRABILIS AND E.COLI. LAB TESTS FOR C-REACTIVE PROTEIN, GOT, LDH, LEUCOCYTES, URINE GLUCOSE, KETONE BODIES AND ERYTHROCYTES WERE PERFORMED. ECG SHOWED SINUS RHYTHM AT 106 BPM, NORMAL TYPE, NO SIGNIFICANT REPOLARISATION DISORDER. PATIENT WAS TAKEN TO THE INTENSIVE CARE UNIT WHERE SHE WAS INITIALLY DIAGNOSED WITH LYELL'S SYNDROME. THE TYPE OF SKIN DISORDER WAS A BULLOUS OR VESICULAR ERUPTION SPECIFIED AS TOXIC EPIDERMAL NECROLYSIS. THE VESICULATION AND EPIDERMOLYSIS INVOLVED THE SKIN AND MUCOSA OF THE HEAD, ARMS AND TRUNK AND WITH SYNECHIA OF THE CONJUNCTIVA THE PATIENT WAS GIVEN 5 GM OF GAMMA GLOBULINS (IMMUNOGLOBULINS) IV. ENDOTRACHEAL INTUBATION AND TRACHEOTOMY WERE CARRIED OUT BECAUSE OF PULMONARY IMPAIRMENT. THE PATIENT ALSO RECEIVED MOXIFLOXACIN HYDROCHLORIDE (AVALOX), CLARITHROMYCIN AND NOREPINEPHRINE HYDROCHLORIDE (ARTERENOL). GERANIUM WAS DISCONTINUED.

13 JAN 2003: OSELTAMIVIR, IBUPROFEN AND PARACETAMOL WERE DISCONTINUED.

14 JAN 2003: THE PATIENT WAS HOSPITALISED IN THE ICU WITH WHAT APPEARED TO BE LYELL'S SYNDROME. THE GENERAL CONDITION OF THE PATIENT WORSENE. THERE WAS A FAST PROGRESSION OF BULLOUS ERUPTION WITH ALMOST COMPLETE DETACHMENT OF SKIN IN THE AREA OF THE HEAD AND THE UPPER BODY DOWN TO THE GROIN. THE PATIENT WAS TRANSFERRED TO ANOTHER UNIT. CHEST X-RAY SHOWED AN INFILTRATION IN THE LEFT LUNG (RETROSPECTIVELY DIAGNOSED).

15 JAN 2003: THE PATIENT DEVELOPED WORSENING RESPIRATORY FUNCTION AND WAS THEREFORE INTUBATED.

17 JAN 2003: A TRACHEOTOMY WAS PERFORMED. THE PATIENT EXPERIENCED SEVERE INVOLVEMENT OF ALL MUCOUS MEMBRANES (BRONCHIAL, GASTRO-INTESTINAL AND VAGINAL) AS WELL AS OF THE CONJUNCTIVAE. DURING FURTHER COURSE, THE GENERAL CONDITION OF THE PATIENT STABILIZED AND SHE COULD BE EXTUBATED AGAIN.

DATES UNKNOWN: A PLATELET COUNT, LDH AND CRP LEVEL WERE OBTAINED.

29 JAN 2003: CLARITHROMYCIN AND MOXIFLOXACIN WERE DISCONTINUED.

UNKNOWN DATE: THE PATIENT DEVELOPED VAGINAL CANDIDOSIS WHICH WAS TREATED WITH FLUCONAZOLE (FLUCONAZOL), IV.

07 FEB 2003: THE VAGINAL CANDIDOSIS WAS TREATED WITH CLOTRIMAZOLE (CLOTRIMAZOL) CREAM.

10 FEB 2003: THE PATIENT WAS ADMITTED TO THE HOSPITAL UNIT AGAIN. GENERAL CONDITION WAS STILL REDUCED. EXAMINATION REVEALED DISTINCT ERYTHRODERMIA WITH EXANTHEMA AROUND THE EYES, ON THE BACK, THORAX, LOWER ABDOMEN DOWN TO THE GROIN AS WELL AS ON THE EXTREMITIES AND NO SYNECHIAS IN THE EYES. THE PATIENT FELT TOO SLOW AND HAD SHORT MEMORY DISORDER. THE LAB VALUES SHOWED DISTINCTLY ELEVATED CHOLESTASIS PARAMETERS AND A CHOLECYSTOLITHIASIS WAS DIAGNOSED. COMPARED TO THE FORMER LAB VALUES A DISTINCT REGRESSION WAS SEEN. THE ELEVATED CHOLESTASIS PARAMETERS WERE SUSPECTED TO BE DUE TO THE PARENTAL FEEDING DURING THE LONG LASTING INTUBATION. DURING THE FURTHER HOSPITAL COURSE, THE GENERAL CONDITION OF THE PATIENT CONTINUOUSLY IMPROVED.

ON AN UNSPECIFIED DATE IN 2003, THE PATIENT'S DIAGNOSIS WAS CHANGED FROM LYELL'S SYNDROME TO STEVENS-JOHNSON'S DISEASE WHICH REQUIRED HOSPITALISATION AND WAS CAUSING A PERSISTENT DISABILITY TO HER EYES.

UNKNOWN DATE: BLOOD COUNT WAS UNSUSPICIOUS. LAB TEST FOR BILIRUBIN, SGOT, GPT, ALKALINE PHOSPHATASE, AMYLASE, CHOLINESTERASE, LIPASE AND POTASSIUM WERE PERFORMED. LAB TEST FOR SODIUM, CALCIUM, RENAL AND COAGULATION PARAMETERS WERE NORMAL. ECG REVEALED SINUS RHYTHM, VERTICAL HEART POSITION, RS IN V3 AND V4 AND NO REPOLARISATION. CHEST X-RAY SHOWED NO PATHOLOGICAL FINDINGS. ABDOMINAL ULTRASOUND SHOWED THE WALL OF THE GALLBLADDER THICKENED TO 0.5CM WHICH RESEMBLED CHRONIC CHOLECYSTITIS, THREE GALL STONES AND METEORISM WERE ALSO SHOWN. CONTROL ULTRASOUND SHOWED SIMILAR FINDINGS.

18 FEB 2004: AN NMR IMAGING REVEALED NO PATHOLOGICAL FINDINGS FOR THE DUTUS CHOLEDOCHUS.

21 FEB 2004: THE PATIENT WAS DISCHARGED HOME WITH CLOTRIMAZOLE (CANESTEN CRÄME), THREE TIMES A DAY, OFLOXACIN (FLOXAL) EYE DROPS THREE TIMES A DAY, DEXAMETHASONE (DEXASINE) EYE DROPS TID AND DEXPANTHENOL (BEPANTHENOL) 'TONGUE' 5 TIMES DAILY. ON AN UNSPECIFIED DATE THE STEVENS-JOHNSON'S DISEASE IMPROVED AND SUBSEQUENTLY RESOLVED WITH SEQUELAE.

AT THE TIME OF THIS REPORT, A SKIN BIOPSY HAD NOT BEEN PERFORMED AND IT WAS STATED THAT THE FIRST CLINICAL SIGNS OF ERYTHRODERMIA AND VESICULATION WERE DOUBTFULLY APPEARING BEFORE THE START OF OSELTAMIVIR. A PHYSICIAN LISTED THE INITIAL DIAGNOSIS OF LYELL'S SYNDROME AS RELATED TO OSELTAMIVIR, WHILST THE LAWYER LISTED THE EVENT OF STEVENS-JOHNSON DISEASE AS RELATED TO THE USE OF OSELTAMIVIR. IT WAS ALSO REPORTED THAT PARACETAMOL AND ASPIRIN (ASS) COULD BE SUSPECTED. THE REPORTER STATED THAT IBUPROFEN, PARETAMOL, CHLORPHENAMINE AND OSELTAMIVIR WERE REPORTED AS POSSIBLE TRIGGERS FOR THE INITIAL DIAGNOSIS OF LYELL'S SYNDROME. THE DIAGNOSIS OF INFLUENZA WAS NOT CONFIRMED BY A LABORATORY TEST. DOSES, ROUTES AND REGIMENS WERE NOT PROVIDED UNLESS OTHERWISE SPECIFIED. SOME OF THE ABOVE INFORMATION WAS OBTAINED FROM A DISCHARGE LETTER.

UPDATE INFORMATION WAS RECEIVED ON 09 JANUARY 2005. THE FOLLOWING INFORMATION WAS ADDED TO THE CASE. EVENT CHANGED FROM LYELL'S SYNDROME TO STEVENS-JOHNSON DISEASE, UPDATED LABELLING AND SERIOUSNESS CRITERIA. ALSO AMENDED THIS TO A LEGAL CASE AND UPDATED TEXT AS WELL AS ADDING NEW REPORTER DETAILS.

*** DATA ELEMENTS THAT WERE TRUNCATED NOW FOLLOW ***

<ichicsr.safetyreport.patient.test(testname=10005485)>

<testresult>

STERILE

SHOWED ANTINUCLEAR ANTIBODY AND ANTINEUTROPHIL CYTOPLASMIC ANTIBODY TO BE NEGATIVE

</testresult>

<ichicsr.safetyreport.patient.test(testname=10008498)>

<testresult>

SHOWED AN INFILTRATION IN THE LEFT LUNG (RETROSPECTIVELY DIAGNOSED).

</testresult>

<ichicsr.safetyreport.patient.test(testname=10014084)>

<testresult>

SHOWED SINUS RHYTHM AT 106 BPM, NORMAL TYPE, NO SIGNIFICANT REPOLARISATION DISORDER.

</testresult>

<ichicsr.safetyreport.patient.test(testname=10014084)>

<testresult>

REVEALED SINUS RHYTHM, VERTICAL HEART POSITION, RS IN V3 AND V4 AND NO REPOLARISATION.

</testresult>

<ichicsr.safetyreport.patient.test(testname=10049506)>

<testresult>

HAEMOGRAM, COAGULATION PARAMETERS AND ELECTROLYTES WERE UNSUSPICIOUS.

</testresult>

<ichicsr.safetyreport.patient.test(testname=10023653)>

<testresult>

388 RESULTS OBTAINED UPON ADMISSION.

UNITS REPORTED AS U/L.

</testresult>

<ichicsr.safetyreport.patient.test(testname=10029815)>

<testresult>

REVEALED NO PATHOLOGICAL FINDINGS FOR THE DUTUS CHOLEDOCHUS.

</testresult>

<ichicsr.safetyreport.patient.test(testname=10052038)>

<testresult>

SHOWED THE WALL OF THE GALLBLADDER THICKENED TO 0.5CM WHICH RESEMBLED CHRONIC CHOLECYSTITIS, THREE GALL STONES AND METEORISM WERE ALSO SHOWN.

</testresult>

Sender's comment:

CIOMS TEXT: STEVENS-JOHNSON DISEASE IS LISTED IN THE CDS FOR OSELTAMIVIR. BASED ON THE INFORMATION RECEIVED FOR THIS CASE, A CAUSAL RELATIONSHIP BETWEEN THE REPORTED EVENT AND TREATMENT WITH OSELTAMIVIR IS ASSESSED AS POSSIBLE. COMPANY CAUSALITY WAS ASSESSED BASED ON THE NARANJO ALGORITHM.

HOWEVER, A POSSIBLE ALTERNATIVE EXPLANATION COULD INCLUDE THE CONCOMITANT USE OF IBUPROFEN.; ***LOCAL

ASSESSMENT: STEVENS-JOHNSON DISEASE = LABELED

ALTERNATIVE EXPLANATION: CONCOMITANT USE OF IBUPROFEN

BFRAM NO.: 030009383

EINZELFALLBEOBACHTUNG. KEIN HANDLUNGSBEDARF!

Reporter's comment:ROCHE, 04.01.2007 329349

Sender diagnosis: not reported

Narrative

THIS SPONTANEOUS CASE WAS REPORTED BY 5 PHYSICIANS AND THE ARZNEIMITTELKOMMISSION DER DEUTSCHEN AERZTESCHAFT (GERMANY) VIA THE BUNDESINSTITUT FÜR ARZNEIMITTEL UND MEDIZINPRODUKTE (GERMANY) AS WELL AS A LAWYER, AND CONCERNS A 21-YEAR-OLD FEMALE PATIENT WHO DEVELOPED 'STEVENS-JOHNSON DISEASE', TRANSAMINASES INCREASED, PNEUMONIA, CANDIDA INFECTION OF VAGINA, SYNECHIA, CLONING OF FEET, FLEXION SPASMS OF FEET, FALL, CRITICAL ILLNESS MYOPATHY, CHOLESTEROL INCREASED, TRIGLYCERIDES INCREASED, OCULAR SCAR PEMPHIGOD, SYMBLEPHARON, TRICHIASIS, CONJUNCTIVITIS, EXANTHEMA, CHOLELITHIASIS, VERTIGO AND HYPOTENSION WHILE/AFTER RECEIVING OSELTAMIVIR (TAMIFLU), IBUPROFEN AND PARACETAMOL (GRIPPOSTAD) FOR RESPIRATORY INFECTION AND GERANIUM (UMCKALOABO) FOR FEVER AND GRIPPAL INFECTION.

ON 10 JAN 2003, THE PATIENT EXPERIENCED FEVER UP TO 40°C AS WELL AS HEADACHE AND LIMB PAIN. SHE STARTED ORAL IBUPROFEN (800MG PER DAY) AND ON THE NEXT DAY, THE PATIENT STARTED GERANIUM. HER CONDITION DID NOT IMPROVE. 12 JAN 2003: IN THE MORNING SHE STARTED TO EXPERIENCE EXANTHEMA OF THE FACE AND HER FEVER WAS PERSISTING. SHE WENT TO THE EMERGENCY DOCTOR. THE PATIENT COMMENCED OSELTAMIVIR (75 MG, DAILY, ORALLY) AND PARACETAMOL (1 GM, DAILY, ORALLY). PARACETAMOL WAS TAKEN AS A COMPOUND MEDICATION WITH CAFFEINE (COFFEIN) AND CHLORPHENAMINE (CHLORPHENAMIN). AFTER TAKING THE SECOND CAPSULE OF OSELTAMIVIR, THE PATIENT DEVELOPED AN EXANTHEMA, WHICH COVERED MORE THAN 50% OF THE SKIN. SHE HAD MORE THAN 30 DISSEMINATED LESIONS, WHICH INCLUDED MUCOSAL AND NAIL/HAIR LESIONS. HER SYMPTOMS INCLUDED BURNING OR PAIN, FEVER, INFECTION AND HYPOTENSION. PHOTOSENSITIVITY WAS NOT SUSPECTED. UPON ADMISSION THE PATIENT WAS IN ACUTE REDUCED GENERAL CONDITION, RESTLESS AND EXPERIENCED SMALL MACULAR EXANTHEMA OF THE FACE, NECK AND UPPER THORAX, CHIMOSIS OF EYES, RED TONGUE AND TACHYCARDIA. THE PATIENT WAS TREATED WITH 250MG PREDNISOLONE (SOLUDECORTIN) IV, CLEMASTINE (TAVEGYL), IV, 1 AMPULE AS SHORT INFUSION, ONE TAKEN IN THE MORNING AND ONE TAKEN AT NIGHT AND STEROFUNDIN (ELECTROLYTES), IV, 4 X 500ML. THE EXANTHEMA IMPROVED. HOWEVER, IT APPEARED AFTER AN HOUR AGAIN. IN ADDITION, THE PATIENT DEVELOPED PAINFUL STOMATITIS AS WELL AS PAIN IN THE URETHRA. IT WAS REPORTED THAT HAEMOGRAM, COAGULATION PARAMETERS, ELECTROLYTES, CREATININE AND UREA WERE UNSUSPICIOUS. BLOOD CULTURE SHOWED ANTINUCLEAR ANTIBODY AND ANTINEUTROPHIL CYTOPLASMIC ANTIBODY TO BE NEGATIVE. URINE CULTURE SHOWED PROTEIN MIRABILIS AND E.COLI. ECG SHOWED SINUS RHYTHM AT 106 BPM, NORMAL TYPE, NO SIGNIFICANT REPOLARISATION DISORDER. PATIENT WAS TAKEN TO THE INTENSIVE CARE UNIT WHERE SHE WAS INITIALLY DIAGNOSED WITH LYELL'S SYNDROME. THE TYPE OF SKIN DISORDER WAS A BULLOUS OR VESICULAR ERUPTION SPECIFIED AS TOXIC EPIDERMAL NECROLYSIS. THE VESICULATION AND EPIDERMOLYSIS INVOLVED THE SKIN AND MUCOSA OF THE HEAD, ARMS AND TRUNK AND WITH SYNECHIA OF THE CONJUNCTIVA THE PATIENT WAS GIVEN 5 GM OF GAMMA GLOBULINS (IMMUNOGLOBULINS) IV. ENDOTRACHEAL INTUBATION AND TRACHEOTOMY WERE CARRIED OUT BECAUSE OF PULMONARY IMPAIRMENT. THE PATIENT ALSO RECEIVED MOXIFLOXACIN HYDROCHLORIDE (AVALOX), CLARITHROMYCIN AND NOREPINEPHRINE HYDROCHLORIDE (ARTERENOL). GERANIUM WAS DISCONTINUED. 13 JAN 2003: OSELTAMIVIR, IBUPROFEN AND PARACETAMOL WERE DISCONTINUED. 14 JAN 2003: THE PATIENT WAS HOSPITALISED IN THE ICU WITH WHAT APPEARED TO BE LYELL'S SYNDROME. THE PATIENT HAD PROGRESSING CONFLUENT RED MACULAE ON HER UPPER LEGS, LOWER ARMS, LOWER LEGS, EXCORIATIONS IN LARGE AREAS OF THE FACE AND NECK AND FORMATION OF SEROUS BULLAE ON THE FRONT TRUNK. UPON ADMISSION THE PATIENT'S LABORATORY VALUES WERE: HAEMOGLOBIN 14.1 G/DL, THROMBOCYTES 121000/MCL, LEUKOCYTES 7900/MCL, GAMMA-GLUTAMYL TRANSFERASE (GGT) 22 U/L, GLUTAMATE PYRUVATE TRANSAMINASE (GPT) 28U/L, GLUTAMIC OXALOACETIC TRANSAMINASE (GOT) 28U/L, LACTATE DEHYDROGENASE 511 U/L. ON 15 JANUARY 2003 INVESTIGATION OF A PUNCTURE SPECIMEN OF A BULLA OF THE SKIN REVEALED NUMEROUS LYMPHOCYTES, GRANULOCYTES, PLASMA CELLS AND MACROPHAGES IN THE EXUDATE. THE GENERAL CONDITION OF THE PATIENT WORSENER AFTER ADMISSION. THERE WAS A SIGNIFICANT SHIFT TO THE LEFT OF THE DIFFERENTIAL BLOOD COUNT WITHOUT LEUKOCYTOSIS (LESS THAN 10000/MCL), HAEMOGLOBIN REDUCED TO LESS THAN 8G/DL. THE PATIENT WAS GIVEN A TOTAL OF EIGHT UNITS CONCENTRATED ERYTHROCYTES. C-REACTIVE PROTEIN INCREASED UP TO 18.2MG/DL, LACTATE DEHYDROGENASE INCREASED UP TO 511U/L. THERE WAS A TRANSIENT HYPERLIPASAEMIA UP TO 632 U/L. CHOLESTASIS PARAMETERS AND TRANSAMINASES INCREASED: GGT 943U/L, GPT 241U/L, GOT 131U/L AND ALKALINE PHOSPHATASE 408U/L. THERE WAS A FAST PROGRESSION OF BULLOUS ERUPTION WITH ALMOST COMPLETE DETACHMENT OF SKIN IN THE AREA OF THE HEAD AND THE UPPER BODY DOWN TO THE GROIN. THE PATIENT WAS TRANSFERRED TO ANOTHER UNIT. CHEST X-RAY SHOWED INFILTRATIONS IN THE LEFT AND RIGHT LUNG AND PNEUMONIA WAS RETROSPECTIVELY

DIAGNOSED. 15 JAN 2003: THE PATIENT DEVELOPED WORSENING RESPIRATORY FUNCTION AND WAS THEREFORE INTUBATED. SHE STARTED TREATMENT WITH INTRAVENOUS 7-S-IMMUNOGLOBULINS (1G/KG/DAY). THE SAME DAY EARLY SIGNS OF SYNECHIAS COULD BE SEEN IN OPHTHALMOLOGICAL EXAMINATIONS. HER CORNEA WERE SMOOTH AND A FUNDOSCOPY SHOWED PAPILLA WERE SHARPLY DEFINED AND VITAL. 16 JAN 2003 A BRONCHOSCOPY REVEALED LARGE AREAS WITH MUCOUS MEMBRANE LESIONS IN THE ENTIRE BRONCHIAL SYSTEM. ACUTE INFECTION, NO PUS, NO ACTIVE BLEEDING SOURCE. LAVAGE OF LINGULA: ROSE COLOURED FLUID. 17 JAN 2003: A TRACHEOTOMY WAS PERFORMED. THE PATIENT EXPERIENCED SEVERE INVOLVEMENT OF ALL MUCOUS MEMBRANES (BRONCHIAL, GASTRO-INTESTINAL AND VAGINAL) AS WELL AS OF THE CONJUNCTIVAE. A BRONCHOALVEOLAR LAVAGE REVEALED NO AEROBIC OR ANAEROBIC BACTERIA, NO ACID RESISTANT RODS, NO DETECTION OF PNEUMOCYSTIS CARINII NOR CYTOMEGALY VIRUS ANTIGEN. PCR WAS NEGATIVE FOR CHLAMYDIA PNEUMONIA AND MYCOBACTERIUM TUBERCULOSIS. NO LEGIONELLA WERE FOUND. DURING FURTHER COURSE, THE GENERAL CONDITION OF THE PATIENT STABILIZED AND SHE COULD BE EXTUBATED AGAIN. IN THE FURTHER COURSE CHEST X-RAYS CONFIRMED REGRESSION OF INFILTRATES AND DETECTED A SLIGHT PLEURAL EFFUSION ON THE LEFT SIDE. INCREASE OF FORMATION OF SYNECHIAS WERE NOTED TEMPORALLY ON BOTH SIDES. NORMAL TENSION WAS SEEN ON BOTH SIDES UPON PALPITATION. ON 20 JAN 2003, TREATMENT WITH INTRAVENOUS 7-S-IMMUNOGLOBULINS WAS STOPPED. THE SAME DAY A SUPRAPUBIC SWAP OF THE BLADDER CATHETER REVEALED STAPH. CAPRAE SENSITIVE TO CLARYTHROMYCIN. FROM 20 JAN 2003, EPITHELIUM STARTED TO FORM OVER THE EROSIONS ON THE PATIENT'S SKIN. A SKIN BIOPSY OF A REDDENED SPOT NEXT TO AN ABDOMINAL BULLA SHOWED AN INTERFACE DERMATITIS OF THE MULTIFORM TYPE AS FOUND IN DRUG INDUCED TOXIC EPIDERMAL NECROLYSIS. ON 21 JAN 2003 A VAGINAL SWAP SHOWED NO BACTERIAL OR YEASTS (TEST RE-TAKEN ON 27 JAN 2003 WITH THE SAME RESULT). ON 22 JAN 2003, THE SKIN CHANGES HAD IMPROVED. AT THIS TIME APPROXIMATELY 60% OF THE SKIN SURFACE (WALLACE RULE) HAD PEELED OFF. LATER A NEW EPITHELIUM FORMED WITHOUT ANY SIGNS OF INFECTIONS. THE PATIENT WAS TREATED WITH MOXIFLOXACIN AND CLARITHROMYCIN WHEREUPON THE PULMONARY INFILTRATES REGRESSED COMPLETELY. ON 28 JAN 2003, THE PATIENT EXPERIENCED TRANSIENT CLONI AND FLEXION SPASMS OF THE FEET, WITH PRONATION AND SUPINATION. A MIDBRAIN OR UPPER BRAIN STEM PROCESS WAS SUSPECTED. THE PATIENT HAD NO FEVER ANYMORE AND ON 29 JAN 2003 CLARITHROMYCIN AND MOXIFLOXACIN WERE DISCONTINUED. FROM 30 JAN 2003 PULMONARY INFILTRATES WERE NOT DETECTABLE BY CHEST X-RAY. ON 1 AND 3 FEB 2003, ABDOMINAL SONOGRAPHIES SHOWED AN ENLARGED LIVER AND SPLEEN, NO FREE PERIHEPATIC FLUID, INTRA AND EXTRAHEPATIC BILE DUCTS NORMAL AND GALL BLADDER DIAMETER OF 21MM. ON 5 FEB 2003, THE TRACHEAL CANULA WAS REMOVED. THE PATIENT DEVELOPED VAGINAL CANDIDOSIS WHICH WAS TREATED WITH FLUCONAZOLE (FLUCONAZOL), IV 200MG PER DAY. ON 6 FEB 2003, A BLOOD CULTURE REVEALED STAPH. EPIDERMIS SENSITIVE ONLY TO GLYCOPEPTIDE AND PROTEUS MIRABILIS SENSITIVE TO QUINOLONE ON CENTRAL VENOUS CATHETER. ON 7 FEB 2003, SHE FELL AND SHOWED AN ANISOCORIA (RIGHT PUPIL GREATER THAN LEFT). THE SAME DAY THE PATIENT'S FLEXION SPASMS WERE NOT DETECTED. BY 8 FEB 2003 THE ANISOCORIA COULD NOT BE DETECTED, AN X-RAY OF THE SHOULDER SHOWED NO FRACTURES. ON 7 FEB 2003 FLUCONAZOLE WAS STOPPED AND CLOTRIMAZOLE (CLOTRIMAZOL) CREAM STARTED. AT THIS TIME AN INCREASE OF THE PATIENT'S CHOLESTEROL PARAMETERS AND TRANSAMINASES WAS DETECTED. 10 FEB 2003: THE PATIENT WAS ADMITTED TO THE HOSPITAL UNIT AGAIN. AT THIS TIME THE INCREASE IN CHOLESTEROL PARAMETERS AND TRANSAMINASES WAS PERSISTING. HER GENERAL CONDITION WAS STILL REDUCED. THE PATIENT ALSO PRESENTED WITH EARLY SIGNS OF SYNECHIA IN THE AREA OF THE CONJUNCTIVAE. TREATMENT WITH ILLIG CONTACT LENSES WERE PRESCRIBED. EXAMINATIONS REVEALED DISTINCT ERYTHRODERMIA WITH EXANTHEMA AROUND THE EYES, ON THE BACK, THORAX, LOWER ABDOMEN DOWN TO THE GROIN AS WELL AS ON THE EXTREMITIES. THE ILLIG CONTACT LENSES WERE TOO PAINFUL TO THE PATIENT AND WERE REMOVED, AND THE PATIENT WAS TREATED WITH DEXPANTHENOL (BEPANTHEN) EYE CREAM. THE PATIENT FELT TOO SLOW AND HAD SHORT MEMORY DISORDER. THE LAB VALUES SHOWED DISTINCTLY ELEVATED CHOLESTASIS PARAMETERS AND A CHOLECYSTOLITHIASIS WAS DIAGNOSED. COMPARED TO THE FORMER LAB VALUES A DISTINCT REGRESSION WAS SEEN. THE ELEVATED CHOLESTASIS PARAMETERS WERE SUSPECTED TO BE DUE TO THE PARENTAL FEEDING DURING THE LONG LASTING INTUBATION. ON 10 FEB 2003 THE PATIENT WAS TRANSFERRED TO A NORMAL WARD. THE SKIN AND MUCOSA CHANGES HAD IMPROVED MARKEDLY WITH INTRAVENOUS TREATMENT. THERE WERE NO SIGNS OF SUPERINFECTION. LABORATORY TEST RESULTS WERE: HAEMOGLOBIN 12.3G/DL, THROMBOCYTES 396000/MCL, LEUKOCYTES 8000/MCL WITH INCONSPICUOUS DIFFERENTIAL COUNT, GGT 847U/L, AP 293U/L, GPT 264U/L, GOT 135U/L, LIPASE 162U/L. AT THIS TIME THE PATIENT WAS BEING TREATED WITH SUBCUTANEOUS ENOXAPARIN SODIUM (CLEXANE, 40MG PER DAY), ORAL RANITIDINE HYDROCHLORIDE (ZANTIC, 150MG PER DAY), ORAL AMITRIPTYLINE HYDROCHLORIDE (SAROTEN, 25MG PER DAY), CLOTRIMAZOLE (CANESTEN) CREAM AND SUPPOSITORY, DEXPANTHENOL EYE CREAM AND FLOXACIN (DEXASINE) EYE DROPS (FOUR TIMES PER DAY). DURING THE FURTHER HOSPITAL COURSE, THE GENERAL CONDITION OF THE PATIENT CONTINUOUSLY IMPROVED. THE PNEUMONIA HAD RESOLVED UNDER ANTIBIOTIC TREATMENT WITHOUT COMPLICATIONS. THE SYNECHIAS WERE PERSISTING. ON AN UNSPECIFIED DATE IN 2003, THE PATIENT'S DIAGNOSIS WAS CHANGED FROM LYE'S SYNDROME TO STEVENS-JOHNSONS DISEASE WHICH REQUIRED HOSPITALISATION AND WAS CAUSING A PERSISTENT DISABILITY TO HER EYES. LAB TEST FOR SODIUM, CALCIUM, RENAL AND COAGULATION PARAMETERS WERE NORMAL. ECG REVEALED SINUS RHYTHM, VERTICAL HEART POSITION, RS IN V3 AND V4 AND NO REPOLARISATION. CHEST X-RAY SHOWED NO PATHOLOGICAL FINDINGS. ABDOMINAL ULTRASOUND SHOWED THE WALL OF THE GALLBLADDER THICKENED TO 0.5CM WHICH RESEMBLED CHRONIC CHOLECYSTITIS, THREE GALL STONES AND METEORISM WERE ALSO SHOWN. CONTROL ULTRASOUND SHOWED SIMILAR FINDINGS. 18 FEB 2004: AN NMR IMAGING REVEALED NO PATHOLOGICAL FINDINGS FOR THE DUTUS CHOLEDOCHUS. 21 FEB 2003: THE PATIENT WAS DISCHARGED HOME WITH CLOTRIMAZOLE (CANESTEN CREME), THREE TIMES A DAY, OFLOXACIN (FLOXAL) EYE DROPS THREE TIMES A DAY, DEXAMETHASONE (DEXASINE) EYE DROPS TID AND DEXPANTHENOL (BEPANTHENOL) 'TONGUE' 5 TIMES DAILY. FROM 25 FEB 2003 TO 18 MAR 2003, THE PATIENT STAYED IN A NEUROLOGICAL REHABILITATION CLINIC. UPON ADMISSION TO THE CLINIC THE PATIENT COMPLAINED ABOUT GENERAL WEAKNESS, TREMBLING OF HANDS AND FINGERS, A WEAKNESS OF THE UPPER LEGS, SLIGHT MEMORY DISTURBANCES AND CONFUSION OF THE SENSE OF TIME. THE SKIN OF HER UPPER BODY AND FACE WAS HIGHLY REDDENED. ALL OTHER EXAMINATIONS WERE NORMAL. A CRITICAL ILLNESS MYOPATHY WAS SUSPECTED. A NEUROLOGICAL EXAMINATION SHOWED WEAKNESS OF BOTH UPPER ARMS, OF THE FLEXION OF THE RIGHT HIP, OF THE ABDUCTION OF BOTH HIPS, OF THE FOOT AND TOE EXTENSORS ON THE RIGHT SIDE. ALL MUSCLE STRETCH REFLEXES WERE INCREASED, NO PATHOLOGICAL REFLEXES. SENSIBILITY INTACT. FROM A NEUROLOGICAL POINT OF VIEW THERE WAS ALSO SLIGHT CONCENTRATION LIMITATION. A

NEUROPSYCHOLOGICAL EXAMINATION SHOWED SLIGHT LIMITATIONS IN THE BASAL AND COMPLEX DEMANDS ON ATTENTIVENESS. THE PATIENT RECEIVED INTENSIVE INTERDISCIPLINARY MULTIMODAL THERAPY CONSISTING OF INDIVIDUAL NEUROPHYSIOLOGICAL GYMNASTICS, ERGOTHERAPY, CONCENTRATION AND MEMORY TRAINING, TRAINING OF THE EQUILIBRIUM, BIKING ERGOMETRY, CONSERVATIVE MASSES AND TRAINING IN JACOBSON MUSCLE RELAXATION TECHNIQUE. IN THE FURTHER COURSE, THE PATIENT'S CONDITION IMPROVED SIGNIFICANTLY. ON 3 MARCH 2003, AN OPHTHALMOLOGICAL EXAMINATION SHOWED STIPPLED FLUORESCING CHANGES OF THE CORNEA BUT NO SYNCHIAS ANYMORE. UPON DISCHARGE, NO PARESES COULD BE DETECTED, ALL EQUILIBRIUM AND COORDINATION FUNCTIONS WERE ADEQUATELY RELIABLE, THE PHYSICAL RESILIENCE HAD IMPROVED SIGNIFICANTLY. FROM A NEUROLOGICAL POINT OF VIEW THERE WERE NO LIMITATIONS AFFECTING THE PATIENT'S EVERY DAY LIFE ANYMORE. SINCE 18 MARCH 2003 THE PATIENT WAS TAKING TREATMENT WITH DEXASINE EYE DROPS, DEXPANTHENOL EYE CREAM, TOBRAMYCIN (TOBRAMAXIN EYE CREAM, ONCE PER DAY), PREDNICARBATE EMOLLIENT (DERMATOP CREAM, ONCE TO TWICE PER DAY) AND NYSTATIN ZINC OXIDE (MULTILIND VAGINAL HEALING CREAM, TWICE PER DAY). ON 5 MAY 2003 THE PATIENT WAS TAKING TREATMENT WITH HYLOCOMOD EYE DROPS (SEVERAL TIMES PER DAY), VITAMIN A PALMITATE (OCULOTECT EYE DROPS, SEVERAL TIMES PER DAY), DEKORTIN (250MG) AND INTRAVENOUS DIMETINDENE MALEATE (FENISTIL). AT THIS TIME THE PATIENT WAS SEEN BY AN OPHTHALMOLOGIST. SHE HAD DEVELOPED IRRITATION OF THE EYES WITH A STRONG SENSATION OF A FOREIGN BODY IN BOTH EYES, MORE PRONOUNCED ON THE RIGHT SIDE. AN INITIAL EXAMINATION REVEALED MILD CONCRESCENCES IN THE AREA OF THE CARBUNCLE AND THE PLICAE SEMILUNARES. THE RIGHT LACRIMAL POINT WAS PARTIALLY SCARRED. THE CORNEA SHOWED STIPPLING OF THE EPITHELIUM POSITIVE TO FLUORESCENCE MAINLY IN THE LOWER THIRD. DESQUAMATED GOBLET CELLS WERE FOUND. A FUNDOSCOPY SHOWED SHARPLY DEFINED AND VITAL PAPILLAE ON BOTH SIDES, A DRY AND NORMAL MACULA WITH NORMAL RETINA. THE OPHTHALMOLOGIST DIAGNOSED AN OCULAR SCAR PEMPHIGOID. FOLLOWING A CONSULTATION ON 25 AUGUST 2003 THE RIGHT EYE HAD SLIGHT CAPILLARY INJECTION OF THE CONJUNCTIVA, TEMPORAL SYMBLEPHARON, MARGIN OF THE LID JAGGED, LOWER LACRIMAL POINT SCARRED, CORNEAS SMOOTH AND CLEAR, TRICHIASIS OF LOWER LID; THE LEFT EYE HAD CAPILLARY INJECTION OF THE CONJUNCTIVA, SYMBLEPHARON BELOW AND TEMPORAL LACRIMAL POINT OPEN, WITH SMOOTH AND CLEAR CORNEA. ON 19 SEP 2003 A FURTHER CONSULTATION WITH THE OPHTHALMOLOGIST SHOWED IN THE RIGHT EYE: NO IRRITATION, SYMBLEPHARON AS BEFORE, KERATINISATION OF MARGIN OF THE LID AS BEFORE, LOWER LACRIMAL POINT SCARRED, TRICHIASIS OF THE LOWER LID. IN THE LEFT EYE: NO IRRITATION AND SYMBLEPHARON AS BEFORE. ON 16 JAN 2004 THE OPHTHALMOLOGIC EXAMINATION SHOWED CONJUNCTIVITIS AND STABLE FINDINGS. THE PATIENT WAS AGAIN HOSPITALISED FROM 27 OCT 2004 TO 28 OCT 2004 DUE TO AN UNCLEAR ALLERGIC REACTION, SYMPTOMATIC CHOLELITHIASIS AND UNCLEAR ELEVATION OF TRANSAMINASES. THE PATIENT WAS ON HOLIDAY AND CONSULTED THE HOSPITAL FOR SEVERE UPPER ABDOMINAL COLICKY PAIN. SHE THEN EXPERIENCED A REDNESS OF THE WHOLE BODY WITH KNOWN ALLERGIES TO IBUPROFEN, CHLORPHENAMIN, OSELTAMIVIR, TAMOXIFEN CITRATE (TAMIFEN), CODEINE (CODICAPS), PARACETAMOL, LEDOTUSSIN AND BALKIS SCHNUPFEN. SHE WAS TREATED WITH 250MG DEKORTIN AND INTRAVENOUS FENISTIL WHEREUPON THE EXANTHEMA REGRESSED. DURING THE ADMISSION THE PATIENT EXPERIENCED VERTIGO AND A DROP OF BLOOD PRESSURE TO 90 SYSTOLIC. THE HYPOTENSION REGRESSED UNDER INFUSION. THE PATIENT STOPPED COMPLAINING ABOUT UPPER ABDOMINAL PAIN. AN ABDOMINAL SONOGRAPHY REVEALED DISTINCT CHOLELITHIASIS WHICH WAS KNOWN TO THE PATIENT. TRANSAMINASES WERE ELEVATED WHICH WERE NOT TYPICAL FOR A CHOLESTASIS AS THE ALKALINE PHOSPHATASE VALUE WAS NORMAL. THE PATIENT WAS DISCHARGED IN A STABLE CONDITION. ON THE DAY OF DISCHARGE A SONOGRAPHY OF THE ABDOMEN SHOWED THAT THE GALL BLADDER WAS ALMOST COMPLETELY FILLED WITH SEVERAL CONCREMENTS OF A DIAMETER OF APPROXIMATELY 1CM. THERE WAS AN INTENSE ECHO SHADOW. AT THE TIME OF THE REPORT THE STEVENS-JOHNSONS DISEASE IMPROVED AND SUBSEQUENTLY RESOLVED WITH SEQUELAE; THE ELEVATED CHOLESTATIS PARAMETERS WERE DECLINING; THE VALUES FOR CHOLESTEROL AND TRIGLYCERIDES WERE INCREASED AND THE OUTCOME OF THESE EVENTS WAS UNKNOWN; PNEUMONIA, CANDIDA INFECTION OF THE VAGINA, SYNECHIA, CLONI OF FEET, FLEXION SPASMS OF FEET, FALL, CRITICAL ILLNESS MYOPATHY, CONJUNCTIVITIS, VERTIGO AND HYPOTENSION HAD RESOLVED; INCREASED TRANSAMINASES, SYMBLEPHARON AND TRICHIASIS WERE PERSISTING; AND THE OUTCOME OF CONJUNCTIVITIS AND CHOLELITHIASIS WAS REPORTED AS UNKNOWN.

AT THE TIME OF THIS REPORT, IT WAS STATED THAT THE FIRST CLINICAL SIGNS OF ERYTHRODERMIA AND VESICULATION WERE DOUBTFULLY APPEARING BEFORE THE START OF OSELTAMIVIR. A PHYSICIAN LISTED THE INITIAL DIAGNOSIS OF LYELL'S SYNDROME AS RELATED TO OSELTAMIVIR, WHILST THE LAWYER LISTED THE EVENT OF STEVENS-JOHNSON DISEASE AS RELATED TO THE USE OF OSELTAMIVIR. IT WAS ALSO REPORTED THAT PARACETAMOL AND ASPIRIN (ASS) COULD BE SUSPECTED. THE REPORTER STATED THAT IBUPROFEN, PARETAMOL, CHLORPHENAMINE AND OSELTAMIVIR WERE REPORTED AS POSSIBLE TRIGGERS FOR THE INITIAL DIAGNOSIS OF LYELL'S SYNDROME. THE CAUSALITY ASSESSMENTS FOR PNEUMONIA, CANDIDA INFECTION OF THE VAGINA, SYNECHIA, FALL, CRITICAL ILLNESS MYOPATHY, CHOLESTEROL INCREASED, TRIGLYCERIDES INCREASED, OCULAR SCAR PEMPHIGOD, SYMBLEPHARON, TRICHIASIS, CONJUNCTIVITIS, EXANTHEMA, CHOLELITHIASIS, VERTIGO AND HYPOTENSION WERE NOT PROVIDED. TRANSAMINASES INCREASED, CLONI OF FEET AND FLEXION SPASMS OF FEET WERE ASSESSED AS NOT RELATED. THE REPORTER STATED THAT THE MASSIVE INVOLVEMENT OF THE BRONCHIAL, GASTROINTESTINAL AND VAGINAL MUCOSAE PROBABLY AIDED A CANDIDA INFECTION OF THE VAGINA. THE REPORTER STATED THAT THE TRANSIENT CLONI AND FLEXION SPASMS OF THE FEET WERE PROBABLY DUE TO INSUFFICIENT ANALGESIA AND SEDATION; THE CAUSE OF INCREASED TRANSAMINASES WAS PROBABLY DUE TO THERAPY WITH FLUCONAZOLE. THE DIAGNOSIS OF INFLUENZA WAS NOT CONFIRMED BY A LABORATORY TEST. DOSES, ROUTES AND REGIMENS WERE NOT PROVIDED UNLESS OTHERWISE SPECIFIED. SOME OF THE ABOVE INFORMATION WAS OBTAINED FROM A DISCHARGE LETTER. FOLLOWING MEDICAL ASSESSMENT, THE COMPANY RE-ASSESSED THE NON-SERIOUS EVENTS OF PNEUMONIA, TRANSAMINASES INCREASED, CRITICAL ILLNESS MYOPATHY, EXANTHEMA AND CHOLELITHIASIS AS MEDICALLY SIGNIFICANT.

UPDATE INFORMATION WAS RECEIVED, AND THE FOLLOWING DATA WERE ADDED TO THE CASE: TRANSAMINASES INCREASED, PNEUMONIA, CANDIDA INFECTION OF VAGINA, SYNECHIA, CLONI OF FEET, FLEXION SPASMS OF FEET, FALL, CRITICAL ILLNESS MYOPATHY, CHOLESTEROL INCREASED, TRIGLYCERIDES INCREASED, OCULAR SCAR PEMPHIGOD, SYMBLEPHARON, TRICHIASIS, CONJUNCTIVITIS, EXANTHEMA, CHOLELITHIASIS, VERTIGO AND HYPOTENSION WERE ADDED AS EVENTS; TWO ADDITIONAL

PHYSICIANS WERE ADDED AS REPORTERS, LABORATORY TESTS WERE ADDED; VARICELLA INFECTION, HIP SURGERY AND DISLOCATION, TONSILECTOMY, PENICILIN ALLERGY, AND SLIGHT MYOPIA WERE ADDED TO THE PATIENT'S MEDICAL HISTORY; AND DETAILS OF THE PATIENT'S CLINICAL COURSE WERE ADDED.

NO FURTHER INFORMATION WAS PROVIDED.

*** DATA ELEMENTS THAT WERE TRUNCATED NOW FOLLOW ***

<ichicsrmessageheader.patient.summary>

<sendercomment>

MONIA, EXANTHEMA, CANDIDA INF. OF VAG., SYNECHIA, CRITICAL ILLNESS MYOPATHY, OCULAR SCAR PEMPHIGOID, SYMBLEPHARON, TRICHIASIS AND CONJUNCTIVITIS = STEVENS JOHNS. SYNDR.

ADDITIONAL POSSIBLE EXPLAN. FOR PNEUMONIA = HISTORY OF PNEUMONIA, PULMONARY INFECTION AND RESPIRATORY INFECTION. ALTERNATIVE EXPLAN. FOR INCREASED TRANSAMINASES = CONCOMITANT FLUCONAZOLE TREATMENT.

ALTERNATIVE EXPLANATION FOR CLONI OF FEET AND FLEXION SPASMS OF FEET = INSUFFICIENT ANALGESIA AND SEDATION

ALTERNATIVE EXPLANATION FOR CHOLESTEROL INCREASED AND TRIGLYCERIDES INCREASED = PARENTERAL FEEDING DURING LONG LASTING INTUBATION.

PARACETAMOL AND GERANIUM = RELATED

BFARM NO.: 030009383

EINZELFALLBEOBACHTUNG. KEIN HANDLUNGSBEDARF!

</sendercomment>

Sender's comment:

CIOMS TEXT: STEVENS-JOHNSON DISEASE, PNEUMONIA, INCREASED TRANSAMINASES, EXANTHEMA AND VERTIGO ARE LISTED IN THE CDS FOR OSELTAMIVIR. CANDIDA INFECTION OF VAGINA, SYNECHIA, CLONI OF FEET, FLEXION SPASMS OF FEET, FALL, CRITICAL ILLNESS MYOPATHY, CHOLESTEROL INCREASED, TRIGLYCERIDES INCREASED, OCULAR SCAR PEMPHIGOD, SYMBLEPHARON, TRICHIASIS, CONJUNCTIVITIS, CHOLELITHIASIS AND HYPOTENSION ARE NOT LISTED IN THE CDS FOR OSELTAMIVIR. BASED ON THE INFORMATION RECEIVED FOR THIS CASE, A CAUSAL RELATIONSHIP BETWEEN THE REPORTED EVENTS AND TREATMENT WITH OSELTAMIVIR IS ASSESSED AS POSSIBLE. COMPANY CAUSALITY WAS ASSESSED BASED ON THE NARANJO ALGORITHM. HOWEVER, A POSSIBLE ALTERNATIVE EXPLANATION FOR STEVENS-JOHNSON DISEASE COULD INCLUDE THE CONCOMITANT USE OF IBUPROFEN. A POSSIBLE ALTERNATIVE EXPLANATION FOR PNEUMONIA, EXANTHEMA, CANDIDA INFECTION OF VAGINA, SYNECHIA, CRITICAL ILLNESS MYOPATHY, OCULAR SCAR PEMPHIGOD, SYMBLEPHARON, TRICHIASIS AND CONJUNCTIVITIS IS STEVENS-JOHNSON SYNDROME. ADDITIONAL POSSIBLE EXPLANATIONS FOR PNEUMONIA INCLUDE THE PATIENT'S MEDICAL HISTORY OF PNEUMONIA, PULMONARY INFECTION AND RESPIRATORY INFECTION. A POSSIBLE ALTERNATIVE EXPLANATION FOR INCREASED TRANSAMINASES INCLUDES THE PATIENT'S CONCOMITANT FLUCONAZOLE TREATMENT. A POSSIBLE ALTERNATIVE EXPLANATION FOR CLONI OF FEET AND FLEXION SPASMS OF FEET INCLUDES INSUFFICIENT ANALGESIA AND SEDATION. A POSSIBLE ALTERNATIVE EXPLANATION FOR CHOLESTEROL INCREASED AND TRIGLYCERIDES INCREASED INCLUDES PARENTAL FEEDING DURING LONG LASTING INTUBATION.; ***LOCAL ASSESSMENT: FOLLOW-UP ZUM 20. JAN. 2006

STEVENS-JOHNSON DISEASE, INCREASED TRANSAMINASES, EXANTHEMA, VERTIGO = LABELED

PNEUMONIA, CANDIDA INFECTION OF VAGINA, SYNECHIA, FALL, MYOPATHY, CHOLESTEROL INCREASED, TRIGLYCERIDES INCREASED, OCULAR SCAR PEMPHIGOID, SYMBLEPHARON, TRICHIASIS, CONJUNCTIVITIS, CHOLELITHIASIS, HYPOTENSION = NOT LABELED

ALTERNATIVE EXPLANATION FOR STEV. JOHNS. SYNDROME: CONCOMITANT USE OF IBUPROFEN

ALTERNATIVE EXPLANATION FOR PNEU

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03002597

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 10 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	20030110				recovered/resolved
2 Leucopenia	20030110				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 nurofen fuer kinder fiebersaft ibuprofen	Oral	/	20030110-20030110	Pyrexia of unknown origin
			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03006572

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 40 Year	weight (kg): 69	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug eruption	20030307		12 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen al 600 ibuprofen	Oral	001 Df dosage form/Day	20030304-20030308	Unspecified disorder of the teeth and supporting structures

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03009947

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 14 Year	weight (kg): 42	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme	20030312		14 Day		recovered/resolved
2 Conjunctivitis	20030312		14 Day		recovered/resolved
3 Disease aggravation	20030312		14 Day		recovered/resolved
4 Stomatitis	20030312		14 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxi 1000-1 a pharma amoxicillin trihydrate	Unknown	/	20030310-20030312	Bronchitis, not specified as acute or chronic
2 dolormin ibuprofen lysinate	Unknown	/	20030310-20030312	Bronchitis, not specified as acute or chronic

Drugs (concomitant)

3 acc 200 acetylcysteine	Unknown	/	-	
4 lemocin cetrimonium bromide; lidocaine; tyrothricin	Unknown	/	-	Bronchitis, not specified as acute or chronic
5 paracetamol paracetamol	Unknown	/	-	Pyrexia of unknown origin

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Acute pharyngitis			k.A.	concomitant disease
Asthma bronchial			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03013545

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 61 Year weight (kg): 80 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme	20030710		14 Day		recovered/resolved
2 Erythema exudativum	20030710		14 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bextra valdecoxib	Oral	080 Mg milligram(s)/Day	20030705-20030711	Other unspecified back disorders
2 ibuprofen klinge 600 ibuprofen	Oral	001 Df dosage form/Day	20030707-20030708	Other unspecified back disorders

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Other ill-defined heart diseases			k.A.	concomitant disease
Unspecified adverse effect of drug, medicament and biologi			k.A.	concomitant disease
Other specified cardiac dysrhythmias			k.A.	concomitant disease
Carpal tunnel syndrome			k.A.	concomitant disease
Unspecified adverse effect of drug, medicament and biologi			k.A.	concomitant disease
Pure hypercholesterolaemia			k.A.	concomitant disease
Essential hypertension, unspecified			k.A.	concomitant disease
Backache, unspecified			k.A.	concomitant disease
Constipation			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03014799

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 67 Year	weight (kg): 78	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20001002		1 Month		recovered/resolved
2 Stevens Johnson syndrome	20001002		1 Month		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	/	-	Unspecified disorder of the
ibuprofen			-	teeth and supporting structures

Drugs (concomitant)

2 timolol augentropfen	Intraocular	/	-	Glaucoma, unspecified
timolol maleate			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Oesophagitis NOS			k.A.	concomitant disease
Other chronic nonalcoholic liver disease			k.A.	concomitant disease
Cyst of kidney, acquired			k.A.	concomitant disease
Diabetes mellitus			k.A.	concomitant disease
Anaemia, unspecified			k.A.	concomitant disease
Glaucoma, unspecified			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03014804

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 81 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20010116		23 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol 300 mg allopurinol	Oral	/	-	Gout, unspecified
2 herz ass-ratiopharm acetylsalicylic acid	Oral	/	-	Heart failure, unspecified
3 ibuprofen ibuprofen	Oral	/	-	Gout, unspecified
4 lasix 40 furosemide	Oral	040 Mg milligram(s)/Day	20001222-20010119	Heart failure, unspecified
5 novalgine tropfen metamizole sodium	Oral	020 Gtt drop(s)/Day	20001222-20010119	Other general symptoms
6 oxazepam oxazepam	Oral	/	-	Other and unspecified special symptoms or syndromes, not
7 tramal long tramadol hydrochloride	Oral	/	-	Other general symptoms

Drugs (concomitant)

8 aldactone 50 spironolactone	Oral	50 Mg milligram(s)/	20001222-20010119	
9 lactulose lactulose	Oral	/	-	Constipation
10 lanitop mite tabletten metildigoxin	Oral	001 Df dosage form/Day	20001222-20010119	Heart failure, unspecified

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000318

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 1 Year	weight (kg): 8	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gastroenteritis	20010520				recovered/resolved
2 Stevens Johnson syndrome	20010527		11 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 nurofen fuer kinder fiebersaft ibuprofen	Oral	/	-	Pyrexia of unknown origin
2 paracetamol paracetamol	Unknown	/	-	Pyrexia of unknown origin

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Lichenification and lichen simplex chronicus			k.A.	concomitant disease
Water excessive loss of			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000319

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 26 Year	weight (kg): 63	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20011020		20 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 augmentan amoxicillin trihydrate; clavulanic acid	Oral	/	-	Pneumonia due to other specified organism
2 ibuprofen ibuprofen	Oral	/	-	Headache

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Allergic rhinitis, cause unspecified			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000329

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 35 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20011202		19 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuhexal ibuprofen	Oral	/	-	Other general symptoms
2 tegretal 400 carbamazepine	Oral	/	-	Convulsions

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-04000345

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Initials: Sex: Male Birth: privacy Age: 44 Year weight (kg): 75 BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	20020305		1 Month		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol 300 mg	Oral	/	-	Hyperuricaemia
allopurinol			-	
2 amoxicillin	Oral	/	-	Acute bronchitis
amoxicillin			-	
3 ibuprofen	Oral	/	-	
ibuprofen			-	

Drugs (concomitant)

4	bezafibrate	Oral	/	-	Other and unspecified hyperlipidaemia
	bezafibrate			-	
5	metoprolol	Oral	/	-	Essential hypertension, unspecified
	metoprolol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-04000286

Patient

Reactions

Drugs (suspected or interacting)

Medical History and concurrent conditions

Primary source

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000566

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 52 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Conjunctivitis	20040109		4 Day		recovering/resolving
2 Red eye	20040109		4 Day		recovering/resolving
3 Eye irritation	20040109		4 Day		recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Oral	600 Mg milligram(s)/Day	200401-200401 -	Other specified complications of procedures, not elsewhere

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified adverse effect of drug, medicament and biologi			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-04003852

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 68 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blood sedimentation increased	2001				Unknown
2 Conjunctivitis	2001				Unknown
3 Localised infection	20010405				not recovered/not resolved
4 Oedema peripheral	20010405				recovering/resolving
5 Osteoarthritis spinal	2001				not recovered/not resolved
6 Cardiac failure congestive	2001				recovered/resolved
7 Onychomycosis	2001				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Unknown	/	-200104 -	
2 teveten 600mg eprosartan mesilate	Oral	001 Df dosage form/Day	20000202-20040202 4 Year	Essential hypertension, unspecified

Drugs (concomitant)

3 acc 600 acetylcysteine	Oral	001 Df dosage form/Day	- -
4 antra mups omeprazole magnesium	Unknown	001 Df dosage form/Day	- -
5 baldrian valeriana officinalis root	Unknown	003 Df dosage form/Day	- -
6 iscover clopidogrel sulfate	Oral	001 Df dosage form/Day	- -
7 lasix furosemide	Oral	020 Mg milligram(s)/Day	- -
8 nootrop 800 piracetam	Unknown	003 Df dosage form/Day	- -
9 pantozol 40 pantoprazole sodium	Oral	001 Df dosage form/Day	- -
10 sotalex mite sotalol hydrochloride	Oral	001 Df dosage form/Day	- -
11 zocor 20 simvastatin	Oral	001 Df dosage form/Day	- -
12 zyloforton	Unknown	003 Df dosage form/Day	- -
13 zyloric 300 allopurinol	Oral	001 Df dosage form/Day	- -

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Obstructive hydrocephalus			Unknown	concomitant disease
TIA			Unknown	concomitant disease: z. n. tia
Apoplexy			Unknown	concomitant disease: z. n. schlaganfall 09/93
Cerebral vascular disturbance			Unknown	concomitant disease
Obesity			Unknown	concomitant disease
Chronic sinusitis, unspecified			Unknown	concomitant disease
Coronary atherosclerosis			Unknown	concomitant disease
Premature beats			Unknown	concomitant disease
Cardiac dysrhythmias			Unknown	concomitant disease
Hyperuricaemia			Unknown	concomitant disease
Pure hypercholesterolaemia			Unknown	concomitant disease
Abnormal glucose tolerance test			Unknown	concomitant disease
Osteoarthritis, localized, not specified whether primary or s			Unknown	concomitant disease
TUR			Unknown	concomitant disease: z. n. tur 1986
Incontinence of urine			Unknown	concomitant disease
Hypertensive renal disease, unspecified			Unknown	concomitant disease
Diverticula of colon			Unknown	concomitant disease
Back disorder			Unknown	concomitant disease

Primary source

Qualification: Physician observestudytype: Clinical trials
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04004371

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 8 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	2004				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolormin fuer kinder ibuprofensaft 2% ibuprofen	Oral	600 Mg milligram(s)/	- -	Unspecified viral infection in conditions classified elsewhere

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05002967

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Necrolysis epidermal					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	600 Mg milligram(s)/Day	-	Other general symptoms
ibuprofen			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05005331

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
No						Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 4 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Staphylococcus aureus bacteremia	20050216				Unknown
2 Lyell syndrome	20050216				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Nurofen fuer Kinder Fiebersaft ibuprofen		/	2005- -	Angina tonsillaris

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05005333

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell syndrome	2005				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Nurofen fuer Kinder Fiebersaft ibuprofen		/	2005- -	Angina tonsillaris

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05006468

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 39 Year	weight (kg): 68	BMI_int: 20.31	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Eosinophilia	20050313	20050402	21 Day	2 Day	recovered/resolved
2 Stevens-Johnson syndrome	20050313	20050404	23 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cotrimoxazol sulfamethoxazole; trimethoprim	Oral	/	20010601-	
2 ibuprofen ibuprofen	Oral	/	-	Toothache
3 kaletra lopinavir; ritonavir	Unknown	/	20050113-20050316 63 Day	HIV infection
4 paracetamol paracetamol	Oral	/	-	Pain NOS

Drugs (concomitant)

5 combivir lamivudine; zidovudine	Oral	/	20050113-20050316 63 Day
6 prothipendyl prothipendyl	Oral	/	20050312-20050312 -

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Polytoxicomania			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05008120

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Female Birth: privacy Age: 30 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Exanthematic drug eruption	20050603	20050603	1 Day		recovered/resolved
2 Hand dermatitis	20050603	20050603	1 Day		recovered/resolved
3 Itching	20050603	20050603	1 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen 400 ibuprofen	Oral	/	- -	Toothache

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05014838

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 81 Year	weight (kg): 72	BMI_int: 26.45	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Petechia	20050905		36 Day		recovered/resolved
2 Hypokalaemia	20050905		36 Day		recovered/resolved
3 Hyponatraemia	20050905		36 Day		recovered/resolved
4 Stomatitis	20050905		36 Day		recovered/resolved
5 Thrombocytopenia	20050905		36 Day		recovered/resolved
6 Stevens Johnson syndrome	20050905		36 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol		300 Mg milligram(s)/Day	-20050929	Hyperuricaemia
allopurinol			-	
2 ibuprofen		800 Mg milligram(s)/	-	Pain
ibuprofen			-	
3 Marcumar		/	-	
phenprocoumon			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Tricuspid insufficiency			Yes	
Mitral insufficiency			Yes	
Atrial fibrillation			Yes	
Coronary artery disease NOS			Yes	
Hypertensive heart disease			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05016723

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 61 Year	weight (kg): 68	BMI_int: 27.59	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Anxiety	2003				Unknown
2 Blood pressure increased	2003				Unknown
3 Constipation	2003				recovered/resolved
4 Dizziness	2003				recovered/resolved
5 Eyebrow loss of	2003				Unknown
6 Leukoencephalopathy	20040720				Unknown
7 Thalamic infarction	20040709				recovered/resolved with sequel
8 Palpitations	2003				recovered/resolved
9 Rash pruritic	2003				Unknown
10 Sleeplessness	2003				recovered/resolved
11 Tachycardia	2003				Unknown
12 Wound healing delayed	200307				Unknown
13 Oral mucosal blistering	2003				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ARIXTRA fondaparinux sodium	Subcutaneous	1 /Day Injection	20030705-20030722 18 Day	
2 Clexane enoxaparin	Subcutaneous	40 Mg milligram(s)/Day Injection	20030702-20030704 3 Day	
3 hypromellose hypromellose	Ophthalmic	2* /Day Ophthalmic insert	- -	
4 ibuprofen ibuprofen	Oral	1200 Mg milligram(s)/Day Tablet	- -	
5 jodid potassium iodide	Oral	200 µg microgram(s)/Day Tablet	- -	
6 KLIMONORM estradiol valerate; levonorgestrel	Oral	1 Df dosage form/Day	- -	
7 METOHEXAL metoprolol tartrate	Oral	50 Mg milligram(s)/Day Tablet	- -	
8 TRAMAL tramadol hydrochloride	Oral	200 Mg milligram(s)/Day	20030711-20030722 12 Day	Pain
9 VIOXX rofecoxib	Oral	50 Mg milligram(s)/Day Tablet	20030704-20030706 3 Day	Pain in hip
10 VIOXX rofecoxib	Oral	25 Mg milligram(s)/Day Tablet	20030707-2004 -	Pain in hip

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Bronchitis asthmatic			Yes	
Cardiac failure congestive			Yes	
Familial risk factor			Yes	
Anterolisthesis			Yes	
Iodine deficiency goitre			Yes	
Hospitalisation			Yes	
Smoker			Yes	
Total hip replacement			No	
Coronary artery disease			Yes	
Drug hypersensitivity			Yes	
Duodenal ulcer			No	
Food allergy			Yes	
Hypertension			Yes	
Hyperuricaemia			Yes	
Muscle atrophy			Yes	
Myopathy			Yes	
Myositis			Yes	
Osteochondrosis			Yes	
Osteoporosis			Yes	
Pollinosis			Yes	
Spondylosis			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

009507513, 22.01.2007 DE-MERCK-0511DEU00145

Sender diagnosis: not reported

Narrative

Information has been received from a hospital physician, a neurologist and a 62-year-old female patient (smoker) with muscle atrophy, iodine deficiency goitre, coronary artery disease, cardiac failure congestive, osteoporosis, anterolisthesis (2001), osteochondrosis, spondylosis, myositis and myopathy since 1980, bronchitis asthmatic (suspected), hyperuricaemia, drug hypersensitivity, pollinosis, food allergy, familial risk factor for hypertension, hypertension and a history of duodenal ulcer (1986) who, on 04-JUL-2003 while hospitalized, was placed on therapy with rofecoxib, 50 mg tablet, 50 mg, once a day for the treatment of hip pain after a total hip replacement right (03-JUL-2003). On 07-JUL-2003 rofecoxib was reduced to 25 mg tablet, 25 mg once a day.

Concomitant therapy included potassium iodide (JODID), metoprolol tartrate (METOHEXAL), hypromellose, ibuprofen, estradiol valerate (+) levonorgestrel (KLIMONORM), enoxaparin sodium (CLEXANE (enoxaparin sodium)), fondaparinux sodium (ARIXTRA) and tramadol HCl (TRAMAL).

In July and/or August 2003, during the hospital stay because of the hip-TEP right and the following treatment in rehabilitation center (from 24-JUL-2003 until 12-AUG-2003), the patient developed itching skin with rash (was treated with dimethindene maleate (FENISTIL), feeling of dizziness, sleeplessness, palpitations, tachycardia, anxiety, constipation, hair loss of eyebrows, blisters of oral mucosa and increase of blood pressure. The patient developed delayed wound healing which was treated with floxacillin sodium (STAPHYLEX), outcome was not reported. Constipation was treated with unspecified medication. Sleeplessness was treated with nitrazepam. At the beginning of November 2003, therapy with rofecoxib was discontinued. The patient recovered from constipation, sleeplessness, palpitation and dizziness. Outcome of itching skin with rash, tachycardia, anxiety, hair loss of eyebrows, blisters of oral mucosa and increase of blood pressure was not reported.

On 09-JUL-2004, the patient developed numbness feeling of left arm, hand and shoulder, left side of body and in the area of the neck (hemihypesthesia left) and loss of fine motor skills. MRT on 20-JUL-2004 revealed subacute thalamic infarction right with a size of 1 to 1.2 cm. The MRT also showed leukoencephalopathy with single foci in medullary layer (both sides) of primarily vascular origin. The patient was placed on therapy with pentoxifylline 600 mg and aspirin 100 mg daily. The neurologist reported that the patient recovered with sequelae due to dysaesthesia and

decreased strength left side.

The patient felt that thalamic infarction, itching skin with rash, feeling of dizziness, sleeplessness, palpitations, tachycardia, anxiety, constipation, hair loss of eyebrows, blisters of oral mucosa and increase of blood pressure were related to therapy with rofecoxib. She did not rate the drug relationship concerning the leukoencephalopathy.

The hospital physician felt that constipation, sleeplessness, palpitations and dizziness were not related to therapy with rofecoxib, but possibly related to all other concomitant therapies, particularly floxacillin sodium (STAPHYLEX) and to the patient's several pre-existing hypersensitivities. The neurologist felt that thalamic infarction was probably related to therapy with rofecoxib, but did not rate drug relationship regarding the other symptoms.

Additional information has been requested.

Upon internal review, leukoencephalopathy and thalamic infarction were determined to be Other Important Medical Events.

Sender's comment:

This is a case reported to us after the voluntarily market withdrawal of VIOXX°. A female patient was placed on VIOXX° over a period of about five months because of postoperative pain after hip TEP placement. Eight months after stopping the VIOXX° intake she suffered a stroke. Diagnostic tests revealed signs of old generalised vascular encephalopathy. Other risk factors include smoking, hypertension and co- medication (Klimonorm°). Considering the missing timely relationship a causal relationship between the intake of VIOXX° and the stroke is not to be suspected. VIOXX° has been worldwide voluntarily withdrawn from market on September 30th 2004.

Reporter's comment:

009507513, 04.12.2007 DE-MERCK-0511DEU00145

Sender diagnosis: not reported

Narrative

Information has been received from a lawyer of a health insurance company, a hospital physician, a neurologist and a 62-year-old female patient (smoker) with muscle atrophy, iodine deficiency goitre, coronary artery disease, cardiac failure congestive, osteoporosis, anterolisthesis (2001), osteochondrosis, spondylosis, myositis and myopathy since 1980, bronchitis asthmatic (suspected), hyperuricaemia, drug hypertensitivity, pollinosis, food allergy, familial risk factor for hypertension, hypertension and a history of duodenal ulcer (1986) who, on 04-JUL-2003 while hospitalized, was placed on therapy with rofecoxib, 50 mg tablet, 50 mg, once a day for the treatment of hip pain after a total hip replacement right (03-JUL-2003). On 07-JUL-2003 rofecoxib was reduced to 25 mg tablet, 25 mg once a day.

Concomitant therapy included potassium iodide (JODID), metoprolol tartrate (METOHEXAL), hypromellose, ibuprofen, estradiol valerate (+) levonorgestrel (KLIMONORM), enoxaparin sodium (CLEXANE (enoxaparin sodium)), fondaparinux sodium (ARIXTRA) and tramadol HCl (TRAMAL).

In July and/or August 2003, during the hospital stay because of the hip-TEP right and the following treatment in rehabilitation center (from 24-JUL-2003 until 12-AUG-2003), the patient developed itching skin with rash (was treated with dimethindene maleate (FENISTIL), feeling of dizziness, sleeplessness, palpitations, tachycardia, anxiety, constipation, hair loss of eyebrows, blisters of oral mucosa and increase of blood pressure. The patient developed delayed wound healing which was treated with floxacillin sodium (STAPHYLEX), outcome was not reported. Constipation was treated with unspecified medication. Sleeplessness was treated with nitrazepam.

In approximately April 2004 therapy with rofecoxib was discontinued, last prescription on 04-MAR-2004.

The patient recovered from constipation, sleeplessness, palpitation and dizziness.

Outcome of itching skin with rash, tachycardia, anxiety, hair loss of eyebrows, blisters of oral mucosa and increase of blood pressure was not reported.

On 09-JUL-2004, the patient developed numbness feeling of left arm, hand and shoulder, left side of body and in the area of the neck (hemihypesthesia left) and loss of fine motor skills. MRT on 20-JUL-2004 revealed subacute thalamic infarction right with a size of 1 to 1.2 cm. The MRT also showed leukoencephalopathy with single foci in medullary layer (both sides) of primarily vascular origin. The patient was placed on therapy with pentoxifylline 600 mg and aspirin 100 mg daily. The neurologist reported that the patient recovered with sequelae due to dysaesthesia and decreased strength left side.

The patient felt that thalamic infarction, itching skin with rash, feeling of dizziness, sleeplessness, palpitations, tachycardia, anxiety, constipation, hair loss of eyebrows, blisters of oral mucosa and increase of blood pressure were related to therapy with rofecoxib. She did not rate the drug relationship

concerning the leukoencephalopathy.

The lawyer felt that thalamic infarction was related to therapy with rofecoxib, he did not rate the other symptoms.

The hospital physician felt that constipation, sleeplessness, palpitations and dizziness were not related to therapy with rofecoxib, but possibly related to all other concomitant therapies, particularly floxacillin sodium (STAPHYLEX) and to the patient's several pre-existing hypersensitivities.

The neurologist felt that thalamic infarction was probably related to therapy with rofecoxib, but did not rate drug relationship regarding the other symptoms.

Additional information has been requested.

Upon internal review, leukoencephalopathy and thalamic infarction were determined to be Other Important Medical Events.

Sender's comment:

This is a case reported to us after the voluntarily market withdrawal of VIOXX°. A patient suffered a stroke about 2 months after therapy with VIOXX° had been discontinued. Diagnostic tests revealed signs of old generalized vascular encephalopathy. Other risk factors include smoking, hypertension and co- medication. Considering the missing timely relationship as well as the risk factors a causal relationship between the intake of VIOXX° and the stroke appears rather unlikely. VIOXX° has been worldwide voluntarily withdrawn from market on September 30th 2004.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06004704

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 81 Year	weight (kg): 71.6	BMI_int: 26.3	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overdose					Unknown
2 Hyponatraemia					Unknown
3 Hypokalaemia					Unknown
4 Exsiccosis					Unknown
5 Stomatitis					recovering/resolving
6 Thrombocytopenia					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol allopurinol	Unknown	/	-	Drug use for unknown indication
2 ibuprofen ibuprofen	Unknown	80 Mg milligram(s)/	20050905-20050905 1 Day	Musculoskeletal pain
3 marcumar phenprocoumon	Unknown	/	-	Atrial fibrillation

Drugs (concomitant)

4 digitoxin digitoxin	/	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertensive heart disease			Yes	
Coronary artery disease			Unknown	
Musculoskeletal pain			Unknown	
Atrial fibrillation			Yes	
Kidney stones	2000		Unknown	SURGICAL TREATMENT 5 YEARS AGO

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

ROCHE, 30.01.2006 426644

Sender diagnosis: not reported

Narrative

THIS SPONTANEOUS CASE RECEIVED FROM BFARM (GERMAN REGULATORY AUTHORITY) CONCERNS A 81 YEAR OLD FEMALE PATIENT WHO EXPERIENCED STOMATITIS, THROMBOCYTOPENIA WITH PETECHIA, EXSICCOSIS, HYPOKALAEMIA, HYPONATRAEMIA AND DIGITOXIN OVERDOSE DURING THE USE OF PHENPROCOUMON (MARCUMAR) FOR CHRONIC ATRIAL FIBRILLATION, ALLOPURINOL (ALLOPURINOL) FOR UNSPECIFIED INDICATION AND IBUPROFEN FOR CHRONIC BACK AND KNEE PAIN.

THE PATIENT HAS A MEDICAL HISTORY OF CORONARY ARTERY DISEASE, HYPERTENSIVE HEART DISEASE (HEART FAILURE) WHICH WAS ONGOING, ATRIAL FIBRILLATION WHICH WAS ONGOING, AND KIDNEY STONES WHICH REQUIRED SURGICAL TREATMENT IN 2000.

ON AN UNSPECIFIED DATE TREATMENT WITH PHENPROCOUMON WAS STARTED. ON 05 SEPTEMBER 2005 THE PATIENT STARTED TREATMENT WITH IBUPROFEN, 800 MG DUE TO CHRONIC BACK PAIN AND KNEE PAIN. AFTER A SINGLE DOSE OF IBUPROFEN, THE PATIENT DEVELOPED SEVERE STOMATITIS WHICH DID NOT IMPROVE DESPITE DISCONTINUATION OF IBUPROFEN AND LOCAL

TREATMENT WITH CAMOMILE-LAVAGE. SIXTEEN DAYS LATER THE PATIENT WAS ADMITTED TO HOSPITAL DUE TO EXTENSIVE EXSICCOSIS, IMPAIRMENT OF ELECTROLYTE BALANCE AND DIGITOXIN OVERDOSE, AFTER A VIRTUAL LACK OF FOOD FOR TWO WEEKS DUE TO THE SEVERE ORAL AND PERIORAL STOMATITIS. THE PATIENT FURTHER PRESENTED WITH RED, BURNING PUSTULES ON FEET, ANKLES AND DISTAL LEGS FOR WHICH SHE HAD RECEIVED OUTPATIENT TREATMENT WITH CETRIZIN, HOWEVER WITHOUT ANY IMPROVEMENT. UPON ADMISSION THE PATIENT PRESENTED IN REDUCED GENERAL CONDITION, WITH PALE SKIN AND DISTINCTLY REDUCED TURGOR OF SKIN. BLOOD PRESSURE WAS 100/80 MMHG, PULSE 45 BPM. ORAL MUCOSA AND LIPS SHOWED APHTHOUS ULCERS AND BLEEDINGS, THE TONGUE WAS COATED. FEET, ANKLES AND DISTAL THIGHS SHOWED A PETECHIAL EXANTHEMA. THERE WAS A SLOW IMPROVEMENT OF THE STOMATITIS AFTER CONSERVATIVE THERAPY INCLUDING LOCAL TREATMENT AND TEN DAYS OF PARENTERAL FEEDING. THEREAFTER, ENTERAL DIET WAS SLOWLY RESTARTED. DURING THE PATIENT'S HOSPITALISATION, TREATMENT WITH PHENPROCOUMON WAS INTERRUPTED AND SWITCHED TO TREATMENT WITH LOW-MOLECULAR HEPARIN. IT WAS ENVISAGED TO RESTART PHENPROCOUMON DURING THE PATIENT'S STAY IN THE SHORT TERM CARE FACILITY. DURING THE HOSPITALISATION, CHRONIC BACK AND KNEE PAINS WERE TREATED WITH DUROGESIC PATCH, WHICH WAS WITHDRAWN AFTER THE PATIENT BECAME NAUSEOUS AND FELT DIZZY. IT WAS RECOMMENDED TO TREAT FURTHER PAIN WITH MORPHINE RECEPTOR ANTAGONISTS OR PARACETAMOL HOWEVER, NON-STEROIDAL ANTIPHLOGISTICS SHOULD NOT BE USED UNDER ANY CIRCUMSTANCES. TREATMENT FOR HEART FAILURE WITH DIURETICS AND ACE INHIBITOR WERE WELL TOLERATED. THE STOMATITIS WAS THOUGHT TO BE MOST LIKELY OF TOXIC-ALLERGIC AETIOLOGY AND IN ALL LIKELIHOOD CAUSED BY ALLOPURINOL OR IBUPROFEN, IN THE CONTEXT OF STEVEN-JOHNSON SYNDROME OR NON BULLOUS ERYTHEMA EXSUDATIVUM MULTIFORME. ANOTHER POSSIBLE CAUSE SUCH AS CHANGES IN THE CONTEXT OF A MALIGNANCY WERE DISCUSSED BUT LATER REJECTED, SINCE A BONE MARROW PUNCTURE WAS INCONSPICUOUS AND THE EVENT IMPROVED UNDER CONSERVATIVE TREATMENT. THE SKIN CHANGES ON FEET, ANKLES AND LEGS WERE DIAGNOSED AS PETECHIAL CHANGES IN THE CONTEXT OF THE THROMBOCYTOPENIA. THERE WERE NO SIGNS FOR POST -INFECTIOUS ARTHRITIS OR DESTRUCTIONS OF BONES OR JOINTS. ON 11 OCTOBER 2005 THE PATIENT WAS DISCHARGED FROM HOSPITAL INTO A SHORT TERM CARE FACILITY. THE PATIENT WAS ON THE FOLLOWING MEDICATION UPON DISCHARGE; TORASEMIDE (TOREM) 10 MG, RAMIPRIL (DELIX) 5 MG, CLEXANE MULTIDOSE 0.6 ML, SCHECK - SOLUTION 1:10 TO RINSE THE MOUTH AND KAMISTAD UNGUENT. AT THE TIME OF THIS REPORT THE STOMATITIS HAD IMPROVED AND THE REPORTER ASSESSED THE STOMATITIS, THROMBOCYTOPENIA WITH PETECHIA, EXSICCOSIS, HYPOKALAEMIA, HYPONATRAEMIA AND DIGITOXIN OVERDOSE AS NOT RELATED TO PHENPROCOUMON, ALLOPURINOL AND IBUPROFEN. NO FURTHER INFORMATION WAS PROVIDED.

*** DATA ELEMENTS THAT WERE TRUNCATED NOW FOLLOW ***

<ichicsr.safetyreport.patient.test(testname=10008498)>

<testresult>

DISTINCT DIAPHRAGMATIC ELEVATION LEFT SIDE, SEVERE RIGHT CONVEX TORSION SCOLIOSIS OF THE LOWER THORACIC SPINE. OTHERWISE NORMAL.

</testresult>

<ichicsr.safetyreport.patient.test(testname=10014084)>

<testresult>

BRADYCARDIAL ATRIAL FIBRILLATION AT 40 BPM, INTERMITTANT LEFT BUNDLE BRANCH BLOCK, INFERIOR DEPRESSION OF ST SEGMENT.

</testresult>

<ichicsr.safetyreport.patient.test(testname=10014084)>

<testresult>

ATRIAL FIBRILLATION WITH 67 BPM TO 142 BPM, FREQUENT VENTRICULAR EXTRASYSTOLE (VES) LIKE DEFORMATIONS OF QRS, SOME VES, INTERMITTANT LEFT BUNDLE BRANCH BLOCK, EPISODES OF SUPRA VENTRICULAR TACHYCARDIA.

</testresult>

<ichicsr.safetyreport.patient.test(testname=10049506)>

<testresult>

BONE MARROW PUNCTURE - REACTIVE HYPERPLASIA OF GRANULOPOIESIS, UNSPECIFIC INFLAMMATORY REACTION OF BONE MARROW, NORMAL MEGACRYOPOIESIS

</testresult>

<ichicsr.safetyreport.patient.test(testname=10062163)>

<testresult>

BORRELIA, YERSINIA AND CAMPYLOBACTER ANTIBODIES NEGATIVE. NO EVIDENCE OF CURRENT INFECTION

</testresult>

<ichicsr.safetyreport.patient.test(testname=10042145)>

<testresult>

NO EVIDENCE OF SALMONELLA, SHIGELLA, YERSINIA OR CAMPYLOBACTER INFECTION

</testresult>

<ichicsr.safetyreport.patient.test(testname=10053368)>

<testresult>

LEFT VENTRICULAR HYPERTROPHY, DILATED ATRIA, AORTIC VALVE AND MITRAL RING SCLEROSIS (DEGENERATIVE), SLIGHT AORTIC VALVE INSUFFICIENCY, MITRAL AND TRICUSPIDAL VALVE INSUFFICIENCY. GRADE 3, MODERATE PULMONARY HYPERTENSION, VISIBLE PARTS OF AORTA ASCENDENS AND AORTIC ARCH NOT DILATED. GOOD SYSTOLIC LEFT VENTRICULAR FUNCTION.

</testresult>

<ichicsr.safetyreport.patient.test(testname=10045434)>

<testresult>

ULTRASOUND OF VEINS RIGHT LEG: NO EVIDENCE OF DEEP VEIN THROMBOSIS

</testresult>

<ichicsr.safetyreport.patient.test(testname=10052684)>

<testresult>

CURRENTLY NO EVIDENCE OF HYDRONEPHROSIS OF BOTH KIDNEYS, ENLARGED LIVER, DISPLACING THE RIGHT KIDNEY TO CAUDAL-VENTRAL, CREATININE VALUE NORMAL.

</testresult>

<ichicsr.safetyreport.patient.test(testname=10048064)>

<testresult>

SHOULDER JOINT - REVEALED ONLY AGE APPROPRIATE DESTRUCTIONS OF BONES AND JOINTS

</testresult>

<ichicsr.safetyreport.patient.test(testname=10048064)>

<testresult>

UPPER RIGHT ANKLE JOINT - REVEALED ONLY AGE APPROPRIATE DESTRUCTIONS OF BONES AND JOINTS

</testresult>

<ichicsr.safetyreport.patient.test(testname=10048064)>

<testresult>

SHOULDER JOINT - REVEALED ONLY AGE APPROPRIATE DESTRUCTIONS OF BONES AND JOINTS

</testresult>

Sender's comment:

CIOMS TEXT: EXSICCOSIS, HYPOKALAEMIA AND HYPONATRAEMIA ARE NOT LISTED IN THE COMPANY REFERENCE DOCUMENT (ISPI) FOR PHENPROCOUMON OR THE IDS CORE FOR ALLOPURINOL. THE COMPANY AGREES WITH THE REPORTED ASSESSMENT THAT EXSICCOSIS, HYPOKALAEMIA AND HYPONATRAEMIA ARE UNRELATED TO PHENPROCOUMON AND ALLOPURINOL. A POSSIBLE ALTERNATIVE EXPLANATION IS THE VIRTUAL LACK OF FOOD FOR TWO WEEKS DUE TO THE SEVERE ORAL AND PERIORAL STOMATITIS .

Reporter's comment:

case: DE-BFARM-06037836

Patient

Reactions

Drugs (suspected or interacting)

Primary source

86 / 305

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06047618

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 27 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Allergic skin reaction					Unknown
2 Face red					Unknown
3 Facial rash					Unknown
4 Itching					Unknown
5 Blistering					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolormin	Oral	2 Df dosage form/Day	-	
ibuprofen lysinate			1 Day	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06059135

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 5 Year	weight (kg): 16.8	BMI_int: 14.14	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythroderma	20061023			2 Day	recovered/resolved
2 Face edema	20061023			2 Day	recovered/resolved
3 Blistering	20061023			2 Day	recovered/resolved
4 Bullous dermatitis	20061023			2 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 nurofen ibuprofen	Oral	450 Mg milligram(s)/Day	20061022-20061023	Balanitis
			2 Day	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06060644

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 56 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic skin eruption	20060401	20060425	25 Day	4 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Oral	3 Df dosage form/	20060328-20060401	Influenza
2 tamiflu oseltamivir phosphate	Oral	/	5 Day 20060328-20060401	Influenza
			5 Day	

Drugs (concomitant)

3 amylmetacresol amylmetacresol	Oral	1 Df dosage form/	20060328-	Influenza
4 mucosolvan ambroxol hydrochloride		/	- - -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Allergic asthma			Unknown	
Influenza			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

ROCHE, 25.12.2006 443850

Sender diagnosis: not reported

Narrative

THIS SPONTANEOUS CASE WAS REPORTED BY A GENERAL PRACTITIONER AND CONCERNS A 56-YEAR-OLD FEMALE PATIENT WHO EXPERIENCED TOXIC DRUG INDUCED STAPHYLODERMIC EXANTHEMA DURING THE USE OF OSELTAMIVIR (TAMIFLU) AND IBUPROFEN (MANUFACTURER UNKNOWN) FOR INFLUENZA.

ON 28 MARCH 2006, THE PATIENT STARTED ORAL OSELTAMIVIR, TWO DOSE FORMS DAILY, AND ORAL IBUPROFEN, THREE DOSE FORMS. FOUR DAYS LATER, THE PATIENT DEVELOPED FEVER BLISTERS ON HANDS, FEET AND IN THE AXILLAE. THE PATIENT WAS DIAGNOSED WITH TOXIC DRUG INDUCED STAPHYLODERMIC EXANTHEMA. OSELTAMIVIR AND IBUPROFEN WERE DISCONTINUED ON THE SAME DAY. THE PATIENT RECEIVED AN INFUSION WITH PHYSIOLOGICAL NACL SOLUTION AND CORTISONE (CORTISON). AT THE END OF APRIL 2006, TOXIC DRUG INDUCED STAPHYLODERMIC EXANTHEMA RESOLVED.

THE GENERAL PRACTITIONER REGARDED TOXIC DRUG INDUCED STAPHYLODERMIC EXANTHEMA AS MEDICALLY SIGNIFICANT AND ASSESSED THE CAUSAL RELATIONSHIP BETWEEN TOXIC DRUG INDUCED STAPHYLODERMIC EXANTHEMA AND THE USE OF OSELTAMIVIR AS POSSIBLY RELATED AND THE CAUSAL RELATIONSHIP BETWEEN TOXIC DRUG INDUCED STAPHYLODERMIC EXANTHEMA AND THE USE OF IBUPROFEN AS QUESTIONABLY RELATED.

NO FURTHER INFORMATION WAS AVAILABLE.

UPDATE INFORMATION WAS RECEIVED AND THE FOLLOWING DATA WAS ADDED TO THE CASE: EVENT WAS CHANGED FROM SKIN REACTION (NON-SERIOUS) TO TOXIC DRUG INDUCED STAPHYLODERMIC EXANTHEMA (SERIOUS); SUSPECT DRUG OF IBUPROFEN; CONCOMITANT MEDICATION OF MUCOSOLVAN AND AMYLMETACESYL (MAC); THERAPY DETAILS FOR THE DRUGS OF OSELTAMIVIR,

IBUPROFEN AND AMYLMETACRESYL (MAC); MEDICAL HISTORY OF ALLERGIC ASTHMA BRONCHIALE; THE PATIENT'S DEMOGRAPHICS (DATE OF BIRTH, INITIALS); REPORTER CONTACT DETAILS.

Sender's comment:

CIOMS TEXT: TOXIC DRUG INDUCED STAPHYLODERMIC EXANTHEMA IS LISTED IN THE CDS FOR OSELTAMIVIR. BASED ON THE INFORMATION RECEIVED FOR THIS CASE, A CAUSAL RELATIONSHIP BETWEEN THE REPORTED EVENT AND TREATMENT WITH OSELTAMIVIR IS ASSESSED AS POSSIBLE. COMPANY CAUSALITY WAS ASSESSED BASED ON THE NARANJO ALGORITHM. HOWEVER, AN ALTERNATIVE EXPLANATION FOR THE EVENT OF TOXIC DRUG INDUCED STAPHYLODERMIC EXANTHEMA INCLUDE THE MEDICATION OF IBUPROFEN. HOWEVER, FURTHER INFORMATION REGARDING MEDICAL HISTORY WOULD ALLOW THE CASE TO BE ASSESSED FURTHER.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07077069

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 57 Year	weight (kg): 76	BMI_int: 26.93	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	200702				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibuprofen-ratiopharm 800 (Filmdabletten)	Oral	2400 Mg milligram(s)/Day	200702-	Pain in thumb
ibuprofen		Film-coated tablet	-	

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Lawyer observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)[ratiopharm GmbH, 24.08.2007 DE-RATIOPH-2007R084113](#)

Sender diagnosis: not reported

Narrative

Reported terms: Stevens-Johnson syndrome(Stevens-Johnson syndrome)

Case reference number 2007R084113 is a spontaneous case report sent by a lawyer which refers to a male aged 58 years. Case was confirmed by a health professional.

The patient's past medical history included no medical records. There is no information about risk factors. There is no information about concomitant diseases. No prior drugs are known to have been taken by the patient.

The patient started taking Ibuprofen-ratiopharm 800 (Filmdabletten) (IBUPROFEN), indication pain in thumb on ??-Feb-2007. The patient developed Stevens-Johnson syndrome (Stevens-Johnson syndrome)(start date ??-Feb-2007).

The following action was taken: for Ibuprofen-ratiopharm 800 (Filmdabletten) (IBUPROFEN) - Drug withdrawn

The outcome for Stevens-Johnson syndrome (Stevens-Johnson syndrome) is Not Recovered / Not Resolved.

Rechallenge information for Ibuprofen-ratiopharm 800 (Filmdabletten) is not applicable. Dechallenge information for Ibuprofen-ratiopharm 800 (Filmdabletten) is not applicable.

The reporter's assessment for drug Ibuprofen-ratiopharm 800 (Filmdabletten) related with Stevens-Johnson syndrome is as follows: Unknown.
CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07089163

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 9 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 nurofen ibuprofen	Oral	600 Mg milligram(s)/Day	20070730-20070730	-

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08008772

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 66 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20030331			21 Day	not recovered/not resolved
2 Chills	20030330			20 Day	Unknown
3 Headache	20030331			21 Day	not recovered/not resolved
4 Swelling face	20030331			21 Day	not recovered/not resolved
5 Swallowing difficult	20030331			21 Day	not recovered/not resolved
6 Itching	20030331			21 Day	not recovered/not resolved
7 Skin disorder	20030331			21 Day	not recovered/not resolved
8 Cystitis acute	20030318			8 Day	Unknown
9 Lip erosion	20030402			23 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass acetylsalicylic acid	Oral	100 Mg milligram(s)/	19980101-	Thrombosis prophylaxis
2 cotrimoxazol sulfamethoxazole; trimethoprim		/	20030324-20030329 6 Day	Cystitis
3 ibuprofen ibuprofen	Oral	/	20000101-	Sciatica
4 OFLOX-AZU ofloxacin	Oral	100 Mg milligram(s)/Day	20030317-20030322 6 Day	Urinary infection
5 voltaren dispers diclofenac sodium	Oral	/Day Dispersible tablet	20030311-	Pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Ischial neuralgia			Unknown	
Sleep disorder			Unknown	
unbekannt			Unknown	
Bronchitis			Unknown	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 22.01.2008 DE-002147023-PHNU2008DE00542

Sender diagnosis: not reported

Narrative

Initial report received on 14 Jan 2008 (report from Intensive Monitoring Project, Schäpf-Mockenhaupt-SCAR Project = RegiSCAR project, interview number 9150022). This patient was treated with several drugs during 2003 and 2004. Voltaren Dispers (diclofenac) was used irregularly since 11 Mar 2003. On 18 Mar 2003 a acute cystitis was reported (treated withOflox-Azu from 17 Mar 2003 to 22 Mar 2003 and Cotrimazole from 24 Mar 2003 to 29 Mar 2003). On 30 Mar 2003 chills were reported and on 31 Mar 2003 the patient developed headache, difficulty swallowing, face swelling, itching and generalized targets and spots. On 02 Apr 2003 the patient was hospitalized with lip erosions. A diagnosis of Steven-Johnson-Syndrome was made. Outcome was reported as not yet recovered. Voltaren Dispers, Oflox-Azu and Cotrimazole were reported as suspected. Outcome was reported as not yet recovered.

Sender's comment:

Serious adverse event [medically significant] "Stevens-Johnson syndrome" assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Cystitis was assessed as not suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08013533

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Male Birth: privacy Age: 15 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20041231	20050118	19 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxihexal amoxicillin trihydrate	Oral	/	-	Influenza
2 bronchicum (old form) ammonia; cimicifuga racemosa root; cortex quebracho; ephedrine hydrochloride; eucalyptus oil; grindelia; menthol; pimpinella root; sodium bromide; thyme; thyme extract	Oral	/	-	Influenza
3 cec cefaclor monohydrate	Oral	/	-	Cough
4 chlorhexamed chlorhexidine gluconate	Oral	/	-	Pain mouth
5 dobendan cetylpyridinium chloride	Oral	/	-	Streptococcal sore throat
6 ibuhexal ibuprofen	Oral	/	-	Pyrexial cold

Primary source

Qualification: Other Health Professional

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08014046

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 39 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Conjunctivitis	20041021				not recovered/not resolved
2 Fever	20041022				not recovered/not resolved
3 Lip erosion	20041022				not recovered/not resolved
4 Skin discolouration	20041023				not recovered/not resolved
5 Blisters	20041024				not recovered/not resolved
6 Oral mucosa erosion	20041025				not recovered/not resolved
7 Stevens-Johnson syndrome	20041020	20041109	21 Day		not recovered/not resolved
8 Eye red	20041020				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol allopurinol	Oral	/	20041007-20041020 14 Day	Hyperuricaemia
2 ibuprofen ibuprofen	Oral	/	20041014-20041020 7 Day	Pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Supplementation therapy			No	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08014073

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 83 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20050728	20050819	23 Day	29 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibu-ksk 600 mg ibuprofen	Oral	3 Df dosage form/Day	20050630-	Ache
2 kepinol sulfamethoxazole; trimethoprim	Oral	/	20050727-20050729 3 Day	Cystitis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Ache			Unknown	
Coronary disease			Unknown	
Cardiac failure			Unknown	
Constipation			Unknown	
Depression			Unknown	
Hypothyroidism			Unknown	
Thrombosis prophylaxis			Unknown	
Pollakiuria			Unknown	
Diabetes mellitus			Unknown	
Urinary infection			Unknown	
Parkinson's disease			Unknown	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

Dr. R. Pfleger Chemische Fabrik GmbH, 27.02.2008 DE-PFLEGER-000051

Sender diagnosis: not reported

Narrative

This report was received from Dokumentationszentrum schwerer Hautreaktionen in der Bundesrepublik Deutschland, Universitäts-Hautklinik, Freiburg via BfArM (DE-BFARM-08014073) as a solicited report within the TEN/SJS Studie.

The patient (female, initials: M.K. birth date: 20.03.1922) was hospitalised on 30.07.2005 because of Stevens Johnson syndrome (LLT), (PT: Stevens-Johnson syndrome). The reaction was not life-threatening, the outcome is stated as not yet recovered. In the corresponding period the patient took several drugs which were listed along with the relative indications in the attachement.

The treatment with Kepinol p.o. (active ingredient: cotrimoxazole; pharmaceutical form not further specified), lasted from 27.07.2005 to 29.07.2005 (2 tbl. bid) because of cystitis. On 28.07.2005 cocades and maculae occurred on the skin. For the next day bullae of the skin were described. On 30.07.2005 fever, Nikolsky's sign and erosions of the lips, oral and genital mucosa appeared. The patient was hospitalised.

In addition to Kepinol the patient took several other drugs (active ingredients):

IBU KSK 600mg (Ibuprofen)
 Delix 5mg (Ramipril)
 Furosemid 40mg (Furosemide)
 Amantadin 100mg (Amantadine)
 Citalopram 40mg (Citalopram)
 L-Thyroxin 100µg (Levothyroxin)

ASS 100mg (Acetylsalicylic acid)
Vesikur 5mg (Solifenacin)
Actraphane InjLsg 30/70 (Insulin human)
Movicol BTL (Macrogol3350, NaCl, NaHCo3, KCl)
Madopar 125 T, Madopar Depot (Levodopa, Benserazide)<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Stevens-Johnson-Syndrome is a known adverse effect of Kepinol and it is therefore mentioned in the corresponding package leaflets as and the SPC (Fachinformation). But in addition to Kepinol the patient took several other drugs which can also cause Stevens-Johnson-Syndrom in the questionable time period:

Ibuprofen (IBU)

Ramipril (Delix)

Furthermore the patient suffered from an infection of the bladder. Also infections could trigger Stevens-Johnson-Syndrom.

Therefore a causal relationship between the intake of Kepinol and the occurrence of Stevens-Johnson-Syndrom is judged as possible.

The benefit-risk-ratio of Kepinol p.o. is not altered by this case report.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08014098

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 68 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20030417	deathcause: Toxic epidermal necrolysis
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Macule	20030404			37 Day	Unknown
2 Toxic epidermal necrolysis	20030404	20030417	14 Day	39 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ALDACTONE spironolactone	Oral	/ Tablet	20030227-20030408	Ascites
2 AQUAPHOR xipamide	Oral	20 Mg milligram(s)/Day Tablet	20030304-	Edema
3 CALCILAC KT calcium carbonate; colecalciferol	Oral	2 Df dosage form/Day Tablet	20030222-	Osteoporosis
4 diclofenac diclofenac	Oral	1 Df dosage form/Day Tablet	20030403-20030403	Pain
5 HEPA-MERZ-SIRUP ornithine aspartate	Oral	6 Df dosage form/Day Syrup	20030222-	Liver cirrhosis
6 ibuprofen ibuprofen	Oral	1 Df dosage form/ Tablet	20030214-	Pain
7 KONAKION N phytomenadione	Oral	30 Gtt drop(s)/Day	20030222-	Vitamin K deficiency
8 MCP TROPFEN metoclopramide	Oral	108 Gtt drop(s)/Day	20030222-	Reflux esophagitis
9 milgamma benfotiamine; cyanocobalamin	Oral	3 Df dosage form/Day	20030222-	Vitamin B complex deficiency
10 MILGAMMA benfotiamine; cyanocobalamin	Oral	3 Df dosage form/Day Tablet	20030222-	Vitamin B6 deficiency
11 NEXIUM esomeprazole magnesium	Oral	20 Mg milligram(s)/Day Modified-release tablet	20030225-	Helicobacter infection
12 NOVAMINSULFON metamizole sodium	Oral	1 Df dosage form/	20030214-	Pain
13 OBSIDAN propranolol hydrochloride	Oral	37.5 Mg milligram(s)/Day Tablet	20030312-	Portal vein pressure increased
14 OMEP omeprazole	Oral	40 Mg milligram(s)/Day TABLET	20030319-20030408	Prophylaxis against gastrointestinal ulcer
15 ULCOGANT sucralfate	Oral	4 Df dosage form/Day Oral suspension	20030224-	Gastrointestinal ulcer

Drugs (concomitant)

16	ALDACTONE spironolactone	Unknown	100 Mg milligram(s)/Day	-
17	ALDACTONE spironolactone	Unknown	150 Mg milligram(s)/Day	-
18	ALDACTONE spironolactone	Unknown	100 Mg milligram(s)/Day	-
19	calcilac folic acid	Unknown	/Day	20030222-
20	calcilac folic acid	Unknown	2* /Day	20030222-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Ascites			No	
Gastrointestinal ulcer NOS			No	
Vitamin B complex deficiency			No	
Pain NOS			No	
Osteoporosis			No	
Oesophagitis			No	
Oedema			No	
Liver cirrhosis			No	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

ROCHE, 13.02.2008 DE-ROCHE-545518

Sender diagnosis: not reported

Narrative

INTENSIVE MONITORING

THIS CASE WAS REPORTED BY A PHYSICIAN AND CONCERNS A 67-YEAR-OLD FEMALE PATIENT EXPERIENCED FATAL TOXIC EPIDERMAL NECROLYSIS WHILE TAKING PHYTOMENADIONE (KONAKION) FOR VITAMIN K SUBSTITUTION, SPIRONOLACTONE (ALDACTONE) FOR ASCITES, IBUPROFEN FOR PAINS, METAMIZOLE (NOVAMINSULFON) FOR PAINS, FERROUS GLYCINE/PROPRANOLOL (OBSIDAN) FOR DECREASE OF BLOOD PRESSURE, CERESIN/GLYCEROL/LEVOMENOL/LIQUID PARAFFIN/PANTHENOL/WOOL ALCOHOL/XIPAMIDE/YELLOW SOFT PARAFFIN (AQUAPHOR) FOR OEDEMA, ORNITHINE (HEAP-MERZ) FOR LIVER CIRRHOSIS, METOCLOPRAMIDE (MCP TROPFEN) FOR PEPTIC OESOPHAGITIS, SUCRALFATE (ULCOGANT) FOR GASTROINTESTINAL ULCERS, ESOMEPRAZOLE (NEXIUM) FOR ERADICATION, CALCIUM CARBONATE/COLECALCIFEROL (CALCILAC KT) FOR OSTEOPOROSIS, BENFOTIAMINE/CYANOCOBALAMIN/PYRIDOXINE/THIAMINE (MILGAMMA) FOR "VITAMIN B1/B6 DEFICIT", OMEPRAZOLE (OMEP) FOR GASTRIC PROTECTION AND DICLOFENAC FOR PAINS.

IT WAS REPORTED THAT THE PATIENT HAD RECEIVED DICLOFENAC IN THE PAST AND WAS WELL TOLERATED.

ON 14 FEBRUARY 2003, THE PATIENT COMMENCED ORAL IBUPROFEN (TWICE A DAY AS NEEDED) AND ORAL METAMIZOLE (TWICE A DAY AS NEEDED). ON 22 FEBRUARY 2003, THE PATIENT COMMENCED ORAL PHYTOMENADIONE ("3X10 DROPS"), ORAL ORNITHINE ("1-1-1 BAG, 2-2-2 BAG"), ORAL METOCLOPRAMIDE ("3X36 DROPS"), ORAL CALCIUM CARBONATE/COLECALCIFEROL ("1-0-1,1-0-0") AND ORAL BENFOTIAMINE/CYANOCOBALAMIN/PYRIDOXINE/THIAMINE (THREE TIMES DAILY). ON 24 FEBRUARY 2003, THE PATIENT COMMENCED ORAL SUCRALFATE ("2X1 BAG, 4X1 BAG") AND THE FOLLOWING DAY STARTED ORAL ESOMEPRAZOLE (20 MG IN THE MORNING). ON 27 FEBRUARY 2003, THE PATIENT COMMENCED ORAL SPIRONOLACTONE ("2X50MG, 3X50MG AND 2X50MG"). ON 4 MARCH 2003, THE PATIENT COMMENCED ORAL CERESIN/GLYCEROL/LEVOMENOL/LIQUID PARAFFIN/PANTHENOL/WOOL ALCOHOL/XIPAMIDE/YELLOW SOFT PARAFFIN (20 MG IN THE MORNING). ON 12 MARCH 2003, THE PATIENT COMMENCED ORAL FERROUS GLYCINE/PROPRANOLOL ("3X1/2"). ON 19 MARCH 2003, THE PATIENT COMMENCED ORAL OMEPRAZOLE (40 MG). ON 3 APRIL 2003, THE PATIENT RECEIVED ORAL DICLOFENAC (ONE DOSE FORM). ON 4 APRIL 2003, THE PATIENT EXPERIENCED LIFE-THREATENING TOXIC EPIDERMAL NECROLYSIS (CO-MANIFESTATION INCLUDED LIPS AND ORAL MUCOSA EROSIONS). THE FOLLOWING DAY, SHE HAD SKIN PAIN AND FIRST SKIN MODIFICATIONS. ON 6 APRIL 2003, THE PATIENT HAD FIRST SKIN EROSION. ON 7 APRIL 2003, THE PATIENT HAD GENITAL AND NASAL EROSION. ON 8 APRIL 2003, SPIRONOLACTONE AND OMEPRAZOLE WERE DISCONTINUED. ON 9 APRIL 2003, THE PATIENT WAS HOSPITALISED AND TREATMENT WITH PHYTOMENADIONE, IBUPROFEN, METAMIZOLE, FERROUS GLYCINE/PROPRANOLOL, CERESIN/GLYCEROL/LEVOMENOL/LIQUID PARAFFIN/PANTHENOL/WOOL ALCOHOL/XIPAMIDE/YELLOW SOFT PARAFFIN, ORNITHINE,

METOCLOPRAMIDE, SUCRALFATE, ESOMEPRAZOLE, CALCIUM CARBONATE/COLECALCIFEROL AND EFOTIAMINE/PYRIDOXIN WAS DISCONTINUED. ON 17 APRIL 2003, THE PATIENT DIED.

THE PHYSICIAN ASSESSED THE DRUG-EVENT RELATIONSHIP BETWEEN TOXIC EPIDERMAL NECROLYSIS AND SPIRONOLACTONE, METAMIZOLE, FERROUS GLYCINE/PROPRANOLOL, CERESIN/GLYCEROL/LEVOMENOL/LIQUID PARAFFIN/PANTHENOL/WOOL ALCOHOL/XIPAMIDE/YELLOW SOFT PARAFFIN, SUCRALFATE, ESOMEPRAZOLE AND OMEPRAZOLE AS POSSIBLE RELATED AND TO PHYTOMENADIONE, IBUPROFEN, ORNITHINE, METOCLOPRAMIDE, CALCIUM CARBONATE/COLECALCIFEROL AND BENFOTIAMINE/CYANOCOBALAMIN/PYRIDOXINE/THIAMINE AS QUESTIONABLY RELATED AND TO DICLOFENAC AS VERY WELL POSSIBLY RELATED. THE PHYSICIAN FURTHER ASSESSED THE FATAL TOXIC EPIDERMAL NECROLYSIS AS NOT RELATED TO THE INFECTION BUT POSSIBLY RELATED TO LIVER CIRRHOSIS.

NO FURTHER INFORMATION WAS AVAILABLE.

Sender's comment:

CIOMS TEXT: FATAL TOXIC EPIDERMAL NECROLYSIS IS NOT LISTED IN THE COMPANY REFERENCE DOCUMENT (ISPI) FOR PHYTOMENADIONE. BASED ON THE INFORMATION RECEIVED FOR THIS CASE, A CAUSAL RELATIONSHIP BETWEEN TOXIC EPIDERMAL NECROLYSIS AND TREATMENT WITH PHYTOMENADIONE IS ASSESSED AS POSSIBLE. COMPANY CAUSALITY WAS ASSESSED BASED ON THE NARANJO ALGORITHM. HOWEVER, A POSSIBLE ALTERNATIVE EXPLANATION FOR THE EVENT INCLUDES THE CONCOMITANT USE OF IBUPROFEN, FERROUS GLYCINE/PROPRANOLOL, ESOMEPRAZOLE AND DICLOFENAC.

FATAL TOXIC EPIDERMAL NECROLYSIS IS NOT LISTED IN THE COMPANY REFERENCE DOCUMENT (IDS) FOR SPIRONOLACTONE. BASED ON THE INFORMATION RECEIVED FOR THIS CASE, A CAUSAL RELATIONSHIP BETWEEN TOXIC EPIDERMAL NECROLYSIS AND TREATMENT WITH SPIRONOLACTONE IS ASSESSED AS POSSIBLE. COMPANY CAUSALITY WAS ASSESSED BASED ON THE NARANJO ALGORITHM. HOWEVER, A POSSIBLE ALTERNATIVE EXPLANATION FOR THE EVENT INCLUDES THE CONCOMITANT USE OF IBUPROFEN, FERROUS GLYCINE/PROPRANOLOL, ESOMEPRAZOLE AND DICLOFENAC.; ***LOCAL ASSESSMENT: INTERVIEW NR. 9150023

FATAL TOXIC EPIDERMAL NECROLYSIS = NOT LABELED FOR PHYTOMENADIONE AND FOR SPIRONOLACTONE

ALTERNATIVE EXPLANATION INCLUDES CONCOMITANT USE OF IBUPROFEN, FERROUS GLYCINE/PROPRANOLOL, ESOMEPRAZOLE AND DICLOFENAC

EINZELFALLBEOBACHTUNG. KEIN HANDLUNGSBEDARF.

Reporter's comment:

AZPROD0000, 22.02.2008 DE-AstraZeneca-2008PK00355

Sender diagnosis: not reported

Narrative

{TOXIC EPIDERMAL NECROLYSIS}

A report was received from the German documentation centre for severe skin reactions via the German health authority BfArM concerning a 68 year old female patient. The patient was being treated with NEXIUM ORAL (esomeprazole magnesium) 20 mg QD oral for hp eradication, OMEP (omeprazole), diclofenac, ULCOGANT (SUCRALFATE) (sucralfate), AQUAPHOR TABLET (xipamide), OBSIDAN (PROPRANOLOL) (propranolol hydrochloride), metamizole and ALDACTONE (spironolactone).

Concomitant drugs included ibuprofen for pain, lactulose for liver cirrhosis, metoclopramide for reflux esophagitis, phytomenadione for vitamin k substitution, calcilac kt for osteoporosis and milgamma for vitamine b1/b6 deficiency.

After starting treatment on 25-FEB-2003 the patient experienced lip and oral mucosa erosions on 04-Apr-2003, initial skin reactions with pain on 05-Apr-2003, genital and nasal as well as general skin erosions on 06-Apr-2003 and positive Nikolsky phenomenon on 07-Apr-2003. She was hospitalised on 09-Apr-2003 and diagnosis of toxic epidermal necrolysis was made. The patient died from the disease on 17-APR-2003.

Experts of the documentation centre for severe skin reactions assessed a causal relationship to esomeprazole, omeprazole, sucralfate, xipamide, propranolol, metamizole, diclofenac and spironolactone as possible.

Serious criterion was death.

Sender's comment:

Toxic epidermal necrolysis is listed as a possible adverse event for esomeprazole. The case is poorly documented and the patient was treated with several suspect drugs. Thus it is not possible to establish an unequivocal causal relationship to esomeprazole. Report according to article 63b AMG:

The case is incompletely documented. Toxic epidermal necrolysis is listed as a possible adverse event for esomeprazole in the German product information, but the patient was treated with several other drugs which are also suspected. Thus it is impossible to establish a causal relationship to a single compound.

Causality assessment: UnassessableNO

Reporter's comment:

Merck KGaA, 22.02.2008 DE-Merck KGaA-6040908

Sender diagnosis: not reported

Narrative

This is a case from the Schoepf registry at the Dermatological University Hospital Freiburg, Germany which registers and evaluates all cases of hospitalisation for SJS/TEN in Germany and the drugs given in temporal relationship with the adverse event received via the German Health Authority (# 08014098) on 19 Feb 2008.

**

A 68-year-old female patient was treated with Ulcogant suspension (sucralfate, 2x1, 4x1) for gastric ulcer since 24 Feb 2003.

On 02 Apr 2003 she experienced lip erosions, oral stomatitis, nasal and genital erosions, skin reactions and pain of skin. She showed symptoms of Nikolski phenomena. On 09 Apr 2003 she was hospitalized and Steven-Johnson Syndrome was diagnosed.

On 17 Apr 2003 the event stopped and the patient died on an unknown date.

Medical history included cirrhosis of liver.

Other suspected drugs Aldactone (spironolactone), Ibuprofen (ibuprofen), Novaminsulfon drops (metamizol), Obsidan (propranolol), Aquaphor (xipamid), Hepa-Merz-Syrup (lactulose), MCP drops (metoclopramid), Nexium (esomeprazole), Konakion N (phytomenadion), Calcilac KT (CaCO₃, colecalciferol), Milgamma (benfotiamin, pyridoxin), Omep (omeprazole) and Diclofenac (diclofenac.)

The reporter assessed Steven-Johnson-Syndrome as probably related to Diclofenac and as possibly related to Ulcogant, Aldactone, Novaminsulfon, Obsidan, Aquaphor, Nexium and Omep. Ibuprofen, Hepa-Merz-Syrup, MCP drops, Konakion N, Calcilac KT, Milgamma were assessed as doubtful related to SJS.

Sender's comment:

A 68-year-old female developed toxic epidermal necrolysis (TEN) with fatal outcome 39 days after introduction of ulcogant (sucralfat, 2 or 4 dosage forms/d - conflicting information received) for ulcerations of upper digestive tract, 49 days after introduction of ibuprofen (taken as required) for pain and 1 day after having taken diclofenac once (dose not provided) for pain.

She was hospitalized 5 days after first signs of TEN. Actions taken on drugs not provided. She died later.

Co-suspected drugs are: Aldactone (spironolactone, dosage between 25 and 150 mg - not clearly stated) for ascites, started 36d prior to- and stopped 4d after occurrence of event; metamizol (taken as required) started 36d prior to event for pain; Obsidan (propranolol, 37,5 mg/d) for hypertension of V. portae, start 23d prior to event; Aquaphor (xipamide, 20 mg/d) for oedema, start 31d prior to event; Hepa-Merz Sirup (lactulose, dosage not provided) for hepatic cirrhosis, start 31d prior to event; metoclopramide (dosage not provided) for oesophageal reflux, start 31d prior to event; Konakion N (phytomenadion, dosage not provided) for substitution, start 31d prior to event; Calcilac KT (calciumcarbonate / colecalciferol, dosage not provided) for osteoporosis, start 31d prior to event; Milgamma (benfotiamin/pyridoxin, dosage not provided) for hypo-aminosis, start 31d prior to event; Esomeprazole (20mg/d) for helicobacter pylori eradication, start 38d prior to event; Omep (omeprazole, 40 mg/d) as prophylaxis, start 15d prior to - and discontinued 4d after occurrence of event; Concomitant diseases: Liver cirrhosis possible (not clearly stated).

Company comment: Event serious, temporal relationship compatible, latencies: 39d for ulcogant, 49d for ibuprofen, 1d for diclofenac. Event is unlisted for ulcogant but listed for ibuprofen and diclofenac as per company reference documents. Alternative explanation: Metamizol, esomeprazole and omeprazole may have contributed (event listed). Liver cirrho

Reporter's comment:

Lilly Deutschland GmbH, 04.03.2008 DE-ELI LILLY AND COMPANY-DE200802005173

Sender diagnosis: not reported

Narrative

This spontaneous case reported by a physician via a regulatory agency concerns a 68 year old female patient of unknown origin.

The medical history of the patient was not reported. Concomitant medications included spironolactone, ibuprofen, metamizol, propranolol, lactulose, metoclopramide, sucralfate, esomepromazole, phytomenadion, colecalciferol with calcium carbonate, benfotiamin with pyridoxin, omeprazole and diclofenac, all for unreported indications.

The patient received xipamide (Aquaphor), 20mg, orally, once daily, in the morning, for the treatment of oedema, beginning on 04-Mar-2003. On 04-Apr-2003, approximately one month after starting xipamide, the patient experienced lips erosions and MSH erosions (to be clarified). On 05-Apr-2003, the patient experienced pain of skin, first skin changes. On 06-Apr-2003, the patient experienced first erosions of the skin, erosions genital and nasal. On 07-Apr-2003, the patient experienced Nikolsky signs. On 09-Apr-2003, the patient was hospitalized. On 04-Apr-2003, the patient was reported to have developed toxic epidermal necrolysis with macula. The patient died on an unreported date. Cause of death was not specified. No information on corrective treatment or laboratory data was provided. It was not reported if an autopsy was performed. It was not reported if xipamide was continued til death.

No reporter contact, lot number not provided.

The reporting physician did not provide an opinion of relatedness between the event toxic epidermal necrolysis and xipamide.

Sender's comment:

Bewertung - 4. März 2008

Verdachtsfall: schwerwiegend und unbekannt

Der Kausalzusammenhang ist denkbar.

Allerdings bewirkt der Fall keine Aenderung des wissenschaftlichen Erkenntnisstandes. Weitergehende Massnahmen nach § 29 Abs. 1 AMG werden nicht fuer erforderlich gehalten.

Reporter's comment:

SANOFI-SYNTHELABO, 07.03.2008 DE-SANOFI-SYNTHELABO-A01200802511

Sender diagnosis: not reported

Narrative

Reaction as reported: toxic epidermal necrolysis with lip erosions, oral stomatitis, nasal and genital erosions, skin reactions and pain of skin

LLT: TOXIC EPIDERMAL NECROLYSIS / 10044223

PT: TOXIC EPIDERMAL NECROLYSIS / 10044223

Narrative:

Case received from the Health authorities through Chugai Pharm (2008CSK_00050) and Merck (6040908) on 25-Feb-2008.

This is a case from the Schoepf registry at the dermatological University Hospital Freiburg which registers and evaluates all cases of hospitalization for Stevens-Johnson-syndrome (SJS)/ toxic epidermal necrolysis (TEN) in Germany and the drugs in temporal relationship with the adverse event received via the German Health authority on 19-Feb-2008.

A 68-year-old female patient had been treated with sucralfate, for gastric ulcer since 24-Feb-2003. On 04-Apr-2003, she experienced lip erosions, oral stomatitis, nasal and genital erosions, skin reactions and pain of skin. She showed symptoms of Nikolski phenomena. On 09-Apr-2003, she was hospitalized and Steven-Johnson syndrome was diagnosed.

On 17-Apr-2003, the reaction stopped and the patient died on an unknown date.

Medical history included cirrhosis of liver.

Other suspected drugs: spironolactone, ibuprofen (Ibuprofen), metamizol (Novaminsulfon drops), propranolol, xipamid, lactulose, metoclopramid, esomeprazole, phytomenadion, calcium carbonate + colecalciferol, benfotiamin + pyridoxin, omeprazole and diclofenac (Diclofenac).

The reporter assessed Stevens-Johnson-syndrome as: probably related to diclofenac and as possibly related to sucralfate, spironolactone, metamizol, propranolol, xipamid, esomeprazole, omeprazole and doubtful for ibuprofen, lactulose, metoclopramid, phytomenadion and calcium carbonate + colecalciferol, benfotiamin + pyridoxine.

Evaluation by Merck:

A 68-year-old female developed toxic epidermal necrolysis (TEN) with fatal outcome 39 days after introduction of sucralfate (Ulcogant, 2 or 4 dosages forms/d conflicting information received) for ulcerations of upper digestive tract, 49 days after introduction of ibuprofen (taken as required) for pain and 1 day after having taken diclofenac once (dose not provided) for pain.

She was hospitalized 5 days after first signs of TEN. Action taken on drugs was not provided. She died later.

Co-suspected drugs were: spironolactone (dosage between 25 and 150mg, not clearly stated) for ascites, started 36 d prior to and stopped 4 d after occurrence of the reaction; metamizol (taken as required) started 36 d prior to reaction for pain; propranolol 37.5 mg/d for hypertension of V. portae, started 23d prior to reaction; xipamide 20 mg/d for oedema, started 31 d prior to event; lactulose for hepatic cirrhosis, start 31 d prior to reaction; metoclopramide for oesophageal reflux, start 31 d prior to reaction, phytomenadion for substitution, start 31 d prior to event; calciumcarbonate+ colecalciferol for osteoporosis, start 31 d prior to reaction; benfotiamin + pyridoxine for hypo-vitaminosis, start 31 d prior to reaction; esomeprazole (20 mg/d) for helicobacter pylori eradication, start 38 d prior to reaction; omeprazole 40 mg/d as prophylaxis, start 15 d prior to and discontinued 4d after occurrence of reaction;

Concomitant disease: liver cirrhosis possible (not clearly stated).

Company comment: reaction serious, temporal relationship compatible, latencies: 39 d for sucralfate, 49 d for ibuprofen, 1 d for diclofenac. reaction unlisted for sucralfate but listed for ibuprofen and diclofenac as per company reference documents.

Alternative explanation: Metamizol, esomeprazole and omeprazole may have contributed (reaction listed). Liver cirrhosis may have contributed to fatal outcome.

Causality: Possible for all drugs.

This case was also captured in Aventis database with reference 200810959EU.

Sender's comment:

Reporter's comment:

Merck KGaA, 18.04.2008 DE-Merck KGaA-6040908

Sender diagnosis: not reported

Narrative

This is a case from the Schoepf registry at the Dermatological University Hospital Freiburg, Germany which registers and evaluates all cases of hospitalisation for SJS/TEN in Germany and the drugs given in temporal relationship with the adverse event received via the German Health Authority (# 08014098) on 19 Feb 2008.

**

A 68-year-old female patient was treated with Ulcogant suspension (sucralfate, 2x1, 4x1) for gastric ulcer since 24 Feb 2003.

On 02 Apr 2003 she experienced lip erosions, oral stomatitis, nasal and genital erosions, skin reactions and pain of skin. She showed symptoms of Nikolski phenomena. On 09 Apr 2003 she was hospitalized and Steven-Johnson Syndrome was diagnosed.

On 17 Apr 2003 the event stopped and the patient died on an unknown date.

Medical history included cirrhosis of liver.

Other suspected drugs Aldactone (spironolactone), Ibuprofen (ibuprofen), Novaminsulfon drops (metamizol), Obsidan (propranolol), Aquaphor (xipamid), Hepa-Merz-Syrup (lactulose), MCP drops (metoclopramid), Nexium (esomeprazole), Konakion N (phytomenadion), Calcilac KT (CaCO₃, colecalciferol), Milgamma (benfotiamin, pyridoxin), Omeprazole (omeprazole) and Diclofenac (diclofenac.)

The reporter assessed Steven-Johnson-Syndrome as probably related to Diclofenac and as possibly related to Ulcogant, Aldactone, Novaminsulfon,

Obsidan, Aquaphor, Nexium and Omep. Ibuprofen, Hepa-Merz-Syrup, MCP drops, Konakion N, Calcilac KT, Milgamma were assessed as doubtful related to SJS.

Correction of company assessment on 11 Apr 2008.

The event was not the cause of the death, the event resolved but the patient died on unknown date of unknown cause.

Sender's comment:

A 68-year-old female developed toxic epidermal necrolysis (TEN) with fatal outcome 39 days after introduction of ulcogant (sucralfat, 2 or 4 dosage forms/d - conflicting information received) for ulcerations of upper digestive tract, 49 days after introduction of ibuprofen (taken as required) for pain and 1 day after having taken diclofenac once (dose not provided) for pain. She was hospitalized 5 days after first signs of TEN. Actions taken on drugs not provided. She died later. Co-suspected drugs are: Aldactone (spironolactone, dosage between 25 and 150 mg - not clearly stated) for ascites, started 36d prior to- and stopped 4d after occurrence of event; metamizol (taken as required) started 36d prior to event for pain; Obsidan (propranolol, 37,5 mg/d) for hypertension of V. portae, start 23d prior to event; Aquaphor (xipamide, 20 mg/d) for oedema, start 31d prior to event; Hepa-Merz Sirup (lactulose, dosage not provided) for hepatic cirrhosis, start 31d prior to event; metoclopramide (dosage not provided) for oesophageal reflux, start 31d prior to event; Konakion N (phytomenadion, dosage not provided) for substitution, start 31d prior to event; Calcilac KT (calciumcarbonate / colecalciferol, dosage not provided) for osteoporosis, start 31d prior to event; Milligamma (benfotiamin/pyridoxin, dosage not provided) for hypo- vitaminosis, start 31d prior to event; Esomeprazole (20mg/d) for helicobacter pylori eradication, start 38d prior to event; Omep (omeprazole, 40 mg/d) as prophylaxis, start 15d prior to - and discontinued 4d after occurrence of event; Concomitant diseases: Liver cirrhosis possible (not clearly stated). Company comment: Event serious, temporal relationship compatible, latencies: 39d for ulcogant, 49d for ibuprofen, 1d for diclofenac. Event is unlisted for ulcogant but listed for ibuprofen and diclofenac as per company reference documents. Alternative explanation: Metamizol, esomeprazole and omeprazole may have contributed (event listed). Liver cirrhosis

Reporter's comment:

SANOFI-SYNTHELABO, 25.04.2008 DE-SANOFI-SYNTHELABO-A01200802511

Sender diagnosis: not reported

Narrative

Reaction as reported: toxic epidermal necrolysis with lip erosions, oral stomatitis, nasal and genital erosions, skin reactions and pain of skin
LLT: TOXIC EPIDERMAL NECROLYSIS / 10044223
PT: TOXIC EPIDERMAL NECROLYSIS / 10044223

Narrative:

Corrected CIOMS on 23-Apr-2008: after internal review by the partner, the reaction was identified as toxic epidermal necrolysis and not Stevens-Johnson syndrome. The outcome was fatal, the patient died from the reaction.

Case received from the Health authorities through Chugai Pharm (2008CSK_00050) and Merck (6040908) on 25-Feb-2008.

This is a case from the Severe Cutaneous Adverse Reactions registry at the dermatological University Hospital Freiburg which registers and evaluates all cases of hospitalization for Stevens-Johnson-syndrome (SJS)/ toxic epidermal necrolysis (TEN) in Germany and the drugs in temporal relationship with the adverse event received via the German Health authority on 19-Feb-2008.

A 68-year-old female patient had been treated with sucralfate, for gastric ulcer since 24-Feb-2003. On 04-Apr-2003, she experienced lip erosions, oral stomatitis, nasal and genital erosions, skin reactions and pain of skin. She showed symptoms of Nikolski phenomena. On 09-Apr-2003, she was hospitalized and Steven-Johnson syndrome was diagnosed.

On 17-Apr-2003, the reaction stopped and the patient died on an unknown date.

Medical history included cirrhosis of liver.

Other suspected drugs: spironolactone, ibuprofen (Ibuprofen), metamizol (Novaminsulfon drops), propranolol, xipamid, lactulose, metoclopramid, esomeprazole, phytomenadion, calcium carbonate + colecalciferol, benfotiamin + pyridoxin, omeprazole and diclofenac (Diclofenac).

The reporter assessed Stevens-Johnson-syndrome as: probably related to diclofenac and as possibly related to sucralfate, spironolactone, metamizol, propranolol, xipamid, esomeprazole, omeprazole and doubtful for ibuprofen, lactulose, metoclopramid, phytomenadion and calcium carbonate + colecalciferol, benfotiamin + pyridoxine.

Evaluation by Merck:

A 68-year-old female developed toxic epidermal necrolysis (TEN) with fatal outcome 39 days after introduction of sucralfate (Ulcogant, 2 or 4 dosages forms/d conflicting information received) for ulcerations of upper digestive tract, 49 days after introduction of ibuprofen (taken as required) for pain and 1 day after having taken diclofenac once (dose not provided) for pain.

She was hospitalized 5 days after first signs of TEN. Action taken on drugs was not provided. She died later.

Co-suspected drugs were: spironolactone (dosage between 25 and 150mg, not clearly stated) for ascites, started 36 d prior to and stopped 4 d after occurrence of the reaction; metamizol (taken as required) started 36 d prior to reaction for pain; propranolol 37.5 mg/d for hypertension of V. portae, started 23d prior to reaction; xipamide 20 mg/d for oedema, started 31 d prior to event; lactulose for hepatic cirrhosis, start 31 d prior to reaction; metoclopramide for oesophageal reflux, start 31 d prior to reaction, phytomenadion for substitution, start 31 d prior to event; calciumcarbonate+ colecalciferol for osteoporosis, start 31 d prior to reaction; benfotiamin + pyridoxine for hypo-vitaminosis, start 31 d prior to reaction; esomeprazole (20 mg/d) for helicobacter pylori eradication, start 38 d prior to reaction; omeprazole 40 mg/d as prophylaxis, start 15 d prior to and discontinued 4d after occurrence of reaction;

Concomitant disease: liver cirrhosis possible (not clearly stated).

Company comment: reaction serious, temporal relationship compatible, latencies: 39 d for sucralfate, 49 d for ibuprofen, 1 d for diclofenac. reaction unlisted for sucralfate but listed for ibuprofen and diclofenac as per company reference documents.

Alternative explanation: Metamizol, esomeprazole and omeprazole may have contributed (reaction listed). Liver cirrhosis may have contributed to fatal outcome.

Causality: Possible for all drugs.

This case was also captured in Aventis database with reference 200810959EU.

Corrected CIOMS on 23-Apr-2008.

The reaction was toxic epidermal necrolysis and not Stevens-Johnson syndrome. The outcome was fatal, the patient died from the reaction.

Sender's comment:

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08014535

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 8 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20030915	20031001	17 Day	6 Day	recovered/resolved
2 Mucosal erosion	20030916			7 Day	recovered/resolved
3 Blister	20030916			7 Day	recovered/resolved
4 Swelling of lips	20030915			6 Day	recovered/resolved
5 Pruritus	20030916			7 Day	recovered/resolved
6 Pain of skin	20030916			7 Day	recovered/resolved
7 Oedema mouth	20030915			6 Day	recovered/resolved
8 Erythema	20030916			7 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ERYHEXAL erythromycin	Oral	/	20030910-20030913 4 Day	Infection
2 NASIC dexpanthenol; xylometazoline hydrochloride	Nasal	/	20030910-20030913 4 Day	Rhinitis
3 NUROFEN ibuprofen	Oral	3 Df dosage form/Day	20030910-20030917 8 Day	Pain
4 SINUPRET ascorbic acid; gentiana; primula flowers; rumicis asetosa; sambucus flowers; verben herb	Oral	9 Df dosage form/Day	20030910-20030913 4 Day	Sinusitis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Influenza like illness	20030910		Unknown	Eye inflammation, fever up to 40 degree celsius
Sinusitis	20030910		Unknown	
Rhinitis	20030910		Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 28.02.2008 DE-SZ09-GXKR2008DE01793

Sender diagnosis: not reported

Narrative

Registry report from the Freiburg Documentation Center for Severe Skin Reactions received on 19 Feb 2008 from the German health authority (BfArM case number: DE-BFARM-08014535): This patient with medical history of influenza-like illness with eye inflammation, fever up to 40 degrees, sinusitis and rhinitis developed Erythema exsudativum multiforme maius during treatment with Eryhexal forte Saft (erythromycin) for infection taken orally at an unknown dosage from 10 Sep 2003 until 13 Sep 2003. Concomitantly the patient received other drugs of which Sinupret (herbal drug), Nasic Nasenspray (xylometazolin and dexpanthenol) and Nurofen (ibuprofen) were considered also possibly causative. On 15 Sep 2003 lip swelling and mouth oedema occurred, on 16 Sep 2003 pain of skin, pruritus, coxades generalized, blisters as well as lip, oral, nasal and genital mucosal erosions occurred. On 17 Sep 2003 the patient was hospitalised. The adverse event lasted until 01 Oct 2003 and the patient recovered. The reporter assessed the causality between adverse event and administration of Eryhexal forte Saft as quite possible. The causality between adverse event and underlying disease (influenza-like illness) was assessed as probable by the reporter.

Sender's comment:

Serious adverse event report of Erythema multiforme (hospitalisation) assessed as listed according to the Core Data Sheet Information. Reporting physician causality is suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-08017695

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 6 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum multiforme	20041231	20050114	15 Day	7 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AMOXYPEN SAFT amoxicillin trihydrate		3*5 ml millilitre(s)/ Syrup	20041227-20050102	Infection
2 BENURON paracetamol	Oral	500 Mg milligram(s)/	20041225-20041226	Fever
3 DOLORMIN SAFT ibuprofen	Oral	100 Mg milligram(s)/ Syrup	20041224-20041224	Flu
4 MUCOSOLVAN SAFT 30 MG/5 ML ambroxol hydrochloride	Oral	3*5 ml millilitre(s)/ Syrup	20041224-20041226	Flu
5 PARACETAMOL paracetamol	Oral	500 Mg milligram(s)/ Day	20041225-	Mycoplasma infection

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Fever			Unknown	
Influenza			Unknown	
Mycoplasma infection NOS			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Primary Source auf zugesandtem BFARM-Meldebogen leider geschwÃ¤rzt; warum?	

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 03.03.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-01172DE

Sender diagnosis: not reported

Narrative

This case was identified and registered by the "Centre of Documentation of severe skin diseases" in Freiburg, Germany and reported to the manufacturers as well as to the Federal Health Office. The "Centre" collects and files nationwide case reports of severe skin diseases.

Therefore, these reports must not be confused with documentations of suspected adverse drug reactions.

It was reported that a 6 year-old female child developed erythema exsudativum multiforme majus (EEMM) from 31Dec2004 until 14Jan2005 (but not recovered).

Flu with Fever up to 38.5 C started on 23Dec2004. On 24Dec2004 treatment with Mucosolvan (ambroxol) syrup 3x5 ml and Dolormin (ibuprofen) syrup 1x5 ml was initiated.

In addition the child received Benuron (paracetamol) 1x500 mg per os from 25Dec2004 until 26Dec2004. Mycoplasma infection was diagnosed treated with Amoxypen (amoxicillin) syrup 3x5 ml from 27Dec2004 until 01Jan2005.

On 01Jan2005 burning and redness of mouth mucosa with burning and swelling lips occurred.

Erythema exsudativum multiforme majus with skin pain, eye redness and erosions of mouth mucosa was diagnosed. The child was hospitalised on 02Jan2005 with pruritus, cockades and spots followed by lip erosions and skin blisters.

These adverse event was formally not marked as life-threatening.

The reporting source rated causal relationship between Mucosolvan and erythema exsudativum multiforme majus as possible.

Sender's comment:

Regional Centre Drug Safety Boehringer Ingelheim

Comment on case 2008-DE-01172DE / MUCOSOLVAN SAFT 30 MG/5 ML

Erythema exsudativum multiforme majus (EEMM) is unlisted for Mucosolvan syrup. Other factors have been reported in this case which may have caused the events, such as the other suspect drugs paracetamol, ibuprofen and, with even more likelihood, amoxicillin or the underlying mycoplasma infection.

As with all unlisted events, the company will continue to monitor all respective reports received and, based on cumulative experience, will re-evaluate the available evidence on an ongoing basis.

No change in the company core safety information or other measures are recommended at this time

PILZDEYN

Case assessment created by PL on 29 Feb 2008

Reporter's comment:

GRUNENTHAL, 06.03.2008 DE-GRUNENTHAL-2008 10811

Sender diagnosis: not reported

Narrative

SUBJECT/PATIENT ID

Manufacturer Control Number: 2008-10811

ADVERSE EVENT DETAILS

This solicited case from special registry for severe skin reactions from GERMANY, that was reported by a other health professional via regulatory authority was received at GRT on 25-FEB-2008 and concerns a 6 years old female patient (height unknown / weight unknown).

The relevant medical history included: Pyrexia, Influenza, Mycoplasma infection.

This case was reported by the Documentation Centre for Severe Skin reactions (Freiburg/Germany) via BfArM and concerns a 6 year old female patient. The patient was treated with Dolormin syrup (ibuprofen) and Mucosolvan syrup (ambroxol) for grippal infection and with Benuron

(paracetamol) for fever. In addition she was treated with Amoxypen syrup (amoxicillin) for infection from 27-Dec-2004 to 01-Jan-2005. The patient was suffering from mycoplasma infection. On 23-Dec-2004 the patient experienced fever up to 38.5 degrees Celsius and grippal infection. On 31-Dec-2004, 5 days after first administration of AMOXYPEN UNK burning/reddening of the oral mucosa and burning/swelling of the lips occurred followed by painful skin, reddening of the eyes and erosions of the oral mucosa on 01-Jan-2005. On 02-Jan-2005, pruritus, cockades and maculae appeared. The patient was hospitalized. On 03-Jan-2005, she experienced skin blisters and erosions of the lips. The reported symptoms were considered not to be life-threatening. Outcome: not yet recovered.

The relevant concomitant medication included:

IBUPROFEN / DOLORMIN;

AMBROXOL HYDROCHLORIDE / MUCOSOLVAN;

PARACETAMOL / BENURON;

Third party case numbers

E2B Authority Number: DE-BFARM-08017695

(GLS)-DE-GRT: DE-08-DS-0122

Sender's comment:

Stevens-Johnson syndrome is assessed as serious due to the criterion hospitalization.

Stevens-Johnson syndrome is listed in the reference safety information of amoxicillin trihydrate.

The adverse event occurred in a reasonable temporal relationship to the administration of amoxicillin trihydrate. In addition the patient suffered from a mycoplasma infection presenting a cause for the development of Stevens-Johnson syndrome as well.

In conclusion Stevens-Johnson syndrome is assessed to be possibly related to the administration of amoxicillin trihydrate.

The information in this single case does not appear to alter the favourable benefit to risk ratio of amoxicillin trihydrate.

Reporter's comment:

McNeil Germany, Local Safety Officers, 07.03.2008 DE-JNJFOC-20080206506

Sender diagnosis: not reported

Narrative

This spontaneous report from a physician concerns a 6-year-old female from Germany: 00025-08.

The patient's weight, height and medical history were unknown. The patient was treated with ibuprofen (100 mg/5 ml/suspension, oral) 100 mg once total, on 24-DEC-2004 for common cold. Non-company suspect drugs included ambroxol hydrochloride (5 ml, 3 times total, 24-DEC-2004 to 25-DEC-2004) for common cold, amoxicillin trihydrate (5 ml, 3 times total, 27-DEC-2004 to 01-JAN-2005) for infection, and paracetamol (500 mg once total on 25-DEC-2004) for fever. Concomitant medications were not reported. On 31-DEC-2004, the patient experienced erythema exsudativum multiforme major. On 03-JAN-2005, the patient was admitted to the hospital. The physician considered the event of erythema exsudativum multiforme major most suspected to amoxicillin, and also considered the other suspect medications as possibly related. Additionally, an infection with mycoplasma was assessed as probable. Treatment with ibuprofen, ambroxol hydrochloride, amoxicillin trihydrate and paracetamol was withdrawn. As of MAR-2005, the patient was not fully recovered.

This report was serious (hospitalization).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Single case assessment (reported term/preferred term): IBUPROFEN

ERYTHEMA MULTIFORME MAJOR /STEVENS-JOHNSON SYNDROME: expected per local SmPc

The benefit-risk-ratio remains unchanged based on the single case assessment of this case. No measures according to § 29 (1) AMG have to be taken.

Reporter's comment:

Bene-Arzneimittel, 10.03.2008 DE-Bene-Arzneimittel-B220

Sender diagnosis: not reported

Narrative

siehe auch Patient's History;

Behandlung u.a. mit Ben-u-ron p.o. erfolgte aufgrund Fieber bis 38,5Â° und grippalem Infekt ("Mykoplasmeninfektion: ja, wahrscheinlich"); 5 Tage nach einmaliger Einnahme von Ben-u-ron (sowie unmittelbar nach 4- bis 5-tägiger Behandlung mit Amoxicillin) wurden folgende Hauterscheinungen beobachtet: "Brennen/Rötung MSH, Brennen/Schwellung Lippen, Schmerzen der Haut, Augen gerötet, MSH-Erosionen, Pruritus, Kokarden und Flecken, schließlich Hautblasen und Lippenerosionen". 7 Tage nach Einnahme von Ben-u-ron p.o. stationäre Aufnahme, wobei klin. Situation nicht als lebensbedrohlich eingestuft wurde.

Sender's comment:

Hauterscheinungen traten erst 5 Tage nach einmaliger Einnahme von Ben-u-ron p.o. auf.

Laut Dokumentation ist der Patient "noch nicht wiederhergestellt".....entspricht vermutlich einem Dokumentationsdefizit, da die Hauterscheinungen sicherlich nicht mehrere Jahre andauerten.....eine valide Stellungnahme ist nur auf Basis vollständiger Unterlagen möglich. Neue wissenschaftliche Erkenntnisse liegen nicht vor bzw. können durch diesen Fall nicht gewonnen werden.

Reporter's comment:

individuelle Bewertung durch Dermatologisches Expertengremium aufgrund spezifischem Bewertungsalgorithmus: Kat."5" = "Zusammenhang möglich" aufgrund engem zeitlichen Zusammenhang mit Einnahme von ben-u-ron. Begleitmedikationen waren Ibuprofen, Ambroxol und Amoxicillin; letzteres wurde vom Exp.-gremium als "verdächtiger Auslöser" eingestuft (Kat."4" = "Zusammenhang gut möglich")

Bene-Arzneimittel, 10.03.2008 DE-Bene-Arzneimittel-B220

Sender diagnosis: not reported

Narrative

siehe auch Patient's History;

Behandlung u.a. mit Ben-u-ron p.o. erfolgte aufgrund Fieber bis 38,5Â° und grippalem Infekt ("Mykoplasmeninfektion: ja, wahrscheinlich"); 5 Tage nach einmaliger Einnahme von Ben-u-ron (sowie unmittelbar nach 4- bis 5-tägiger Behandlung mit Amoxicillin) wurden folgende Hauterscheinungen beobachtet: "Brennen/Rötung MSH, Brennen/Schwellung Lippen, Schmerzen der Haut, Augen gerötet, MSH-Erosionen, Pruritus, Kokarden und Flecken, schließlich Hautblasen und Lippenerosionen". 7 Tage nach Einnahme von Ben-u-ron p.o. stationäre Aufnahme, wobei klin. Situation nicht als lebensbedrohlich eingestuft wurde.

Sender's comment:

Hauterscheinungen traten erst 5 Tage nach einmaliger Einnahme von Ben-u-ron p.o. auf.

Laut Dokumentation ist der Patient "noch nicht wiederhergestellt".....entspricht vermutlich einem Dokumentationsdefizit, da die Hauterscheinungen sicherlich nicht mehrere Jahre andauerten.....eine valide Stellungnahme ist nur auf Basis vollständiger Unterlagen möglich. Neue wissenschaftliche Erkenntnisse liegen nicht vor bzw. können durch diesen Fall nicht gewonnen werden.

Reporter's comment:

individuelle Bewertung durch Dermatologisches Expertengremium aufgrund spezifischem Bewertungsalgorithmus: Kat."5" = "Zusammenhang möglich" aufgrund engem zeitlichen Zusammenhang mit Einnahme von ben-u-ron. Begleitmedikationen waren Ibuprofen, Ambroxol und Amoxicillin; letzteres wurde vom Exp.-gremium als "verdächtiger Auslöser" eingestuft (Kat."4" = "Zusammenhang gut möglich")

McNeil Germany, Local Safety Officers, 20.03.2008 DE-JNJFOC-20080206506

Sender diagnosis: not reported

Narrative

This spontaneous report from a physician concerns a 6-year-old female from Germany: 00025-08.

The patient's weight, height and medical history were unknown. The patient was treated with ibuprofen (100 mg/5 ml/suspension, oral) 100 mg once total, on 24-DEC-2004 for common cold. Non-company suspect drugs included ambroxol hydrochloride (5 ml, 3 times total, 24-DEC-2004 to 25-DEC-2004) for common cold, amoxicillin trihydrate (5 ml, 3 times total, 27-DEC-2004 to 01-JAN-2005) for infection, and paracetamol (500 mg once total on 25-DEC-2004) for fever. Concomitant medications were not reported. On 31-DEC-2004, the patient experienced erythema exsudativum multiforme major. On 03-JAN-2005, the patient was admitted to the hospital. The physician considered the event of erythema exsudativum multiforme major most suspected to amoxicillin, and also considered the other suspect medications as possibly related. Additionally, an infection with mycoplasma was assessed as probable. Treatment with ibuprofen, ambroxol hydrochloride, amoxicillin trihydrate and paracetamol was withdrawn. As of MAR-2005, the patient was not fully recovered.

This report was serious (hospitalization).

Additional information received on 12-MAR-2008 indicated the body surface area involved was not calculated, and no testing regarding culprit drug or infection was done. No leukocyte stimulation testing was done. There were no concomitant diseases. It was reported that co-suspect drugs ambroxol hydrochloride and paracetamol were possibly responsible for the erythema exsudativum multiforme major and amoxicillin trihydrate was even more than possibly considered to be responsible. <dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Single case assessment (reported term/preferred term):

ERYTHEMA MULTIFORME MAJOR /STEVENS-JOHNSON SYNDROME: unexpected per local SmPc

The benefit-risk-ratio remains unchanged based on the single case assessment of this case. No measures according to § 29 (1) AMG have to be taken.

Reporter's comment:

Bene-Arzneimittel, 02.10.2009 DE-Bene-Arzneimittel-B220

Sender diagnosis: not reported

Narrative

siehe auch Patient's History;

Behandlung u.a. mit Ben-u-ron p.o. erfolgte aufgrund Fieber bis 38,5Â° und grippalem Infekt ("Mykoplasmeninfektion: ja, wahrscheinlich"); 5 Tage nach einmaliger Einnahme von Ben-u-ron (sowie unmittelbar nach 4- bis 5-tägiger Behandlung mit Amoxicillin) wurden folgende Hauterscheinungen beobachtet: "Brennen/Rötung MSH, Brennen/Schwellung Lippen, Schmerzen der Haut, Augen gerötet, MSH-Erosionen, Pruritus, Kokarden und Flecken, schließlich Hautblasen und Lippenerosionen". 7 Tage nach Einnahme von Ben-u-ron p.o. stationäre Aufnahme, wobei klin. Situation nicht als lebensbedrohlich eingestuft wurde.

Sender's comment:

Hauterscheinungen traten erst 5 Tage nach einmaliger Einnahme von Ben-u-ron p.o. auf.

Laut Dokumentation ist der Patient "noch nicht wiederhergestellt".....entspricht vermutlich einem Dokumentationsdefizit, da die Hauterscheinungen sicherlich nicht mehrere Jahre andauerten.....eine valide Stellungnahme ist nur auf Basis vollständiger Unterlagen möglich. Neue wissenschaftliche Erkenntnisse liegen nicht vor bzw. können durch diesen Fall nicht gewonnen werden.

Reporter's comment:

individuelle Bewertung durch Dermatologisches Expertengremium aufgrund spezifischem Bewertungsalgorithmus: Kat."5" = "Zusammenhang möglich" aufgrund engem zeitlichen Zusammenhang mit Einnahme von ben-u-ron. Begleitmedikationen waren Ibuprofen, Ambroxol und Amoxicillin; letzteres wurde vom Exp.-gremium als "verdächtigster Auslöser" eingestuft (Kat."4" = "Zusammenhang gut möglich")

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08016252

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 6 Year	weight (kg): 17	BMI_int: 13.55	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gangrenous appendicitis	200703				recovered/resolved
2 Abscess leg	200703				recovered/resolved
3 Hepatosplenomegaly	200703				recovered/resolved
4 Mucosal inflammation	200703				recovered/resolved
5 Decubitus	200703				recovered/resolved
6 Stomatitis	200703				recovered/resolved
7 Loss of weight	200703				recovered/resolved
8 Respiratory insufficiency	200703				recovered/resolved
9 Streptococcal tonsillitis	200703				recovered/resolved
10 Staphylococcal sepsis	200703				recovered/resolved
11 Pancytopenia	200703				recovered/resolved
12 Anaemia	200703				recovered/resolved
13 Agranulocytosis	200703				recovered/resolved
14 Thrombocytopenia	200703				recovered/resolved
15 Leukopenia	200703				recovered/resolved
16 Colonic dysfunction	200703				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Nurofen Junior Fiebersaft ibuprofen	Oral	/	200711-20071203 -	Fever
2 Nurofen Junior Fiebersaft ibuprofen	Oral	/	20070312-20070328 17 Day	Febrile infection

Drugs (concomitant)

3 boostrix diphtheria toxoid; pertussis vaccine acellular; tetanus toxoid	Intramuscular	/	20070301- -	Vaccination
4 capval noscapine resin	Oral	/	20070312- -	
5 contramutan aconite; adonis vernalis glycosides; belladonna alkaloids; echinacea spp; eupatorium perfoliatum; myristica	Oral	/	20070312- -	
6 infectocillin 500 soft phenoxymethylpenicillin potassium	Oral	/	20070320-20070326 7 Day	
7 klacid clarithromycin	Oral	/	20071202-20071203 2 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Sepsis			Unknown	
Tonsillitis			Unknown	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Not available to sender (unknown)

case: DE-BFARM-08019990

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		No

Patient

Initials:	Sex: Male	Birth: privacy	Age: 67 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20040927			12960 Minute	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Acetylsalizylsaeure acetylsalicylic acid		/	-	-
2 ANTRA omeprazole		/	-	-
3 AUGMENTAN I.V. amoxicillin sodium; clavulanate potassium		/	-	-
4 BAYOTENSIN nitrendipine		/	-	-
5 c-vitamin ascorbic acid		/	-	-
6 DOCITON propranolol hydrochloride		/	-	-
7 ibuprofen ibuprofen		/	-	-
8 LOPIRIN "BRISTOL-MYERS SQUIBB" captopril		/	-	-
9 METHOTREXAT methotrexate		/	-	-
10 mono-embolex certoparin sodium		/	-	-
11 NOVALGIN metamizole		/	-	-
12 Oxygesic 20 mg oxycodone hydrochloride	Oral	/	20040919-20040922 4 Day	Pain
13 praxiten oxazepam		/	-	-
14 REMERGIL mirtazapine		/	-	-
15 tavor lorazepam		/	-	-
16 ULTRALAN fluocortolone; fluocortolone caproate		/	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pain			Yes	Pain
Depression			Yes	Depression
Hypertension			Yes	Hypertension
Polyarthritis			Yes	Chronic polyarthritis
Arrhythmia			Unknown	Arrhythmia
Hospitalisation	20040919		Unknown	Hospitalisation
Arthroplasty NOS			Unknown	Hemiarthroplasty

Primary source

Qualification: Other Health Professional observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

NAPPMUNDI, 21.02.2008 DE-NAPPMUNDI-DEU 2008 0004070

Sender diagnosis: not reported

Narrative

Report reference number DEU-2008-0004070 was received on 14FEB2008 from the documentation centre of severe skin reaction in Freiburg, Germany (reference number E08013434 via the German Regulatory Authority, BfArM (reference number DE-BFARM-08013434). This report relates to a 67 year-old male patient.

The patient's current conditions included pain, depression, hypertension and chronic polyarthritis. The patient was hospitalised from 19SEP2004 on for an unknown reason.

Medical history included arrhythmia and hemiarthroplasty. The patient received Oxygesic 20mg (oxycodone hydrochloride, dosage not reported, given from 19SEP2004 until 23SEP2004) for pain. As further suspected drug were reported Antra (omeprazole), Tavor (lorazepam), Praxiten (oxazepam), acetylsalicylic acid, Ultralan (fluocortolone/fluocortolone caproate), Dociton (propranolol), methotrexate, Augmentan (amoxicillin/clavulanate), ascorbic acid, Mono-Embolex (heparin-fraction/sodium salt), Bayotensin (nitrendipine), Lopirin (captopril), ibuprofen, Novalgin (metamizole), and Remergil (mirtazapine).

On 27SEP2004 - 6 days after Oxygesic had been discontinued - the patient experienced Stevens Johnson Syndrome with skin pain, pruritus, labial swelling, redness, fever, erosions, blister/Nikolsky's sign. The patient was treated as inpatient on the internal ward until 21OCT2004. The outcome of the adverse event was not reported.

The reporter found the adverse event to be possibly related to treatment with Oxygesic and also rated 16 additional medicinal products to be well possibly (two products), possibly (five products) or doubtfully (nine products) related to the Steven-Johnson-Syndrome.

This case was rated to be serious because it involved hospitalisation and due to its medical significance.

No additional information is expected.

Sender's comment:

All serious AE in this ICSR are unlabelled against the German SPC.

Causality assessment by Mundipharma Germany is presented in the respective ICSR standard fields.

This ICSR does not add significant information to the safety profile of the respective substance(s) and does not alter the risk/benefit assessment of the Mundipharma medicinal product(s) involved. Therefore, measures concerning the marketing authorisation status are not required.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08020023

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 00070 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20040927	20041021	25 Day		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 augmentin amoxicillin trihydrate; clavulanic acid	Intravenous (not otherwis	00000002 G gram(s)/	-	Infection prophylaxis
2 BAYOTENSIN nitrendipine	Oral	/	-	Hypertension
3 CAPOZIDE captopril; hydrochlorothiazide	Oral	00000001 Df dosage form	20040919-	Hypertension
4 ibuprofen ibuprofen	Oral	00000400 Mg milligram(s)	-	Pain
5 MONO-EMBOLEX heparin	Subcutaneous	/	-	Thrombosis prophylaxis
6 novalgin metamizole		/	-	
7 oxygesic oxycodone hydrochloride	Oral	00000020 Mg milligram(s)	-	Pain
8 REMERGIL mirtazapine	Oral	00000030 Mg milligram(s)	-	Depression

Drugs (concomitant)

9	ANTRA omeprazole	Oral	00000020 Mg milligram(s)	20020102-	-
10	assialgan acetylsalicylic acid; codeine phosphate; phenacetin	Oral	00000100 Mg milligram(s)	20020106-	Prophylaxis
11	ciprobay ciprofloxacin hydrochloride	Oral	00000500 Mg milligram(s)	-	Fever
12	dociton propranolol hydrochloride	Oral	00000040 Mg milligram(s)	20030707-	Arrhythmia
13	fenistil dimetindene maleate	Oral	/	-	Pruritus
14	LACTULOSE lactulose	Oral	00000001 ml millilitre(s)/	-	Constipation
15	METHOTREXATE methotrexate		00000001 Df dosage form	19960101-	Chronic polyarthritis
16	PRAXITEN oxazepam	Oral	00000001 Mg milligram(s)	20030707-	Depression
17	RIOPAN magaldrate	Oral	/	-	Hyperacidity
18	tavor lorazepam	Oral	00000010 Mg milligram(s)	20020104-	Depression
19	ULCOGANT sucralfate	Oral	/	-	Hyperacidity
20	ultralan fluocortolone	Oral	00000005 Mg milligram(s)	20030111-	Chronic polyarthritis
21	vitamin c ascorbic acid	Oral	/	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Chronic polyarthritis			Unknown	
Arrhythmia			Unknown	
Hyperacidity			Unknown	
Fever			Unknown	
Constipation			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 26.02.2008 DE-Bristol-Myers Squibb Company-14080279

Sender diagnosis: not reported

Narrative

A health professional reported via German Health Authority (BFARM E08012434) and BMS Germany (2008021502) that a 70-year-old male was hospitalized following medically significant Steven-Johnson syndrome while taking ibuprofen, captopril + hctz, oxycodone, mirtazapine, metanizole, nitrendipine, heparin sodium and amoxicillin + clavulanic acid. The patient was started on 2.2 grams of amoxicillin + clavulanic acid as part of infection prophylaxis, 20 milligrams (mg) of oxycodone for pain, heparin sodium as part of thrombosis prophylaxis, 400 mg of oral ibuprofen for pain, nitrendipine for hypertension, 30 mg of mirtazapine for depression. The patient did not recover as on March 2005.

The patient medical history included: gastric protection, chronic polyarthrits, arrhythmia, fever, hyperacidity, constipation.

BMS Medical Evaluation Comment:

This 70-year-old male experienced Steven-Johnson's syndrome(SJS) while on multiple drugs including ibuprofen and captopril + hctz. The event of SJS may be associated with captopril +hctz. Additionally, the cosuspect drugs nitrendipine and metamizole as well as the concomitant ciprofloxacin

could have played a contributory role in SJS. However, a causal role of ibuprofen in this reported event could not be excluded.

Sender's comment:

This report is submitted to you in the name of the marketing authorisation holder Bristol-Myers Squibb GmbH & Co. KGaA.

captopril + hctz

Event number; label*; causality**

#1; expected; PSB

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

Comments: No further comments.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08052050

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 10 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell syndrome	20080229			2 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 nurofen brausegranulat ibuprofen	Oral	200 Mg milligram(s)/Day	20080228-20080229	2 Day

Drugs (concomitant)

2 cetirizine cetirizine	/	-
3 dynexan aluminium formate; arnica montana; chamomile; salvia; tetracaine hydrochloride	/	-
4 refobacin creme gentamicin sulfate	/	-
5 tannosynt urea-cresol-sulfonate sodium	/	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Angioedema			Unknown	
Anaphylactic reaction			Unknown	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08057159

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 54 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fever	20070721	20070725	5 Day		recovered/resolved
2 Pain of extremities	20070721	20070725	5 Day		recovered/resolved
3 Skin infection	20070721	20070725	5 Day		recovered/resolved
4 Back pain	20070721	20070725	5 Day		recovered/resolved
5 Contact dermatitis	20070715	20070725	11 Day		recovered/resolved
6 Blisters	20070721	20070725	5 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 betnesol betamethasone sodium phosphate		/	200707- -	Contact eczema
2 CETIRIZINE cetirizine		/	200707- -	Contact eczema
3 DOLORMIN SCHMERZGEL ibuprofen lysinate	Topical	/	200707-20070715 -	Drug use for unknown indication

Primary source

Qualification: Physician observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

McNeil Germany, Local Safety Officers, 19.05.2008 DE-JNJFOC-20080501980

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a physician and concerns a 54 year old female from Germany: 00157/08.

The patient's weight and medical history were not reported. The patient was treated with 2.5 % ketoprofen (gel, topical) dose and indication unknown initiated in JUL-2007. Non-company suspect drugs included betamethasone sodium phosphate cream and systemic cetirizine, both doses unknown initiated in JUL-2007 for contact eczema. Concomitant medications were not reported.

On 15-JUL-2007, the patient went to the hospital (not further specified) with a contact eczema on her knees, that developed after application of ketoprofen gel. She was treated with Betnesol V cream (betamethasone) and systemic cetirizine, which led to progression of symptoms. She also experienced fever, pain in extremities and back pain without signs of common cold.

On 21-JUL-2007, the patient was hospitalized with fever of 40 degrees Celsius. The inner sides of her knees showed an erythema with loose blisters of up to 5 centimeters in diameter, partly with pus. Bacteriologic examination of the blisters showed Streptococci group G and Staphylococcus aureus. Laboratory data was provided as follows: ALT (sgpt) 21-JUL-2007 40+ IU, 23-JUL-2007 50+ IU, C-reactive protein 21-JUL-2007 93.3+ mg/l, 23-JUL-2007 117+ mg/l, 25-JUL-2007 32+ mg/l, gamma glutamyl transferase 21-JUL-2007 68+ IU, 23-JUL-2007 71+ IU, leucocytes (NR: 4.3 - 10.5 g/l) 21-JUL-2007 10.52+ g/l, 23-JUL-2007 8.47 g/l, 25-JUL-2007 6.55 g/l, temperature 21-JUL-2007 40.2 degrees Celsius. Treatment with ketoprofen was discontinued on 15-JUL-2007. The patient was treated with systemic antibiotic erythromycin and acetaminophen was initiated. The patient recovered and was discharged from the hospital on 25-JUL-2007.

This report was serious (hospitalization).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Single case assessment (reported term/preferred term):

CONTACT DERMATITIS/DERMATITIS CONTACT: expected per local SmPc

BLISTERS/BLISTER: expected per local SmPc

INFECTED SKIN BLISTERS/SKIN INFECTION: unexpected per local SmPc

FEVER/PYREXIA: unexpected per local SmPc

PAIN IN EXTREMITIES and BACK PAIN were assessed to be non-serious events.

The benefit-risk-ratio remains unchanged based on the single case assessment of this case. No measures according to § 29 (1) AMG have to be taken.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08086729

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Male	Birth: privacy	Age: 29 Year	weight (kg): 71	BMI_int: 24.28	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blisters	20080716			2 Day	recovered/resolved
2 Dyspnoea	20080715			1 Day	recovered/resolved
3 Redness generalised	20080716			2 Day	recovered/resolved
4 Paraesthesia	20080716			2 Day	recovered/resolved
5 Oedema peripheral	20080716			2 Day	recovered/resolved
6 Extremities hot feeling of	20080716			2 Day	recovered/resolved
7 Head pressure	20080716			2 Day	recovered/resolved
8 Heart throbbing	20080716			2 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 OPTALIDON 200 (NCH)	Oral	/	20080715-20080715	Migraine
ibuprofen			1 Day	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 22.07.2008 DE-NOVNCHHQ-CHNY2008DE02765

Sender diagnosis: not reported

Narrative

Initial consumer report received on 16 Jul 2008. This male consumer used Optalidon 200 (ibuprofen) on 15 Jul 2008 and noticed a quick relief of his migraine. On the same day, up to 15 minutes after oral intake, he experienced heart throbbing, skin redness and vesicles on whole body, dyspnoea, pressure feeling in head as if all his blood was pumped into his head, as well as reddening and local swelling of hands and feet. Optalidon 200 was discontinued on the same day. Following drug treatment with Fenistil Injektionslösung (dimetindene maleate) and an unspecified prednisolone compound, all reactions except the circulatory reactions disappeared within one hour. Final outcome was condition improving.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant hazard) not confirmed by a health professional. The events Dyspnoea, Palpitation, Generalised erythema, Oedema peripheral and Blister were assessed as unlabelled according to the product information. The information provided in this individual case does not warrant a change to the product information text. The event Headache was already labelled in the product information.

Reporter's comment:NOVARTIS, 03.11.2008 DE-NOVNCHHQ-CHNY2008DE02765

Sender diagnosis: not reported

Narrative

07.03.2016

BfArM - Pharmacovigilance

124 / 305

Initial consumer report received on 16 Jul 2008. This male consumer used Optalidon 200 (ibuprofen) on 15 Jul 2008 and noticed a quick relief of his migraine. On the same day, up to 15 minutes after oral intake, he experienced heart throbbing, skin redness and vesicles on whole body, dyspnoea, pressure feeling in head as if all his blood was pumped into his head, as well as reddening and local swelling of hands and feet. Optalidon 200 was discontinued on the same day. Following drug treatment with Fenistil Injektionslösung (dimetindene maleate) and an unspecified prednisolone compound, all reactions except the circulatory reactions disappeared within one hour. Final outcome was condition improving.

Follow-up report received from consumer on 28 Oct 2008. This 29-year-old consumer used Optalidon N (propyphenazone, caffeine citrate) on 16 Jul 2008 and not Optalidon 200 as previously reported. He used it for tension headache and migraine and approximately 10 minutes after intake, he experienced heat in feet and hands, prickle and swelling in feet and hands, then heart throbbing and pressure feeling in the head. Ten minutes later, he developed blisters on belly, thighs and arms. He visited a general practitioner who administered him intravenously Fenistil Injektionslösung and prednisolone (unknown manufacturer). The reactions lasted for approximately 1 to 1.5 hour. Final outcome was complete recovery for all the events. The patient added that causality assessment by attending physician was probable.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant hazard) not confirmed by a health professional. The events Dyspnoea, Palpitation, Generalised erythema, Oedema peripheral, Blister, Feeling hot and Paraesthesia were assessed as unlabelled according to the product information due to greater specificity/severity. The term Hypersensitivity reaction and Anaphylaxis-like symptoms are already included in the product information. The event Headache was assessed as unlabelled according to the product information. However other alternative causes (pre-existing tension headache and migraine) provide a possible explanation for the reported adverse event.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08108612

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
No						Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 87 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blistering	20080822			1 Day	recovering/resolving
2 Redness	20080822			1 Day	recovering/resolving
3 Pruritus aggravated	20080822	20080822		1 Hour	recovering/resolving
4 Urticaria	20080822			1 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibuprofen AL 400 ibuprofen	Oral	400 Mg milligram(s)/Day	20080822-20080822	Tooth pain
			1 Day	

Drugs (concomitant)

2 septanest mit adrenalin 1/100 000 articaine hydrochloride; epinephrine bitartrate	1.7 ml millilitre(s)/Day	20080822-20080822	Tooth extraction
		-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-08124768

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Initials: Sex: Female Birth: privacy Age: weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Therapeutic product ineffective					recovered/resolved
2 Walking difficulty					recovered/resolved
3 Back pain					not recovered/not resolved
4 Joint pain					recovered/resolved
5 Muscle ache					recovered/resolved
6 Cough					recovered/resolved
7 Swelling arm					recovered/resolved
8 Swelling of legs					recovered/resolved
9 Weight gain					recovered/resolved
10 Cramps leg					recovered/resolved
11 Ischialgia					recovered/resolved
12 Hair loss					recovered/resolved
13 Gut pain					not recovered/not resolved
14 Aggravation of existing disorder					recovered/resolved
15 Feeling blue					recovered/resolved
16 Depression					recovered/resolved
17 Memory disturbance					recovered/resolved
18 Scales					recovered/resolved
19 Apathy					recovered/resolved
20 Performance status decreased					recovered/resolved
21 Nasal mucosal disorder					recovered/resolved
22 Headache					recovered/resolved
23 Nausea					recovered/resolved
24 Genital infection fungal					recovered/resolved
25 Discharge vaginal					recovered/resolved
26 Genital pruritus female					recovered/resolved

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Clivioelle 1 mg/0,5 mg Tabletten estradiol valerate; norethisterone	Oral	/	-	
2 ibuprofen 600 ibuprofen		/	-	Pain

Qualification:	Other Health Professional	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08148338

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 48 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20060106	20060130	25 Day		recovered/resolved
2 Convulsion	200512				Unknown
3 Brain neoplasm	20051216				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DOLORMIN (ibuprofen lysine) ibuprofen lysinate	Oral	/ 245	20051201-20051231 31 Day	Back pain
2 DORMICUM (midazolam) midazolam	Intravenous (not otherwis	7.5 Mg milligram(s)/Day 120	20051230-20051230 -	Sedation
3 fortecortin dexamethasone	Oral	/	20051216- -	Prophylaxis
4 fortecortin dexamethasone	Oral	/	20051216- -	Prophylaxis
5 iodine I 125 iodinated (125 i) human serum albumin	Unknown	/	20051230- -	Brain neoplasm
6 LAXOBERAL sodium picosulfate	Oral	12 Gtt drop(s)/Day	20051231-20051231 -	Constipation
7 NEXIUM esomeprazole magnesium	Oral	1 Df dosage form/Day	20051229-20060103 6 Day	Gastric ulcer prophylaxis
8 perfalgan paracetamol	Intravenous (not otherwis	1 G gram(s)/Day 120	20051230-20051230 -	Pain
9 TAVOR (lorazepam) lorazepam	Oral	1 Df dosage form/Day	20051229-20051229 1 Day	Sleep disorder
10 Valeriana valeriana officinalis root	Oral	1 Df dosage form/Day	20051230-20060101 3 Day	Sleep disorder
11 ZENTROPIL phenytoin	Oral	300 Mg milligram(s)/Day	20051216-20060103 19 Day	Convulsion
12 ZENTROPIL phenytoin	Oral	300 Mg milligram(s)/Day	20060104- -	Convulsion
13 ZOCOR simvastatin	Oral	20 Mg milligram(s)/Day 245	20051212- -	Hypercholesterolaemia

Drugs (concomitant)

14	ADUMBRAN oxazepam	Oral	1 Df dosage form/Day	20051217-20051222 6 Day	
15	aspirin acetylsalicylic acid	Oral	100 Mg milligram(s)/Day	20051122-20051216	Thrombosis prophylaxis
16	BIFITERAL lactulose	Oral	15 ml millilitre(s)/Day 244	20051219-20051219	Constipation
17	CONCOR bisoprolol fumarate	Oral	5 Mg milligram(s)/Day	20050101- -	Hypertension arterial
18	DELIX PLUS hydrochlorothiazide; ramipril	Oral	Mg milligram(s)/Day	20000101- -	Hypertension arterial
19	PANTOZOL pantoprazole sodium	Oral	40 Mg milligram(s)/Day	20051216-20051222 7 Day	
20	sortis atorvastatin calcium	Oral	1 Df dosage form/Day 245	20050101-20051211	Hypercholesterolaemia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Back pain			Yes	
Constipation			Yes	
Hypercholesterolaemia			Yes	
Hypertension arterial			Yes	
Pain			Yes	
Sleep disorder			Yes	
Surgery			Yes	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

009507513, 18.12.2008 DE-MERCK-0812DEU00197

Sender diagnosis: not reported

Narrative

Information has been received from a registry, documenting at least 95% of all serious skin reactions that occur in Germany, concerning a 48-year-old female patient with back pain, sleep disorder, pain, arterial hypertension and constipation who, on 12-DEC-2005, was placed on therapy with simvastatin, 20 mg tablet, 20 mg, once a day for the treatment of hypercholesterolaemia (duration not reported).

In December 2005, the patient developed seizure for the first time. On 15-DEC-2005, CT with unspecified radiographic contrast medium, injection, given iv (dose not reported) was performed (results not provided). On 16-DEC-2005, the patient was hospitalized and diagnosed with brain neoplasm (not further specified).

Concomitant suspect secondary therapy included ibuprofen lysine (DOLORMIN), tablet, for the treatment of back pain (5 tablets between 01-DEC-2005 and 31-DEC-2005), radiographic contrast medium (unspecified) iv given (dose not reported, applied between 16-DEC-2005 and 22-DEC-2005 and from 29-DEC-2005 until 30-DEC-2005), phenytoin (ZENTROPIL), po, 100 mg three times a day for the treatment of seizure (from 16-DEC-2005 until 03-JAN-2006, and from 04-JAN-2006 - stop date not reported), dexamethasone sodium phosphate (FORTECORTIN), 4 mg and/or 40 mg (start date 16-DEC-2005) as brain edema prophylaxis, lorazepam (TAVOR), 1 dose unit once on 29-DEC-2005 for the treatment of sleep disorder, esomeprazole magnesium (NEXIUM), 1 dose unit once a day, po, as gastric ulcer prophylaxis (from 29-DEC-2005 until 03-JAN-2006), midazolam (DORMICUM), 7.5 mg iv once on 30-DEC-2005 for sedation, unspecified anesthetic medication, subcutaneously given on 30-DEC-2005 for unspecified surgery, acetaminophen (PERFALGAN), iv, 1 gram given on 30-DEC-2005 for the treatment of pain, valerian (SEDONIUM), po, 1 dose daily for the treatment of sleep disorder (from 30-DEC-2005 until 01-JAN-2006), iodine I 125 SEED implant (placed on 30-DEC-2005) for the treatment of brain neoplasm and sodium picosulfate (LAXOBERAL), drops, po, 12 drops once on 31-DEC-2005 for the treatment of constipation.

Other prior and concomitant therapy included atorvastatin calcium (SORTIS), aspirin, bisoprolol fumarate (CONCOR), hydrochlorothiazide (+) ramipril (DELIX PLUS), pantoprazole sodium (PANTOZOL), oxazepam (ADUMBRAN) and lactulose (BIFITERAL).

On 06-JAN-2006, the patient developed pruritus, spots, erosions of lips and oral mucosa and, on 08-JAN-2006, erosions of the nasal mucosa, on 10-JAN-2006, blisters and, on 20-JAN-2006, erosions of genital mucosa. Final diagnosis was Stevens-Johnson syndrome. Action taken regarding therapy with simvastatin was not reported. On 30-JAN-2006, the patient recovered from Stevens-Johnson syndrome, outcome regarding seizure and brain

neoplasm was not provided.

The reporting physician felt that Stevens-Johnson syndrome was possibly related to therapy with simvastatin and/or ibuprofen lysine and/or radiographic contrast medium (unspecified) and/or dexamethasone sodium phosphate and/or lorazepam and/or esomeprazole magnesium and/or midazolam and/or anesthetic (unspecified) and/or acetaminophen and/or valerian and/or iodine I 125 and/or sodium picosulfate. The reporting physician felt that Stevens-Johnson syndrome was very likely related to therapy with phenytoin (ZENTROPIL). Drug relationship regarding seizure and brain neoplasm was not reported.

Additional information is not expected.

Upon internal review, Stevens-Johnson syndrome was determined to be an Other Important Medical Event.

Sender's comment:

Steven-Johnson syndrome, brain neoplasm and convulsion are serious unlabeled events for therapy with ZOCOR°. The same month she was started on ZOCOR a 48-year-old female patient developed convulsions and was diagnosed with brain neoplasm. The following month she developed a SJS. Given the time relation and pharmacologic action of ZOCOR° a link is rated as not probable for convulsions and brain neoplasm. The data are too poor to assess a link for SJS. The safety profile remains unchanged. Self-initiated measures are not indicated.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09000945

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 29 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epidermal necrolysis	20051202	20051230	29 Day		not recovered/not resolved
2 Stevens Johnson syndrome	20051202	20051230	29 Day		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol 300 mg allopurinol	Oral	/	-	Hyperuricaemia
2 ibuprofen ret. 850 mg ibuprofen	Oral	1 Df dosage form/Day	-	Back pain
3 novaminsulfon tropfen metamizole	Oral	/	-	Podagra

Drugs (concomitant)

4 colchicum-dispert colchicine	Oral	/	-	Gout acute
5 voltaren diclofenac sodium	Topical	/	20051017-	Podagra

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09000961

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 75 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20060727	20060811	16 Day	10 Month	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Oral	/	20070604-20070727 54 Day	Pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Endometrial cancer			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09000962

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 12 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20060918	20060922	5 Day	3 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Oral	/	20060916-20060918 3 Day	Pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Supplementation therapy			Unknown	
Borrelia infection			Unknown	Verdacht auf Borreliose

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09000979

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 9 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum multiforme	20070608	20070622	15 Day	5 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolormin Saft ibuprofen lysinate	Oral	/	20070609-20070609	Fever 1 Day
2 dolormin Saft ibuprofen lysinate	Oral	/	20070604-20070608	Fever 5 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Flu			Unknown	grippaler Infekt

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09000984

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 67 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20070810	20070831	22 Day		fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol 300 mg allopurinol	Oral	150 Mg milligram(s)/Day	20070713-20070731 19 Day	Hyperuricaemia
2 amoxicillin comp amoxicillin trihydrate; clavulanate potassium	Oral	3 Df dosage form/Day	20070803-20070810 8 Day	Lymphocytic alveolitis
3 clexane injlsg 0,4 ml enoxaparin	Subcutaneous	2 Df dosage form/Day	20070801- -	Thrombosis prophylaxis
4 hct 25 mg hydrochlorothiazide	Oral	25 Mg milligram(s)/Day	20070802- -	Lung edema
5 ibutad 800 ibuprofen	Oral	800 Mg milligram(s)/Day	20070713-20070801 20 Day	Pain
6 irenat tropfen sodium perchlorate	Oral	60 Gtt drop(s)/Day	20070803- -	
7 Kontrastmittel InjLsg	Intravenous (not otherwis /		20070806-20070806 -	Computerised tomogram
8 narkose(n.n.bez) pentobarbital		/	20070803-20070803 -	Bronchoscopy
9 solu-decortin H 500 mg prednisolone sodium succinate	Intravenous (not otherwis /		20070806-20070809 4 Day	Lung fibrosis
10 spironolacton 50 mg spironolactone	Oral	50 Mg milligram(s)/Day	20070802- -	Lung edema

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Constipation			Unknown	
Lymphocytic alveolitis			Unknown	
Idiopathic pulmonary fibrosis			Unknown	
Hypothyroidism			Unknown	
Portal vein thrombosis			Unknown	susp. portal vein thrombose
Osteoporosis prophylaxis			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09000994

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 20 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20051024	20051109	17 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ACC akut acetylcysteine	Oral	2 Df dosage form/Day	20051020-20051025	Cough
2 Ibu 600 - 1 A Pharma ibuprofen	Oral	600 Mg milligram(s)/	20051020-20051025	Fever
3 Klacid Filmtabletten clarithromycin	Oral	2 Df dosage form/Day	20051020-20051025	Infection

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Bronchitis acute			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09001035

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 8 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum multiforme	20060401	20060420	20 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Nurofen ibuprofen	Oral	/	20060307-20060404	Fever 29 Day
2 salbutamol salbutamol	Respiratory (inhalation)	2 Df dosage form/Day	20060328-20060404	Cough 8 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Mycoplasma infection			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09001075

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 15 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20060428	20060508	11 Day		recovered/resolved
2 Stevens Johnson syndrome	20060428	20060508	11 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin amoxicillin	Oral	2 Df dosage form/Day	20060422-20060426 5 Day	Infection
2 doxycyclin doxycycline	Oral	/	20060426-20060429 4 Day	Bronchopneumonia
3 ibuprofen ibuprofen	Oral	2 Df dosage form/Day	20060422-20060426 5 Day	Fever
4 paracetamol paracetamol	Oral	2 Df dosage form/Day	20060422-20060426 5 Day	Fever

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09001987

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 16 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme major	20060106	20060121	16 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin amoxicillin	Oral	/	-	Infection
2 ibuprofen ibuprofen	Oral	/	-	Fever

Drugs (concomitant)

3 belara chlormadinone acetate; ethinylestradiol	Oral	/	20050701-	Contraception NOS
4 paracetamol paracetamol	Oral	/	-	Fever

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09002855

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 9 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20051102	20051110	9 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 belladonna belladonna alkaloids	Oral	30 Df dosage form/Day	20051026-20051027 2 Day	Infection
2 dolormin ibuprofen lysinate	Oral	1 Df dosage form/Day	- -	Fever
3 klacid clarithromycin	Oral	15 ml millilitre(s)/Day	20051028-20051101 5 Day	Bronchitis
4 paracetamol paracetamol	Oral	/	- -	Fever
5 spasmo-mucosolvan ambroxol hydrochloride; clenbuterol hydrochloride	Oral	3 Df dosage form/Day	20051027-20051101 6 Day	Cough

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Conjunctivitis			Unknown	
Desensitization injection			Unknown	
Throat secretion increased			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09003225

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes Yes

Patient

Initials: Sex: Female Birth: privacy Age: 48 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20060106	20060130	25 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolormin ibuprofen lysinate	Oral	5 Df dosage form/	-	Back pain
2 dormicum 7,5 midazolam maleate	Intravenous (not otherwis	7.5 Mg milligram(s)/	-	Sedation
3 fortecortin 4 mg dexamethasone	Oral	4 Mg milligram(s)/	-	
4 kontrastmittel/MRT	Intravenous (not otherwis	/	-	
5 laxoberal sodium picosulfate	Oral	12 Gtt drop(s)/	-	Obstipation
6 lod-125-seed-implantat iodine (125 i)		/	-	
7 nexium esomeprazole	Oral	1 Df dosage form/	-	
8 perfalgan paracetamol	Intravenous (not otherwis	1 G gram(s)/	-	Ache
9 sedonium valeriana officinalis root	Oral	1 Df dosage form/	-	Disorder sleep
10 tavor lorazepam	Oral	1 Df dosage form/	-	Disorder sleep
11 zentropil phenytoin	Oral	3*100 Mg milligram(s)/	-	Seizure
12 zocor 20 mg simvastatin	Oral	20 Mg milligram(s)/	-	Hypercholesteremia

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09003250

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes

Patient

Initials: Sex: Female Birth: privacy Age: 20 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20070407	20070419	13 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxiclav-puren 500/125 amoxicillin trihydrate; clavulanate potassium	Oral	3 Df dosage form/	20070405-20070408	Infection
2 dolormin ibuprofen lysinate	Oral	/	4 Day 20070311-	Headache
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Allergic reaction			Unknown	
Pain			Unknown	
Joint pain			Unknown	
Mineral supplementation			Unknown	Multivitamintabl. Yomogi.
Contraception			Unknown	
Anal erosion			Unknown	and nasal
Electrolyte substitution therapy			Unknown	
Flu symptoms			Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

case: DE-BFARM-09003350

Patient

Reactions

Drugs (suspected or interacting)

Primary source

Literature:

case: DE-BFARM-09003356

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Initials: Sex: Male Birth: privacy Age: 27 Month weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20061224	20070118	26 Day	23 Month	not recovered/not resolved
2 Necrolysis epidermal toxic (Lyell type)	20061224	20070118	26 Day	23 Month	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cotrimox-wolff forte saft sulfamethoxazole; trimethoprim	Oral	/	20061212-20061224 13 Day	Otitis media
2 nurofen fuer kinder fiebersaft ibuprofen	Oral	/	20061201-20061224 24 Day	Fever
3 paracetamol saft paracetamol	Oral	/	20061128-20061224 27 Day	Fever

MedDRA-term	Begin	End	Cont.	Comment
Common cold			k.A.	
Rhinitis			k.A.	
Otitis media			k.A.	
Fever			k.A.	
Hematoma			k.A.	
Cough			k.A.	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09004419

Serious Yes	Death	Lifethr. No	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 85 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20060706	20060801	27 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	acc Lsg. 200 mg acetylcysteine	Oral	3* /Day	20060629-	-
2	corvaton retard 8 mg molsidomine	Oral	/	20060602-	Cardiac insufficiency
3	dipidolor Inj.Lsg. 3,75 mg piritramide	Intravenous (not otherwis	2* /Day	-	Pain
4	dormicum 75 mg midazolam maleate	Oral	/Day	-	Sedation
5	fraxiparin Inj.Lsg. nadroparin calcium	Subcutaneous	/	20060602-	Thrombosis prophylaxis
6	hct gamma 25 mg hydrochlorothiazide	Oral	/Day	-	Cardiac insufficiency
7	ibuhexal 600 mg ibuprofen	Oral	3* /Day	-	Pain
8	isoket retard 20 mg	Oral	2* /Day	-	Cardiac insufficiency
9	isoket 5 mg	Oral	/Day	-	Cardiac insufficiency
10	lasix furosemide	Oral	/	20060602-	Cardiac insufficiency
11	lisinopril 10 mg lisinopril dihydrate	Oral	/	-	Cardiac insufficiency
12	metformin 850 mg metformin hydrochloride	Oral	/	-	Diabetes mellitus
13	metohexal 142 mg metoprolol tartrate	Oral	/	-	Cardiac insufficiency
14	nexium 20 mg esomeprazole	Oral	/	-	Stomachache
15	nitrospray glyceryl trinitrate	Oral	/	20060703-20060703	Decompensated heart failure
16	nitrospray glyceryl trinitrate	Oral	/	20060627-20060627	Decompensated heart failure
17	novaminsulfon Trpf. metamizole	Oral	/	20060602-	Pain
18	oxazepam 10 mg oxazepam	Oral	/	-	Sleep disorder
19	ranitic ranitidine hydrochloride	Oral	/	20060627-20060627	Ache stomach
20	rocephin 2 g ceftriaxone sodium	Intravenous (not otherwis	/	20060629-	Pneumonia
21	tramal Trpf. 20 tramadol hydrochloride	Oral	/	-	Pain
22	vanco cell inj.lsg. 1 g vancomycin hydrochloride	Intravenous (not otherwis	/	20060623-	Infection

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypothyroidism			Unknown	
Anemia			Unknown	
Renal insufficiency			Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09040949

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex:	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20051105	20051120	16 Day	11 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 atrovent ipratropium bromide	Respiratory (inhalation)	40 Gtt drop(s)/Day	20051020-20051024	Bronchitis
2 CEFUROXIM cefuroxime sodium	Oral	2 ml millilitre(s)/Day	20051019-20051028	Bronchitis
3 ECOLICIN "CHAUVIN" colistin mesilate sodium; erythromycin lactobionate	Ophthalmic	3* /Day 71	20051030-20051105	
4 ferro sanol ferroglycine sulfate complex	Oral	30 Gtt drop(s)/Day	20050907-20051025	Iron deficiency
5 Ketanest ketamine	Intravenous (not otherwis	4 Mg milligram(s)/ 231	20051026-20051026	
6 nurofen ibuprofen	Oral	100 Mg milligram(s)/	20051027-	Fever
7 nurofen ibuprofen	Oral	100 Mg milligram(s)/	20050701-20051019	
8 pulmicort budesonide	Respiratory (inhalation)	/	20050727-20051024	Pulmonary function impairment
9 sultanol inhalationsloesung salbutamol sulfate	Respiratory (inhalation)	32 Gtt drop(s)/Day	20051020-20051024	Bronchitis
10 Vitamine nmb vitamins nos	Oral	1 Df dosage form/Day	20051019-	Nutritional support

Drugs (concomitant)

11 ass acetylsalicylic acid		450 Mg milligram(s)/Day	20051105-	Fever
12 fluorid sodium fluoride	Oral	1 Df dosage form/Day 245	20050807-20051019	
13 nurofen ibuprofen	Rectal	100 Mg milligram(s)/Day	20051019-20051020	Fever
14 otriven xylometazoline hydrochloride	Nasal	/Day	20051021-20051021	
15 otriven xylometazoline hydrochloride	Nasal	/Day	20051019-20051019	Common cold
16 SINUPRET ascorbic acid; gentiana; primula flowers; rumicis asetosa; sambucus flowers; verberna herb	Oral	/	20051010-20051019	Bronchitis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Bronchitis			Unknown	
Fever convulsions	20051019		Unknown	
Common cold			Unknown	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 31.03.2009 DE-PFIZER INC-2008151549

Sender diagnosis: not reported

Narrative

This is a report from the German Documentation Centre of Severe Skin Reactions, a sponsored project set up to collect suspected drug induced severe skin reactions leading to hospital admission.

This 16-month old male patient with medical history of recurrent bronchitis and common cold was admitted to hospital on 19Oct2005 due to fever convulsions. On 29Oct2005 the patient developed maculopapulous exanthema which persisted until 02Nov2005. On 05Nov2005 diagnosis of Stevens-Johnson-Syndrome was made with symptoms of lacquer lips pruritus, reddening of eyes, spots and efflorescences and swollen tongue. In addition, on 07Nov2005, erosions and blistering of lips, oral mucous and genital mucous membranes occurred. The reporting physician considered a possible causal relationship between the Stevens-Johnson-Syndrome and esketamine (KETANEST) 4 mg, intravenous given only once on 26Oct2005 for sedation. In addition a possible causal relationship was seen with chloralhydrat suppository, given from 18Oct2005 until 19Oct2005, on 24Oct2005, from 27Oct2005 until 01Nov2005 and from 03Nov2005 onwards with changing doses (0-0-1, 1-0-0, 1-0-1) as prophylaxis of convulsion, with cefuroxim (2 x 1 ml daily) administered from 19Oct2005 until 28Oct2005 and clarithromycin (2 x 4 ml daily) given from 29Oct2005 until 05Nov2005 both due to bronchitis, with midazolam 2.2 mg intravenous given once on 26Oct2005 for sedation and with metamizole (NOVAMINSULFON) oral, 6-0-0 gtt on 30Oct2005, 7-0-0 gtt on 04Nov2005 and 7-7-7 gtt on 07Nov2005 due to fever. A questionable causal relationship was seen with budesonide (PULMICORT) (1-0-1, 1-1-1, respiratory) administered from 27Jul2005 until 24Oct2005 for "pulmonary stabilization", with aminoacetic acid/ferrous sulfate (FERROSANOL) 3 x 10 gtt daily, oral given from 07Sep2005 until 25Oct2005 as substitution, with cromoglycate sodium (DNCG) (1-0-1-respiratory) given from 15Aug2005 until 19Oct2005 and from 24Oct2005 until 05Nov2005 due to recurrent bronchitis, with paracetamol 250 mg suppository (1-0-0, 1-0-1) given from 01Jul2005 until 21Oct2005, from 27Oct2005 until 01Nov2005 and from 03Nov2005 onwards due to fever, with ibuprofen (NUROFEN) 100 mg oral (0-0-1, 1-0-0,1-0-1) administered from 01Jul2005 until 19Oct2005 and from 27Oct2005 onwards due to fever, with vitamins (MULTISANOSTOL) oral 1 x daily given from 19Oct2005 onwards as substitution, with ipratropium bromide (ATROVENT) (4 x 10 gtt daily-respiratory) given from 20Oct2005 until 24Oct2005, with salbutamol (SULTANOL) (4 x 8 gtt daily respiratory) administered from 20Oct2005 until 24Oct2005 due to bronchitis, with ambroxol oral (2 x 2.5 mg daily) given from 21Oct2005 until 24Oct2005 due for secretolysis, with colistin/erythromycin (ECOLICIN) eye drops (3 x 1 daily) from 30Oct2005 to 05Nov2005 for an unknown indication and with cloralhydrat 600 mg oral (1-0-0, 1-0-1) administered from 10Oct2005 onwards as prophylaxis for convulsion. Concomitant medications included sodium fluoride (FLUORID) 1 tablet daily oral from 07Aug2005 to 19Oct2005 for substitution, mixture of ascorbic acid, gentian, primula flowers, rumicis asetosa, sambucus flowers, verbena herb (SINUPRET) oral from 10Oct2005 to 19Oct2005 for recurrent bronchitis, ibuprofen (NUROFEN) 100 mg daily rectal from 19Oct2005 to 20Oct2005 for fever, xylometazoline hydrochloride (OTRIVEN) 1 daily nasal on 19Oct2005 for common cold and acetylsalicylic acid (ASS) 3 x 150 mg daily from 05Nov2005 for fever.

On 05Nov2005 the patient was started on dimetindene (FENISTIL) oral 3 x tsp daily for pruritus, on trimethoprim/sulfamethoxazole (COTRIM) 5 ml oral (0-0-1, 1-0-1) for infection prophylaxis and on immunoglobuline human (INTRATECT) intravenous (only administered once on 06Nov2005). Alternating common cold and acute bronchitis were considered as probable cause of the Steven Johnson Syndrome. The patient recovered completely from the adverse event on 20Nov2005.

Follow-up status: case closed (03FEB2009).

Sender's comment:

The causal role of each individual suspect can not be established.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09042916

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 69 Year	weight (kg): 100	BMI_int: 33.03	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pemphigus					recovering/resolving
2 Neurodermatitis					recovering/resolving
3 Pruritus					recovering/resolving
4 Rash psoriasiform					recovering/resolving
5 Blistering					recovering/resolving
6 Macular rash					recovering/resolving
7 Rash	200812				recovering/resolving
8 Dizziness					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DIOVAN valsartan	Oral	/	2007-200811	Hypertension NOS
2 DIOVAN valsartan	Oral	80 Mg milligram(s)/Day	200812-200903 3 Month	
3 ibuprofen ibuprofen	Oral	/	-200901	Arthrosis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Arthrosis			Unknown	
Crust			Unknown	type: Atopic

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 03.04.2009 DE-002147023-PHHY2009DE11049

Sender diagnosis: not reported

Narrative

Initial report received from a retired physician who was also the patient's wife on 24 Mar 2009: This patient started treatment with Diovan (valsartan) 80 mg once daily, for hypertension from an unspecified date in 2007. The patient had discontinued Diovan (valsartan) after a treatment duration of 1.5 years because of dizziness. About 4 weeks later Diovan was restarted. After Diovan had been restarted a rash occurred (event onset approximately at the beginning of Dec 2008) which at first looked as an exanthema due to neurodermatitis. At the beginning of 2009 the patient then developed a rash at the back which looked like mosquito bites (accompanied by pruritus). Then a large macula occurred on the leg. The patient was examined by a dermatologist and treatment with cortisone was started. But the rash spread all over the body and additionally blistering of hands and feet occurred (looking similar to pemphigus). The symptoms were treated with cortisone (oral and topical) and cetirizine. Meanwhile the rash looked psoriasiform. Diovan was discontinued 14 days ago. Since then the events improved considerably.

Sender's comment:

Serious spontaneous report

Neurodermatitis and pemphigus [medically significant], assessed as not listed in the Core Data Sheet. The information provided in this individual case does not warrant a change in the Core Data Sheet text. The topic will be monitored closely..

Rash [medically significant] already listed in the Core Data Sheet.

All spontaneous reports are considered suspected based on reporting purposes or in accordance with national regulatory requirements

All remaining reported leading events and associated symptoms were assessed as non serious by the reporting healthcare professional/Novartis Medical Safety Physician.

Reporter's comment:

NOVARTIS, 02.07.2009 DE-002147023-PHHY2009DE11049

Sender diagnosis: not reported

Narrative

Initial report received from a retired physician who was also the patient's wife on 24 Mar 2009: This patient started treatment with Diovan (valsartan) 80 mg once daily, for hypertension from an unspecified date in 2007. The patient had discontinued Diovan (valsartan) after a treatment duration of 1.5 years because of dizziness. About 4 weeks later Diovan was restarted. After Diovan had been restarted a rash occurred (event onset approximately at the beginning of Dec 2008) which at first looked as an exanthema due to neurodermatitis. At the beginning of 2009 the patient then developed a rash at the back which looked like mosquito bites (accompanied by pruritus). Then a large macula occurred on the leg. The patient was examined by a dermatologist and treatment with cortisone was started. But the rash spread all over the body and additionally blistering of hands and feet occurred (looking similar to pemphigus). The symptoms were treated with cortisone (oral and topical) and cetirizine. Meanwhile the rash looked psoriasiform. Diovan was discontinued 14 days ago. Since then the events improved considerably.

Follow up report received from a physician in medical practice on 22 Jun 2009: The patient initiated Diovan (valsartan) 80 mg approximately at the beginning of 2006. Additionally he had sporadically taken ibuprofen (manufacturer not reported) for years. Diovan was discontinued at the beginning of Dec 2008. The event, which had started in Jan 2009, was now described as follows: "Slowly progredient, intensely itching exanthema in the end affecting trunk and extremities with alterations of finger nails: Erythema exsudativum multiforme-like exanthema". Lupus erythematosus and pemphigus were excluded histologically. Ibuprofen was discontinued in Jan 2009. No further information was provided regarding the restarting of Diovan, which had been mentioned in the FU report, it was only mentioned that the event improved after discontinuation of Diovan. The information regarding the final outcome was ambiguous as the reporter noted 'not yet recovered' as outcome but also noted that the event had lasted until Jun 2009. The reporter assessed the causal relationship with regard to Diovan as possible but considered the available information insufficient to fully assess the case. Ibuprofen was assessed as co suspect.

Sender's comment:

Serious spontaneous report

Neurodermatitis and pemphigus [medically significant], assessed as not listed in the Core Data Sheet. The information provided in this individual case does not warrant a change in the Core Data Sheet text. The topic will be monitored closely..

Rash [medically significant] already listed in the Core Data Sheet.

All spontaneous reports are considered suspected based on reporting purposes or in accordance with national regulatory requirements

All remaining reported leading events and associated symptoms were assessed as non serious by the reporting healthcare professional/Novartis Medical Safety Physician.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09089591

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 No

Patient

Initials: Sex: Male Birth: privacy Age: 18 Month weight (kg): 12.6 BMI_int: 20.71 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug rash	20090706			2 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 nurofen soft 4% ibuprofen	Oral	45 ml millilitre(s)/Day 244	20090705-20090706 2 Day	Fever

Drugs (concomitant)

2 PCM 125 paracetamol	Rectal	3 Df dosage form/Day 242	- -	Fever
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Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-09108231

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 24 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome					Unknown
2 Leukocytosis	200909				Unknown
3 C-reactive protein increased	200909				Unknown
4 Fuchs' syndrome	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylsalicylic acid acetylsalicylic acid	Oral	/	-	Common cold
2 Amoxicillin amoxicillin	Oral	3000 Mg milligram(s)/Day	-	Pharyngitis
3 diclofenac diclofenac	Oral	150 Mg milligram(s)/Day	-	Pharyngitis
4 ibuprofen ibuprofen	Unknown	/	-	Fever
5 ibuprofen ibuprofen	Oral	/	-	Common cold
6 Paracetamol paracetamol	Oral	/	-	Common cold

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Apathy			Unknown	
Fever sore			Unknown	
Odynophagia			Unknown	
Rhinitis			Unknown	
Common cold			Yes	With odynophagy, rhinitis, general apathy and fever

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Jakob M, Stuhmann N, Jordan J, Bootz F, Schröck A. [Stomatitis, cheilitis and conjunctivitis after a common cold.] HNO 2009; doi 10.1007/s00106-009-1931-2.	
Qualification:	Other Health Professional	observestudytype:
Literature:	Jakob M., Stuhmann N., Jordan J., Bootz F., Schröck A. Stomatitis, cheilitis and conjunctivitis after a common cold. HNO 2009 , 57:914-917	

Narratives (nur bei elektronischem Eingang)

JUTA, 18.08.2009 DE-JUTA-1515-09

Sender diagnosis: not reported

Narrative

A 24-year-old female patient developed Stevens Johnson syndrome (Fuchs' syndrome) during treatment with amoxicillin and diclofenac and after treatment with paracetamol, ibuprofen and acetylsalicylic acid. The patient complained of odynophagy, rhinitis, general apathy and fever which had persisted for 5 days. After initial treatment with paracetamol, ibuprofen and acetylsalicylic acid without improvement amoxicillin and diclofenac were prescribed. In addition to leukocytosis and increased C-reactive protein, there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The lips were red with hemorrhagic scabs.

Sender's comment:

Allergic skin reactions are expected adverse drug reactions for amoxicillin. The comedication and the basic disease are alternative explanations. No further measures are considered to be necessary.

Reporter's comment:

Riemser Arzneimittel AG, 24.11.2009 DE-RIEMSER-20090338

Sender diagnosis: not reported

Narrative

This spontaneous literature case concerned a previous healthy female who experienced Fuchs syndrome while receiving paracetamol, ibuprofen and acetylsalicylic acid for a common cold and amoxicillin and diclofenac for acute pharyngitis. The medical history of the patient was unremarkable especially for allergic skin reactions.

On an unspecified date, the patient suffered from a common cold with odynophagia, rhinitis, fatigue and pyrexia and started treatment with paracetamol, ibuprofen and acetylsalicylic acid without medical advice. Three days later, there was no improvement of her symptoms and she consulted an otorhinolaryngologist. The diagnosis of an acute pharyngitis was made and treatment with amoxicillin (1000 mg tid) and diclofenac (50 mg tid) was initiated. During the following two days, the patient suffered from a painless redness of her eyes and from a progression of odynophagia. Thus, she was referred to the Department of Otorhinolaryngology of the University Hospital Bonn, Germany. On admission, the patient presented in a severely reduced general condition with pyrexia. Nutrition and fluid intake was not possible due to excessive odynophagia. There were pronounced erosive, reddened, edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes. The lips were red with hemorrhagic scabs. In addition, a conjunctivitis was noticed. Laboratory parameters were remarkable for leukocytosis and elevated CRP. The diagnosis of Fuchs syndrome, a special form of Stevens-Johnson syndrome, was made and the patient was treated with systemic corticosteroids (100 mg daily). Furthermore, antiseptic and anaesthetic mouth rinsings as well as dexpanthenol-containing ointments were prescribed. It is obvious, that all drugs were discontinued. During her hospital stay, the patient recovered completely and was discharged three weeks after admission.

Follow up information was received and the following data were added to the case: clinical course and outcome of the patient; causality assessment between the reported event and the used drugs; medical history of the patient.

Sender's comment:

The RIEMSER Arzneimittel AG holds the marketing authorization for paracetamol-containing preparations (e.g. Neopyrin forte) and for amoxicillin-containing preparations (e.g. AmoxiHefa 1000 mg). Because there was no information about the tradenames of the used paracetamol and amoxicillin in this case, it cannot be excluded, that RIEMSER products were administered. Fuchs syndrome is not listed for Neopyrin forte and AmoxiHefa. However, hypersensitivity reactions are listed for Neopyrin forte and for AmoxiHefa 1000 mg, severe allergic skin reactions are listed. The causal relationship between the reported event and paracetamol and amoxicillin was assessed as possible by the company. Alternative explanation is the concomitant medication of ibuprofen, acetylsalicylic acid and diclofenac. Furthermore, the underlying common cold might be a contributing factor for the Fuchs syndrome. The seriousness of this case is assessed as serious because a hospitalization was required. Additionally, the case was classified as medically significant by the company. Based upon this single case, the overall safety profile of the RIEMSER paracetamol- and amoxicillin-containing preparations has not been changed.

Reporter's comment:

The reporting physician assessed, that each of the used drugs might be the causative agent for the occurred Fuchs syndrome in this case.

AXCOUNT, 22.12.2009 DE-AXCOUNT-00019

Sender diagnosis: not reported

Narrative

A 24-year-old patient complained of odynophagy, rhinitis, general apathy and fever (38,5 Â°C) which had persisted for 5 days. Treatment had initially

been a combination of paracetamol, ibuprofen and acetylsalicylic acid and when no improvement occurred, this medication was stopped after 3 days and amoxicillin and diclofenac were prescribed for sore throat. Following, odynophagy was exacerbated and nutrition and fluid intake was not possible. In addition to leukocytosis and increased C-reactive protein, there were oedematous alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The diagnosis was Fuchs' syndrome. The reaction outcome was not mentioned.

Sender's comment:

This case was classified serious because to its medical importance (since the nutrition and fluid intake were not possible due to excessive odynophagy in this case). Regarding the information on this case, it can not be excluded that Amoxicillin account 1000 mg, Tabletten was involved in the adverse reaction because this case occurred in Germany. Fuchs' syndrome is not covered by the current SPC and will be closely monitored by the MAH in the future.

Reporter's comment:

The reporters stated that Fuchs' syndrome was medication induced. The exact role of amoxicillin in the development of Fuchs' syndrome is not nearly posed.

Bausch & Lomb Incorporated, 05.01.2010 DE-BAUSCH-2009BL006723

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a 24-year-old female patient complained of odynophagy, rhinitis, general apathy and fever (38.5 degrees C) which had persisted for 5 days. Treatment had initially been a combination of PARACETAMOL, IBUPROFEN and ACETYLSALICYLIC ACID (self-medication, manufacturers and doses unknown) and when no improvement occurred on the third day the medication was changed to 1000 mg AMOXICILLIN three times per day and 50 mg DICLOFENAC three times per day by an ENT doctor who made the diagnosis of angina of the salpingopalatine fold. Nutrition and fluid intake was not possible due to excessive odynophagy. In addition to leukocytosis (LEUCOCYTOSIS) (13.12 g/l) and increased C-reactive protein (C-REACTIVE PROTEIN INCREASED) (193 mg/l), there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The lips were red with hemorrhagic scabs. The diagnosis was Fuchs' syndrome (FUCHS' SYNDROME) which is a severe medication-induced skin reaction. Abstinence from the agent (medication) responsible and early cortisone therapy are decisive. The patient received cortisone therapy of 100 mg per day. The erosive alterations of oral mucosa were treated with antiseptic, anaesthetizing and caring mouth irrigations, the lip alterations with dexpanthenol containing ointments. The outcome was not reported. No further information is available.

Source:

JAKOB M, STUHRMANN N, JORDAN J, BOOTZ F, SCHROECK A. STOMATITIS, CHEILITIS AND CONJUNCTIVITIS AFTER A COMMON COLD. HNO 2009 SEP; 57(9): 914-7.

Sender's comment:

A literature report was received indicating a serious unexpected case of FUCHS' SYNDROME in connection with the use of several drugs including paracetamol (manufacturer unknown). As Bausch and Lomb also have a marketing authorisation for paracetamol (Vivimed N gegen Fieber und Kopfschmerzen) this case is reported.

This case was assessed as serious (medical significant), unexpected and possibly related as a causal relationship between the application of paracetamol and the occurrence of this adverse drug reaction cannot be excluded. FUCHS' SYNDROME is a sub-type of the Stevens Johnsons Syndrome with a very rare frequency of about 1 out of one million patients. The Fuchs syndrome like Stevens Johnsons Syndrome may be caused by different drugs including antibiotics and NSAIDs. In this case report the patient had used several drugs that are suspected to cause this type of adverse reaction like penicilline, and several NSAIDs (ibuprofen, ASS, diclofenac, paracetamol). Therefore the contribution of paracetamol to this adverse reaction remains unclear but cannot be ruled out. In the SmPC of our paracetamol-containing product a potential of generalised hypersensitivity reactions after the use of paracetamol is already described. However this special drug reaction is not mentioned, therefore this case is assessed unexpected.

As this report is an isolated case no further measures for risk mitigation are considered necessary.

Reporter's comment:

Dr. August Wolff Arzneimittel, 20.01.2010 DE-DRWOLFF-2010-050

Sender diagnosis: not reported

Narrative

A case of Fuchs syndrome with stomatitis, cheilitis and conjunctivitis was reported in a 24-year-old female patient. The patient had a past medical history of common cold with odynophagy, rhinitis, general apathy and fever. Five days before presentation, she had taken paracetamol, ibuprofen and acetylsalicylic acid for treatment. As her symptoms did not improve, amoxicillin (3x1000mg/d) and diclofenac (3x50 mg/d) were prescribed by her treating otorhinolaryngologist based on a diagnosis of acute pharyngitis two days before presentation. The patient presented to the hospital with stomatitis, cheilitis and conjunctivitis. Physical examination revealed pronounced erosive, reddened oedematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes. Relevant laboratory test results included leucocytes 13.12 G/L and CRP 193 mg/L. The patient was diagnosed with Fuchs syndrome. No further case details were reported.

Sender's comment:

This adverse reaction is unexpected for Amoxi-Wolff products. Fuchs syndrome is a subtype of Stevens-Johnson-Syndrome. However, as this term (SJS) is not sufficiently reflected in the current SPCs of Amoxi-Wolff products, the company will revise the wording.

Reporter's comment:

Dr. August Wolff Arzneimittel, 20.01.2010 DE-DRWOLFF-2010-050

Sender diagnosis: not reported

Narrative

A case of Fuchs syndrome with stomatitis, cheilitis and conjunctivitis was reported in a 24-year-old female patient. The patient had a past medical history of common cold with odynophagy, rhinitis, general apathy and fever. Five days before presentation, she had taken paracetamol, ibuprofen and acetylsalicylic acid for treatment. As her symptoms did not improve, amoxicillin (3x1000mg/d) and diclofenac (3x50 mg/d) were prescribed by her treating otorhinolaryngologist based on a diagnosis of acute pharyngitis two days before presentation. The patient presented to the hospital with stomatitis, cheilitis and conjunctivitis. Physical examination revealed pronounced erosive, reddened oedematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes. Relevant laboratory test results included leucocytes 13.12 G/L and CRP 193 mg/L. The patient was diagnosed with Fuchs syndrome. No further case details were reported.

Sender's comment:

This adverse reaction is unexpected for Amoxi-Wolff products. Fuchs syndrome is a subtype of Stevens-Johnson-Syndrome. However, as this term (SJS) is not sufficiently reflected in the current SPCs of Amoxi-Wolff products, the company will revise the wording.

Reporter's comment:

Dr. August Wolff Arzneimittel, 21.01.2010 DE-DRWOLFF-2010-050

Sender diagnosis: not reported

Narrative

A case of Fuchs syndrome with stomatitis, cheilitis and conjunctivitis was reported in a 24-year-old female patient. The patient had a past medical history of common cold with odynophagy, rhinitis, general apathy and fever. Five days before presentation, she had taken paracetamol, ibuprofen and acetylsalicylic acid for treatment. As her symptoms did not improve, amoxicillin (3x1000mg/d) and diclofenac (3x50 mg/d) were prescribed by her treating otorhinolaryngologist based on a diagnosis of acute pharyngitis two days before presentation. The patient presented to the hospital with stomatitis, cheilitis and conjunctivitis. Physical examination revealed pronounced erosive, reddened oedematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes. Relevant laboratory test results included leucocytes 13.12 G/L and CRP 193 mg/L. The

patient was diagnosed with Fuchs syndrome. No further case details were reported.

Sender's comment:

This adverse reaction is unexpected for Amoxi-Wolff products. Fuchs syndrome is a subtype of Stevens-Johnson-Syndrome. However, as this term (SJS) is not sufficiently reflected in the current SPCs of Amoxi-Wolff products, the company will revise the wording.

Reporter's comment:

mibe GmbH Arzneimittel, 08.03.2010 DE-MIBE-N02BA01-01032010-A

Sender diagnosis: not reported

Narrative

A 24-year-old female patient complained of odynophagy, rhinitis, general apathy and fever (38.5 degrees C) which had persisted for 5 days. Treatment had initially been a combination of paracetamol, ibuprofen and acetylsalicylic acid and when no improvement occurred amoxicillin was prescribed. Nutrition and fluid intake was not possible due to excessive odynophagy. In addition to leukocytosis and increased C-reactive protein, there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The lips were red with hemorrhagic scabs. The diagnosis was Fuchs' syndrome which is a severe medication-induced skin

Sender's comment:

mibe comment:

Fuchs syndrome describes a variant of erythema multiforme majus which mainly involves the mucosal surfaces. Severe skin reactions such as Stevens-Johnson syndrome are known side effects on acetylsalicylic acid and ibuprofen and listed in the current SPC of Temagin ASS, Jenaprofen, Ibu-MTT and Fibralflex. Only allergic skin reactions with rash are listed for paracetamol. This case is assessed as serious and the causal relationship as possible for several suspected drugs. No safety concern is elicited from this case.

Reporter's comment:

MIP International Pharma Research GmbH, 31.05.2011 DE-MIP-hamxpo090111-hdcopo090111

Sender diagnosis: not reported

Narrative

A 24-year-old female patient complained of odynophagy, rhinitis, general apathy and fever (38.5 °C) which had persisted for 5 days. Treatment had initially been a combination of paracetamol, ibuprofen and acetylsalicylic acid and when no improvement occurred amoxicillin and diclofenac were prescribed. Nutrition and fluid intake was not possible due to excessive odynophagy. In addition to leukocytosis and increased C-reactive protein, there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The lips were red with hemorrhagic scabs. The diagnosis was Fuchs' syndrome which is a severe medication-induced skin reaction. Abstinence from the agent (medication) responsible and early cortisone therapy are decisive.

Sender's comment:

The case was detected in the MEDLINE database on 18th May 2011. In the case, no product name or MAH could be identified. The case could theoretically be in connection with amoxicillin- or diclofenac-containing products of the MAH. Therefore, the case was recorded for both active substances in the database of the MAH as case hamxpo090111 and case hdcopo090111. The diagnosed Fuchs' syndrome is a special form of

Stevens-Johnson-Syndrom concerning the eye and oral mucosa. According to the treating physician, the Fuchs' Syndrom belongs to serious drug-induced skin reactions. Skin and subcutaneous disorders like exanthema, rash and allergic reactions are known ADRs of both drugs. The cases were classified as labelled for diclofenac and as unlabelled for amoxicillin. The case was classified as serious due to the medically important condition. Based on the available data, the MAH evaluated the causality as possible for both drugs. The reported case does not change the risk/benefit assessment for Amoxi-saar 500/750/1000 mg Tabletten as well as Diclo-saar 50/100 mg Tabletten. Stevens-Johnson Syndrome is also labelled for the co-medication ASS and Ibuprofen.

Reporter's comment:

McNeil Germany, Local Safety Officers, 17.04.2013 DE-JNJFOC-20130407230

Sender diagnosis: not reported

Narrative

Report was received from literature.

Jakob M, Stuhmann N, Jordan J, Bootz F, Schrock A. Stomatitis, cheilitis and conjunctivitis after a common cold. Hno 2009; 57(9):914-917

This report concerns a 24-year-old female patient. The patient's height, weight and medical history were not reported. The patient had complained of odynophagy, rhinitis, general apathy and fever (38.5degreeC) which had persisted for 5 days. The patient was treated with the combination of Ibuprofen (unspecified formulation and route, batch unspecified) dose and frequency unspecified and non-company suspect drug(s) paracetamol (unspecified formulation and route, batch unspecified) dose and frequency unspecified and acetylsalicylic acid (unspecified formulation and route, batch unspecified) dose and frequency unspecified and when no improvement occurred amoxicillin (unspecified formulation and route, batch unspecified) dose and frequency unspecified was prescribed for odynophagy, rhinitis, general apathy and fever. The patient's nutrition and fluid intake was not possible due to excessive odynophagy. In addition to leukocytosis and increased C-reactive protein, there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The lips were red with hemorrhagic scabs. The diagnosis was Fuchs syndrome which was a severe medication induced skin reaction. Abstinence from the agent (medication) responsible and early cortisone therapy were decisive. Action taken with ibuprofen, paracetamol, acetylsalicylic acid and amoxicillin were not reported. The patient's outcome was not reported for Fuchs syndrome at the time of this report.

This report is serious (medically significant).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Reporter's comment:

PFIZERINC, 23.04.2013 DE-PFIZER INC-2013120405

Sender diagnosis: not reported

Narrative

This is a literature report from the Hno 2009; 57(9):914-917, entitled "Stomatitis, cheilitis and conjunctivitis after a common cold", based on information received by Pfizer from Johnson and Johnson (manufacturer control number DE-JNJFOC-20130407230), license party for ibuprofen.

Report was received from literature. Jakob M, Stuhmann N, Jordan J, Bootz F, Schrock A. Stomatitis, cheilitis and conjunctivitis after a common cold. Hno 2009; 57(9):914-917. This report concerns a 24-year-old female patient. The patient's height, weight and medical history were not reported. The patient had complained of odynophagy, rhinitis, general apathy and fever (38.5degreeC) which had persisted for 5 days. The patient was treated with the combination of Ibuprofen (unspecified formulation and route, batch unspecified) dose and frequency unspecified and non-company suspect drug (s) paracetamol (unspecified formulation and route, batch unspecified) dose and frequency unspecified and acetylsalicylic acid (unspecified formulation and route, batch unspecified) dose and frequency unspecified and when no improvement occurred amoxicillin (unspecified formulation and route, batch unspecified) dose and frequency unspecified was prescribed for odynophagy, rhinitis, general apathy and fever. The patient's nutrition and fluid intake was not possible due to excessive odynophagy. In addition to leukocytosis and increased C-reactive protein, there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis.

The lips were red with hemorrhagic scabs. The diagnosis was Fuchs syndrome which was a severe medication induced skin reaction. Abstinence from the agent (medication) responsible and early cortisone therapy were decisive. Action taken with ibuprofen, paracetamol, acetylsalicylic acid and amoxicillin were not reported. The patient's outcome was not reported for Fuchs syndrome at the time of this report. This report is serious (medically significant).

Causality Assessment per reporter for the event is "Possible".

Johnson and Johnson Causality assessment: the event of Fuchs syndrome was considered possibly related to ibuprofen.

Pfizer is Marketing Authorization Holder of ibuprofen in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of ibuprofen has submitted the same report to the regulatory authorities.

Sender's comment:

Based on the information provided, a possible contributory role of the suspect drug to the reported event Fuch's syndrome cannot be excluded. The impact of this report on the benefit/risk profile of the Pfizer product is evaluated as part of Pfizer procedures for safety evaluation, including the review and analysis of aggregate data for adverse events. Any safety concern identified as part of this review, as well as any appropriate action in response, will be promptly notified to Regulatory Authorities, Ethics Committees and Investigators, as appropriate.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09140312

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 5 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blue lips	20091003			1 Week	recovering/resolving
2 Cerebral haemorrhage	20091003			1 Week	recovering/resolving
3 Blood in urine	20091003			1 Week	recovering/resolving
4 Blood in stool	20091003			1 Week	recovering/resolving
5 Idiopathic thrombocytopenic purpura	20091005			1 Week	recovering/resolving
6 Haematoma	20091003			1 Week	recovering/resolving
7 Petechiae	20091003			1 Week	recovering/resolving
8 Haematemesis	20091005			6 Day	recovering/resolving
9 Haemorrhage urinary bladder	20091005			6 Day	recovering/resolving
10 Chronic headaches	20091005			6 Day	recovering/resolving
11 Vesicles	20091003			1 Week	recovering/resolving
12 Thrombocytopenia	20091005			6 Day	recovering/resolving
13 Splenomegaly	20091005			6 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Infectocef (cephaclor) cefaclor		/	20090929- -	Laryngopharyngitis
2 Infectocef (cephaclor) cefaclor		/	20090929- -	CRP increased
3 Nurofen Junior Fiebersaft ibuprofen	Oral	200 Mg milligram(s)/6 Hou 170	20090927- -	Fever

Drugs (concomitant)

4 NeoTussan (dextromethorphan) dextromethorphan	/	- -	
5 Rectodelt prednisone	/	- -	Pseudocroup

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

RBHC, 22.10.2009 DE-RB-022307-09

Sender diagnosis: not reported

Narrative

Report no 1 is a serious case received from RB Germany on 08-Oct-2009, a consumer case ref: RBD-23726790.

The mother of a 5-year-old boy reported the ADR directly to Reckitt Benckiser Germany. Further information has been obtained by a telephone call with the mother on 08-OCT-2009. This report is based entirely on the mother's statements and is not confirmed by any HCP.

REPORT TERMS AND DESCRIBE REACTIONS

1. Blue lips
2. Bloody vesicles on the lips

3. Petechiae
4. Thrombocytopenia
5. Immune thrombocytopenia
6. Severe headache
7. Blood in urine
8. Blood in the stool
9. Suspicion of cerebral haemorrhage
10. Splenomegaly
11. Blood coagulum in urinary bladder
12. Haematemesis.

NARRATIVE

Please note, in this case the suspected drug is "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml) which is referred to as "Nurofen" in this narrative.

On Sunday 27-SEP-2009 a 5-year-old boy developed high fever up to 40.7 °C. A paediatrician on duty (contacted by the boy's mother) diagnosed by phone the boy with pseudo-croup based on the described symptoms and therefore prescribed Nurofen and Rectodelt suppositories (prednisone).

Nurofen was administered in a dose of 5 mL every 6 hours including night time. A reduction of the fever was not achieved. A paediatrician contacted on Monday 28-SEP-2009 confirmed the diagnosis of pseudo-croup and instructed to continue the initial therapy. The fever remained high in spite of the therapy.

On Tuesday 29-SEP-2009 laryngopharyngitis was diagnosed by the paediatrician who then prescribed in addition Infectocef 250 mg (cefacrore) for antibiotics. Again this therapy did not result in an amelioration of the high fever condition. Nevertheless the therapy was continued on Wednesday 30-SEP-2009. The boy was observed to be sleepy and it was difficult to wake him. During a further visit to the paediatrician the previous therapy was continued. However, the body temperature remained at 40 to 41 °C. Yet another paediatrician, visited on Thursday 01-OCT-2009, observed that the laryngopharyngitis abated. He confirmed a normal course and that the high fever is to be expected in this case. On Friday 02-OCT-2009 the same paediatrician was consulted again. He declared that the laryngopharyngitis was nearly over. On this day the fever receded to 39.6°C. On Saturday 03-OCT-2009 at 09:00 o'clock the mother observed on her boy blue lips with bloody vesicles. The fever had nearly disappeared at that time. In the morning of this day the intake of Nurofen and Infectocef 250 mg was stopped. In the evening petechiae in various sizes were observed on the whole body. That evening at 20:30 o'clock the paediatrician on duty was consulted. He confirmed the petechial bleedings and ordered the immediate admission to the hospital in Herford, Germany (Kreiskrankenhaus Herford, Schwarzenmoorstr.70, 32049 Herford). There a haemogram was made and the thrombocyte count revealed less than 5000/µl and on the same evening an immunoglobuline therapy was initiated. During that night blood in urine and in stool was detected and also an increase of petechial haemorrhage. The initiated therapy was continued on Sunday 04-OCT-2009. During the night of Sunday to Monday 05-OCT-2009 the boy suffered from severe headaches. Cerebral bleeding was suspected which in fact could not be confirmed later on. According to the mother of the boy the headaches were caused by a too rapid rate of infusion of the immunoglobulines. An ultrasound examination revealed a blood coagulum in the urinary bladder of about 3 cm in diameter as well as a splenomegaly. The same night haematemesis occurred and the body temperature increased to 38.5°C. Thereupon Benuron 300 mg (paracetamole) was administered intravenously leading to a quick remission of the fever. On Monday 05-OCT-2009 the thrombocyte count remained still at less than 5000/µl. The treatment with immunoglobulines was not effective according to the mother. Thus the parents moved the boy out of the hospital assuming full responsibility and brought him to another hospital in Paderborn, Germany (Krankenhaus St. Vincenz Frauen-u. Kinderklinik, Husener Straße 81, 33098 Paderborn). Upon admission there a life-threatening thrombocytopenia was diagnosed with a count of 0/µl and on the night of Monday to Tuesday 05-OCT-2009 erythrocyte concentrates were administered. On Tuesday a corticoid therapy with a dose of 15 mg Decortin (prednisolone) twice daily was started. The hospital made the diagnosis of an immunothrombocytopenic purpura possibly having occurred in connection with a viral infection. On Tuesday morning the thrombocyte count had increased to 23000/µl followed by a further increase to 60000/µl on Wednesday 06-OCT-2009. On the following Thursday 07-OCT-2009 the parents took the boy home again on their own responsibility after the thrombocyte count had stabilized at 60000/µl. At this count no risk was seen. The corticoid therapy was continued at a dose of 15 mg in the morning and the evening. The further ambulant treatment was assumed by the paediatrician. After having read the patient information leaflet the mother suspected a causal relationship of this event with the administration of Nurofen. At the time of our phone conversation with the mother the medical condition of the boy was satisfactory.

Furthermore she told us, that Nurofen suspension has been administered to her son since he was six months old without any problems and that the drug always has induced a decrease of temperature within 30 to 45 minutes and has led to normal body temperature within 1 to 2 days.

Extent and quality of the reported case Information

The source of all the information contained in this report was the patient's mother. To the date of this report no confirmation by any of the involved physicians was available.

Seriousness

Serious

Causality assessment

Based on the one sided source of information this case is not reliably assessable.

Labelling/expectedness (reference document: German SPC of "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" of MAR 2008)

1. Blue lips: verbatim unlabelled
2. Bloody vesicles on the lips: verbatim unlabelled, however superficial lesions in mouth are labelled.
3. Petechiae: verbatim unlabelled, however superficial lesions in mouth are labelled.
4. Thrombocytopenia: dyshaematopoiesis including thrombocytopenia is labelled.
5. Immune thrombocytopenia: verbatim unlabelled, however dyshaematopoiesis including thrombocytopenia is labelled

6. Severe headache: headache is labelled.
7. Blood in urine: unlabelled, however in the context of a severe thrombocytopenia intestinal haemorrhage can occur.
8. Blood in the stool: labelled with regard to gastrointestinal ADRs; independently from gastrointestinal ADRs intestinal haemorrhage can occur in case of thrombocytopenia.
9. Suspicion of cerebral haemorrhage: unlabelled, this event was a tentative diagnosis, which couldn't be confirmed in this case but which in the context of a severe thrombocytopenia belongs to the known clinical complications.
10. Splenomegaly: unlabelled; this finding can be expected in association with thrombocytopenia.
11. Blood coagulum in urinary bladder: unlabelled, however in the context of a severe thrombocytopenia intestinal haemorrhage can occur.
12. Haematemesis: labelled with regard to gastrointestinal ADRs; independently from gastrointestinal ADRs intestinal haemorrhage can occur in case of a severe thrombocytopenia.

From a medical point of view the aetiopathogenesis of the clinical signs and findings observed in this case are in accordance with the resulting clinical sequelae of a severe thrombocytopenia. The occurrence of dyshaematopoiesis

including thrombocytopenia is labelled as a risk in the currently approved SPC of "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen". Altogether this labelling is assessed to cover the reported reactions sufficiently.

VII. ADDITIONAL ADMINISTRATIVE INFORMATION

Scientific level, risk-benefit-ratio, risk prevention measures (with regard to the medicinal product and its authorization status)

Due to the one sided source of information without any medical confirmation changes of the scientific level and/or the risk-benefit-ratio cannot be concluded from this individual case, hence no safety relevant measure to take by the MAH on own responsibility is currently deemed necessary.

Remarks

This case is under further evaluation.

From the antibiotic Infectocef (cefaclore) the risk of inducing thrombocytopenia is known too and therefore labelled in its SPC.

Report No 2 received 15-Oct-2009, case medically confirmed by Physician.

Another suspect drug added: Infectocef.

Two concomitants added: InfectoCortiKrupp and NeoTussan.

All adverse events updated to Thrombocytopenia, Petechiae and Haemorrhages.

Additional information was obtained by phone conversation with the paediatrician on 15-OCT-2009 and another paediatrician, involved on 16-OCT-2009. This CIOMS I followup 2 is connected with the initial and the follow-up 1 CIOMS of this case.

REPORT TERMS AND DESCRIBE REACTIONS

1. Thrombocytopenia
2. Petechiae
3. Haemorrhages

NARRATIVE

Please note, the suspected drug in this case is "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured) which is referred to as "Nurofen" in this narrative.

A paediatrician (doctor on emergency service) reported that on 03-OCT-2009 he was consulted by the parents with a 5-year-old boy for the first time. The boy presented with petechiae and he advised the parents to take him to the hospital immediately. On his ADR report he stated as suspected drug Nurofen and remarked that former administration of this medicinal product had been well tolerated. During a phone call on 15-OCT-2009 he additionally reported "Infectocef" (cephaclor) as suspected drug (prescribed by another paediatrician). After having taken the boy out of hospital against medical advice the parents and the boy consulted him once more for control of thrombocytes on 08-OCT-2009. The laboratory investigation revealed a count of thrombocytes of 64.000 / μ l. A further appointment for laboratory control in his practice was not kept. He assumed that the parents had changed the attending physician again. During the phone conversation the paediatrician emphasized not to be able to give a statement concerning causality.

He considered an involvement of both medicinal products (Nurofen and Infectocef) as possible. He suspected a thrombocytopenia during a viral infection as the most likely cause. Furthermore he reported the course of the event as regressive and the outcome as unknown.

A second paediatrician involved in this case reported during a phone conversation on 16-OCT-2009 that he saw the boy on 29-SEP-2009, the third day with fever. The laboratory findings of this day showed a leucocytosis as well as an elevated CRP. Therefore he prescribed "Infectocef" (cephaclor). Concerning the causality of the thrombocytopenia he stated that no assessment was possible.

Seriousness

Serious

Causality Assessment

1. Thrombocytopenia: possible
2. Petechiae: possible
3. Haemorrhages: possible

Labelling/Expectedness (reference document: German SPC of Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen of MAR 2008)

1. Thrombocytopenia: labelled
2. Petechiae: verbatim unlabelled, however superficial lesions in the mouth are labelled.
3. Haemorrhages: verbatim unlabelled, however gastrointestinal bleedings, nose bleeding and cutaneous haemorrhages are labelled. This labelling is assessed to cover the reported reaction sufficiently.

State of scientific level, risk-benefit-ratio, risk prevention measures

This individual case does not affect the scientific level and/or the risk-benefit ratio of Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum

Einnehmen, hence no safety relevant measures with regard to the marketing authorization status to take by the MAH on own responsibility is currently deemed necessary.

Remarks

This case is under evaluation.

Sender's comment:

Reporter's comment:

RBHC, 12.11.2009 DE-RB-022307-09

Sender diagnosis: not reported

Narrative

Report no 1 is a serious case received from RB Germany on 08-Oct-2009, a consumer case ref: RBD-23726790.

The mother of a 5-year-old boy reported the ADR directly to Reckitt Benckiser Germany. Further information has been obtained by a telephone call with the mother on 08-OCT-2009. This report is based entirely on the mother's statements and is not confirmed by any HCP.

REPORT TERMS AND DESCRIBE REACTIONS

1. Blue lips
2. Bloody vesicles on the lips
3. Petechiae
4. Thrombocytopenia
5. Immune thrombocytopenia
6. Severe headache
7. Blood in urine
8. Blood in the stool
9. Suspicion of cerebral haemorrhage
10. Splenomegaly
11. Blood coagulum in urinary bladder
12. Haematemesis.

NARRATIVE

Please note, in this case the suspected drug is "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml) which is referred to as "Nurofen" in this narrative.

On Sunday 27-SEP-2009 a 5-year-old boy developed high fever up to 40.7 °C. A paediatrician on duty (contacted by the boy's mother) diagnosed by phone the boy with pseudo-croup based on the described symptoms and therefore prescribed Nurofen and Rectodelt suppositories (prednisone).

Nurofen was administered in a dose of 5 mL every 6 hours including night time. A reduction of the fever was not achieved. A paediatrician contacted on Monday 28-SEP-2009 confirmed the diagnosis of pseudo-croup and instructed to continue the initial therapy. The fever remained high in spite of the therapy.

On Tuesday 29-SEP-2009 laryngopharyngitis was diagnosed by the paediatrician who then prescribed in addition Infectocef 250 mg (cefaclore) for antibiosis. Again this therapy did not result in an amelioration of the high fever condition. Nevertheless the therapy was continued on Wednesday 30-SEP-2009. The boy was observed to be sleepy and it was difficult to wake him. During a further visit to the paediatrician the previous therapy was continued. However, the body temperature remained at 40 to 41 °C. Yet another paediatrician, visited on Thursday 01-OCT-2009, observed that the laryngopharyngitis abated. He confirmed a normal course and that the high fever is to be expected in this case. On Friday 02-OCT-2009 the same paediatrician was consulted again. He declared that the laryngopharyngitis was nearly over. On this day the fever receded to 39.6°C. On Saturday 03-OCT-2009 at 09:00 o'clock the mother observed on her boy blue lips with bloody vesicles. The fever had nearly disappeared at that time. In the morning of this day the intake of Nurofen and Infectocef 250 mg was stopped. In the evening petechiae in various sizes were observed on the whole body. That evening at 20:30 o'clock the paediatrician on duty was consulted. He confirmed the petechial bleedings and ordered the immediate admission to the hospital in Herford, Germany (Kreiskrankenhaus Herford, Schwarzenmoorstr.70, 32049 Herford). There a haemogram was made and the thrombocyte count revealed less than 5000/μl and on the same evening an immunoglobuline therapy was initiated. During that night blood in urine and in stool was detected and also an increase of petechial haemorrhage. The initiated therapy was continued on Sunday 04-OCT-2009. During the night of Sunday to Monday 05-OCT-2009 the boy suffered from severe headaches. Cerebral bleeding was suspected which in fact could not be confirmed later on. According to the mother of the boy the

headaches were caused by a too rapid rate of infusion of the immunoglobulines. An ultrasound examination revealed a blood coagulum in the urinary bladder of about 3 cm in diameter as well as a splenomegaly. The same night haematemesis occurred and the body temperature increased to 38.5°C. Thereupon Benuron 300 mg (paracetamol) was administered intravenously leading to a quick remission of the fever. On Monday 05-OCT-2009 the thrombocyte count remained still at less than 5000/µl. The treatment with immunoglobulines was not effective according to the mother. Thus the parents moved the boy out of the hospital assuming full responsibility and brought him to another hospital in Paderborn, Germany (Krankenhaus St. Vincenz Frauen-u. Kinderklinik, Husener Straße 81, 33098 Paderborn). Upon admission there a life-threatening thrombocytopenia was diagnosed with a count of 0/µl and on the night of Monday to Tuesday 05-OCT-2009 erythrocyte concentrates were administered. On Tuesday a corticoid therapy with a dose of 15 mg Decortin (prednisolone) twice daily was started. The hospital made the diagnosis of an immunothrombocytopenic purpura possibly having occurred in connection with a viral infection. On Tuesday morning the thrombocyte count had increased to 23000/µl followed by a further increase to 60000/µl on Wednesday 06-OCT-2009. On the following Thursday 07-OCT-2009 the parents took the boy home again on their own responsibility after the thrombocyte count had stabilized at 60000/µl. At this count no risk was seen. The corticoid therapy was continued at a dose of 15 mg in the morning and the evening. The further ambulant treatment was assumed by the paediatrician. After having read the patient information leaflet the mother suspected a causal relationship of this event with the administration of Nurofen. At the time of our phone conversation with the mother the medical condition of the boy was satisfactory.

Furthermore she told us, that Nurofen suspension has been administered to her son since he was six months old without any problems and that the drug always has induced a decrease of temperature within 30 to 45 minutes and has led to normal body temperature within 1 to 2 days.

Extent and quality of the reported case Information

The source of all the information contained in this report was the patient's mother. To the date of this report no confirmation by any of the involved physicians was available.

Seriousness

Serious

Causality assessment

Based on the one sided source of information this case is not reliably assessable.

Labelling/expectedness (reference document: German SPC of "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" of MAR 2008)

1. Blue lips: verbatim unlabelled
2. Bloody vesicles on the lips: verbatim unlabelled, however superficial lesions in mouth are labelled.
3. Petechiae: verbatim unlabelled, however superficial lesions in mouth are labelled.
4. Thrombocytopenia: dyshaematopoiesis including thrombocytopenia is labelled.
5. Immune thrombocytopenia: verbatim unlabelled, however dyshaematopoiesis including thrombocytopenia is labelled
6. Severe headache: headache is labelled.
7. Blood in urine: unlabelled, however in the context of a severe thrombocytopenia intestinal haemorrhage can occur.
8. Blood in the stool: labelled with regard to gastrointestinal ADRs; independently from gastrointestinal ADRs intestinal haemorrhage can occur in case of thrombocytopenia.
9. Suspicion of cerebral haemorrhage: unlabelled, this event was a tentative diagnosis, which couldn't be confirmed in this case but which in the context of a severe thrombocytopenia belongs to the known clinical complications.
10. Splenomegaly: unlabelled; this finding can be expected in association with thrombocytopenia.
11. Blood coagulum in urinary bladder: unlabelled, however in the context of a severe thrombocytopenia intestinal haemorrhage can occur.
12. Haematemesis: labelled with regard to gastrointestinal ADRs; independently from gastrointestinal ADRs intestinal haemorrhage can occur in case of a severe thrombocytopenia.

From a medical point of view the aetiopathogenesis of the clinical signs and findings observed in this case are in accordance with the resulting clinical sequelae of a severe thrombocytopenia. The occurrence of dyshaematopoiesis including thrombocytopenia is labelled as a risk in the currently approved SPC of "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen". Altogether this labelling is assessed to cover the reported reactions sufficiently.

VII. ADDITIONAL ADMINISTRATIVE INFORMATION

Scientific level, risk-benefit-ratio, risk prevention measures (with regard to the medicinal product and its authorization status)

Due to the one sided source of information without any medical confirmation changes of the scientific level and/or the risk-benefit-ratio cannot be concluded from this individual case, hence no safety relevant measure to take by the MAH on own responsibility is currently deemed necessary.

Remarks

This case is under further evaluation.

From the antibiotic Infectocef (cefacrore) the risk of inducing thrombocytopenia is known too and therefore labelled in its SPC.

Report No 2 received 15-Oct-2009, case medically confirmed by Physician.

Another suspect drug added: Infectocef.

Two concomitants added: InfectoCortiKrupp and NeoTussan.

All adverse events updated to Thrombocytopenia, Petechiae and Haemorrhages.

Additional information was obtained by phone conversation with the paediatrician on 15-OCT-2009 and another paediatrician, involved on 16-OCT-2009. This CIOMS I followup 2 is connected with the initial and the follow-up 1 CIOMS of this case.

REPORT TERMS AND DESCRIBE REACTIONS

1. Thrombocytopenia
2. Petechiae
3. Haemorrhages

NARRATIVE

Please note, the suspected drug in this case is "Nurofen Junior Fiebersaft Erdbeer 4%

Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured) which is referred to as "Nurofen" in this narrative.

A paediatrician (doctor on emergency service) reported that on 03-OCT-2009 he was

consulted by the parents with a 5-year-old boy for the first time. The boy presented with petechiae and he advised the parents to take him to the hospital immediately. On his ADR report he stated as suspected drug Nurofen and remarked that former administration of this medicinal product had been well tolerated. During a phone call on 15-OCT-2009 he additionally reported "Infectocef" (cephaclor) as suspected drug (prescribed by another paediatrician). After having taken the boy out of hospital against medical advice the parents and the boy consulted him once more for control of thrombocytes on 08-OCT-2009. The laboratory investigation revealed a count of thrombocytes of 64.000 / μ l. A further appointment for laboratory control in his practice was not kept. He assumed that the parents had changed the attending physician again. During the phone conversation the paediatrician emphasized not to be able to give a statement concerning causality.

He considered an involvement of both medicinal products (Nurofen and Infectocef) as possible. He suspected a thrombocytopenia during a viral infection as the most likely cause. Furthermore he reported the course of the event as regressive and the outcome as unknown.

A second paediatrician involved in this case reported during a phone conversation on 16-OCT-2009 that he saw the boy on 29-SEP-2009, the third day with fever. The laboratory findings of this day showed a leucocytosis as well as an elevated CRP. Therefore he prescribed "Infectocef" (cephaclor). Concerning the causality of the thrombocytopenia he stated that no assessment was possible.

Seriousness

Serious

Causality Assessment

1. Thrombocytopenia: possible
2. Petechiae: possible
3. Haemorrhages: possible

Labelling/Expectedness (reference document: German SPC of Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen of MAR 2008)

1. Thrombocytopenia: labelled
2. Petechiae: verbatim unlabelled, however superficial lesions in the mouth are labelled.
3. Haemorrhages: verbatim unlabelled, however gastrointestinal bleedings, nose bleeding and cutaneous haemorrhages are labelled. This labelling is assessed to cover the reported reaction sufficiently.

State of scientific level, risk-benefit-ratio, risk prevention measures

This individual case does not affect the scientific level and/or the risk-benefit ratio of Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen, hence no safety relevant measures with regard to the marketing authorization status to take by the MAH on own responsibility is currently deemed necessary.

Remarks

This case is under evaluation.

Report No 3 received from RB Germany, Date 05-NOV-2009, Ref: RBD-23726790. Hospital discharge report received, Date 03-NOV-2009.

Information added / updated:

Primary Sources: Hospital information added.

Adverse Events: As reported 'Thrombocyteopenia' changed to 'Idiopathic Thrombocyteopenia Purpura' and In Diagnosis ticked Yes. 'Haemorrhages' changed to 'Haematoma'.

Suspect Drugs: 'Infectocef' start date, duration, unit dose and indications 'Laryngopharyngitis with Leukocytosis' and 'Increase of CRP' added. 'Infection' removed.

Concomitant Medications: 'InfectoCortiKrupp' and 'Neo Tussan' removed. 'Rectodelt' added and Indication 'Pseudo-croup' added.

Company Diagnosis: Changed to 'Idiopathic Thrombocyteopenic Purpura'.

Lab Data added.

Case information originating from the hospital discharge report of the hospital in Paderborn (Krankenhaus St. Vincenz, Frauen- u. Kinderklinik, Husener Straße 81, 33098 Paderborn):

In the hospital discharge report, dated with 27-OCT-2009, the diagnosis of an idiopathic thrombocytopenic purpura was reported. The report did not indicate a causal relationship between this diagnosis and the administration of Nurofen or "Infectocef" (cephaclor). Findings on admission were: a haematoma at the left eye, multiple haematomata of different age as well as old and new petechiae on the whole integument and petechiae on hard palate and on genital and scrotum, furthermore palpable cervical lymph nodes. The body temperature was 38.2 C°. Concerning therapy and medical course the treatment with prednisolone, which the hospital in Herford, Germany, had started was continued. Additionally the boy got immunoglobulines and once a thrombocyte concentrate, because the laboratory findings on admission showed a thrombocyte count of 0/ μ l. Laboratory findings furthermore indicated a mild viral infection. During therapy the thrombocyte count increased and showed on the second day of hospitalization 23000/ μ l and on the third day 60000 / μ l. Thereafter the immunoglobuline therapy was finished, whereas the prednisolone therapy was continued. On the third day the parents of the boy decided to take him out of hospital against medical advice and despite the warning of the risk of cerebral bleeding.

Laboratory findings

The following laboratory results originate from the record on discharge of the hospital in Paderborn (Krankenhaus St. Vincenz, Frauen- u. Kinderklinik, Husener Straße 81, 33098 Paderborn). Those were communicated without specifying laboratory method and normal values.

05-OCT-2009: leukocytes 13220/ μ l, Hb 11.6 g/dl, thrombocytes 0/ μ l, neutrophiles 75.8%, lymphocytes 12.2%, glucose 162 mg/dl, CRP 1.16 mg/dl.

06-OCT-2009: Hb 11.4 g/dl, thrombocytes 23000/ μ l, CRP 0.8 mg/dl.

07-OCT-2009: Hb 11.7 g/dl, thrombocytes 60000/ μ l, CRP 0.47 mg/dl, glucose 85 mg/dl.

Urinary findings

05-OCT-2009: ph 7.0, leukocytes 25/ μ l, protein 100 mg/dl, glucose 300 mg/dl, erythrocytes >350/ μ l. In the sediment: leukocytes 1305/ μ l, erythrocytes 495/ μ l, bacteria +positive.

06-OCT-2009: ph 8.0, erythrocytes 350/ μ l.

07-OCT-2009: ph 8.0, erythrocytes 90/ μ l.

Typing of blood

05-OCT-2009: blood group 0, Rhesus factor positive (D positive). Kell factor negative. Rh system: CcD. ee. Autoagglutination testing: negative. Antibody screening negative.

On the occasion of a further phone conversation (21-OCT-2009) the mother strictly denied the administration of "InfectoCortiKrupp" (prednisolone) and "NeoTussan" (dextromethorphan), which was reported by one of the involved paediatricians.

From a medical point of view the pathogenesis of the clinical signs and findings observed in this case are in accordance with clinical sequelae of a severe thrombocytopenia. The occurrence of dyshaematopoiesis including thrombocytopenia is labelled as a risk in the currently approved SPC of "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen". Altogether this labelling is assessed to cover the reported reactions sufficiently. Current status of scientific knowledge - risk-benefit-ratio - risk prevention measures (with regard to the medicinal product and its authorization status) This individual case does not affect the scientific level and/or the risk-benefit-ratio of "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen", hence no safety relevant measures with regard to the marketing authorization status to take by the MAH on own responsibility is currently deemed necessary.

Remarks

The most frequent form of thrombopenic purpura during childhood is the idiopathic thrombopenic purpura which occurs without any recognizable cause. An immunological thrombocytopenia showing antithrombocytic antibodies can also develop without involvement of drugs e.g. in the context of infections case. The clinical course in this case showing no quick recovery after drug discontinuation as well as the good tolerability of Nurofen administration since years argue against a drug induced pathogenesis.

Outcome: Recovering / Resolving.

Sender's comment:

Reporter's comment:

RBHC, 24.11.2009 DE-RB-022307-09

Sender diagnosis: not reported

Narrative

Report no 1 is a serious case received from RB Germany on 08-Oct-2009, a consumer case ref: RBD-23726790.

The mother of a 5-year-old boy reported the ADR directly to Reckitt Benckiser Germany. Further information has been obtained by a telephone call with the mother on 08-OCT-2009. This report is based entirely on the mother's statements and is not confirmed by any HCP.

REPORT TERMS AND DESCRIBE REACTIONS

1. Blue lips
2. Bloody vesicles on the lips
3. Petechiae
4. Thrombocytopenia
5. Immune thrombocytopenia
6. Severe headache

7. Blood in urine
8. Blood in the stool
9. Suspicion of cerebral haemorrhage
10. Splenomegaly
11. Blood coagulum in urinary bladder
12. Haematemesis.

NARRATIVE

Please note, in this case the suspected drug is "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml) which is referred to as "Nurofen" in this narrative.

On Sunday 27-SEP-2009 a 5-year-old boy developed high fever up to 40.7°C. A paediatrician on duty (contacted by the boy's mother) diagnosed by phone the boy with pseudo-croup based on the described symptoms and therefore prescribed Nurofen and Rectodelt suppositories (prednisone).

Nurofen was administered in a dose of 5 mL every 6 hours including night time. A reduction of the fever was not achieved. A paediatrician contacted on Monday 28-SEP-2009 confirmed the diagnosis of pseudo-croup and instructed to continue the initial therapy. The fever remained high in spite of the therapy.

On Tuesday 29-SEP-2009 laryngopharyngitis was diagnosed by the paediatrician who then prescribed in addition Infectocel 250 mg (cefaclore) for antibiotics. Again this therapy did not result in an amelioration of the high fever condition. Nevertheless the therapy was continued on Wednesday 30-SEP-2009. The boy was observed to be sleepy and it was difficult to wake him. During a further visit to the paediatrician the previous therapy was continued. However, the body temperature remained at 40 to 41 °C. Yet another paediatrician, visited on Thursday 01-OCT-2009, observed that the laryngopharyngitis abated. He confirmed a normal course and that the high fever is to be expected in this case. On Friday 02-OCT-2009 the same paediatrician was consulted again. He declared that the laryngopharyngitis was nearly over. On this day the fever receded to 39.6°C. On Saturday 03-OCT-2009 at 09:00 o'clock the mother observed on her boy blue lips with bloody vesicles. The fever had nearly disappeared at that time. In the morning of this day the intake of Nurofen and Infectocel 250 mg was stopped. In the evening petechiae in various sizes were observed on the whole body. That evening at 20:30 o'clock the paediatrician on duty was consulted. He confirmed the petechial bleedings and ordered the immediate admission to the hospital in Herford, Germany (Kreiskrankenhaus Herford, Schwarzenmoorstr.70, 32049 Herford). There a haemogram was made and the thrombocyte count revealed less than 5000/μl and on the same evening an immunoglobuline therapy was initiated. During that night blood in urine and in stool was detected and also an increase of petechial haemorrhage. The initiated therapy was continued on Sunday 04-OCT-2009. During the night of Sunday to Monday 05-OCT-2009 the boy suffered from severe headaches. Cerebral bleeding was suspected which in fact could not be confirmed later on. According to the mother of the boy the headaches were caused by a too rapid rate of infusion of the immunoglobulines. An ultrasound examination revealed a blood coagulum in the urinary bladder of about 3 cm in diameter as well as a splenomegaly. The same night haematemesis occurred and the body temperature increased to 38.5°C. Thereupon Benuron 300 mg (paracetamol) was administered intravenously leading to a quick remission of the fever. On Monday 05-OCT-2009 the thrombocyte count remained still at less than 5000/μl. The treatment with immunoglobulines was not effective according to the mother. Thus the parents moved the boy out of the hospital assuming full responsibility and brought him to another hospital in Paderborn, Germany (Krankenhaus St. Vincenz Frauen-u. Kinderklinik, Husener Straße 81, 33098 Paderborn). Upon admission there a life-threatening thrombocytopenia was diagnosed with a count of 0/μl and on the night of Monday to Tuesday 05-OCT-2009 erythrocyte concentrates were administered. On Tuesday a corticoid therapy with a dose of 15 mg Decortin (prednisolone) twice daily was started. The hospital made the diagnosis of an immunothrombocytopenic purpura possibly having occurred in connection with a viral infection. On Tuesday morning the thrombocyte count had increased to 23000/μl followed by a further increase to 60000/μl on Wednesday 06-OCT-2009. On the following Thursday 07-OCT-2009 the parents took the boy home again on their own responsibility after the thrombocyte count had stabilized at 60000/μl. At this count no risk was seen. The corticoid therapy was continued at a dose of 15 mg in the morning and the evening. The further ambulant treatment was assumed by the paediatrician. After having read the patient information leaflet the mother suspected a causal relationship of this event with the administration of Nurofen. At the time of our phone conversation with the mother the medical condition of the boy was satisfactory.

Furthermore she told us, that Nurofen suspension has been administered to her son since he was six months old without any problems and that the drug always has induced a decrease of temperature within 30 to 45 minutes and has led to normal body temperature within 1 to 2 days.

Extent and quality of the reported case Information

The source of all the information contained in this report was the patient's mother. To the date of this report no confirmation by any of the involved physicians was available.

Seriousness

Serious

Causality assessment

Based on the one sided source of information this case is not reliably assessable.

Labelling/expectedness (reference document: German SPC of "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" of MAR 2008)

1. Blue lips: verbatim unlabelled
2. Bloody vesicles on the lips: verbatim unlabelled, however superficial lesions in mouth are labelled.
3. Petechiae: verbatim unlabelled, however superficial lesions in mouth are labelled.
4. Thrombocytopenia: dyshaematopoiesis including thrombocytopenia is labelled.
5. Immune thrombocytopenia: verbatim unlabelled, however dyshaematopoiesis including thrombocytopenia is labelled
6. Severe headache: headache is labelled.
7. Blood in urine: unlabelled, however in the context of a severe thrombocytopenia intestinal haemorrhage can occur.
8. Blood in the stool: labelled with regard to gastrointestinal ADRs; independently from gastrointestinal ADRs intestinal haemorrhage can occur in case of thrombocytopenia.

9. Suspicion of cerebral haemorrhage: unlabelled, this event was a tentative diagnosis, which couldn't be confirmed in this case but which in the context of a severe thrombocytopenia belongs to the known clinical complications.
10. Splenomegaly: unlabelled; this finding can be expected in association with thrombocytopenia.
11. Blood coagulum in urinary bladder: unlabelled, however in the context of a severe thrombocytopenia intestinal haemorrhage can occur.
12. Haematemesis: labelled with regard to gastrointestinal ADRs; independently from gastrointestinal ADRs intestinal haemorrhage can occur in case of a severe thrombocytopenia.

From a medical point of view the aetiopathogenesis of the clinical signs and findings observed in this case are in accordance with the resulting clinical sequelae of a severe thrombocytopenia. The occurrence of dyshaematopoiesis including thrombocytopenia is labelled as a risk in the currently approved SPC of "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen". Altogether this labelling is assessed to cover the reported reactions sufficiently.

VII. ADDITIONAL ADMINISTRATIVE INFORMATION

Scientific level, risk-benefit-ratio, risk prevention measures (with regard to the medicinal product and its authorization status)

Due to the one sided source of information without any medical confirmation changes of the scientific level and/or the risk-benefit-ratio cannot be concluded from this individual case, hence no safety relevant measure to take by the MAH on own responsibility is currently deemed necessary.

Remarks

This case is under further evaluation.

From the antibiotic Infectocef (cefaclor) the risk of inducing thrombocytopenia is known too and therefore labelled in its SPC.

Report No 2 received 15-Oct-2009, case medically confirmed by Physician.

Another suspect drug added: Infectocef.

Two concomitants added: InfectoCortiKrupp and NeoTussan.

All adverse events updated to Thrombocytopenia, Petechiae and Haemorrhages.

Additional information was obtained by phone conversation with the paediatrician on 15-OCT-2009 and another paediatrician, involved on 16-OCT-2009. This CIOMS I followup 2 is connected with the initial and the follow-up 1 CIOMS of this case.

REPORT TERMS AND DESCRIBE REACTIONS

1. Thrombocytopenia
2. Petechiae
3. Haemorrhages

NARRATIVE

Please note, the suspected drug in this case is "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured) which is referred to as "Nurofen" in this narrative.

A paediatrician (doctor on emergency service) reported that on 03-OCT-2009 he was consulted by the parents with a 5-year-old boy for the first time. The boy presented with petechiae and he advised the parents to take him to the hospital immediately. On his ADR report he stated as suspected drug Nurofen and remarked that former administration of this medicinal product had been well tolerated. During a phone call on 15-OCT-2009 he additionally reported "Infectocef" (cephaclor) as suspected drug (prescribed by another paediatrician). After having taken the boy out of hospital against medical advice the parents and the boy consulted him once more for control of thrombocytes on 08-OCT-2009. The laboratory investigation revealed a count of thrombocytes of 64.000 / μ l. A further appointment for laboratory control in his practice was not kept. He assumed that the parents had changed the attending physician again. During the phone conversation the paediatrician emphasized not to be able to give a statement concerning causality.

He considered an involvement of both medicinal products (Nurofen and Infectocef) as possible. He suspected a thrombocytopenia during a viral infection as the most likely cause. Furthermore he reported the course of the event as regressive and the outcome as unknown.

A second paediatrician involved in this case reported during a phone conversation on 16-OCT-2009 that he saw the boy on 29-SEP-2009, the third day with fever. The laboratory findings of this day showed a leucocytosis as well as an elevated CRP. Therefore he prescribed "Infectocef" (cephaclor). Concerning the causality of the thrombocytopenia he stated that no assessment was possible.

Seriousness

Serious

Causality Assessment

1. Thrombocytopenia: possible
2. Petechiae: possible
3. Haemorrhages: possible

Labelling/Expectedness (reference document: German SPC of Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen of MAR 2008)

1. Thrombocytopenia: labelled
2. Petechiae: verbatim unlabelled, however superficial lesions in the mouth are labelled.
3. Haemorrhages: verbatim unlabelled, however gastrointestinal bleedings, nose bleeding and cutaneous haemorrhages are labelled. This labelling is assessed to cover the reported reaction sufficiently.

State of scientific level, risk-benefit-ratio, risk prevention measures

This individual case does not affect the scientific level and/or the risk-benefit ratio of Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen, hence no safety relevant measures with regard to the marketing authorization status to take by the MAH on own responsibility is currently deemed necessary.

Remarks

This case is under evaluation.

Report No 3 received from RB Germany, Date 05-NOV-2009, Ref: RBD-23726790. Hospital discharge report received, Date 03-NOV-2009.

Information added / updated:

Primary Sources: Hospital information added.

Adverse Events: As reported 'Thrombocytopenia' changed to 'Idiopathic Thrombocytopenia Purpura' and In Diagnosis ticked Yes. 'Hæmorrhages' changed to 'Haematoma'.

Suspect Drugs: 'Infectocef' start date, duration, unit dose and indications 'Laryngopharyngitis with Leukocytosis' and 'Increase of CRP' added. 'Infection' removed.

Concomitant Medications: 'InfectoCortiKrupp' and 'Neo Tussan' removed. 'Rectodelt' added and Indication 'Pseudo-croup' added.

Company Diagnosis: Changed to 'Idiopathic Thrombocytopenic Purpura'.

Lab Data added.

Case information originating from the hospital discharge report of the hospital in Paderborn (Krankenhaus St. Vincenz, Frauen- u. Kinderklinik, Husener Straße 81, 33098 Paderborn):

In the hospital discharge report, dated with 27-OCT-2009, the diagnosis of an idiopathic thrombocytopenic purpura was reported. The report did not indicate a causal relationship between this diagnosis and the administration of Nurofen or "Infectocef" (cephaclore). Findings on admission were: a haematoma at the left eye, multiple haematomata of different age as well as old and new petechiae on the whole integument and petechiae on hard palate and on genital and scrotum, furthermore palpable cervical lymph nodes. The body temperature was 38.2 °C. Concerning therapy and medical course the treatment with prednisolone, which the hospital in Herford, Germany, had started was continued. Additionally the boy got immunoglobulines and once a thrombocyte concentrate, because the laboratory findings on admission showed a thrombocyte count of 0/µl. Laboratory findings furthermore indicated a mild viral infection. During therapy the thrombocyte count increased and showed on the second day of hospitalization 23000/µl and on the third day 60000 /µl. Thereafter the immunoglobuline therapy was finished, whereas the prednisolone therapy was continued. On the third day the parents of the boy decided to take him out of hospital against medical advice and despite the warning of the risk of cerebral bleeding.

Laboratory findings

The following laboratory results originate from the record on discharge of the hospital in Paderborn (Krankenhaus St. Vincenz, Frauen- u. Kinderklinik, Husener Straße 81, 33098 Paderborn). Those were communicated without specifying laboratory method and normal values.

05-OCT-2009: leukocytes 13220/µl, Hb 11.6 g/dl, thrombocytes 0/µl, neutrophiles 75.8%, lymphocytes 12.2%, glucose 162 mg/dl, CRP 1.16 mg/dl.

06-OCT-2009: Hb 11.4 g/dl, thrombocytes 23000/µl, CRP 0.8 mg/dl.

07-OCT-2009: Hb 11.7 g/dl, thrombocytes 60000/µl, CRP 0.47 mg/dl, glucose 85 mg/dl.

Urinary findings

05-OCT-2009: ph 7.0, leukocytes 25/µl, protein 100 mg/dl, glucose 300 mg/dl, erythrocytes >350/µl. In the sediment: leukocytes 1305/µl, erythrocytes 495/µl, bacteria +positive.

06-OCT-2009: ph 8.0, erythrocytes 350/µl.

07-OCT-2009: ph 8.0, erythrocytes 90/µl.

Typing of blood

05-OCT-2009: blood group 0, Rhesus factor positive (D positive). Kell factor negative. Rh system: CcD.ee. Autoagglutination testing: negative.

Antibody screening negative.

On the occasion of a further phone conversation (21-OCT-2009) the mother strictly denied the administration of "InfectoCortiKrupp" (prednisolone) and "NeoTussan" (dextromethorphan), which was reported by one of the involved paediatricians.

From a medical point of view the pathogenesis of the clinical signs and findings observed in this case are in accordance with clinical sequelae of a severe thrombocytopenia. The occurrence of dyshaematopoiesis including thrombocytopenia is labelled as a risk in the currently approved SPC of "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen". Altogether this labelling is assessed to cover the reported reactions sufficiently. Current status of scientific knowledge - risk-benefit-ratio - risk prevention measures (with regard to the medicinal product and its authorization status) This individual case does not affect the scientific level and/or the risk-benefit-ratio of "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen", hence no safety relevant measures with regard to the marketing authorization status to take by the MAH on own responsibility is currently deemed necessary.

Remarks

The most frequent form of thrombopenic purpura during childhood is the idiopathic thrombopenic purpura which occurs without any recognizable cause. An immunological thrombocytopenia showing antithrombocytic antibodies can also develop without involvement of drugs e.g. in the context of infections case. The clinical course in this case showing no quick recovery after drug discontinuation as well as the good tolerability of Nurofen administration since years argue against a drug induced pathogenesis.

Outcome: Recovering / Resolving.

Report No 4 received 12-NOV-2009. Email confirmation from DSO regarding reported adverse events.

Information added / Updated:

Adverse events added:

'Blue lips
'Bloody vesicles on the lips'
'Immune thrombocytopenia'
'Severe headache'
'Blood in urine'
'Blood in the stool'
'Suspicion of cerebral haemorrhage'
'Splenomegaly'
'Blood coagulum in urinary bladder'
'Haematemesis'

1. Thrombocytopenia*
2. Petechiae *
3. Haemorrhages*
4. Blue lips **
5. Bloody vesicles on the lips **
6. Immune thrombocytopenia **
7. Severe headache **
8. Blood in urine **
9. Blood in the stool **
10. Suspicion of cerebral haemorrhage **
11. Splenomegaly**
12. Blood coagulum in urinary bladder**
13. Haematemesis**

*Reaction medically confirmed (ADR report from one of the involved paediatricians in private practice).

**Statements of the mother of the boy without medical confirmation.

Extend and quality of the reported case information

The statements of the mother of the concerned boy and the data obtained from medical institutions do not fully correspond. Nevertheless the obtained information is sufficient for case evaluation.

Outcome: Recovering / Resolving.

Sender's comment:

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09169590

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 58 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Allergic reaction	200911				not recovered/not resolved
2 Eye swelling					Unknown
3 Itching					Unknown
4 Drug eruption	200911				recovered/resolved
5 Dry throat					Unknown
6 Erythema					Unknown
7 Lip swelling					Unknown
8 Quincke's edema	20091129			11 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuflam ibuprofen	Oral	600 Mg milligram(s)/Day 82	20091119- -	Joint inflammation

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)[sanofi-aventis, 10.12.2009 DE-SANOFI-AVENTIS-2009SA005258](#)

Sender diagnosis: not reported

Narrative

Initial report received from the patient herself on 30-Nov-2009. The female patient was prescribed ibuprofen (Ibuflam 600mg) on 19-Nov-2009 for inflammation in her shoulder joint. During treatment with ibuprofen, the patient developed mild swelling in the area of both eyes, on 28-Nov-2009, symptoms worsened slightly, and the patient developed mild swelling of her upper lip. On 29-Nov-2009, swelling in area of eyes and of upper lip intensified, the patient suffered from dry throat. On 30-Nov-2009, the patient woke up with swollen eyes and lips, and an erythema on her upper arms and her back. She visited a physician and was hospitalized with the diagnosis of allergic reaction. The patient did not take any concomitant medication. Further information was not provided.

Sender's comment:

Sanofi-Aventis Company Comment dated 04-DEC-2009:

Based on the available information and safety profile of ibuprofen a contributory role cannot be excluded. Allergic reactions are known to occur under treatment with ibuprofen.10-DEC-2009 Sanofi-Aventis Germany

All reported clinical diagnoses are considered locally labelled for all suspected products and thus consistent with the known benefit risk ratio of the product(s). Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

sanofi-aventis, 18.12.2009 DE-SANOFI-AVENTIS-2009SA005258

Sender diagnosis: not reported

Narrative

Initial report received from the patient herself on 30-Nov-2009. The female patient was prescribed ibuprofen (Ibuflam 600mg) on 19-Nov-2009 for inflammation in her shoulder joint. During treatment with ibuprofen, the patient developed mild swelling in the area of both eyes, on 28-Nov-2009, symptoms worsened slightly, and the patient developed mild swelling of her upper lip. On 29-Nov-2009, swelling in area of eyes and of upper lip intensified, the patient suffered from dry throat. On 30-Nov-2009, the patient woke up with swollen eyes and lips, and an erythema on her upper arms and her back. She visited a physician and was hospitalized with the diagnosis of allergic reaction. The patient did not take any concomitant medication. Further information was not provided.

Addendum received for follow-up on 14-Dec-2009:

This follow-up information was provided from the 59-year-old female patient herself. She reported that she developed an allergic reaction during therapy with ibuprofen. Therefore she was in emergency room. She received cortisone tablets ambulant as corrective treatment.

Sender's comment:

Sanofi-Aventis Company Comment dated 17-DEC-2009: New information does not change previous assessment of this case.

Based on the available information and safety profile of ibuprofen a contributory role cannot be excluded. Allergic reactions are known to occur under treatment with ibuprofen. 18-DEC-2009 Sanofi-Aventis Germany

All reported clinical diagnoses are considered locally labelled for all suspected products and thus consistent with the known benefit risk ratio of the product(s). Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

sanofi-aventis, 18.01.2010 DE-SANOFI-AVENTIS-2009SA005258

Sender diagnosis: not reported

Narrative

Initial report received from the patient herself on 30-Nov-2009. The female patient was prescribed ibuprofen (Ibuflam 600mg) on 19-Nov-2009 for inflammation in her shoulder joint. During treatment with ibuprofen, the patient developed mild swelling in the area of both eyes, on 28-Nov-2009, symptoms worsened slightly, and the patient developed mild swelling of her upper lip. On 29-Nov-2009, swelling in area of eyes and of upper lip intensified, the patient suffered from dry throat. On 30-Nov-2009, the patient woke up with swollen eyes and lips, and an erythema on her upper arms and her back. She visited a physician and was hospitalized with the diagnosis of allergic reaction. The patient did not take any concomitant medication. Further information was not provided.

Addendum received for follow-up on 14-Dec-2009:

This follow-up information was provided from the 59-year-old female patient herself. She reported that she developed an allergic reaction during therapy with ibuprofen. Therefore she was in emergency room. She received cortisone tablets ambulant as corrective treatment.

Addendum received for follow-up on 13-Jan-2010:

This follow-up information was provided from patient's physician who informed us that he was not involved to this case.

Sender's comment:

Sanofi-Aventis Company Comment dated 14-JAN-2010: New information does not change previous assessment of this case.

Based on the available information and safety profile of ibuprofen a contributory role cannot be excluded. Allergic reactions are known to occur under treatment with ibuprofen. 18-JAN-2010 Sanofi-Aventis Germany

All reported clinical diagnoses are considered locally labelled for all suspected products and thus consistent with the known benefit risk ratio of the product(s). Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

sanofi-aventis, 17.02.2010 DE-SANOFI-AVENTIS-2009SA005258

Sender diagnosis: not reported

Narrative

Initial report received from the patient herself on 30-Nov-2009. The female patient was prescribed ibuprofen (Ibuflam 600mg) on 19-Nov-2009 for inflammation in her shoulder joint. During treatment with ibuprofen, the patient developed mild swelling in the area of both eyes, on 28-Nov-2009, symptoms worsened slightly, and the patient developed mild swelling of her upper lip. On 29-Nov-2009, swelling in area of eyes and of upper lip intensified, the patient suffered from dry throat. On 30-Nov-2009, the patient woke up with swollen eyes and lips, and an erythema on her upper arms and her back. She visited a physician and was hospitalized with the diagnosis of allergic reaction. The patient did not take any concomitant medication. Further information was not provided.

Addendum received for follow-up on 14-Dec-2009:

This follow-up information was provided from the 59-year-old female patient herself. She reported that she developed an allergic reaction during therapy with ibuprofen. Therefore she was in emergency room. She received cortisone tablets ambulant as corrective treatment.

Addendum received for follow-up on 13-Jan-2010:

This follow-up information was provided from patient's physician who informed us that he was not involved to this case.

Additional information received on 11-Feb-2010:

From the general practitioner we received ADR form and hospital report.

Contrary to the initial report ibuprofen 600 mg was taken twice a day since 19-Nov-2009. Indication: shoulder lesion left.

On 30-Nov-2009 the patient visited to the hospital for examination of her complaints. The according hospital report disclosed Quincke's edema and drug induced generalised exanthema with itching. Therefore the patient's initial report was medically confirmed. Corrective treatment: prednisolone 60 mg given for two days.

Thereafter the patient recovered; however the exact date was unknown as the patient never returned to the hospital or general practitioner. No further information expected.

Sender's comment:

Sanofi-Aventis Company Comment for follow-up dated 15-FEB-2010: New information does not change previous assessment of this case. Based on the available information and safety profile of ibuprofen a contributory role cannot be excluded. Allergic reactions are known to occur under treatment with ibuprofen. 17-FEB-2010 Sanofi-Aventis Germany

All reported clinical diagnoses are considered locally labelled for all suspected products and thus consistent with the known benefit risk ratio of the product(s). Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10064335

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 86 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20081003	deathcause: Stevens-Johnson syndrome
deathdate: 20081003	deathcause: Multiorgan failure

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20080818	20081003	47 Day	22 Day	fatal
2 Stevens-Johnson syndrome	20080818			23 Day	fatal
3 Methicillin-resistant Staphylococcus aureus sepsis					fatal
4 PSA increased					fatal
5 Hypoalbuminemia					fatal
6 Mycoplasma antibody positive	20080917			626 Day	fatal
7 Impaired liver function					fatal
8 Bone marrow toxicity					fatal
9 Condition aggravated	20080921			630 Day	fatal
10 Exsiccosis					fatal
11 Edema of legs					fatal
12 Edema generalized	20080907			616 Day	fatal
13 Systemic inflammatory response syndrome					fatal
14 Vein disorder					fatal
15 Thrombophlebitis	20080818			596 Day	fatal
16 Pneumonia fungal					fatal
17 Bronchopneumonia					fatal
18 Capillary leak syndrome	2008				fatal
19 Condition aggravated	20080921			630 Day	fatal
20 Decompensation cardiac					fatal
21 Edema generalized	20080907			616 Day	fatal
22 Edema of legs					fatal
23 Hypoalbuminemia					fatal
24 Impaired liver function					fatal
25 Lung infiltration	20080919			628 Day	fatal
26 Mucous membrane disorder					fatal
27 Multiorgan failure	2008			642 Day	fatal
28 Pancytopenia	2008				fatal
29 PSA increased					fatal
30 Respiratory insufficiency	20080924			633 Day	fatal
31 Thrombophlebitis	20080818			596 Day	fatal
32 Vein disorder					fatal
33 Systemic inflammatory response syndrome					fatal
34 Bone marrow toxicity					fatal
35 Exsiccosis					fatal
36 Pneumonia fungal					fatal
37 Mycoplasma antibody positive	20080917			626 Day	fatal
38 Renal insufficiency aggravated					fatal
39 Stevens-Johnson syndrome	20080818			23 Day	fatal
40 Multiorgan failure	2008			642 Day	fatal

	DE-BFARM-10064335		
41	Pancytopenia	2008	fatal
42	Capillary leak syndrome	2008	fatal
43	Renal insufficiency aggravated		fatal
44	Decompensation cardiac		fatal
45	Respiratory insufficiency	20080924	633 Day fatal
46	Lung infiltration	20080919	628 Day fatal
47	Bronchopneumonia		fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Acetylsalizylsaeure acetylsalicylic acid	Oral	/	20080727- -	Thrombosis prophylaxis
2 ALLOPURINOL allopurinol	Oral	300 Mg milligram(s)/Day	20080724- -	Hyperuricaemia
3 ass acetylsalicylic acid	Oral	100 Mg milligram(s)/	20080727- -	Thrombosis prophylaxis
4 betaisodona povidone-iodine	Topical	/	20080728- -	
5 CYNT moxonidine	Oral	/	20080728- -	Cardiac insufficiency
6 DETRUSITOL tolterodine l-tartrate	Oral	2 mg/kg milligram(s)/kilogr	20080727- -	Incontinence
7 DEXA POLYSPECTRAN N dexamethasone sodium phosphate; neomycin sulfate; polymyxin b sulfate	Ophthalmic	/	20080724- -	Conjunctivitis
8 FERROUS GLUCONATE ferrous gluconate		/	- -	
9 Folsaeure folic acid	Oral	/	20080727- -	Supplementation therapy
10 ibuprofen ibuprofen	Oral	/ 5	20080727- -	Pain
11 novodigal beta-acetyldigoxin	Oral	0.2 Mg milligram(s)/Day	20080727- -	Cardiac insufficiency
12 TORASEMIDE torasemide	Oral	10 Mg milligram(s)/Day	20070101- -	Hypertension arterial

Drugs (concomitant)

13	MONO-EMBOLEX heparin-fraction, sodium salt	/	20080819-20080820 2 Day	Thrombophlebitis
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Mycoplasma serology positive			Unknown	
Alcohol abuse			Yes	
Nicotine abuse			Yes	
Chronic atrial fibrillation			Yes	
Apoplexy			No	
Cardiac insufficiency			Yes	
Cardiac pacemaker insertion			Yes	
Chronic conjunctivitis			Yes	
Drug allergy			No	
Hypertension arterial			Yes	
Hyperuricaemia			Yes	
Hypoalbuminaemia			Yes	
PSA increased			Yes	
Renal insufficiency			Yes	
Sick sinus syndrome			Yes	
Thrombophlebitis of the leg			Yes	
Exsiccosis			Yes	
Sulfonamide allergy			No	

Primary source

Qualification: Physician observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)Whitehall Much GmbH, 06.05.2010 DE-WYE-G06048110

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735.

A contactable physician reported about an 87 year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27Jul2008 due to incontinence. 600 mg ibuprofen per oral route from 27Jul2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28Jul2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01Jan2007 due to arterial hypertension. 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24Jul2008 due to hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVODIGAL) per oral route from 27Jul2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27Jul2008 for thrombosis prophylaxis, povidone-iodine (BETASODONA ointment) topically from 28Jul2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (Hoercomplex Batch CJU) per oral route from 28Jul2008 for an unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (Folplus STUCOPHARM) per oral route from 27Jul2008 for Substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma an an unknown date. On 18Aug2008 the patient developed spots on skin and Steven-Johnson-Syndrome was diagnosed on the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and on 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN lotion). On 28Aug2008 the patient was hospitalised and redness of eyes was diagnosed. On 01 Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed. It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered on 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causal relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was seen by the project experts.

No follow-up needed. Follow-up provided automatically by dZh.

Sender's comment:

Bekanntes unerwünschtes Ereignis (Stevens-Johnson Syndrome) unter der Anwendung von Ibuprofen.
 Ein Kausalzusammenhang mit der Einnahme von Ibuprofen ist möglich, ist aber aufgrund der Komedikation nicht eindeutig zu beurteilen.
 Basierend auf der aktuellen Information führt dieser Bericht nicht zu einer Änderung des Nutzen-Risiko-Verhältnisses für Ibuprofen.
 Zu diesem Zeitpunkt sind keine weiteren Massnahmen vorgesehen.

Reporter's comment:

PFIZERINC, 11.05.2010 DE-PFIZER INC-2010052756

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735.
 A contactable physician reported about an 87 year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27Jul2008 due to incontinence, 600 mg ibuprofen per oral route from 27Jul2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28Jul2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01Jan2007 due to arterial hypertension, 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24Jul2008 due to hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVODIGAL) per oral route from 27Jul2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27Jul2008 for thrombosis prophylaxis, povidone-iodine (BETAISODONA, ointment) topically from 28Jul2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (HOERCOMPLEX, Batch CJU) per oral route from 28Jul2008 for an unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (FOLPLUX, STUCOPHARM) per oral route from 27Jul2008 for substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma on an unknown date. On 18Aug2008 the patient developed spots on skin and Steven-Johnson-Syndrome was diagnosed on the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and on 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN, lotion). On 28Aug2008 the patient was hospitalised and redness of eyes was diagnosed. On 01Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed. It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered on 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causal relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was seen by the project experts.

This case contains a Wyeth co-suspect product. The Wyeth case may qualify for regulatory reporting for the Wyeth co-suspect product. This case may therefore be a duplicate report.

Pfizer is Marketing Authorization Holder of dexta-polyspectran eye drops in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of Dexta-polyspectran eye drops has submitted the same report to the regulatory authorities.

Sender's comment:

Contribution of tolterodine, ibuprofen, dexta-polyspectran, moxonidine, torasemide, allopurinol, beta-acetyldigoxin, acetylsalicylic acid, povidone-iodine, ferrous gluconate and folic acid to the reported events cannot be excluded. Stevens-Johnson-Syndrome ist in der Produktinformation als mögliche Nebenwirkung nicht beschrieben. Wir sehen zum gegenwärtigen Zeitpunkt keinen Anlass für eine geänderte Bewertung des Nutzen-/Risikoverhältnisses.

Reporter's comment:

PFIZERINC, 17.05.2010 DE-PFIZER INC-2010052756

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735.

A contactable physician reported about an 87 year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27Jul2008 due to incontinence, 600 mg ibuprofen per oral route from 27Jul2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28Jul2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01Jan2007 due to arterial hypertension, 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24Jul2008 due to hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVODIGAL) per oral route from 27Jul2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27Jul2008 for thrombosis prophylaxis, povidone-iodine (BETAISODONA, ointment) topically from 28Jul2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (HOERCOMPLEX, Batch CJU) per oral route from 28Jul2008 for an unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (FOLPLUX, STUCOPHARM) per oral route from 27Jul2008 for substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma on an unknown date. On 18Aug2008 the patient developed spots on skin and Steven-Johnson-Syndrome was diagnosed on the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and on 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN, lotion). On 28Aug2008 the patient was hospitalized and redness of eyes was diagnosed. On 01Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed. It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered on 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causal relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was seen by the project experts.

This case contains a Wyeth co-suspect product. The Wyeth case may qualify for regulatory reporting for the Wyeth co-suspect product. This case may therefore be a duplicate report.

Pfizer is Marketing Authorization Holder of dexa-polyspectran eye drops in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of Dexa-polyspectran eye drops has submitted the same report to the regulatory authorities.

Follow-up (28Apr2010): New information received from License Partner, J & J Pharmaceutical Research & Development Company includes: License partner company's case assessment.

Sender's comment:

J & J Pharmaceutical Research & Development Company Causality Assessment:

The causality of Ibuprofen in relationship with event Death was assessed as possible and with event Stevens-Johnson Syndrome was also assessed as possible.

Contribution of tolterodine, ibuprofen, dexa-polyspectran, moxonidine, torasemide, allopurinol, beta-acetyldigoxin, acetylsalicylic acid, povidone-iodine, ferrous gluconate and folic acid to the reported events cannot be excluded. Stevens-Johnson-Syndrome ist in der Produktinformation als moegliche Nebenwirkung nicht beschrieben. Wir sehen zum gegenwaertigen Zeitpunkt auch nach Erhalt der Follow-up Information keinen Anlass fuer eine geaenderte Bewertung des Nutzen-/Risikoverhaeltnisses.

Reporter's comment:

PFIZERINC, 27.05.2010 DE-PFIZER INC-2010052756

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin

reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735.

A contactable physician reported about an 87 year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27Jul2008 due to incontinence, 600 mg ibuprofen per oral route from 27Jul2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28Jul2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01Jan2007 due to arterial hypertension, 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24Jul2008 due to hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVODIGAL) per oral route from 27Jul2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27Jul2008 for thrombosis prophylaxis, povidone-iodine (BETAISODONA, ointment) topically from 28Jul2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (HOERCOMPLEX, Batch CJU) per oral route from 28Jul2008 for an unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (FOLPLUX, STUCOPHARM) per oral route from 27Jul2008 for substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma on an unknown date. On 18Aug2008 the patient developed spots on skin and Steven-Johnson-Syndrome was diagnosed on the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and on 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN, lotion). On 28Aug2008 the patient was hospitalized and redness of eyes was diagnosed. On 01Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed. It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered on 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causal relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was seen by the project experts.

This case contains a Wyeth co-suspect product. The Wyeth case may qualify for regulatory reporting for the Wyeth co-suspect product. This case may therefore be a duplicate report.

Pfizer is Marketing Authorization Holder of dexamethasone-polyspectran eye drops in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of Dexamethasone-polyspectran eye drops has submitted the same report to the regulatory authorities.

Follow-up (28Apr2010): New information received from License Partner, J & J Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen, includes: License partner company's case assessment.

Follow-up (13May2010):

This is follow-up report received from Johnson & Johnson Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen. New information includes: Additional information now medically confirmed received from a other health professional on 13May2010. The patient died from Stevens-Johnson Syndrome on an unknown date.

Sender's comment:

J & J Pharmaceutical Research & Development Company Causality Assessment:

The causality of Ibuprofen in relationship with event Death was assessed as possible and with event Stevens-Johnson Syndrome was also assessed as possible.

Contribution of tolterodine, ibuprofen, dexamethasone-polyspectran, moxonidine, torasemide, allopurinol, beta-acetyldigoxin, acetylsalicylic acid, povidone-iodine, ferrous gluconate and folic acid to the reported event cannot be excluded. Stevens-Johnson-Syndrom mit toedlichem Ausgang ist in der Produktinformation als moegliche Nebenwirkung nicht beschrieben. Wir sehen zum gegenwaertigen Zeitpunkt keinen Anlass fuer eine geaenderte Bewertung des Nutzen-/Risikoverhaeltnisses. Keine Aenderung der vorherigen Stellungnahme durch die neu hinzugekommenen Informationen.

Reporter's comment:

Whitehall Much GmbH, 29.06.2010 DE-WYE-G06048110

Sender diagnosis: not reported

Narrative

case reversioned - additional data received

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735.

A contactable physician reported about an 87 year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27Jul2008 due to incontinence. 600 mg ibuprofen per oral route from 27Jul2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28Jul2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01Jan2007 due to arterial hypertension. 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24Jul2008 due to hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVIDIGAL) per oral route from 27Jul2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27Jul2008 for thrombosis prophylaxis, povidone-iodine (BETAISODONA ointment) topically from 28Jul2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (Hoercomplex Batch CJU) per oral route from 28Jul2008 for an unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (Folplus STUCOPHARM) per oral route from 27Jul2008 for Substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma an an unknown date. On 18Aug2008 the patient developed spots on skin and Steven-Johnson-Syndrome was diagnosed on the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and an 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN lotion). On 28Aug2008 the patient was hospitalised and redness of eyes was diagnosed. On 01 Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed . It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered on 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causal relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was Seen by the project experts.

This case contains a Wyeth co-suspected product. The Wyeth case may qualify for regulatory reporting for the Wyeth co-suspect product. This case may therefore be a duplicate report.

Pfizer is Marketing Authorization Holder of dexta-polyspectran eye drops in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of dexta-polyspectran eye drops has submitted the same report to the regulatory authorities.

Follow-up (28Apr2010): New Information received from license Partner, J & J Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen, includes: License partner company's case assessment.

Follow-up (13May2010):

This is follow-up report received from Johnson & Johnson Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407637, licence party for Ibuprofen. New Information Includes: Additional Information now medically confirmed received from another health professional on 13May2010. The patient died from Stevens-Johnson Syndrome on an unknown date.

Follow-up (17Jun2010):

The contactable physician reported that according to the patient's carer it was not clear whether the patient took detrusitol, allopurinol, torasemide and digoxin (NOVIDIGAL). The carer stated that the patient only took acetylsalicylic acid (ASS 100 mg). The patient had a medical history of unclear known right-sided focal hepatic lesion (sonography of abdomen on 09Sep2008), condition after apoplexy with aphasia, chronic atrial fibrillation with suspicion of coronary heart disease, differential diagnosis hypertensive cardiac disease with arterial hypertension, condition after implantation of cardiac pacemaker due to sick sinus syndrome, chronic conjunctivitis, thrombophlebitis of right lower leg (on 18Aug2008), renal insufficiency, exsiccosis, hypoalbuminemia left heart insufficiency, condition after alcohol abuse and nicotine abuse, hyperuricemia and Increased prostatic specific antigen (PSA, diagnosed in Nov2006). Further a past drug history of allergy to benzocaine and sulfonamide was reported. The patient was concomitantly treated with infusions of heparin-fraction, sodium salt (MONO-EMBOLEX) on 19Aug2008 and on 20 Aug2008 due to thrombophlebitis of right lower leg. During hospitalisation the patient underwent several examinations. Laboratory investigation from 28Aug2008 yielded the following results: Serum urea 125 mg/dl (normal range 16-55), serum creatine (Jaffe) 2.26 mg/dl (normal range 0.81-1.44), serum aspartate aminotransferase (AST) 75 IU/l (normal range 0-50), Serum alanine aminotransferase (ALT) 52 IU/l (normal range 0-50), serum gamma-glutamyltransferase (GGT) 126 IU/l (normal range 0-60) and a lymphocyte count of 7 % (normal range 20-50). On 01 Sep2008 laboratory investigations showed the following results: Serum urea 140 mg/dl (normal range 18-55), serum creatinine (Jaffe) 1.60 mg/dl (normal range 0.81-1.44), serum AST 69 IU/l (normal range 0-50), serum ALT 101 IU/l (normal range 0-50), serum LDH 433 IU/l (normal range 0-250), serum GGT 140 IU/l (normal range 0-60), a thrombocyte count of 250/nl (normal range 140-440) and a lymphocyte count of 7 % (normal range 20-50). On 03Sep2008 laboratory investigations showed the following results: Serum urea 107 mg/dl (normal range 18-55), Serum creatinine (Jaffe) 1.51 mg/dl (normal range 0.81-1.44), serum AST 47 IU/l (normal range 0-50), serum ALT 113 IU/l (normal range 0-50), serum LDH 384 IU/l (normal range 0-250) and serum GGT 171 IU/l (normal range 0-60). On 15Sep2008 laboratory investigations showed the following results: Serum urea 98 mg/dl (normal range 18-55), serum creatinine (Jaffe) 1.56 mg/dl (normal range 0.81-1.44), serum AST 191UA (normal range 0-50), serum ALT 32 IU/l (normal range 0-50), serum LDH 263 IU/l (normal range 0-250), serum GGT 183 IU/l (normal range 0-60) and a thrombocyte count of 60/nl (normal range 140-440). On 17Sep2008 laboratory investigations showed the following results: Serum creatinine (Jaffe) 1.8 mg/dl (normal range 0.81-1.44), serum AST 19 IU/l (normal range 0-50) and serum ALT 32 IU/l (normal range 0-50), The results from that date for serum urea, serum LDH, serum GGT and thrombocyte count was illegible in the source docs. In Nov2006 the patient had been diagnosed with prostatic specific antigen (PSA) level of 13 ng/ml. In Aug2008 or Sep2006 during hospital stay a PSA value of 17 ng/ml was detected. During stay in the dermatologic ward (admission was on 28Aug2008 as previously reported) the patient was treated

with 80 mg prednisolone per oral route and with 40 mg pantoprazole for gastric protection. The patient's body was topically treated with dexamethasone sodium phosphate (BETNESOL V) and the face was treated with methylprednisolone aceponate (ADVANTAN). The eyes were treated with topically 4 times daily ofloxacin (FLOXAN) ointment and oral hygiene was performed. Increased creatinine of 2.26 mg/dl was successfully treated with intravenous volume substitution. Histology of back (as reported) was performed on 02 Sep 2008 showing multiform drug reaction. Direct immunofluorescence (DIF) from 02 Sep 2008 was negative. On 07 Sep 2008 the patient received 20 mg intravenous furosemide (LASIX) due to massive edema. Edema of legs reaching up to the scrotum were then (no date provided) treated with 25 mg hydrochlorothiazide (ESIDRIX) per oral route. Hypoalbuminemia was treated with human albumin 10% for 3 days. Increased C-reactive protein was diagnosed on 12 Sep 2008 and treated from that day onwards for 7 days with intravenous ceftriaxone sodium (ROCEPHIN). The cause for increasing leukocytopenia and thrombocytopenia was unclear (the patient was not treated with heparin at that time). An internist was consulted and the most probable cause for leukocytopenia and thrombocytopenia was seen in the patient's skin or immune reaction. On 17 Sep 2008 the patient was diagnosed with increased level of Mycoplasma antibodies in serum, and therefore underwent treatment with once daily 300 mg oral roxithromycin (RULID). The reporting physician stated that Mycoplasma is a known trigger of Stevens-Johnson syndrome. On 19 Sep 2008 the patient was switched to intravenous parenteral nutrition. The patient had mucosal lesions and a bad venous status due to edema. On 19 Sep 2008 x-ray of chest was performed revealing extensive infiltrations. After repeated consultation of the internist the patient received cilastatin sodium imipenem (ZIENAM), flucanazole (DIFLUCAN) and erythromycin. The patient was isolated due to leukocyte value of 1.7/nl and was treated with thrombocyte concentrate due to a thrombocyte count of 141/nl. On 21 Sep 2008 the patient was transferred from the dermatologic ward to the intensive care unit due to increased worsening of the patient's general condition and respiratory insufficiency with bronchopneumonia and fungal pneumonia as a consequence of immunosuppression under cortisone (systemic inflammatory response syndrome, SIRS) and a possible cytotoxic bone marrow damage due to drug-induced (furosemide and allopurinol suspected) allergic reaction. The patient had peripheral exsiccosis, signs of capillary leak syndrome and no peripheral edema. Pancytopenia with anemia, neutropenia and thrombocytopenia was treated with red blood cell concentrate, thrombocyte concentrate and stimulation of leukopoiesis with GRANOZYTE (nos). Symptoms of cardiac decompensation were treated with diuretic therapy (furosemide). From 24 Sep 2008 onwards the patient developed sepsis and respiratory insufficiency so that intubation, artificial ventilation and catecholamines were required. The patient was isolated due to infection with multiresistant *Streptococcus aureus* (MRSA). In the further course the patient developed worsening renal insufficiency, impaired liver function, changes in bronchial system (both sides) with severe inflammatory structural changes and suspicion of confluent fungal infection. Microbiological investigation revealed infection with mold-like fungus so that an antimycotic therapy with VOICONAZOL (nos) was started. At that time increased doses of catecholamines were required and the patient developed complete anuria. On 03 Oct 2008 at 12:25 hours the patient died due to multiorgan failure. The reporting physician did not provide a causality assessment of the additionally reported events with fatal outcome. An autopsy report was not available to the reporter.

Case Comment J & J Pharmaceutical Research & Development Company Causality Assessment: The causality of Ibuprofen in relationship with event Death was assessed as possible and with event Stevens-Johnson Syndrome was also assessed as possible.

Contribution of tolterodine, ibuprofen, dexamethasone, moxonidine, torasemide, allopurinol, beta-acetyldigoxin, acetylsalicylic acid, povidone-iodine, ferrous gluconate and folic acid to the reported event cannot be excluded.

Sender's comment:

Bekanntes unerwünschtes Ereignis (Stevens-Johnson Syndrome) unter der Anwendung von Ibuprofen.
Ein Kausalzusammenhang mit der Einnahme von Ibuprofen ist möglich, ist aber aufgrund der Komedikation nicht eindeutig zu beurteilen.
Basierend auf der aktuellen Information führt dieser Bericht nicht zu einer Änderung des Nutzen-Risiko-Verhältnisses für Ibuprofen.
Zu diesem Zeitpunkt sind keine weiteren Massnahmen vorgesehen.

Reporter's comment:

PFIZERINC, 30.06.2010 DE-PFIZER INC-2010052756

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735.

A contactable physician reported about an 87 year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27 Jul 2008 due to incontinence, 600 mg ibuprofen per oral route from 27 Jul 2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28 Jul 2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01 Jan 2007 due to arterial hypertension, 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24 Jul 2008 due to hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVODIGAL) per oral route from 27 Jul 2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27 Jul 2008 for thrombosis prophylaxis, povidone-iodine (BETAISODONA, ointment) topically from 28 Jul 2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (HOERCOMPLEX, Batch CJU) per oral route from 28 Jul 2008 for an

unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (FOLPLUX, STUCOPHARM) per oral route from 27Jul2008 for substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma on an unknown date. On 18Aug2008 the patient developed spots on skin and Steven-Johnson-Syndrome was diagnosed on the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and on 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN, lotion). On 28Aug2008 the patient was hospitalized and redness of eyes was diagnosed. On 01Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed. It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered on 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causal relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was seen by the project experts.

This case contains a Wyeth co-suspect product. The Wyeth case may qualify for regulatory reporting for the Wyeth co-suspect product. This case may therefore be a duplicate report.

Pfizer is Marketing Authorization Holder of dexamethasone sodium phosphate eye drops in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of Dexamethasone sodium phosphate eye drops has submitted the same report to the regulatory authorities.

Follow-up (28Apr2010): New information received from License Partner, J & J Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen, includes: License partner company's case assessment.

Follow-up (13May2010):

This is follow-up report received from Johnson & Johnson Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen. New information includes: Additional information now medically confirmed received from a other health professional on 13May2010. The patient died from Stevens-Johnson Syndrome on an unknown date.

Follow-up (17Jun2010):

This is a follow-up report received from the contactable physician. The physician reported that according to the patient's carer it was not clear whether the patient took tolterodine, allopurinol, torasemide and digoxin (NOVIDIGAL). The carer stated that the patient only took acetylsalicylic acid (ASS, 100 mg). The patient had a medical history of unclear known right-sided focal hepatic lesion (sonography of abdomen on 09Sep2008), condition after apoplexy with aphasia, chronic atrial fibrillation with suspicion of coronary heart disease, differential diagnosis hypertensive cardiac disease with arterial hypertension, condition after implantation of cardiac pacemaker due to sick sinus syndrome, chronic conjunctivitis, thrombophlebitis of right lower leg (on 18Aug2008), renal insufficiency, exsiccosis, hypoalbuminemia, left heart insufficiency, condition after alcohol abuse and nicotine abuse, hyperuricemia and increased prostatic specific antigen (PSA, diagnosed in Nov2006). Further a past drug history of allergy to benzocaine and sulfonamide was reported. The patient was concomitantly treated with injections of heparin-fraction, sodium salt (MONO-EMBOLEX) on 19Aug2008 and on 20 Aug2008 due to thrombophlebitis of right lower leg. During hospitalisation the patient underwent several examinations. Laboratory investigation results on 28Aug2008 included the following: Serum urea 125 mg/dl (normal range 18-55), serum creatinine (Jaff) 2.26 mg/dl (normal range 0.81-1.44), serum aspartate aminotransferase (AST) 75 IU/l (normal range 0-50), serum alanine aminotransferase (ALT) 52 IU/l (normal range 0-50), serum gamma-glutamyltransferase (GGT) 126 IU/l (normal range 0-60) and a lymphocyte count of 7 % (normal range 20-50). On 01Sep2008 laboratory investigations showed the following results: Serum urea 140 mg/dl, serum creatinine (Jaff) 1.80 mg/dl, serum AST 69 IU/l, serum ALT 101 IU/l, serum LDH 433 IU/l (normal range 0-250), serum GGT 140 IU/l, a thrombocyte count of 250/nl (normal range 140-440) and a lymphocyte count of 7 %. On 03Sep2008 laboratory investigations showed the following results: Serum urea 107 mg/dl, serum creatinine (Jaff) 1.51 mg/dl, serum AST 47 IU/l, serum ALT 113 IU/l, serum LDH 384 IU/l and serum GGT 171 IU/l. On 15Sep2008 laboratory investigations showed the following results: Serum urea 98 mg/dl, serum creatinine (Jaff) 1.56 mg/dl, serum AST 19 IU/l, serum ALT 32 IU/l, serum LDH 263 IU/l, serum GGT 183 IU/l and a thrombocyte count of 60/nl. On 17Sep2008 laboratory investigations showed the following results: Serum creatinine (Jaff) 1.8 mg/dl, serum AST 19 IU/l and serum ALT 32 IU/l. The results from that date for serum urea, serum LDH, serum GGT and thrombocyte count were illegible in the source docs. In Nov2006 the patient had been diagnosed with prostatic specific antigen (PSA) level of 13 ng/ml. In Aug2008 or Sep2008 during hospital stay a PSA value of 17 ng/ml was detected. During stay in the dermatologic ward (admission was on 28Aug2008 as previously reported) the patient was treated with 80 mg prednisolone per oral route and with 40 mg pantoprazole for gastric protection. The patient's body was topically treated with bexmethasone sodium phosphate (BETNESOL V) and the face was treated with methylprednisolone aceponate (ADVANTAN). The eyes were treated with topically 4 times daily ofloxacin (FLOXAN) ointment and oral hygiene was performed. Increased creatinine of 2.26 mg/dl was successfully treated with intravenous volume substitution. Histology of back (as reported) was performed on 02 Sep2008 showing multi-form drug reaction. Direct immunofluorescence (DIF) from 02Sep2008 was negative. On 07Sep2008 the patient received 20 mg intravenous furosemide (LASIX) due to massive edema. Edema of legs reaching up to the scrotum were then (no date provided) treated with 25 mg hydrochlorothiazide (ESIDRIX) per oral route. Hypoalbuminemia was treated with human albumin 10% for 3 days. Increased C-reactive protein was diagnosed on 12Sep2008 and treated from that day onwards for 7 days with intravenous ceftriaxone sodium (ROCEPHIN). The cause for increasing leukocytopenia and thrombocytopenia was unclear (the patient was not treated with heparin at that time). An internist was consulted and the most probable cause for leukocytopenia and thrombocytopenia was seen in the patient's skin or immune reaction. On 17Sep2008 the patient was diagnosed with increased level of Mycoplasma antibodies in serum, and therefore underwent treatment with once daily 300 mg oral roxithromycin (RULID). The reporting physician stated that Mycoplasma is a known trigger of Stevens-Johnson syndrome. On 19Sep2008 the patient was switched to intravenous parenteral nutrition. The patient had mucosal lesions and a bad venous status due to edema. On 19Sep2008 x-ray of chest was performed revealing extensive infiltrations. After repeated consultation of the internist

the patient received cilastatin sodium imipenem (ZIENAM), fluconazole (DIFLUCAN) and erythromycin. The patient was isolated due to leukocyte value of 1.7/nl and was treated with thrombocyte concentrate due to a thrombocyte count of 14/nl. On 21Sep2008 the patient was transferred from the dermatologic ward to the intensive care unit due to increased worsening of the patient's general condition and respiratory insufficiency with bronchopneumonia and fungal pneumonia as a consequence of immunosuppression under cortisone (systemic inflammatory response syndrome, SIRS) and a possible cytotoxic bone marrow damage due to drug-induced (furosemide and allopurinol suspected) allergic reaction. The patient had peripheral exsiccosis, signs of capillary leak syndrome and no peripheral edema. Pancytopenia with anemia, neutropenia and thrombocytopenia was treated with red blood cell concentrate, thrombocyte concentrate and stimulation of leukopoiesis with GRANOZYTE. Symptoms of cardiac decompensation were treated with diuretic therapy (etacrynic acid). From 24Sep2008 onwards the patient developed sepsis and respiratory insufficiency so that intubation, artificial ventilation and catecholamines were required. The patient was isolated due to infection with multiresistant *Streptococcus* "aureus". In the further course the patient developed worsening renal insufficiency, impaired liver function, changes in bronchial system (both sides) with severe inflammatory structural changes and suspicion of confluent fungal infection. Microbiological investigation revealed infection with mold-like fungus so that an antimycotic therapy with VOICONAZOL was started. At that time increased doses of catecholamines were required and the patient developed complete anuria. On 03Oct2008 at 12:25 hours the patient died due to multiorgan failure. The reporting physician did not provide a causality assessment of the additionally reported events pancytopenia and capillary leak syndrome. An autopsy report was not available to the reporter.

Sender's comment:

J & J Pharmaceutical Research & Development Company Causality Assessment:

The causality of Ibuprofen in relationship with event Death was assessed as possible and with event Stevens-Johnson Syndrome was also assessed as possible.

Based on follow up information received, contribution of tolterodine, ibuprofen, dexamethasone, moxonidine, torasemide, allopurinol, beta-acetyldigoxin, acetylsalicylic acid, povidone-iodine, ferrous gluconate and folic acid to the reported events cannot be excluded. Alternatively, these events find root in the patient's medical history of various heart disorder, thrombophlebitis, renal insufficiency, hepatic lesions and prostate disorder. Stevens-Johnson-Syndrom, Pancytopenie, Capillary-Leak-Syndrom und Multiorganversagen sind in der Produktinformation als mögliche Nebenwirkung nicht beschrieben. Wir sehen zum gegenwärtigen Zeitpunkt keinen Anlass für eine geänderte Bewertung des Nutzen-/Risikoverhältnisses. Keine Änderung der vorherigen Stellungnahme durch die neu hinzugekommenen Informationen.

Reporter's comment:

PFIZERINC, 02.07.2010 DE-PFIZER INC-2010052756

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735. A contactable physician reported about an 87 year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27Jul2008 due to incontinence, 600 mg ibuprofen per oral route from 27Jul2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28Jul2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01Jan2007 due to arterial hypertension, 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24Jul2008 due to hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVODIGAL) per oral route from 27Jul2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27Jul2008 for thrombosis prophylaxis, povidone-iodine (BETAISODONA, ointment) topically from 28Jul2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (HOERCOMPLEX, Batch CJU) per oral route from 28Jul2008 for an unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (FOLPLUX, STUCOPHARM) per oral route from 27Jul2008 for substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma on an unknown date. On 18Aug2008 the patient developed spots on skin and Steven-Johnson-Syndrome was diagnosed on the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and on 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN, lotion). On 28Aug2008 the patient was hospitalized and redness of eyes was diagnosed. On 01Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed. It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered on 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causal relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive

serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was seen by the project experts.

This case contains a Wyeth co-suspect product. The Wyeth case may qualify for regulatory reporting for the Wyeth co-suspect product. This case may therefore be a duplicate report.

Pfizer is Marketing Authorization Holder of dexta-polyspectran eye drops in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of Dexta-polyspectran eye drops has submitted the same report to the regulatory authorities.

Follow-up (28Apr2010): New information received from License Partner, J & J Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen, includes: License partner company's case assessment.

Follow-up (13May2010):

This is follow-up report received from Johnson & Johnson Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen. New information includes: Additional information now medically confirmed received from other health professional on 13May2010. The patient died from Stevens-Johnson Syndrome on an unknown date.

Follow-up (17Jun2010):

This is a follow-up report received from the contactable physician. The physician reported that according to the patient's carer it was not clear whether the patient took tolterodine, allopurinol, torasemide and digoxin (NOVIDIGAL). The carer stated that the patient only took acetylsalicylic acid (ASS, 100 mg). The patient had a medical history of unclear known right-sided focal hepatic lesion (sonography of abdomen on 09Sep2008), condition after apoplexy with aphasia, chronic atrial fibrillation with suspicion of coronary heart disease, differential diagnosis hypertensive cardiac disease with arterial hypertension, condition after implantation of cardiac pacemaker due to sick sinus syndrome, chronic conjunctivitis, thrombophlebitis of right lower leg (on 18Aug2008), renal insufficiency, exsiccosis, hypoalbuminemia, left heart insufficiency, condition after alcohol abuse and nicotine abuse, hyperuricemia and increased prostatic specific antigen (PSA, diagnosed in Nov2006). Further a past drug history of allergy to benzocaine and sulfonamide was reported. The patient was concomitantly treated with injections of heparin-fraction, sodium salt (MONO-EMBOLEX) on 19Aug2008 and on 20 Aug2008 due to thrombophlebitis of right lower leg. During hospitalisation the patient underwent several examinations. Laboratory investigation results on 28Aug2008 included the following: Serum urea 125 mg/dl (normal range 18-55), serum creatinine (Jaff) 2.26 mg/dl (normal range 0.81-1.44), serum aspartate aminotransferase (AST) 75 IU/l (normal range 0-50), serum alanine aminotransferase (ALT) 52 IU/l (normal range 0-50), serum gamma-glutamyltransferase (GGT) 126 IU/l (normal range 0-60) and a lymphocyte count of 7 % (normal range 20-50). On 01Sep2008 laboratory investigations showed the following results: Serum urea 140 mg/dl, serum creatinine (Jaff) 1.80 mg/dl, serum AST 69 IU/l, serum ALT 101 IU/l, serum LDH 433 IU/l (normal range 0-250), serum GGT 140 IU/l, a thrombocyte count of 250/nl (normal range 140-440) and a lymphocyte count of 7 %. On 03Sep2008 laboratory investigations showed the following results: Serum urea 107 mg/dl, serum creatinine (Jaff) 1.51 mg/dl, serum AST 47 IU/l, serum ALT 113 IU/l, serum LDH 384 IU/l and serum GGT 171 IU/l. On 15Sep2008 laboratory investigations showed the following results: Serum urea 98 mg/dl, serum creatinine (Jaff) 1.56 mg/dl, serum AST 19 IU/l, serum ALT 32 IU/l, serum LDH 263 IU/l, serum GGT 183 IU/l and a thrombocyte count of 60/nl. On 17Sep2008 laboratory investigations showed the following results: Serum creatinine (Jaff) 1.8 mg/dl, serum AST 19 IU/l and serum ALT 32 IU/l. The results from that date for serum urea, serum LDH, serum GGT and thrombocyte count were illegible in the source docs. In Nov2006 the patient had been diagnosed with prostatic specific antigen (PSA) level of 13 ng/ml. In Aug2008 or Sep2008 during hospital stay a PSA value of 17 ng/ml was detected. During stay in the dermatologic ward (admission was on 28Aug2008 as previously reported) the patient was treated with 80 mg prednisolone per oral route and with 40 mg pantoprazole for gastric protection. The patient's body was topically treated with bexamethasone sodium phosphate (BETNESOL V) and the face was treated with methylprednisolone aceponate (ADVANTAN). The eyes were treated with topically 4 times daily ofloxacin (FLOXAN) ointment and oral hygiene was performed. Increased creatinine of 2.26 mg/dl was successfully treated with intravenous volume substitution. Histology of back (as reported) was performed on 02 Sep2008 showing multiform drug reaction. Direct immunofluorescence (DIF) from 02Sep2008 was negative. On 07Sep2008 the patient received 20 mg intravenous furosemide (LASIX) due to massive edema. Edema of legs reaching up to the scrotum were then (no date provided) treated with 25 mg hydrochlorothiazide (ESIDRIX) per oral route. Hypoalbuminemia was treated with human albumin 10% for 3 days. Increased C-reactive protein was diagnosed on 12Sep2008 and treated from that day onwards for 7 days with intravenous ceftriaxone sodium (ROCEPHIN). The cause for increasing leukocytopenia and thrombocytopenia was unclear (the patient was not treated with heparin at that time). An internist was consulted and the most probable cause for leukocytopenia and thrombocytopenia was seen in the patient's skin or immune reaction. On 17Sep2008 the patient was diagnosed with increased level of Mycoplasma antibodies in serum, and therefore underwent treatment with once daily 300 mg oral roxithromycin (RULID). The reporting physician stated that Mycoplasma is a known trigger of Stevens-Johnson syndrome. On 19Sep2008 the patient was switched to intravenous parenteral nutrition. The patient had mucosal lesions and a bad venous status due to edema. On 19Sep2008 x-ray of chest was performed revealing extensive infiltrations. After repeated consultation of the internist the patient received cilastatin sodium imipenem (ZIENAM), fluconazole (DIFLUCAN) and erythromycin. The patient was isolated due to leukocyte value of 1.7/nl and was treated with thrombocyte concentrate due to a thrombocyte count of 14/nl. On 21Sep2008 the patient was transferred from the dermatologic ward to the intensive care unit due to increased worsening of the patient's general condition and respiratory insufficiency with bronchopneumonia and fungal pneumonia as a consequence of immunosuppression under cortisone (systemic inflammatory response syndrome, SIRS) and a possible cytotoxic bone marrow damage due to drug-induced (furosemide and allopurinol suspected) allergic reaction. The patient had peripheral exsiccosis, signs of capillary leak syndrome and no peripheral edema. Pancytopenia with anemia, neutropenia and thrombocytopenia was treated with red blood cell concentrate, thrombocyte concentrate and stimulation of leukopoiesis with GRANOZYTE. Symptoms of cardiac decompensation were treated with diuretic therapy (ethacrynic acid). From 24Sep2008 onwards the patient developed sepsis and respiratory insufficiency so that intubation, artificial ventilation and catecholamines were required. The patient was isolated due to infection with multiresistant *Streptococcus aureus*. In the further course the patient developed worsening renal insufficiency, impaired liver function, changes in bronchial system (both sides) with severe inflammatory structural changes and suspicion of confluent fungal infection. Microbiological investigation revealed infection with mold-like fungus so that an antimycotic therapy with VOICONAZOL was started. At that time increased doses of catecholamines were required and the patient developed complete anuria. On 03Oct2008 at 12:25 hours the patient died due to multiorgan failure. The reporting physician did not

provide a causality assessment of the additionally reported events pancytopenia and capillary leak syndrome. An autopsy report was not available to the reporter.

Follow-up (17Jun2010): This follow-up report is being submitted to amend previously reported information to correct the isolated bacteria from *Streptococcus aureus* to *Staphylococcus aureus*.

Follow-up (25Jun2010): This is follow-up report received from Johnson & Johnson Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen. The causality with ibuprofen was assessed as possible for the events of Stevens-Johnson syndrome, multi-organ failure and pancytopenia by Licensed Partner. The causality for the remaining suspect drugs was stated as unknown.

Sender's comment:

J & J Pharmaceutical Research & Development Company Causality Assessment: The causality of Ibuprofen in relationship with event Death was assessed as possible. The causality with Ibuprofen was assessed as possible for the events Stevens-Johnson Syndrome, multi-organ failure and pancytopenia.

Based on follow up information received, contribution of tolterodine, ibuprofen, dexta-polyspectran, moxonidine, torasemide, allopurinol, beta-acetyldigoxin, acetylsalicylic acid, povidone-iodine, ferrous gluconate and folic acid to the reported events cannot be excluded. Alternatively, these events find root in the patient's medical history of various heart disorder, thrombophlebitis, renal insufficiency, hepatic lesions and prostate disorder. Stevens-Johnson-Syndrom, Pancytopenie, Capillary-Leak-Syndrom, SIRS, Pneumonie, Thrombophlebitis, erhöhte Leberwerte, Hypoalbuminaemie, PSA Erhöhung und Multiorganversagen (auch einzelner Organe) sind in der Produktinformation nicht beschrieben. Zu diesem Zeitpunkt gibt es keinen Anlass fuer eine geaenderte des Nutzen-/Risiko-Bewertung. Bei Oedemen handelt es sich um eine bekannte, in der Produktinformation ausreichend praezise deklarierte Nebenwirkung.

Reporter's comment:

PFIZERINC, 14.01.2011 DE-PFIZER INC-2010052756

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735.

A contactable physician reported about an 87-year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27Jul2008 due to incontinence, 600 mg ibuprofen per oral route from 27Jul2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28Jul2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01Jan2007 due to arterial hypertension, 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24Jul2008 due to hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVODIGAL) per oral route from 27Jul2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27Jul2008 for thrombosis prophylaxis, povidone-iodine (BETAISODONA, ointment) topically from 28Jul2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (HOERCOMPLEX, Batch CJU) per oral route from 28Jul2008 for an unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (FOLPLUX, STUCOPHARM) per oral route from 27Jul2008 for substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma on an unknown date. On 18Aug2008 the patient developed spots on skin and Steven-Johnson-Syndrome was diagnosed on the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and on 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN, lotion). On 28Aug2008 the patient was hospitalized and redness of eyes was diagnosed. On 01Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed. It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered on 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causal relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was seen by the project experts.

This case contains a Wyeth co-suspect product. The Wyeth case may qualify for regulatory reporting for the Wyeth co-suspect product. This case may therefore be a duplicate report.

Pfizer is Marketing Authorization Holder of dexta-polyspectran eye drops in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of Dexta-polyspectran eye drops has submitted the same report to the regulatory authorities.

Follow-up (28Apr2010): New information received from License Partner, J & J Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen, includes: License partner company's case assessment.

Follow-up (13May2010):

This is follow-up report received from Johnson & Johnson Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen. New information includes: Additional information now medically confirmed received from other health professional on 13May2010. The patient died from Stevens-Johnson Syndrome on an unknown date.

Follow-up (17Jun2010):

This is a follow-up report received from the contactable physician. The physician reported that according to the patient's carer it was not clear whether the patient took tolterodine, allopurinol, torasemide and digoxin (NOVIDIGAL). The carer stated that the patient only took acetylsalicylic acid (ASS, 100 mg). The patient had a medical history of unclear known right-sided focal hepatic lesion (sonography of abdomen on 09Sep2008), condition after apoplexy with aphasia, chronic atrial fibrillation with suspicion of coronary heart disease, differential diagnosis hypertensive cardiac disease with arterial hypertension, condition after implantation of cardiac pacemaker due to sick sinus syndrome, chronic conjunctivitis, thrombophlebitis of right lower leg (on 18Aug2008), renal insufficiency, exsiccosis, hypoalbuminemia, left heart insufficiency, condition after alcohol abuse and nicotine abuse, hyperuricemia and increased prostatic specific antigen (PSA, diagnosed in Nov2006). Further a past drug history of allergy to benzocaine and sulfonamide was reported. The patient was concomitantly treated with injections of heparin-fraction, sodium salt (MONO-EMBOLEX) on 19Aug2008 and on 20 Aug2008 due to thrombophlebitis of right lower leg. During hospitalisation the patient underwent several examinations. Laboratory investigation results on 28Aug2008 included the following: Serum urea 125 mg/dl (normal range 18-55), serum creatinine (Jaff) 2.26 mg/dl (normal range 0.81-1.44), serum aspartate aminotransferase (AST) 75 IU/l (normal range 0-50), serum alanine aminotransferase (ALT) 52 IU/l (normal range 0-50), serum gamma-glutamyltransferase (GGT) 126 IU/l (normal range 0-60) and a lymphocyte count of 7 % (normal range 20-50). On 01Sep2008 laboratory investigations showed the following results: Serum urea 140 mg/dl, serum creatinine (Jaff) 1.80 mg/dl, serum AST 69 IU/l, serum ALT 101 IU/l, serum LDH 433 IU/l (normal range 0-250), serum GGT 140 IU/l, a thrombocyte count of 250/nl (normal range 140-440) and a lymphocyte count of 7 %. On 03Sep2008 laboratory investigations showed the following results: Serum urea 107 mg/dl, serum creatinine (Jaff) 1.51 mg/dl, serum AST 47 IU/l, serum ALT 113 IU/l, serum LDH 384 IU/l and serum GGT 171 IU/l. On 15Sep2008 laboratory investigations showed the following results: Serum urea 98 mg/dl, serum creatinine (Jaff) 1.56 mg/dl, serum AST 19 IU/l, serum ALT 32 IU/l, serum LDH 263 IU/l, serum GGT 183 IU/l and a thrombocyte count of 60/nl. On 17Sep2008 laboratory investigations showed the following results: Serum creatinine (Jaff) 1.8 mg/dl, serum AST 19 IU/l and serum ALT 32 IU/l. The results from that date for serum urea, serum LDH, serum GGT and thrombocyte count were illegible in the source docs. In Nov2006 the patient had been diagnosed with prostatic specific antigen (PSA) level of 13 ng/ml. In Aug2008 or Sep2008 during hospital stay a PSA value of 17 ng/ml was detected. During stay in the dermatologic ward (admission was on 28Aug2008 as previously reported) the patient was treated with 80 mg prednisolone per oral route and with 40 mg pantoprazole for gastric protection. The patient's body was topically treated with bexamethasone sodium phosphate (BETNESOL V) and the face was treated with methylprednisolone aceponate (ADVANTAN). The eyes were treated with topically 4 times daily ofloxacin (FLOXAN) ointment and oral hygiene was performed. Increased creatinine of 2.26 mg/dl was successfully treated with intravenous volume substitution. Histology of back (as reported) was performed on 02 Sep2008 showingmultiform drug reaction. Direct immunofluorescence (DIF) from 02Sep2008 was negative. On 07Sep2008 the patient received 20 mg intravenous furosemide (LASIX) due to massive edema. Edema of legs reaching up to the scrotum were then (no date provided) treated with 25 mg hydrochlorothiazide (ESIDRIX) per oral route. Hypoalbuminemia was treated with human albumin 10% for 3 days. Increased C-reactive protein was diagnosed on 12Sep2008 and treated from that day onwards for 7 days with intravenous ceftriaxone sodium (ROCEPHIN). The cause for increasing leukocytopenia and thrombocytopenia was unclear (the patient was not treated with heparin at that time). An internist was consulted and the most probable cause for leukocytopenia and thrombocytopenia was seen in the patient's skin or immune reaction. On 17Sep2008 the patient was diagnosed with increased level of Mycoplasma antibodies in serum, and therefore underwent treatment with once daily 300 mg oral roxithromycin (RULID). The reporting physician stated that Mycoplasma is a known trigger of Stevens-Johnson syndrome. On 19Sep2008 the patient was switched to intravenous parenteral nutrition. The patient had mucosal lesions and a bad venous status due to edema. On 19Sep2008 x-ray of chest was performed revealing extensive infiltrations. After repeated consultation of the internist the patient received cilastatin sodium imipenem (ZIENAM), fluconazole (DIFLUCAN) and erythromycin. The patient was isolated due to leukocyte value of 1.7/nl and was treated with thrombocyte concentrate due to a thrombocyte count of 14/nl. On 21Sep2008 the patient was transferred from the dermatologic ward to the intensive care unit due to increased worsening of the patient's general condition and respiratory insufficiency with bronchopneumonia and fungal pneumonia as a consequence of immunosuppression under cortisone (systemic inflammatory response syndrome, SIRS) and a possible cytotoxic bone marrow damage due to drug-induced (furosemide and allopurinol suspected) allergic reaction. The patient had peripheral exsiccosis, signs of capillary leak syndrome and no peripheral edema. Pancytopenia with anemia, neutropenia and thrombocytopenia was treated with red blood cell concentrate, thrombocyte concentrate and stimulation of leukopoiesis with GRANOZYTE. Symptoms of cardiac decompensation were treated with diuretic therapy (ethacrynic acid). From 24Sep2008 onwards the patient developed sepsis and respiratory insufficiency so that intubation, artificial ventilation and catecholamines were required. The patient was isolated due to infection with multiresistant *Streptococcus aureus*. In the further course the patient developed worsening renal insufficiency, impaired liver function, changes in bronchial system (both sides) with severe inflammatory structural changes and suspicion of confluent fungal infection. Microbiological investigation revealed infection with mold-like fungus so that an antimycotic therapy with VOICONAZOL was started. At that time increased doses of catecholamines were required and the patient developed complete anuria. On 03Oct2008 at 12:25 hours the patient died due to multiorgan failure. The reporting physician did not provide a causality assessment of the additionally reported events pancytopenia and capillary leak syndrome. An autopsy report was not available to the reporter.

Follow-up (17Jun2010): This follow-up report is being submitted to amend previously reported information to correct the isolated bacteria from *Streptococcus aureus* to *Staphylococcus aureus*.

Follow-up (25Jun2010): This is follow-up report received from Johnson & Johnson Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen. The causality with ibuprofen was assessed as possible for the events of Stevens-Johnson syndrome, multi-organ failure and pancytopenia by Licensed Partner. The causality for the remaining suspect drugs was stated as unknown.

Follow-up (04Jan2011): Case 2010052756 and case DE-WYE-G06048110 are found to be duplicates resulting from case sharing between Pfizer and Wyeth after the merger in Oct2009. This is a follow-up report combining information from duplicate reports 2010052756 and DE-WYE-G06048110. The current and all subsequent follow-up information will be reported under 2010052756.

Sender's comment:

J & J Pharmaceutical Research & Development Company Causality Assessment: The causality of Ibuprofen in relationship with event Death was assessed as possible. The causality with Ibuprofen was assessed as possible for the events Stevens-Johnson Syndrome, multi-organ failure and pancytopenia.

Based on follow up information received, contribution of tolterodine, ibuprofen, dexta-polyspectran, moxonidine, torasemide, allopurinol, beta-acetyldigoxin, acetylsalicylic acid, povidone-iodine, ferrous gluconate and folic acid to the reported events cannot be excluded. Alternatively, these events find root in the patient's medical history of various heart disorder, thrombophlebitis, renal insufficiency, hepatic lesions and prostate disorder. The follow-up information received does not alter the previous company clinical evaluation. Stevens-Johnson-Syndrom, Pancytopenie, Capillary-Leak-Syndrom, SIRS, Pneumonie, Thrombophlebitis, erhöhte Leberwerte, Hypoalbuminaemie, PSA Erhöhung und Multiorganversagen (auch einzelner Organe) sind in der Produktinformation nicht beschrieben. Zu diesem Zeitpunkt gibt es keinen Anlass fuer eine geaenderte des Nutzen-/Risiko-Bewertung. Bei Oedemen handelt es sich um eine bekannte, in der Produktinformation ausreichend praezise deklarierte Nebenwirkung.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10076940

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 54 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20090610	20090703	24 Day	9 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 APIDRA insulin glulisine	Subcutaneous	/	20090430-	Diabetes mellitus
2 CARBOSTESIN "ASTRA" bupivacaine hydrochloride		.25 % percent/	20090609-	Analgesia
3 clonidine clonidine	Intravenous (not otherwis	150 µg microgram(s)/Day	20090609-20090609	Pain
4 DIPIDOLOR piritramide	Intravenous (not otherwis	5 Mg milligram(s)/	20090609-20090609	Pain
5 HEPARIN heparin	Intravenous (not otherwis	5000 lu international unit(s)	20090609-20090610	Thrombosis prophylaxis
6 ibuprofen ibuprofen	Oral	1200 Mg milligram(s)/Day	20090528-20090601	
7 ibuprofen ibuprofen	Oral	1200 Mg milligram(s)/Day	20090423-20090527	Pain
8 magnesium stoffangabe (teilweise) unklar	Oral	/	20090101-20090601	Muscle cramps
9 Metformin HCl metformin hydrochloride	Oral	/	20090420-20090601	Diabetes mellitus
10 MIDAZOLAM midazolam	Oral	7.5 Mg milligram(s)/Day	20090609-20090609	Sedation
11 NAROPIN ropivacaine hydrochloride		/	20090609-20090609	Analgesia
12 NOVALGIN metamizole	Intravenous (not otherwis	3 G gram(s)/Day	20090609-	Pain
13 PROPOFOL propofol	Intravenous (not otherwis	/	20090609-20090609	General anaesthesia
14 SCANDICAIN "ASTRA" mepivacaine hydrochloride	Subcutaneous	/	20090609-20090609	Local anaesthesia
15 SUFENTANIL sufentanil		/	20090609-	Analgesia
16 taxofit minerals nos; vitamins nos	Oral	/	20090528-20090601	
17 TRAMAGIT tramadol hydrochloride		100 Mg milligram(s)/	20090528-20090601	Pain
18 unacid ampicillin sodium; sulbactam sodium	Oral	4.5 G gram(s)/Day	20090609-20090610	
19 unacid ampicillin sodium; sulbactam sodium	Oral	4.5 G gram(s)/Day	20090602-20090605	Infection

Drugs (concomitant)

20	ciprofloxacin	Oral	2* /Day	20090509-20090518	Ulcus cruris
	ciprofloxacin			10 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Ulcus cruris			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 18.05.2010 DE-PFIZER INC-2010057514

Sender diagnosis: not reported

Narrative

This is a spontaneous report received from a contactable physician.

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170846.

The patient had been hospitalized at the surgical ward from 02Jun2009 until 05Jun2009 for unknown reason.

A 54-year-old male patient was treated with the following suspect drugs: oral sulfamethoxazole (UNACID) 1.5g 3x/d due to an infection of the ulcer cruris from 02Jun2009 to 05Jun2009 and 09Jun2009 to 10Jun2009, oral ibuprofen (MAH not specified) 600 mg 2x/d due to pain from 23Apr2009 to 27May2009 and 28May2009 to 01Jun2009, oral metformin (MAH not specified) with unknown dose due to diabetes mellitus from 20Apr2009 to 01Jun2009, oral magnesium (MAH not specified) with unknown dose due to muscle cramps from 01Jan2009 to 01Jun2009, subcutaneous insulin glulisine (APIDRA) dose according to blood glucose due to diabetes mellitus from 30Apr2009 onwards, oral tramadol HCl (TRAMAGIT) 100 mg due to pain from 28May2009 to 01Jun2009, oral minerals NOS, vitamins NOS (TAXOFIT) with unknown dose for unknown indication, documented as for the eyes from 28May2009 to 01Jun2009, intravenous piritramide (DIPIDOLOR) 5mg due to pain on 09Jun2009, intravenous clonidine (CLONIDIN) 75microgram 2x/d due to pain on 09Jun2009, oral midazolam (MAH not specified) 7.5mg 1x/d for sedation on 09Jun2009, subcutaneous mepivacaine HCl (SCANDICAIN) with unknown dose for local anaesthesia on 09Jun2009, ropivacaine HCl (NAROPIN) with unknown dose for analgesia on 09Jun2009, intravenous propofol (PROPOFOL) with unknown dose for general anaesthesia on 09Jun2009, intravenous heparin (HEPARIN) 5000IU for prophylaxis of thrombosis from 09Jun2009 to 10Jun2009, intravenous metamizole sodium (NOVALGIN) 1g 3x/d due to pain from 09Jun2009 on, bupivacaine HCl (CARBOSTESIN) 0.25% and sufentanil (SUFENTANIL) both for analgesia from 09Jun2009 on. Medical history included ulcer cruris. Concomitant medication included oral ciprofloxacin (CIPROFLOXACIN) with unknown dose 2x/d due to ulcer cruris from 09May2009 to 18May2009. Past drug history included oral treatment with ibuprofen (MAH not specified) which was tolerated well by patient. The patient was hospitalized on 08Jun2009 in order to undergo bypass surgery which was performed on 09Jun2009. He developed fever, swallowing complaints and spots on 10Jun2009. Pruritus and erosions of the lips developed for the first time on 12Jun2009 and blisters of the skin on 14Jun2009. The patient was transferred from the surgical ward to the intensive care unit on 15Jun2009 and to the ward for dermatology on 16Jun2009. A Stevens-Johnson-syndrome (probable case), with onset date 10Jun2009, was diagnosed. The patient was treated with oral dimetindene (FENISTIL) within unknown dose 1x/d then 2x/d due to exanthema from 10Jun2009 on, oral ciprofloxacin (CIPROBAY) 250mg 1x/d then 250mg 2x/d for prophylaxis of infection from 10Jun2009 on, hexetidine (HEXORAL) mouth rinse due to a sore throat from 11Jun2009 on, tyrothricin, cetrimonium, lidocaine (LEMOCIN) lozenges due to a sore throat from 11Jun2009 on and oral paracetamol with unknown dose 2x/d due to fever on 12Jun2009. The event was not considered life-threatening, however caused prolongation of hospitalization. The event lasted until 03Jul2009 and he was discharged from the hospital the same day. Outcome of the event was completely recovered. Information on medication is available from 15May2009 until 12Jun2009. In September 2009 the project experts assessed the causal relationship as "very possible" with sulfamethoxazole and tramadol HCl as possible" with metamizole sodium and as "dubious" with ibuprofen; metformin; magnesium; insulin glulisine; minerals NOS, vitamins NOS; piritramide; clonidine; midazolam; mepivacaine HCl; ropivacaine HCl; propofol; heparin; bupivacaine HCl; sufentanil. Superinfection of the Ulcer cruris as alternative cause was documented as possible by the project experts.

Sender's comment:

Based on the information received the Company (Pfizer) cannot exclude at this time that the suspect medications contributed to the reported event. This report would not seem to modify the risk-benefit profile of the subject drugs. The case will be reassessed if additional information becomes available. Bei Stevens-Johnson-Syndrom handelt es sich um eine bekannte, in der Produktinformation ausreichend praezise deklarierte Nebenwirkung. Wir sehen zum gegenwaertigen Zeitpunkt keinen Anlass fuer eine geaenderte Bewertung des Nutzen-/Risikoverhaeltnisses.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10072157

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 54 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20090610	20090703	24 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Oral	1200 Mg milligram(s)/Day	20090528-20090601 5 Day	Pain
2 ibuprofen ibuprofen	Oral	1200 Mg milligram(s)/Day	20090423-20090527 35 Day	Pain
3 METFORMIN metformin	Oral	/	20090420-20090601 43 Day	Diabetes mellitus
4 SULTAMICILLIN TOSILATE sultamicillin tosilate	Oral	4.5 G gram(s)/Day	20090609-20090610 2 Day	Ulcus cruris
5 SULTAMICILLIN TOSILATE sultamicillin tosilate	Oral	4.5 G gram(s)/Day	20090602-20090605 4 Day	Ulcus cruris

Drugs (concomitant)					
6	APIDRA insulin glulisine	Subcutaneous	/	20090430- -	Diabetes mellitus
7	CARBOSTESIN bupivacaine hydrochloride	Unknown	/	20090609- -	Analgesia
8	ciprofloxacin ciprofloxacin	Unknown	/	20090509-20090518 10 Day	Ulcus cruris
9	clonidine clonidine	Intravenous (not otherwis	/	20090609- -	Pain
10	DIPIDOLOR piritramide	Intravenous (not otherwis	5 Mg milligram(s)/	20090609- -	Pain
11	HEPARIN heparin	Intravenous (not otherwis	/	20090609-20090610 2 Day	Thrombosis prophylaxis
12	magnesium stoffangabe (teilweise) unklar	Oral	/	20090101-20090601 5 Month	Muscle cramps
13	MIDAZOLAM midazolam	Oral	7.5 Mg milligram(s)/Day	20090609- -	Sedation
14	NAROPIN ropivacaine hydrochloride	Unknown	/	20090609- -	Analgesia
15	NOVALGIN metamizole	Intravenous (not otherwis	3 G gram(s)/Day	20090609- -	Pain
16	PROPOFOL propofol	Intravenous (not otherwis	/	20090609- -	General anaesthesia
17	SCANDICAIN mepivacaine hydrochloride	Subcutaneous	/	20090609- -	Local anaesthesia
18	SUFENTANIL sufentanil	Unknown	/	20090609- -	Analgesia
19	taxofit minerals nos; vitamins nos	Unknown	/	20090528-20090601 5 Day	Drug use for unknown indication
20	TRAMAGIT tramadol hydrochloride	Oral	100 Mg milligram(s)/	20090528-20090601 5 Day	Pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diabetes mellitus			Unknown	
Ulcus cruris			Unknown	

Primary source

Qualification: Physician
Literature: observestudytype:

Narratives (nur bei elektronischem Eingang)

McNeil Germany, Local Safety Officers, 18.05.2010 DE-JNJFOC-20100502237

Sender diagnosis: not reported

Narrative

This spontaneous report from a physician concerns a 54 year old male from Germany: 2010057514.

The patient's weight and height was not reported. The patient's medical history and concurrent conditions included: ulcer cruris and diabetes mellitus. The patient was treated with ibuprofen (unspecified, oral) 600 mg twice a day, initiated on 23-APR-2009 to 27-MAY-2009 and 600 mg twice a day, initiated on 28-MAY-2009 to 01-JUN-2009 for pain. Non-company suspect drugs included: sultamicillin tosilate for infection of ulcer cruris and metformin for diabetes mellitus. Concomitant medications included mepivacaine hydrochloride for local anaesthesia, ropivacaine hydrochloride for analgesia, clonidine for pain, piritramide for pain, taxofit for unknown indication, tramadol hydrochloride for pain, insulin glulisine for diabetes mellitus, magnesium for muscle cramps, ciprofloxacin for ulcer cruris, bupivacaine hydrochloride for analgesia, sufentanil for analgesia, metamizole sodium for pain, heparin for prophylaxis of thrombosis, propofol for general anaesthesia, and midazolam for sedation. On 10-JUN-2009, the patient experienced Stevens Johnson Syndrome. On 08-Jun-2009, the patient was hospitalized for bypass surgery on 09-JUN-2009. On 10-JUN-2009, he developed a fever, swallowing complaints and spots. Stevens Johnsons Syndrome was diagnosed. The patient was treated with oral dimetindene (FENISTIL), oral ciprofloxacin (CIPROBAY), hexetidine (HEXORAL), tyrothricin, cetrimonium, lidocaine (LEMOCIN) lozenges and oral paracetamol. Action taken with ibuprofen, sultamicillin tosilate and metformin was unknown. The patient had recovered from Stevens Johnson Syndrome on 03-JUL-2009 and was discharged from the hospital.

This report was serious (prolonged hospitalization).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Single case assessment (reported term/preferred term): IBUPROFEN (Unspecified Oral) Reported Name: IBUPROFEN

STEVENS JOHNSON SYNDROME / STEVENS-JOHNSON SYNDROME: expected per local SmPc

The benefit-risk-ratio remains unchanged based on the single case assessment of this case. No measures according to § 29 (1) AMG have to be taken.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10074110

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 58 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson reaction	20100322				Unknown
2 Herpes labialis	20100320				Unknown
3 Burn					Unknown
4 Swollen lips					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	1200 Mg milligram(s)/Day	20100319-20100319	Lumbar syndrome
2 ibuprofen	Oral	1800 Mg milligram(s)/Day	20100320-20100320	Lumbar syndrome
		245	1800 Year	

Drugs (concomitant)

3 PANTOZOL		/	-	Premedication
pantoprazole sodium			-	
4 TETRAZEPAM	Oral	50 Mg milligram(s)/Day	20100319-	Lumbar syndrome
tetrazepam		245	-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

sanofi-aventis, 21.05.2010 DE-SANOFI-AVENTIS-2010SA016249

Sender diagnosis: not reported

Narrative

Case upgraded to serious upon receipt for follow-up information on 20-Mar-2010:

Initial report received on 20-Mar-2010:

This spontaneous case information was received from a consumer herself. Due to acute back pain the patient started with ibuprofen (Ibuprofen) 600 mg twice on 19-Mar-2010. On the next day she took three tablets of 600 mg.

Premedication included pantoprazole and tetrazepam was taken in the night from 19-Mar-2010 to 20-Mar-2010 (continuation unknown).

On 20-Mar-2010 the patient noticed burning blisters on under- and upper lip as well as on the nasal base. During a second phone call (on 21-Mar-2010) the patient reported about swollen lips.

Ibuprofen was discontinued and the patient will be visited her treating physician. No further information was received so far.

Additional information received on 12-May-2010 from the treating general practitioner:

The patient received ibuprofen (described dosage: 600 mg twice daily) and concomitantly tetrazepam 50 mg (at bedtime) for lumbar spine syndrome.

On 22-Mar-2010 presentation of the patient with changing on the lips, which was diagnosed as resolving herpes labialis by the general practitioner.

Corrective treatment included aciclovir orally and cutaneously.

The general practitioner listed Stevens Johnson Syndrome as adverse event; however he did not verify this diagnosis.

The further course could not be reported as the patient never returned to the practice of the general practitioner.

Causality by general practitioner: unlikely

Additional information received on 20-May-2010 from a dermatologist,

who informed that the patient suffered from herpes labialis. No allergy was suspected . No further information was received so far.

Sender's comment:

Sanofi-Aventis Company Comment dated 14-MAY-2010:

Based on the available information a causal relationship to ibuprofen cannot be completely ruled out. However the information is not sufficient for a precise assessment to the event of steven-johnson-syndrom. In addition it is unclear if herpes labialis is a new infection or a reinfection of a previous one.

21-May-2010 Sanofi-Aventis Germany

All reported clinical diagnoses are considered locally labelled for all suspected products and thus consistent with the known benefit risk ratio of the product(s). Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

case: DE-BFARM-10157901

Patient

Initials: Sex: Female Birth: privacy Age: 43 Year weight (kg): 60 BMI int: 22.04 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin reaction	20100919			8 Day	Unknown
2 Skin red	20100919			8 Day	Unknown
3 Blistering	20100919			8 Day	Unknown
4 Wound secretion	20100919			8 Day	Unknown
5 Skin chapped	20100919			8 Day	Unknown
6 Skin swelling	20100919			8 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolormin	Cutaneous	2 Df dosage form/Day	20100912-20100919	Administration site pain
ibuprofen lysinate		95	8 Day	

Drugs (concomitant)

2	berodual dosier-aerosol	Respiratory (inhalation)	/	-	Asthma
	fenoterol hydrobromide; ipratropium bromide			-	
3	symbicort turbobaler	Respiratory (inhalation)	2 Df dosage form/Day	-	Asthma
	budesonide; formoterol			-	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10185236

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age:	weight (kg): 75	BMI_int: 25.35	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin reaction					recovered/resolved
2 Oral mucosal blistering					recovered/resolved
3 Nasal mucosal disorder					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolormin extra	Oral	/	-	Pain NOS
ibuprofen lysinate			-	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11044985

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 26 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hand swelling	20110223				Unknown
2 Blisters	20110223				Unknown
3 Blister of fingers, without mention of infection	20110223				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuflam	Oral	800 Mg milligram(s)/Day	20110222-20110223	Headache
ibuprofen		245	2 Day	
2 ibuflam	Oral	800 Mg milligram(s)/Day	20110222-20110223	Joint pain
ibuprofen		245	2 Day	

Drugs (concomitant)

3 CITALOPRAM	Oral	20 Mg milligram(s)/	-	Depression
citalopram		245	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Stomach flu			Unknown	
Depression			Yes	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

sanofi-aventis, 28.02.2011 DE-SANOFI-AVENTIS-2011SA011671

Sender diagnosis: not reported

Narrative

Initial report received on 23-Feb-2011:

This spontaneous case information was received from the 26-year-old consumer himself. For depression the patient was treated concomitantly with citalopram 20 mg. The patient suffered from intestinal influenza and developed headache and joint pain. Therefore he took ibuprofen (Ibuflam) 800 mg. The first tablet had been taken in the evening of 22-Feb-2011 and the second tablet was taken in the next morning. A few mins later the patient realized swollen hands, egg seized blisters on the rump area and burning blisters on the fingers. Because the treating physician was on vacation the patient went into hospital. He received infusions NOS and went home afterwards. The exact outcome was not mentioned.

The patient did not want to be sent a "Patient Consent to Release Confidential and Privileged Information" form. However; he will sent the hospital report.

Sender's comment:

25-FEB-2011 Sanofi Aventis Company Comment: the 26-year-old patient developed swollen hands after intake of ibuprofen 800 mg. Due to the compatible temporal relationship a contributing role of ibuprofen cannot be excluded. However, this case lacks further detailed information regarding the patient's medical history, concomitant diseases and drugs which are required to assess the onset of peripheral oedema may can be a symptom of various causes such as allergic or renal aetiology.28-Feb-11 Sanofi-Aventis Germany

The reported diagnoses are considered locally unlabeled. As only limited information has been obtained so far, it is difficult to assess a cause and effect relationship for the suspected drugs. To date, measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11040174

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 3 Year	weight (kg): 17.8	BMI_int: 13.94	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overlap syndrome	20110203			11 Day	recovered/resolved
2 Toxic epidermal necrolysis	20110203			11 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibuprofensaft ibuprofen	Oral	/	20110124-	Fever
2 INFECTOCEF cefaclor	Oral	/	20110201-20110202 2 Day	Tonsillitis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Febrile infection			Unknown	
EBV infection			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 21.03.2011 DE-SZ09-CHPA2011DE03985

Sender diagnosis: not reported

Narrative

Initial spontaneous report received from the German Health Authority (DE-BfArM-11040174) on 11 Mar 2011, reported by a physician: This 3 year old female patient with a medical history of febrile infection and EBV infection developed overlap syndrome on 03 Feb 2011 during or after treatment with IBUPROFEN (ibuprofen) for fever and INFECTOCEF (cefaclor) for tonsillitis. Treatment with ibuprofen started on 24 Jan 2011 (end date was not reported) and treatment with Infectocef was from 01 Feb 2011 to 02 Feb 2011. Dosage was not provided. No information on concomitant treatment was given. The outcome of the adverse event was reported as recovered / resolved. Causality was assessed as certain by the reporter. Additional information was not available.

Sender's comment:

Ibuprofen and Infectocef: Serious spontaneous report: Overlap syndrome [hospitalisation], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

NOVARTIS, 21.03.2011 DE-002147023-CHPA2011DE03985

Sender diagnosis: not reported

Narrative

Initial spontaneous report received from the German Health Authority (DE-BfArM-11040174) on 11 Mar 2011, reported by a physician: This 3 year old female patient with a medical history of febrile infection and EBV infection developed overlap syndrome on 03 Feb 2011 during or after treatment with IBUPROFEN (ibuprofen) for fever and INFECTOCEF (cefaclor) for tonsillitis. Treatment with ibuprofen started on 24 Jan 2011 (end date was not reported) and treatment with Infectocef was from 01 Feb 2011 to 02 Feb 2011. Dosage was not provided. No information on concomitant treatment was given. The outcome of the adverse event was reported as recovered / resolved. Causality was assessed as certain by the reporter. Additional information was not available.

Sender's comment:

Ibuprofen and Infectocef: Serious spontaneous report: Overlap syndrome [hospitalisation], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

sanofi-aventis, 24.03.2011 DE-SANOFI-AVENTIS-2011SA015783

Sender diagnosis: not reported

Narrative

Initial report:

On 15-Mar-2011 this case was extracted from the website of the German health authority (authority number DE-BFARM-11040174); documenting a case, in which a physician had reported about a 3-year-old girl. The medical history included febrile infection and EBV infection without exact specification of dates. On 24-Jan-2011 ibuprofen fluid (Ibuprofensaft) in unspecified dosage was started due to fever. From 01-Feb-2011 to the next day cefaclor for tonsillitis NOS was given. The action taken for ibuprofen fluid was not provided. On 03-Feb-2011 the overlap syndrome Stevens Johnson syndrome / TEN (15% body surface) was diagnosed, which led to hospitalization. Corrective treatment was not mentioned; however the little girl recovered at unspecified time point.

Reporter causality: certain for both suspected drugs

Sender's comment:

24-Mar-11 Sanofi-Aventis Germany

The reported clinical diagnosis is considered locally labelled for ibuprofen and thus consistent with the known benefit risk ratio of the product. Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

sanofi-aventis, 05.07.2011 DE-SANOFI-AVENTIS-2011SA015783

Sender diagnosis: not reported

Narrative

Initial report:

On 15-Mar-2011 this case was extracted from the website of the German health authority (authority number DE-BFARM-11040174); documenting a case, in which a physician had reported about a 3-year-old girl. The medical history included febrile infection and EBV infection without exact specification of dates. On 24-Jan-2011 ibuprofen fluid (Ibuprofensaft) in unspecified dosage was started due to fever. From 01-Feb-2011 to the next day cefaclor for tonsillitis NOS was given. The action taken for ibuprofen fluid was not provided. On 03-Feb-2011 the overlap syndrome Stevens Johnson syndrome / TEN (15% body surface) was diagnosed, which led to hospitalization. Corrective treatment was not mentioned; however the little girl recovered at unspecified time point.

Reporter causality: certain for both suspected drugs

Additional information received on 29-Jun-2011 from a physician by letter:

New additional information revealed that the suspected product was not a company product Therefore this case will be deleted after reporting to the German Health Authority.

Sender's comment:

05-JUL-11 Sanofi-Aventis Germany

The reported clinical diagnosis is considered locally labelled for ibuprofen and thus consistent with the known benefit risk ratio of the product. However, follow-up information revealed that ibuprofen from another manufacturer was used. Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: US-BFARM-11065850

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 22 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis					recovered/resolved with sequel
2 Hypotension					recovered/resolved with sequel

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Unknown	200 Mg milligram(s)/Day	-	Menstrual discomfort
ibuprofen			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Drug allergy			Unknown	swelling of lips in reponse to nonsteroidal anti-inflammatory drugs (acetaminophen and ibuprofen)

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Toedliche Gefahr auf der Sonnenbank. MMW-Fortschritte der Medizin. 2011;10:1	

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 16.03.2011 US-PFIZER INC-2011053858

Sender diagnosis: not reported

Narrative

This is a literature report from MMW - Fortschritte der Medizin , Title: Toedliche Gefahr auf der Sonnenbank, Number 10/2011, Volume 10, page 1. The English translation for the article has not been requested. A female patient experienced a mild rash after ibuprofen intake. After visiting a tanning salon to treat herself with radiation exposure, she developed toxic epidermal necrolysis with severe blisters and a drop of blood pressure.

Follow up (11Mar2011): original article from ARCH DERMATOL, doi10.1001/ archdermatol, 2011.13: A 22-year-old female patient with Fitzpatrick type II skin treated herself with ibuprofen (manufacturer and dose form unspecified) due to menstrual discomfort. Otherwise she was healthy and did not report taking medications, but had a history of abnormal reactions to nonsteroidal anti-inflammatory drugs, including swelling of the lips in response to acetaminophen and ibuprofen .After having for 2 days red, burning, and blistering skin, including involvement of oral, ocular, and vaginal mucosa she presented to a hospital. Previously the patient had ingested 200 mg of ibuprofen daily for 5 days to treat menstrual discomfort. Within 24 hours of ingestion, she had noticed mild lip swelling and a minimally symptomatic eruption consisting of small red macules and papules on her chest, back, and proximal arms. Two days after the skin eruption had begun, in an attempt to self treat her stable, non progressing rash, she visited a tanning salon she had frequented 4 to 5 times in the past year and received radiation exposure for approximately 8 minutes in a Sundazzler 160 stand-up tanning bed (Heartland Tanning, Inc, Lees Summit, Missouri). The patient wore blue jeans and a brassiere and used an ocular shield during the tanning session. Within approximately 4 hours after the tanning bed radiation exposure, the patient noted increased itchiness of her radiation-exposed skin. The next morning, she experienced severe redness, pain, and the beginnings of blister formations on her radiation-exposed abdomen, back, face, and proximal arms. Moreover, on her lips and in her oral cavity, including in her throat, she developed redness and pain. She also noted mild vaginal irritation. She was afebrile and was able to ingest liquids. The patient then presented to an emergency department at her local hospital. She was thought to have a streptococcal infection or drug eruption and was prescribed intramuscular corticosteroids and oral amoxicillin sodium. She was discharged from the emergency department but she returned approximately 6 hours later because of worsening symptoms. At this time she was hospitalized and within 24 hours, she was intubated and given fluid boluses for episodes of hypotension. A vaginal culture was performed, which did not reveal staphylococcus aureus or group A streptococcus. Results of a throat culture were negative for the latter. After the cultures were performed, the patient was treated with intravenous vancomycin hydrochloride. Due to her worsening symptoms, the patient was transferred to a medical center. On arrival, she was placed in the burn unit, and dermatology, ophthalmology, and gynecology services were consulted. She was afebrile, and her blood pressure was stable. The patient exhibited blister formation on the areas exposed to tanning bed radiation, especially the chest, abdomen, proximal arms, and upper back (approximately 35% of the body surface area). Laboratory tests results revealed mild transaminitis (aspartate aminotransferase 80U/L, normal <45), but were otherwise normal. The dermatology service suspected toxic epidermal necrolysis (TEN) and performed skin biopsies that revealed a lymphocytic infiltrate at the dermoepidermal junction with some apoptotic keratinocytes consistent with TEN. The ophthalmology service noted ocular

involvement also consistent with TEN, and the gynecological service found erythema and swelling of the patients vaginal introitus with several small ulcerations. She was given intravenous cyclosporine, 5 mg/kg/d and was treated supportively in the burn unit. After 2 days, owing to lack of response and some extension of the blisters onto the groin and proximal thighs, treatment with cyclosporine was discontinued, and the patient received 3 days of treatment with intravenous IgG (1 g/kg/d). She slowly recovered and was extubated after 1 week. After a total of nearly 3 weeks, she was discharged home and reported no discomfort except mild ocular light sensitivity.

Pfizer is the Marketing Authorization Holder of Ibuprofen in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of Ibuprofen has submitted the same report to the regulatory authorities.

Sender's comment:

A possible contributory role of the suspect drug ibuprofen cannot be excluded for the reported events toxic epidermal necrolysis and hypotension based on temporal association. This individual report does not alter the risk/benefit profile of the suspect drug.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11068580

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 86 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin reaction localised	201011		5 Day		recovered/resolved
2 Skin red	201011		5 Day		recovered/resolved
3 Blistering	201011		5 Day		recovered/resolved
4 Erythema	20101112	20101116	5 Day	1 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuflam 600 mg ibuprofen	Oral	1200 Mg milligram(s)/Day 82	20101112-20101112	Pain

Drugs (concomitant)

2 AZOPT brinzolamide		/	-	Glaucoma
3 CARBIDOPA/LEVODOPA carbidopa; levodopa	Oral	1 Df dosage form/Day 245	-	Parkinson's disease
4 METOBETA metoprolol tartrate	Oral	100 Mg milligram(s)/Day 245	-	Hypertension
5 PANTOPRAZOLE pantoprazole	Oral	40 Mg milligram(s)/Day 245	-	Prophylaxis

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

[sanofi-aventis, 20.04.2011 DE-SANOFI-AVENTIS-2011SA023065](#)

Sender diagnosis: not reported

Narrative

Initial report received on 14-Apr-2011 from a pharmacist provided via the Drug Commission of the German Medical Association (id. no: 160744). The case involves an 86-year-old female patient who had started therapy with ibuprofen (Ibuflam) 600 mg twice daily at an unspecified date for pain. While she was treated with ibuprofen, she developed severe, extensive erythema with formation of blisters in November 2010. Consequently therapy was withdrawn. Afterwards signs and symptoms improved and resolved completely after five days. Further information including significant past and present medical history, concomitant medication and corrective treatment was not provided.

Reporter causality: not provided.

Sender's comment:

20-Apr-11 Sanofi-Aventis Germany

The reported clinical diagnosis is considered locally labelled for ibuprofen and thus consistent with the known benefit risk ratio of the product. Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

sanofi-aventis, 03.05.2011 DE-SANOFI-AVENTIS-2011SA023065

Sender diagnosis: not reported

Narrative

Initial report received on 14-Apr-2011 from a pharmacist provided via the Drug Commission of the German Medical Association (id. no: 160744). The case involves an 86-year-old male patient, whose present medical history included glaucoma, Parkinson's disease and hypertension treated with brinzolamide, carbidopa/levodopa and metoprolol tartrate, respectively. For prophylaxis the patient used pantoprazole. On 12-Nov-2010 the patient took ibuprofen (Ibuflam) 600 mg twice daily for pain. On the same day he developed severe, extensive erythema with formation of blisters. The patient did not take further tablets of ibuprofen consequently and he recovered until 16-Nov.2010. Further information including corrective treatment was not provided.

Reporter causality: possible.

Additional information from the pharmacist, received on 28-Apr-2011:

Completion of onset date, duration of treatment with the suspected drug, amendment of concomitant diseases and drugs and duration of reaction.

Sender's comment:

03-MAY-11 Sanofi-Aventis Germany

The reported clinical diagnosis is considered locally labelled for ibuprofen and thus consistent with the known benefit risk ratio of the product. Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11096429

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 33 Year	weight (kg): 63	BMI_int: 21.8	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Capillaritis	2007				not recovered/not resolved
2 Scar					not recovered/not resolved
3 Hyperpigmentation skin					not recovered/not resolved
4 Cutaneous vasculitis	2004				not recovered/not resolved
5 Ankle ulcer	2004				not recovered/not resolved
6 Thrombosis	2007				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Acetylsalizylsaeure acetylsalicylic acid	Oral	500 Mg milligram(s)/	-	
2 ibuprofen ibuprofen	Oral	400 Mg milligram(s)/	-	
3 voltaren dispers diclofenac sodium	Oral	/	2001-201105	Headache
4 voltaren resinat diclofenac resinate	Oral	/	2007-	Muscle pain

Drugs (concomitant)

5 folio folinic acid	Oral	150 µg microgram(s)/	-	
6 FOLIO FORTE	Oral	800 µg microgram(s)/	201104-	
7 FOLIO FORTE	Oral	10 µg microgram(s)/	-	
8 vitamin c ascorbic acid		500 Mg milligram(s)/	201103-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Joint pain			Unknown	
Drug allergy			Unknown	
Pain in spine			Unknown	
Ulcer			Unknown	
Pain			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 13.05.2011 DE-002147023-PHHY2011DE37257

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE37257, is an initial spontaneous report received from physician via phone call on 28 Apr 2011. This report refers to a 33-year-old female patient. The patient received Voltaren resinat (diclofenac resinat) orally as needed for muscle and joint pain since 2007 (for about 5

years). About half a year after the start of treatment with Voltaren resinat, the patient developed ulceration on ankles. Reportedly the ulceration had worsened after intake of Voltaren resinat. A specimen of ulcerated skin tissue showed capillary inflammation and thrombosis of capillaries. Peripheral arterial disease and venous disorders had been excluded. Dechallenge of Voltaren Resinat had been planned. The event outcome was reported as ongoing and causality was not explicitly mentioned.

Sender's comment:

Serious spontaneous report- Skin ulcer, [medically significant] assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Based on Review of available reported data does not allow for a proper causality assessment. Reported event(s) is (are) unassessable

Reporter's comment:

NOVARTIS, 11.07.2011 DE-002147023-PHHY2011DE37257

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE37257, is an initial spontaneous report received from physician via phone call on 28 Apr 2011 with follow up received on 27 Jun 2011: This report refers to a 33-year-old female patient. This polymedicated patient had medical history of amoxicillin allergy. Concomitant medication included Vitamin C and Folio forte tablets The patient received Voltaren dispers (diclofenac free acid) tablets orally for headache, vertebral pain and pain from ulcers since 2001 approximately one to two tablets daily. The patient also received ibuprofen (manufacturer unknown) 600 mg and acetylsalicylic acid (manufacturer unknown) 500 mg orally. In end of 2004, the patient developed cutaneous vasculitis at both ankles. The patient developed ulceration on ankles. A specimen of ulcerated skin tissue showed capillary inflammation and thrombosis of capillaries. Peripheral arterial disease and venous disorders had been excluded. The diameter of ulcerations was approximately 5 mm and was relapsing every 3 to 10 months until recently. Ulcerations have merged with a diameter of a single lesion of up to 10 mm. Vasculitis was treated with prednisolone 60 mg and pain with Valoron drops orally. No new lesion had occurred since cessation of voltaren dispers on May 2011. The patient had not yet recovered and sequelae of scar and hyperpigmentation have remained. The causality was assessed by reporting physician as likely to be related to Voltaren dispers, however ibuprofen 600 mg and acetylsalicylic acid 500 mg were mentioned as possible alternative explanations.

Follow-up information received from physician on 27 Jun 2011: Suspect drug changed from Voltaren resinat to Voltaren dispers, updated start date of suspect drug, indication, concomitant medication, medical history of allergy, events, treatment medication, co suspect medication, action taken with suspect drug and causality.

Sender's comment:

Serious spontaneous report,

For Voltaren dispers,

Cutaneous vasculitis, [medically significant] assessed as listed according to the Core Data Sheet.

For acetyl salicylic acid and ibuprofen,

Cutaneous vasculitis [medically significant], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

For acetyl salicylic acid and ibuprofen, the event is confounded by Voltaren dispers.

Reporter's comment:

NOVARTIS, 11.07.2011 DE-SZ09-PHHY2011DE37257

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE37257, is an initial spontaneous report received from physician via phone call on 28 Apr 2011 with follow up received on 27 Jun 2011: This report refers to a 33-year-old female patient. This polymedicated patient had medical history of amoxicillin allergy. Concomitant medication included Vitamin C and Folio forte tablets The patient received Voltaren dispers (diclofenac free acid) tablets orally for headache, vertebral pain and pain from ulcers since 2001 approximately one to two tablets daily. The patient also received ibuprofen (manufacturer unknown) 600 mg and acetylsalicylic acid (manufacturer unknown) 500 mg orally. In end of 2004, the patient developed cutaneous vasculitis at both ankles. The patient developed ulceration on ankles. A specimen of ulcerated skin tissue showed capillary inflammation and thrombosis of capillaries. Peripheral arterial disease and venous disorders had been excluded. The diameter of ulcerations was approximately 5 mm and was relapsing every 3 to 10 months until recently. Ulcerations have merged with a diameter of a single lesion of up to 10 mm. Vasculitis was treated with prednisolone 60 mg and pain with Valoron drops orally. No new lesion had occurred since cessation of voltaren dispers on May 2011. The patient had not yet recovered and sequelae of scar and hyperpigmentation have remained. The causality was assessed by reporting physician as likely to be related to Voltaren dispers, however ibuprofen 600 mg and acetylsalicylic acid 500 mg were mentioned as possible alternative explanations.

Follow-up information received from physician on 27 Jun 2011: Suspect drug changed from Voltaren resinat to Voltaren dispers, updated start date of suspect drug, indication, concomitant medication, medical history of allergy, events, treatment medication, co suspect medication, action taken with suspect drug and causality.

Sender's comment:

Serious spontaneous report,

For Voltaren dispers,

Cutaneous vasculitis, [medically significant] assessed as listed according to the Core Data Sheet.

For acetyl salicylic acid and ibuprofen,

Cutaneous vasculitis [medically significant], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

For acetyl salicylic acid and ibuprofen, the event is confounded by Voltaren dispers.

Reporter's comment:

NOVARTIS, 11.07.2011 DE-NOVNCHHQ-PHHY2011DE37257

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE37257, is an initial spontaneous report received from physician via phone call on 28 Apr 2011 with follow up received on 27 Jun 2011: This report refers to a 33-year-old female patient. This polymedicated patient had medical history of amoxicillin allergy. Concomitant medication included Vitamin C and Folio forte tablets The patient received Voltaren dispers (diclofenac free acid) tablets orally for headache, vertebral pain and pain from ulcers since 2001 approximately one to two tablets daily. The patient also received ibuprofen (manufacturer unknown) 600 mg and acetylsalicylic acid (manufacturer unknown) 500 mg orally. In end of 2004, the patient developed cutaneous vasculitis at both ankles. The patient developed ulceration on ankles. A specimen of ulcerated skin tissue showed capillary inflammation and thrombosis of capillaries. Peripheral arterial disease and venous disorders had been excluded. The diameter of ulcerations was approximately 5 mm and was relapsing every 3 to 10 months until recently. Ulcerations have merged with a diameter of a single lesion of up to 10 mm. Vasculitis was treated with prednisolone 60 mg and pain with Valoron drops orally. No new lesion had occurred since cessation of voltaren dispers on May 2011. The patient had not yet recovered and sequelae of scar and hyperpigmentation have remained. The causality was assessed by reporting physician as likely to be related to Voltaren dispers, however ibuprofen 600 mg and acetylsalicylic acid 500 mg were mentioned as possible alternative explanations.

Follow-up information received from physician on 27 Jun 2011: Suspect drug changed from Voltaren resinat to Voltaren dispers, updated start date of suspect drug, indication, concomitant medication, medical history of allergy, events, treatment medication, co suspect medication, action taken with suspect drug and causality.

Sender's comment:

Serious spontaneous report,

For Voltaren dispers,

Cutaneous vasculitis, [medically significant] assessed as listed according to the Core Data Sheet.

For acetyl salicylic acid and ibuprofen,

Cutaneous vasculitis [medically significant], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

For acetyl salicylic acid and ibuprofen, the event is confounded by Voltaren dispers.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11137408

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 87 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bullous pemphigoid	20110624	201107		0 Month	recovered/resolved
2 Bullous eruption	20110624	201107		0 Month	recovered/resolved
3 Blister	201106				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 IBUFLAM ibuprofen	Oral	600 Mg milligram(s)/ 245	-201106 -	Joint pain
2 IBUFLAM ibuprofen	Oral	600 Mg milligram(s)/Day 245	201106-201106 -	Joint pain

Drugs (concomitant)

3 AMLODIPINE amlodipine	Unknown	10 Mg milligram(s)/	-	Drug use for unknown indication
4 GODAMED acetylsalicylic acid; aminoacetic acid	Unknown	/	-	Drug use for unknown indication
5 HYDROCHLOROTHIAZIDE hydrochlorothiazide	Unknown	/	-	Drug use for unknown indication
6 METOPROLOL metoprolol tartrate	Unknown	100 Mg milligram(s)/	-	Drug use for unknown indication
7 RAMIPRIL ramipril	Unknown	10 Mg milligram(s)/	-	Drug use for unknown indication
8 SIMVASTATIN simvastatin	Unknown	/	-	Drug use for unknown indication
9 TORASEMIDE torasemide	Unknown	10 Mg milligram(s)/	-	Drug use for unknown indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Eczema			Yes	
Multimorbidity			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

sanofi-aventis, 15.07.2011 DE-SANOFI-AVENTIS-2011SA040913

Sender diagnosis: not reported

Narrative

Case upgraded to serious upon receipt of follow-up information on 12-Jul-2011.

The initial information summarized below was received on 27-Jun-2011 and 28-Jun-2011:

This spontaneous case information was received from the consumer reporting on her 88-year-old father. The following drugs are known to have been taken by the patient prior to the event: amlodipine 10 mg, ramipril 10 mg, torasemide 10 mg, metoprolol 100 mg, hydrochlorothiazide, acetylsalicylic

acid/aminoacetic acid and simvastatin. Due to arthrosis pain the patient had taken ibuprofen (Ibuflam) 600 mg sporadically. Later on, in Jun-2011 the dosage was increased to 600 mg daily. Altogether, 30 to 40 tablets were taken.

One week before the initial report, the patient noticed beginning blister formation, which got worse in size and number. Some blisters were up to plum size and filled with water and opened up. The wounds were treated by the nursing service. The blisters worsened despite discontinuation of ibuprofen. The patient was hospitalised in a dermatologic clinic from 29-Jun-2011 to 06-Jul-2011; however the further course was not reported.

Additional information received on 12-Jul-2011:

Case upgraded after receipt of the completed "Patient Consent to Release Confidential and Privileged Information" form by the consumer.

Sender's comment:

15-Jul-11 Sanofi-Aventis Germany

The reported clinical diagnosis is considered locally labelled for ibuprofen and thus consistent with the known benefit risk ratio of the product(s). Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

sanofi-aventis, 12.08.2011 DE-SANOFI-AVENTIS-2011SA040913

Sender diagnosis: not reported

Narrative

Case upgraded to serious upon receipt of follow-up information on 12-Jul-2011.

The initial information summarized below was received on 27-Jun-2011 and 28-Jun-2011:

This spontaneous case information was received from the consumer reporting on her 88-year-old father. The following drugs are known to have been taken by the patient prior to the event: amlodipine 10 mg, ramipril 10 mg, torasemide 10 mg, metoprolol 100 mg, hydrochlorothiazide, acetylsalicylic acid/aminoacetic acid and simvastatin. Due to arthrosis pain the patient had taken ibuprofen (Ibuflam) 600 mg sporadically. Later on, in Jun-2011 the dosage was increased to 600 mg daily. Altogether, 30 to 40 tablets were taken.

One week before the initial report, the patient noticed beginning blister formation, which got worse in size and number. Some blisters were up to plum size and filled with water and opened up. The wounds were treated by the nursing service. The blisters worsened despite discontinuation of ibuprofen. The patient was hospitalised in a dermatologic clinic from 29-Jun-2011 to 06-Jul-2011.

The dermatologist specified the patient as multimorbid. The anamnesis included known eczemas. On 24-Jun-2011 the patient began to present bullous pemphigoid. Furthermore the dermatologist specified bullous drug exanthema as differential diagnosis. The patient recovered in Jul-2011.

Alternative explanation included autoimmune disease without connection of ibuprofen.

Causality by dermatologist: possible for bullous pemphigoid.

Additional information received on 12-Jul-2011:

Case upgraded after receipt of the completed "Patient Consent to Release Confidential and Privileged Information" form by the consumer.

Additional information received from the dermatologist on 08-Aug-2011:

Supplement the confirmation and further specification of the event reported by the patient. Entry of patients data (birth date, concomitant diseases) as well as of two diagnoses by the dermatologist with onset date, alternative explanation and verified outcome.

Sender's comment:

Sanofi-aventis Company Comment for follow-up information dated 09-AUG-2011: New follow up information received does not change the previous assessment of this case.12-AUG-11 Sanofi-Aventis Germany

The reported clinical diagnoses are considered locally labelled for ibuprofen and thus consistent with the known benefit risk ratio of the product. Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11215196

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Female Birth: privacy Age: weight (kg): 60 BMI_int: 21.01 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blistering	20111008				recovered/resolved
2 Skin rash	20111008				recovered/resolved
3 Pruritus	20111008				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen dura 400 mg filmdabletten	Oral	400 Mg milligram(s)/Day	-	Pain
ibuprofen		82	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hay fever			Yes	

Primary source

Qualification: Pharmacist observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12008230

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 28 Year	weight (kg): 90	BMI_int: 26.01	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Conjunctivitis	20120102	20120113	12 Day	9 Day	recovered/resolved
2 Aphtha	20120102	20120113	12 Day	9 Day	recovered/resolved
3 Genital ulceration	2012	2012	5 Week		recovered/resolved
4 Tracheal ulcer	2012	2012	5 Week		recovered/resolved
5 Esophageal ulcer	2012	2012	5 Week		recovered/resolved
6 Drug interaction					Unknown
7 Reduced general condition	2012	2012	5 Week		recovered/resolved
8 Condition aggravated					recovered/resolved
9 Leukocyte count increased	20120102	20120113	12 Day	9 Day	recovered/resolved
10 CRP increased	20120102	20120113	12 Day	9 Day	recovered/resolved
11 Fever	20120102	20120113	12 Day	9 Day	recovered/resolved
12 Oral aphthae	20120102	20120113	12 Day	9 Day	recovered/resolved
13 Drug hypersensitivity	201112				Unknown
14 Behcet's disease	201112				Unknown
15 Balanitis	201112				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuflam ibuprofen	Unknown	/	- 1 Week	Fever
2 ibuflam ibuprofen	Unknown	/	- 1 Week	Cold

Drugs (concomitant)

3 AMOXICILLIN amoxicillin	Unknown	/	20111224-20111226 3 Day	Cold
4 AMOXICILLIN amoxicillin	Unknown	/	20111224-20111226 3 Day	Fever

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Influenza like illness	20111224		Unknown	
Aphtha	201103	2011	No	
Reflux oesophagitis			Unknown	
Hiatal hernia			Unknown	
Conjunctivitis	201103		No	
Oral aphthae	201103		No	
Genital erosion	201103		No	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

TEVA, 19.01.2012 DE-TEVA-315991GER

Sender diagnosis: not reported

Narrative

04-Jan-2012, Spontaneous Serious Report,

A Physician reported the case of a Male patient 28 Years of age who experienced SUSPECTED MORBUS BEHCET, SUSPECTED ATYPICAL DRUG HYPERSENSITIVITY, ORAL/TRACHEAL/ESOPHAGEAL APHTHA, GENITAL APHTHA, BILATERAL CONJUNCTIVITIS, BALANITIS, FEVER AGGRAVATED while on Amoxicillin ratiopharm 1000mg Filmtabletten (AMOXICILLIN) (Teva's product).

On 24-Dec-2011 the patient developed a febrile flu-like infection. On 25-Dec-2011 treatment with amoxicillin was started (Dose unknown). No concomitant medication reported. The disease symptoms were improving, however, the patient developed approx. three days after treatment initiation a conjunctivitis, oral aphtae and a aggravation of the fever. On 02-Jan-2011 the patient was hospitalized with moderately reduced general condition, bilateral conjunctivitis, oral and genital aphtae, balanitis. The following test were performed in the hospital (additional test results were also provided):

HLA B51 negative

Blood culture: negative

Mouth swab: Haemophilus parainfluenza, bacteria of the oral flora, Serratia marcescens

HIV-Test: negative

ANA, ANC, rheumatoid factor, circulating immune complexes, AAB against each CCP negative.

Mycoplasma antibodies: results following.

Bronchial lavage: no evidence of acid-fast bacilli. TBC negative

Due to the severe oral aphtae the patients' food and fluid intake was strongly impaired. The CRP was significantly increased. No pathogens were detected in blood cultures. Initial antibiotic therapy with levofloxacin could hardly improve any clinical and laboratory parameter. After cortisone administration, however, a rapid improvement in general health status and a falling CRP was observed.

Due to the clinical appearance and despite negative HLAB51 and for Behcet's atypical histology and negative pathergy test, the physicians suspected a Behcet's disease. In addition, an unusual drug reaction to amoxicillin was suspected as differential diagnosis. In addition, the patient was also treated with amoxicillin in Mar-2011 for a infection and developed oral and genital aphtae in a milder form.

The patient was discharged on 13-Jan-2012. On 17-Jan-2012, the oral aphtae were not recovered and the outcome of the other AE's were not reported. However, the mother of the patient stated that there where no genital aphtae in 2012 (clearly described in the physician's report).

Treatment of the AE's: Levofloxacin (dose unknown); Prednisone 40 mg/d;

At the time of the report the outcome of the AEs were: SUSPECTED MORBUS BEHCET : Unknown, SUSPECTED ATYPICAL DRUG HYPERSENSITIVITY : Unknown, ORAL/TRACHEAL/ESOPHAGEAL APHTHA : not recovered/not resolved, GENITAL APHTHA : Unknown, BILATERAL CONJUNCTIVITIS : Unknown, BALANITIS : Unknown, FEVER AGGRAVATED : Unknown.

Medical History: FLU-LIKE INFECTION(Concomitant) ,ORAL APHTHA (Past) ,GENITAL APHTHA (Past) ,GRADE TWO REFLUX OESOPHAGITIS(Concomitant) ,MODERATE AXIAL HIATAL HERNIA(Concomitant)

Action Taken with suspect Drugs: Amoxicillin ratiopharm 1000mg Filmtabletten - Drug discontinued

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: M Behcet is rated as not related to Amoxicillin treatment but to a possible genetic predisposition and other exogenous factors. However, atypical drug hypersensitivity associated with aphtae, conjunctivitis, fever and balanitis is rated as possibly related due to plausible temporal relationship and history of similar symptoms after previous Amoxicillin exposure.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case

Sender's comment:

Serious, spontaneous (TEVA product could not be excluded) case report from Germany. Duty for notification according to German Drug Law. Causality assessment: see case narrative TEVA comment. Assessment of expectedness according to current German SmPC and PIL: unexpected. The risk-benefit ratio of the treatment with the drug in general can be regarded as unchanged. Changes in the statements of the German SmPC and PIL regarding special warnings, special precautions for use and undesirable effects due to this case seem not to be justified at present.

Reporter's comment:

TEVA, 16.02.2012 DE-TEVA-315991GER

Sender diagnosis: not reported

Narrative

04-Jan-2012, Spontaneous Serious Report,

A Physician reported the case of a Male patient 28 Years of age who experienced SUSPECTED MORBUS BEHCET, SUSPECTED ATYPICAL DRUG HYPERSENSITIVITY, ORAL/TRACHEAL/ESOPHAGEAL APHTHAE, GENITAL APHTHAE, BILATERAL CONJUNCTIVITIS, BALANITIS, FEVER AGGRAVATED while on Amoxicillin ratiopharm 1000mg Filmtabletten (AMOXICILLIN) (Teva's product).

On 24-Dec-2011 the patient developed a febrile flu-like infection. On 25-Dec-2011 treatment with amoxicillin was started (Dose unknown). No concomitant medication reported. The disease symptoms were improving, however, the patient developed approx. three days after treatment initiation a conjunctivitis, oral aphtae and a aggravation of the fever. On 02-Jan-2011 the patient was hospitalized with moderately reduced general condition, bilateral conjunctivitis, oral and genital aphtae, balanitis. The following test were performed in the hospital (additional test results were also provided):
HLA B51 negative
Blood culture: negative
Mouth swab: Haemophilus parainfluenza, bacteria of the oral flora, Serratia marcescens
HIV-Test: negative
ANA, ANC, rheumatoid factor, circulating immune complexes, AAB against each CCP negative.
Mycoplasma antibodies: results following.
Bronchial lavage: no evidence of acid-fast bacilli. TBC negative

Due to the severe oral aphtae the patients' food and fluid intake was strongly impaired. The CRP was significantly increased. No pathogens were detected in blood cultures. Initial antibiotic therapy with levofloxacin could hardly improve any clinical and laboratory parameter. After cortisone administration, however, a rapid improvement in general health status and a falling CRP was observed.

Due to the clinical appearance and despite negative HLAB51 and for Behcet's atypical histology and negative pathergy test, the physicians suspected a Behcet's disease. In addition, an unusual drug reaction to amoxicillin was suspected as differential diagnosis. In addition, the patient was also treated with amoxicillin in Mar-2011 for a infection and developed oral and genital aphtae in a milder form.

The patient was discharged on 13-Jan-2012. On 17-Jan-2012, the oral aphtae were not recovered and the outcome of the other AE's were not reported. However, the mother of the patient stated that there where no genital aphtae in 2012 (clearly described in the physician's report).

Treatment of the AE's: Levofloxacin (dose unknown); Prednisone 40 mg/d;

At the time of the report the outcome of the AEs were: SUSPECTED MORBUS BEHCET : Unknown, SUSPECTED ATYPICAL DRUG HYPERSENSITIVITY : Unknown, ORAL/TRACHEAL/ESOPHAGEAL APHTHAE : not recovered/not resolved, GENITAL APHTHAE : Unknown, BILATERAL CONJUNCTIVITIS : Unknown, BALANITIS : Unknown, FEVER AGGRAVATED : Unknown.

Medical History: FLU-LIKE INFECTION(Concomitant) ,ORAL APHTHAE (Past) ,GENITAL APHTHAE (Past) ,GRADE TWO REFLUX OESOPHAGITIS(Concomitant) ,MODERATE AXIAL HIATAL HERNIA(Concomitant)

Action Taken with suspect Drugs: Amoxicillin ratiopharm 1000mg Filmtabletten - Drug discontinued

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: M Behcet is rated as not related to Amoxicillin treatment but to a possible genetic predisposition and other exogenous factors. However, atypical drug hypersensitivity associated with aphtae, conjunctivitis, fever and balanitis is rated as possibly related due to plausible temporal relationship and history of similar symptoms after previous Amoxicillin exposure.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case

08-Feb-2012,

Follow up information received from the hospital. The physician reported that the patient also experienced CRP INCREASED and LEUKOCYTE COUNT INCREASED, besides ORAL/TRACHEAL/ESOPHAGEAL APHTHAE, GENITAL APHTHAE, BILATERAL CONJUNCTIVITIS and FEVER AGGRAVATED. All previously mentioned events occurred between 02-Jan-2012 and 13-Jan-2012 and the reporter assessed the causality as probable. The patient was initially treated with Tavanic (Levofloxacin) and Roxithromycin, which was ineffective. After cortisone treatment the symptoms were improving and are recovered at the time of this report.

The events SUSPECTED MORBUS BEHCET, SUSPECTED ATYPICAL DRUG HYPERSENSITIVITY and BALANITIS were not mentioned in the

follow up report.

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: Due to data available (see source documents, differential diagnostics, physicians statements and so on) it is not clear, so far, whether symptoms reported were due to Amoxicillin, to an M.Behcet, or other genesis, thus symptoms reported except M.Behcet are possibly related to Amoxicillin.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case

Sender's comment:

Serious, spontaneous (TEVA product could not be excluded) (follow-up (1)) case report from Germany. Duty for notification according to German Drug Law. Causality assessment: see case narrative TEVA comment. Assessment of expectedness according to current German SmPC and PIL: unexpected. The risk-benefit ratio of the treatment with the drug in general can be regarded as unchanged. Changes in the statements of the German SmPC and PIL regarding special warnings, special precautions for use and undesirable effects due to this case seem not to be justified at present.

Reporter's comment:

TEVA, 08.03.2012 DE-TEVA-315991GER

Sender diagnosis: not reported

Narrative

04-Jan-2012, Spontaneous Serious Report,

A Physician reported the case of a Male patient 28 Years of age who experienced SUSPECTED MORBUS BEHCET, SUSPECTED ATYPICAL DRUG HYPERSENSITIVITY, ORAL/TRACHEAL/ESOPHAGEAL APHTHA, GENITAL APHTHA, BILATERAL CONJUNCTIVITIS, BALANITIS, FEVER AGGRAVATED while on Amoxicillin ratiopharm 1000mg Filtabletten (AMOXICILLIN) (Teva's product).

On 24-Dec-2011 the patient developed a febrile flu-like infection. On 25-Dec-2011 treatment with amoxicillin was started (Dose unknown). No concomitant medication reported. The disease symptoms were improving, however, the patient developed approx. three days after treatment initiation a conjunctivitis, oral aphtae and a aggravation of the fever. On 02-Jan-2011 the patient was hospitalized with moderately reduced general condition, bilateral conjunctivitis, oral and genital aphtae, balanitis. The following test were performed in the hospital (additional test results were also provided):

HLA B51 negative

Blood culture: negative

Mouth swab: Haemophilus parainfluenza, bacteria of the oral flora, Serratia marcescens

HIV-Test: negative

ANA, ANC, rheumatoid factor, circulating immune complexes, AAB against each CCP negative.

Mycoplasma antibodies: results following.

Bronchial lavage: no evidence of acid-fast bacilli. TBC negative

Due to the severe oral aphtae the patients' food and fluid intake was strongly impaired. The CRP was significantly increased. No pathogens were detected in blood cultures. Initial antibiotic therapy with levofloxacin could hardly improve any clinical and laboratory parameter. After cortisone administration, however, a rapid improvement in general health status and a falling CRP was observed.

Due to the clinical appearance and despite negative HLAB51 and for Behcet's atypical histology and negative pathergy test, the physicians suspected a Behcet's disease. In addition, an unusual drug reaction to amoxicillin was suspected as differential diagnosis. In addition, the patient was also treated with amoxicillin in Mar-2011 for a infection and developed oral and genital aphtae in a milder form.

The patient was discharged on 13-Jan-2012. On 17-Jan-2012, the oral aphtae were not recovered and the outcome of the other AE's were not reported. However, the mother of the patient stated that there where no genital aphtae in 2012 (clearly described in the physician's report).

Treatment of the AE's: Levofloxacin (dose unknown); Prednisone 40 mg/d;

At the time of the report the outcome of the AEs were: SUSPECTED MORBUS BEHCET : Unknown, SUSPECTED ATYPICAL DRUG HYPERSENSITIVITY : Unknown, ORAL/TRACHEAL/ESOPHAGEAL APHTHA : not recovered/not resolved, GENITAL APHTHA : Unknown,

BILATERAL CONJUNCTIVITIS : Unknown, BALANITIS : Unknown, FEVER AGGRAVATED : Unknown.

Medical History: FLU-LIKE INFECTION(Concomitant) ,ORAL APHTHA (Past) ,GENITAL APHTHA (Past) ,GRADE TWO REFLUX OESOPHAGITIS(Concomitant) ,MODERATE AXIAL HIATAL HERNIA(Concomitant)

Action Taken with suspect Drugs: Amoxicillin ratiopharm 1000mg Filmtabletten - Drug discontinued

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: M Behcet is rated as not related to Amoxicillin treatment but to a possible genetic predisposition and other exogenous factors. However, atypical drug hypersensitivity associated with aphthae, conjunctivitis, fever and balanitis is rated as possibly related due to plausible temporal relationship and history of similar symptoms after previous Amoxicillin exposure.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case

08-Feb-2012,

Follow up information received from the hospital. The physician reported that the patient also experienced CRP INCREASED and LEUKOCYTE COUNT INCREASED, besides ORAL/TRACHEAL/ESOPHAGEAL APHTHA, GENITAL APHTHA, BILATERAL CONJUNCTIVITIS and FEVER AGGRAVATED. All previously mentioned events occurred between 02-Jan-2012 and 13-Jan-2012 and the reporter assessed the causality as probable. The patient was initially treated with Tavanic (Levofloxacin) and Roxithromycin, which was ineffective. After cortisone treatment the symptoms were improving and are recovered at the time of this report.

The events SUSPECTED MORBUS BEHCET, SUSPECTED ATYPICAL DRUG HYPERSENSITIVITY and BALANITIS were not mentioned in the follow up report.

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: Due to data available (see source documents, differential diagnostics, physicians statements and so on) it is not clear, so far, whether symptoms reported were due to Amoxicillin, to an M.Behcet, or other genesis, thus symptoms reported except M.Behcet are possibly related to Amoxicillin.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case

02-Mar-2012,

Follow up information received from the patient's mother. The treatment with cortisone was discontinued on a unknown date and the patient is able to work as normal.

TEVA comment: no change of latest assessment from 15-Feb-2012

Sender's comment:

Serious, spontaneous (TEVA product could not be excluded) (follow-up (2)) case report from Germany. Duty for notification according to German Drug Law. Causality assessment: see case narrative TEVA comment. Assessment of expectedness according to current German SmPC and PIL: unexpected. The risk-benefit ratio of the treatment with the drug in general can be regarded as unchanged. Changes in the statements of the German SmPC and PIL regarding special warnings, special precautions for use and undesirable effects due to this case seem not to be justified at present.

Reporter's comment:

sanofi-aventis, 25.04.2012 DE-SANOFI-AVENTIS-2012SA027495

Sender diagnosis: not reported

Narrative

Initial Report received on 19-Apr-2012 from a consumer:

This spontaneous case information involves the 28-year-old son of the reporter. The patient had been treated with ibuprofen (Ibuflam 400) as well as concomitantly with amoxicillin for a week because of a cold and fever. No improvement was noticeable but rather an aggravation of the general

condition. On 02-Jan-2012 the patient went to the hospital for two weeks. Allergy test for amoxicillin, endoscopic examination of lungs and gastroscopy showed no results. Corrective treatment: high dosage cortisone. Symptoms held on for about five weeks. The temporary medical report was marked with uncertain cause. Medical history: aphta in mouth and on other mucus membranes, conjunctivitis. No further information was reported.

Sender's comment:

23-Apr-2012 Sanofi-Aventis Germany

Taking into consideration the compatible temporal relationship a causal role of ibuprofen cannot be excluded. However more details especially on medical history, further diagnostics and on specific circumstances surrounding the event would be helpful for a proper medical assessment of the case. Alternative explanation includes the also suspected medication of amoxicillin.

Reporter's comment:

TEVA, 04.05.2012 DE-TEVA-315991GER

Sender diagnosis: not reported

Narrative

04-Jan-2012, Spontaneous Serious Report,

A Physician reported the case of a Male patient 28 Years of age who experienced SUSPECTED MORBUS BEHCET, SUSPECTED ATYPICAL DRUG HYPERSENSITIVITY, ORAL/TRACHEAL/ESOPHAGEAL APHTHAE, GENITAL APHTHAE, BILATERAL CONJUNCTIVITIS, BALANITIS, FEVER AGGRAVATED while on Amoxicillin ratiopharm 1000mg Filmtabletten (AMOXICILLIN) (Teva's product).

On 24-Dec-2011 the patient developed a febrile flu-like infection. On 25-Dec-2011 treatment with amoxicillin was started (Dose unknown). No concomitant medication reported. The disease symptoms were improving, however, the patient developed approx. three days after treatment initiation a conjunctivitis, oral aphtae and a aggravation of the fever. On 02-Jan-2011 the patient was hospitalized with moderately reduced general condition, bilateral conjunctivitis, oral and genital aphtae, balanitis. The following test were performed in the hospital (additional test results were also provided):

HLA B51 negative

Blood culture: negative

Mouth swab: Haemophilus parainfluenza, bacteria of the oral flora, Serratia marcescens

HIV-Test: negative

ANA, ANC, rheumatoid factor, circulating immune complexes, AAB against each CCP negative.

Mycoplasma antibodies: results following.

Bronchial lavage: no evidence of acid-fast bacilli. TBC negative

Due to the severe oral aphtae the patients' food and fluid intake was strongly impaired. The CRP was significantly increased. No pathogens were detected in blood cultures. Initial antibiotic therapy with levofloxacin could hardly improve any clinical and laboratory parameter. After cortisone administration, however, a rapid improvement in general health status and a falling CRP was observed.

Due to the clinical appearance and despite negative HLAB51 and for Behcet's atypical histology and negative pathergy test, the physicians suspected a Behcet's disease. In addition, an unusual drug reaction to amoxicillin was suspected as differential diagnosis. In addition, the patient was also treated with amoxicillin in Mar-2011 for a infection and developed oral and genital aphtae in a milder form.

The patient was discharged on 13-Jan-2012. On 17-Jan-2012, the oral aphtae were not recovered and the outcome of the other AE's were not reported. However, the mother of the patient stated that there where no genital aphthae in 2012 (clearly described in the physician's report).

Treatment of the AE's: Levofloxacin (dose unknown); Prednisone 40 mg/d;

At the time of the report the outcome of the AEs were: SUSPECTED MORBUS BEHCET : Unknown, SUSPECTED ATYPICAL DRUG HYPERSENSITIVITY : Unknown, ORAL/TRACHEAL/ESOPHAGEAL APHTHAE : not recovered/not resolved, GENITAL APHTHAE : Unknown, BILATERAL CONJUNCTIVITIS : Unknown, BALANITIS : Unknown, FEVER AGGRAVATED : Unknown.

Medical History: FLU-LIKE INFECTION(Concomitant) ,ORAL APHTHAE (Past) ,GENITAL APHTHAE (Past) ,GRADE TWO REFLUX OESOPHAGITIS(Concomitant) ,MODERATE AXIAL HIATAL HERNIA(Concomitant)

Action Taken with suspect Drugs: Amoxicillin ratiopharm 1000mg Filmtabletten - Drug discontinued

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: M Behcet is rated as not related to Amoxicillin treatment but to a possible genetic predisposition and other exogenous factors. However, atypical drug hypersensitivity associated with aphthae, conjunctivitis, fever and balanitis is rated as possibly related due to plausible temporal relationship and history of similar symptoms after previous Amoxicillin exposure.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case

08-Feb-2012,

Follow up information received from the hospital. The physician reported that the patient also experienced CRP INCREASED and LEUKOCYTE COUNT INCREASED, besides ORAL/TRACHEAL/ESOPHAGEAL APHTHA, GENITAL APHTHA, BILATERAL CONJUNCTIVITIS and FEVER AGGRAVATED. All previously mentioned events occurred between 02-Jan-2012 and 13-Jan-2012 and the reporter assessed the causality as probable. The patient was initially treated with Tavanic (Levofloxacin) and Roxithromycin, which was ineffective. After cortisone treatment the symptoms were improving and are recovered at the time of this report.

The events SUSPECTED MORBUS BEHCET, SUSPECTED ATYPICAL DRUG HYPERSENSITIVITY and BALANITIS were not mentioned in the follow up report.

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: Due to data available (see source documents, differential diagnostics, physicians statements and so on) it is not clear, so far, whether symptoms reported were due to Amoxicillin, to an M.Behcet, or other genesis, thus symptoms reported except M.Behcet are possibly related to Amoxicillin.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case

02-Mar-2012,

Follow up information received from the patient's mother. The treatment with cortisone was discontinued on a unknown date and the patient is able to work as normal.

TEVA comment: no change of latest assessment from 15-Feb-2012

25-Apr-2012,

Follow up information received from the patient's mother. The patient underwent a amoxicillin allergy blood test (not further specified), which was negative. In addition, the patient was treated with Ibufam 400 (IBUPROFEN; No Teva product) in parallel with amoxicillin (no therapy details reported) and the patient's mother also suspected this drug or a interplay between both drugs as a possible cause for his son's symptoms.

Teva Comment: no change of latest assessment from 02-Mar-2012 despite results of allergy testing

Sender's comment:

Serious, spontaneous (TEVA product could not be excluded) (follow-up (3)) case report from Germany. Duty for notification according to German Drug Law. Causality assessment: see case narrative TEVA comment. Assessment of expectedness according to current German SmPC and PIL: unexpected. The risk-benefit ratio of the treatment with the drug in general can be regarded as unchanged. Changes in the statements of the German SmPC and PIL regarding special warnings, special precautions for use and undesirable effects due to this case seem not to be justified at present.

Reporter's comment:

sanofi-aventis, 11.05.2012 DE-SANOFI-AVENTIS-2012SA027495

Sender diagnosis: not reported

Narrative

Initial Report received on 19-Apr-2012 from a consumer:

This spontaneous case information involves the 28-year-old son of the reporter. The patient had been treated with ibuprofen (Ibufam 400) as well as

concomitantly with amoxicillin for a week because of a cold and fever. No improvement was noticeable but rather an aggravation of the general condition. On 02-Jan-2012 the patient went to the hospital for two weeks. According to the mother of the patient allergy test for amoxicillin, endoscopic examination of lungs and gastroscopy showed no results. Symptoms held for about five weeks.

Hospital report: ibuprofen was not mentioned in this report. An unusual form of allergy to amoxicillin was suspected. Because of the medical findings and clinical suspicion of Morbus Behçet a skin biopsy was initiated. Despite negative result a Morbus Behçet was still considered as possible differential diagnosis. Corrective treatment: successful treatment with high dosage cortisone.

Medical history: aphtha in mouth and on other mucus membranes, conjunctivitis (similar symptoms occurred also during previous amoxicillin intake).

Additional information received on 09-May-2012 from the mother of the patient:

Hospital report: skin biopsy, allergy suspicion, successful corrective treatment

Sender's comment:

10-May-2012 Sanofi-Aventis Germany

Taking into consideration the compatible temporal relationship a causal role of ibuprofen cannot be excluded. Based on the available hospital report a Morbus Behçet can be considered as alternative explanation. In addition an allergic reaction to amoxicillin seems also possible.

Reporter's comment:

sanofi-aventis, 25.06.2012 DE-SANOFI-AVENTIS-2012SA027495

Sender diagnosis: not reported

Narrative

Initial Report received on 19-Apr-2012 from a consumer:

This spontaneous case information involves the 28-year-old son of the reporter (patient's mother). The patient had been treated with ibuprofen (Ibuprofen 400) for a week because of a cold and fever (high-feverish infection with weakness) and amoxicillin from 24-Dec-2011 till 26-Dec-2011. Subsequently, he developed esophageal, oral tracheal and genital aphthae and conjunctivitis (both eyes) and his general condition did not improve but aggravated. He could not eat and drink because of severe oral inflammations. The patient was hospitalized from 02-Jan-2012 until 13-Jan-2012. The patient's mother stated that unfortunately, her son as well as the family had completely forgotten to provide the hospital physician with information, that the patient had also taken ibuprofen besides amoxicillin. According to the mother of the patient allergy test for amoxicillin, endoscopic examination of lungs and gastroscopy revealed no results. Symptoms held for about five weeks.

From the hospital report: ibuprofen was not mentioned within this report. Laboratory findings: significant increase in inflammation parameters; blood culture: no isolation of any pathogen; oesophagoduodenoscopy: oesophageal ulceration; histology: oesophagitis; bronchoscopy: tracheal ulcerations; histology: moderate chronic and acute tracheitis. Abdominal sonography: no focus of infection could be found. Because of the medical findings and clinical suspicion of Behçet's disease a skin biopsy, patch test and autoimmune serology were done. Despite negative result of autoimmune serology, negative patch test and histological finding, which was not typical for Behçet's disease, the latter was still considered as a possible differential diagnosis. An unusual form of allergy to amoxicillin could not be excluded and therefore the latter was taken into consideration as a further differential diagnosis together with erythema exsudativum multiforme (because of conjunctivitis, oral tracheal, oesophageal and genital aphthae). Corrective treatment: initially, antibiotic treatment with levofloxacin did not result in any clinical and laboratory improvement; local treatment of aphthae with preparations mixed by the pharmacist, kanamycin eye-drops for conjunctivitis, high dosage cortisone. A therapeutic attempt with cortisone resulted in rapid clinical improvement and decrease in laboratory parameters. Due to the particular constellation of laboratory findings as with mycoplasma infection, the treatment with roxithromycin was started too.

Medical history: similar symptoms (aphthae in mouth and genital aphthae with conjunctivitis) occurred also during the previous amoxicillin intake in March 2011 because of unspecified infection.

Additional information received on 09-May-2012 from the mother of the patient:

Hospital report: skin biopsy, allergy suspicion, successful corrective treatment

Additional information received on 19-Jun-2012 from hospital physician:

Patient data, medical history, hospital dates, symptoms were amended (according to the ADR-form and hospital discharge report received from the hospital physician).

Sender's comment:

21-JUN-2012 Sanofi Company Comment upon receipt of the follow-up information dated 19-Jun-2012: taking into consideration the compatible temporal relationship a causal role of ibuprofen cannot be excluded. Based on the available hospital report a Behçet's disease was suspected (despite

negative skin biopsy, autoimmune serology and pethergy test) and unusual form of allergy against amoxicillin as well as erythema exudativum multiforme were taken into consideration as differential diagnoses. Concomitant intake of amoxicillin with occurrence of similar symptoms (aphthae in mouth, genital apthae and conjunctivitis) after previous amoxicillin intake in March 2011 because of unspecified infection) could be discussed as other possible cause. Furthermore, mycoplasma infection (according to the particular constellation of laboratory findings) is well known to be associated with erythema exudativum multiforme (differential diagnosis).

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12020301

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 41 Year	weight (kg): 78	BMI_int: 25.47	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Burning skin	20120127			1 Day	recovered/resolved
2 Swelling	20120127			1 Day	recovered/resolved
3 Redness	20120127			1 Day	recovered/resolved
4 Itching all over	20120127			1 Day	recovered/resolved
5 Redness generalised	20120127			1 Day	recovered/resolved
6 Joint swelling	20120127			1 Day	recovered/resolved
7 Burning skin	20120127			1 Day	recovered/resolved
8 Burning sensation in joints	20120127			1 Day	recovered/resolved
9 Blistering	20120127			1 Day	recovered/resolved
10 Blistering	20120127			1 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibu-ratiopharm 400 mg ibuprofen	Oral	400 Mg milligram(s)/Day	20120127-20120127	Headache

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Allergy			Yes	to iodine
Smoker			Unknown	
Headache			Yes	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

TEVA, 13.02.2012 DE-TEVA-321046GER

Sender diagnosis: not reported

Narrative

03-Feb-2012, Spontaneous Serious Report.

(COMMISSION OF GERMAN PHARMACISTS; 167175)

A Pharmacist reported the case of a Male patient 41 Years of age who experienced BLISTERING SKIN OF WHOLE BODY, ITCHING OF WHOLE BODY, REDNESS OF WHOLE BODY, SWELLING OF THE WHOLE BODY, BURNING OF THE WHOLE BODY, BLISTERING JOINTS, REDNESS OF JOINTS, JOINT SWELLING, BURNING IN JOINTS while on Ibu-ratiopharm 400 mg (IBUPROFEN) (Teva's drug)).

At the time of the report the outcome of the AEs were: BLISTERING SKIN OF WHOLE BODY : recovered/resolved, ITCHING OF WHOLE BODY : recovered/resolved, REDNESS OF WHOLE BODY : recovered/resolved, SWELLING OF THE WHOLE BODY : recovered/resolved, BURNING OF THE WHOLE BODY : recovered/resolved, BLISTERING JOINTS : recovered/resolved, REDNESS OF JOINTS : recovered/resolved, JOINT SWELLING : recovered/resolved, BURNING IN JOINTS : recovered/resolved.

The reported events occurred after 30 min. of use of Ibu-ratiopharm 400 mg first on joints and then on the whole body.

Medical History: HEADACHE(Concomitant) ,ALLERGY(Concomitant) ,SMOKER(Concomitant)

Action Taken with suspect Drugs: Ibu-ratiopharm 400 mg - Drug discontinued.

Therapy: Healing earth was used to relieve ADR's.

This case was considered serious based on the following criteria: (Other Serious (Important Medical Events))

Teva Comment: Due to close temporal relation Symptoms reported are possibly related to Ibuprofen in the sense of hypersensitivity reaction.

Sender's comment:

Serious, spontaneous (TEVA product could not be excluded) case report from Germany. Duty for notification according to German Drug Law. Causality assessment: see case narrative TEVA comment. Assessment of expectedness according to current German SmPC and PIL: expected. Within allergic reaction skin rash and itching are listed. Furthermore severe skin disorders are listed in this context. The risk-benefit ratio of the treatment with the drug in general can be regarded as unchanged. Changes in the statements of the German SmPC and PIL regarding special warnings, special precautions for use and undesirable effects due to this case seem not to be justified at present.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-12088444

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 15 Year	weight (kg): 62.2	BMI_int: 25.89	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Juvenile idiopathic arthritis	20120405			661 Day	Unknown
2 DRESS syndrome	20120405	20120416	12 Day	661 Day	recovered/resolved
3 General body pain	20120405	20120416	12 Day	661 Day	recovered/resolved
4 Rash	20120314	20120613	2 Month	639 Day	recovered/resolved
5 Condition aggravated	20120405	20120416	12 Day	661 Day	recovered/resolved
6 Condition aggravated	20120405	20120416	12 Day	661 Day	recovered/resolved
7 Juvenile idiopathic arthritis	20120405			661 Day	Unknown
8 General body pain	20120405	20120416	12 Day	661 Day	recovered/resolved
9 Rash	20120314	20120613	2 Month	639 Day	recovered/resolved
10 DRESS syndrome	20120405	20120416	12 Day	661 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ACZ885 canakinumab		249 Mg milligram(s)/ 194	20100803- -	Juvenile idiopathic arthritis
2 ETANERCEPT etanercept		/	20050104-20060509 16 Month	
3 ibuprofen ibuprofen	Oral	800 Mg milligram(s)/	20100615-20120412 21 Month	
4 MTX methotrexate sodium	Oral	15 Mg milligram(s)/Week	20100615- -	Juvenile idiopathic arthritis

Drugs (concomitant)

5 OMEPRAZOLE omeprazole	Oral	40 Mg milligram(s)/Day	20100615-20120412 21 Month	Gastritis
6 PANTOZOL pantoprazole sodium	Oral	20 Mg milligram(s)/Day	20120412- -	Gastritis
7 PREDNISOLONE prednisolone		/	- -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Gastritis			Unknown	

Primary source

Qualification:	Physician	observestudytype:	Clinical trials
Literature:			

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 27.04.2012 DE-NOVNCHHQ-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in patients with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included Omeprazol, pantoprazol (pantoprazole) and MTX. Study medication (canakinumab) was commenced on 03 Aug 2010 at a dose of 249 mg. From 31 Jan 2012 to 01 Feb 2012, the subject took ibuprofen. On 14 Mar 2012, the subject presented with rash. On 05 Apr 2012, the subject presented with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput), whole body pain and flair. The subject was hospitalized on an unspecified date. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l(normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l),CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flair. On 18 Apr 2012, the subject received most recent dose of study medication. The subject's condition was improving from rash as of 18 Apr 2012. The investigator assessed the events as serious (hospitalization).

The investigator did not suspect a relationship between these events and the study medication but the event DRESS syndrome was assessed as suspected to ibuprofen.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

Ilaris: Agree with PI assessment

Reporter's comment:

NOVARTIS, 30.04.2012 DE-SZ09-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in patients with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included Omeprazol, pantoprazol (pantoprazole) and MTX. Study medication (canakinumab) was commenced on 03 Aug 2010 at a dose of 249 mg. From 31 Jan 2012 to 01 Feb 2012, the subject took ibuprofen. On 14 Mar 2012, the subject presented with rash. On 05 Apr 2012, the subject presented with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput), whole body pain and flair. The subject was hospitalized on an unspecified date. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l(normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l),CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flair. On 18 Apr 2012, the subject received most recent dose of study medication. The subject's condition was improving from rash as of 18 Apr 2012. The investigator assessed the events as serious (hospitalization).

The investigator did not suspect a relationship between these events and the study medication but the event DRESS syndrome was assessed as suspected to ibuprofen.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

Ilaris: Agree with PI assessment

Reporter's comment:

NOVARTIS, 07.05.2012 DE-NOVNCHHQ-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in patients with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included Omeprazol, pantoprazole and MTX. Study medication (canakinumab) was commenced on 03 Aug 2010 at a dose of 249 mg. From 31 Jan 2012 to 01 Feb 2012, the subject took ibuprofen. On 14 Mar 2012, the subject presented with rash. On 05 Apr 2012, the subject was diagnosed with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare. The subject was hospitalized on 08 Apr 2012. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l(normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l),CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flare and was discharged on the same day. On 18 Apr 2012, the subject received most recent dose of study medication. The subject's condition was improving from rash as of 18 Apr 2012. The investigator assessed the events as serious (hospitalization).

The investigator did not suspect a relationship between these events and the study medication but the event DRESS syndrome was assessed as suspected to ibuprofen.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Follow-up report received on 30 Apr 2012: Updated final diagnosis and hospitalization dates.

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

Ilaris: Agree with PI assessment

Reporter's comment:

NOVARTIS, 11.05.2012 DE-SZ09-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in patients with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included Omeprazol, pantoprazole and MTX. Study medication (canakinumab) was commenced on 03 Aug 2010 at a dose of 249 mg. From 31 Jan 2012 to 01 Feb 2012, the subject took ibuprofen. On 14 Mar 2012, the subject presented with rash. On 05 Apr 2012, the subject was diagnosed with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare. The subject was hospitalized on 08 Apr 2012. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l(normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l),CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flare and was discharged on the same day. On 18 Apr 2012, the subject received most recent dose of study medication. The subject's condition was improving from rash as of 18 Apr 2012. The investigator assessed the events as serious (hospitalization).

The investigator did not suspect a relationship between these events and the study medication but the event DRESS syndrome was assessed as suspected to ibuprofen.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Follow-up report received on 30 Apr 2012: Updated final diagnosis and hospitalization dates.

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

Ilaris: Agree with PI assessment

Reporter's comment:

McNeil Germany, Local Safety Officers, 18.05.2012 DE-JNJFOC-20120511860

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician via a business partner Pfizer and concerns a 15-year-old female patient from Germany: 2012110808.

The patient participated in German Biologics in Pediatric Rheumatology Registry (BIKER), a non-interventional study source for protocol number: B1801310 and center ID/subject ID: E060524.

The patient's height was not reported and weight was 62.4 kilograms. The patient's medical history was not reported. The patient was treated with ibuprofen lysinate (film coated tablets, unspecified) 600 mg 2x1 tablet as needed, initiated on an unspecified date for an unspecified indication. Non-company suspect drugs included: methotrexate sodium (unspecified formulation and route) 15 mg once weekly initiated on an unspecified date for an unspecified indication and Canakinumab (immunosuppressants) (unspecified formulation and route), 4 mg/kg 1 in 1 month, for an unspecified indication. Concomitant medications included etanercept, pantoprazole sodium, omeprazole and prednisolone. On 14-MAR-2012, the patient had experienced rash (symptom of DRESS Syndrome). On 05-APR-2012, the patient experienced flare, whole body pain and blood count changes

(symptoms of DRESS Syndrome) for which she was hospitalized. Laboratory data on 08-APR-2012 included: alanine aminotransferase: 44 IU/l (Normal Range: < 35 IU/l), aspartate aminotransferase: 121 IU/l (Normal Range: 16 - 46 IU/l), Blood creatine phospho kinase (CPK): 594 IU/l (Normal Range: <123 IU/l), C-Reactive Protein: 35.5 mg/l (Normal Range: 0 - 5 mg/l), hemoglobin: 13.1 g/dl (Normal Range: 11.7 - 14.7 g/dl), blood lactate dehydrogenase (LDH): 910 IU/l (Normal Range: 135 - 214 IU/l), thrombocyte count $78 \times 10^9/l$ (Normal Range: $150 - 400 \times 10^9/l$), white blood cell count $2.33 \times 10^9/l$ (Normal Range: $4.5 - 11.0 \times 10^9/l$ and Ferritin 3162 g/l (normal range unspecified). Treatment details were not provided. The patient was discharged from the hospital on 16-APR-2012. Treatment with ibuprofen lysinate was withdrawn on 12-APR-2012 and the dose of methotrexate sodium and Canakinumab (immunosuppressants) was not changed. The patient had recovered from rash, flare, whole body pain and blood count changes (symptoms of DRESS Syndrome) on 16-APR-2012. The physician reported that the event was possibly related to methotrexate, canakinumab and ibuprofen lysinate and not related to prednisolone or etanercept. This report was serious (hospitalization).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Single case assessment (reported term/preferred term): IBUPROFEN LYSINATE (Film Coated Tablet)

DRESS SYNDROME / DRUG RASH WITH EOSINOPHILIA : unexpected per local SmPc

The benefit-risk-ratio remains unchanged based on the single case assessment of this case. No measures according to § 29 (1) AMG have to be taken.

Reporter's comment:

NOVARTIS, 21.05.2012 DE-NOVNCHHQ-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in patients with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included prednisolone, omeprazole, pantozol, ibuprofen, etanercept and MTX. Study medication (canakinumab) was commenced on 02 Aug 2010 at a dose of 249 mg. The subject took omeprazole from 15 June 2010 to 12 Apr 2012 and then switched to pantozol from 12 Apr 2012. Ibuprofen was taken as on demand from 15 Jun 2010 to 12 Apr 2012. MTX was started on 15 Jun 2009 and currently ongoing. On 14 Mar 2012, the subject presented with rash. Last dose of study medication was received by the subject on 23 Mar 2012. On 05 Apr 2012, the subject was diagnosed with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare. The subject was hospitalized on 08 Apr 2012. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l(normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l),CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. Treatment with ibuprofen and omeprazole were permanently discontinued on 12 Apr 2012. Treatment with MTX and pantozol were not altered due to these events. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flare and was discharged on the same day. On 18 Apr 2012, the subject received most recent dose of study medication. The subject's condition was improving from rash as of 18 Apr 2012. The investigator assessed these events (except rash) were serious (hospitalization). The investigator assessed the event "rash" as non-serious.

The investigator did not suspect a relationship between these events and the study medication. The investigator assessed the events "DRESS syndrome and rash" were related to ibuprofen.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Follow-up report received on 30 Apr 2012: Updated final diagnosis and hospitalization dates.

Follow-up report received on 15 May 2012: Updated the new suspected co-medications (study medication, etanercept and MTX), start date of study medication, concomitant medication (prednisolone), status of ibuprofen, seriousness criteria of the event "rash" (previously reported as serious) and suspectedness of the event "rash" (previously reported as not suspected).

body pain/flare and rash" with the study medication as not related, last dose of the study medication (21 Mar 2012) and therapy dates of "ibuprofen, omeprazole, MTX and pantozol.

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

Ilaris: Agree with PI assessment

Reporter's comment:

NOVARTIS, 22.05.2012 DE-SZ09-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in patients with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included prednisolone, omeprazole, pantozol, ibuprofen, etanercept and MTX. Study medication (canakinumab) was commenced on 02 Aug 2010 at a dose of 249 mg. The subject took omeprazole from 15 June 2010 to 12 Apr 2012 and then switched to pantozol from 12 Apr 2012. Ibuprofen was taken as on demand from 15 Jun 2010 to 12 Apr 2012. MTX was started on 15 Jun 2009 and currently ongoing. On 14 Mar 2012, the subject presented with rash. Last dose of study medication was received by the subject on 23 Mar 2012. On 05 Apr 2012, the subject was diagnosed with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare. The subject was hospitalized on 08 Apr 2012. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l(normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l),CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. Treatment with ibuprofen and omeprazole were permanently discontinued on 12 Apr 2012. Treatment with MTX and pantozol were not altered due to these events. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flare and was discharged on the same day. On 18 Apr 2012, the subject received most recent dose of study medication. The subject's condition was improving from rash as of 18 Apr 2012. The investigator assessed these events (except rash) were serious (hospitalization). The investigator assessed the event "rash" as non-serious.

The investigator did not suspect a relationship between these events and the study medication. The investigator assessed the events "DRESS syndrome and rash" were related to ibuprofen.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Follow-up report received on 30 Apr 2012: Updated final diagnosis and hospitalization dates.

Follow-up report received on 15 May 2012: Updated the new suspected co-medications (study medication, etanercept and MTX), start date of study medication, concomitant medication (prednisolone), status of ibuprofen, seriousness criteria of the event "rash" (previously reported as serious) and suspectedness of the event "rash" (previously reported as not suspected).

Follow-up report received on 21 May 2012 (prior the circulation of previous report): Updated the causality of the events "DRESS syndrome, whole body pain/flare and rash" with the study medication as not related, last dose of the study medication (21 Mar 2012) and therapy dates of "ibuprofen,

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

Ilaris: Agree with PI assessment

Reporter's comment:

NOVARTIS, 25.06.2012 DE-NOVNCHHQ-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in patients with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included prednisolone, omeprazole, pantozol, ibuprofen, etanercept and MTX. Study medication (canakinumab) was commenced on 03 Aug 2010 at a dose of 262 mg. The subject took omeprazole from 15 Jun 2010 to 12 Apr 2012 and then switched to pantozol from 12 Apr 2012. Ibuprofen was taken as on demand from 15 Jun 2010 to 12 Apr 2012. MTX was started on 15 Jun 2009 and currently ongoing. On 14 Mar 2012, the subject presented with rash. Last dose of study medication was received by the subject on 23 Mar 2012. On 05 Apr 2012, the subject was diagnosed with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare. The subject was hospitalized on 08 Apr 2012. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l (normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l), CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. Treatment with ibuprofen and omeprazole were permanently discontinued on 12 Apr 2012. Treatment with MTX and pantozol were not altered due to these events. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flare and was discharged on the same day. On 18 Apr 2012, the subject received most recent dose of study medication. On 13 Jun 2012, the subject's laboratory tests included ALT of 36 u/l, AST of 31 u/l, CRP of 1.3 mg/l, leukocytes of 3.95/ml, platelets of 223/ml and hemoglobin 12.3 g/dl. The subject also completely recovered from rash on 13 Jun 2012. The investigator assessed these events were serious (hospitalization).

The investigator did not suspect a relationship between these events and the study medication. The investigator assessed the events "DRESS syndrome and rash" were related to ibuprofen.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Follow-up report received on 30 Apr 2012: Updated final diagnosis and hospitalization dates.

Follow-up report received on 15 May 2012: Updated the new suspected co-medications (study medication, etanercept and MTX), start date of study medication, concomitant medication (prednisolone), status of ibuprofen, seriousness criteria of the event "rash" (previously reported as serious) and suspectedness of the event "rash" (previously reported as not suspected).

Follow-up report received on 21 May 2012 (prior the circulation of previous report): Updated the causality of the events "DRESS syndrome, whole body pain/flare and rash" with the study medication as not related, last dose of the study medication (21 Mar 2012) and therapy dates of "ibuprofen, omeprazole, MTX and pantozol.

Follow-up report received on 20 Jun 2012: Updated the laboratory reports dated 13 Jun 2012, outcome of the event "rash" as complete recovery and seriousness criteria for the event "rash" as hospitalization (previously reported as non serious).

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

Ilaris: Agree with PI assessment

Reporter's comment:

NOVARTIS, 26.06.2012 DE-SZ09-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in patients with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included prednisolone, omeprazole, pantozol, ibuprofen, etanercept and MTX. Study medication (canakinumab) was commenced on 03 Aug 2010 at a dose of 262 mg. The subject took omeprazole from 15 Jun 2010 to 12 Apr 2012 and then switched to pantozol from 12 Apr 2012. Ibuprofen was taken as on demand from 15 Jun 2010 to 12 Apr 2012. MTX was started on 15 Jun 2009 and currently ongoing. On 14 Mar 2012, the subject presented with rash. Last dose of study medication was received by the subject on 23 Mar 2012. On 05 Apr 2012, the subject was diagnosed with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare. The subject was hospitalized on 08 Apr 2012. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l (normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l), CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. Treatment with ibuprofen and omeprazole were permanently discontinued on 12 Apr 2012. Treatment with MTX and pantozol were not altered due to these events. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flare and was discharged on the same day. On 18 Apr 2012, the subject received most recent dose of study medication. On 13 Jun 2012, the subject's laboratory tests included ALT of 36 u/l, AST of 31 u/l, CRP of 1.3 mg/l, leukocytes of 3.95/ml, platelets of 223/ml and hemoglobin 12.3 g/dl. The subject also completely recovered from rash on 13 Jun 2012. The investigator assessed these events were serious (hospitalization).

The investigator did not suspect a relationship between these events and the study medication. The investigator assessed the events "DRESS syndrome and rash" were related to ibuprofen.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Follow-up report received on 30 Apr 2012: Updated final diagnosis and hospitalization dates.

Follow-up report received on 15 May 2012: Updated the new suspected co-medications (study medication, etanercept and MTX), start date of study medication, concomitant medication (prednisolone), status of ibuprofen, seriousness criteria of the event "rash" (previously reported as serious) and suspectedness of the event "rash" (previously reported as not suspected).

Follow-up report received on 21 May 2012 (prior the circulation of previous report): Updated the causality of the events "DRESS syndrome, whole body pain/flare and rash" with the study medication as not related, last dose of the study medication (21 Mar 2012) and therapy dates of "ibuprofen, omeprazole, MTX and pantozol.

Follow-up report received on 20 Jun 2012: Updated the laboratory reports dated 13 Jun 2012, outcome of the event "rash" as complete recovery and seriousness criteria for the event "rash" as hospitalization (previously reported as non serious).

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator

causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

Ilaris: Agree with PI assessment

Reporter's comment:

NOVARTIS, 20.09.2012 DE-SZ09-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in subjects with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included prednisolone, omeprazole, pantozol, ibuprofen, etanercept and MTX. Study medication (canakinumab) was commenced on 03 Aug 2010 at a dose of 262 mg. The subject took omeprazole from 15 Jun 2010 to 12 Apr 2012 and then switched to pantozol from 12 Apr 2012. Ibuprofen was taken as on demand from 15 Jun 2010 to 12 Apr 2012. MTX was started on 15 Jun 2009 and currently ongoing. On 14 Mar 2012, the subject presented with rash. Last dose of study medication was received by the subject on 23 Mar 2012. On 05 Apr 2012, the subject was diagnosed with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare. The subject was hospitalized on 08 Apr 2012. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l (normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l), CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. Treatment with ibuprofen and omeprazole were permanently discontinued on 12 Apr 2012. Treatment with MTX and pantozol were not altered due to these events. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flare and was discharged on the same day. On 18 Apr 2012, the subject received most recent dose of study medication. On 13 Jun 2012, the subject's laboratory tests included ALT of 36 u/l, AST of 31 u/l, CRP of 1.3 mg/l, leukocytes of 3.95/ml, platelets of 223/ml and hemoglobin 12.3 g/dl. The subject also completely recovered from rash on 13 Jun 2012. Again on 11 Sep 2012, the subject was hospitalized for worsening of systemic juvenile idiopathic arthritis (second occurrence). Intraarticular injection left foot was planned on 12 Sep 2012. At the time of reporting, outcome of the event "worsening of systemic juvenile idiopathic arthritis (second occurrence)" was not reported. The investigator assessed these events were serious (hospitalization).

The investigator did not suspect a relationship between the events "DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare" with the study medication. The investigator assessed the events "DRESS syndrome and rash" were related to ibuprofen. The investigator did not assess the event "worsening of systemic juvenile idiopathic arthritis (second occurrence)" with the study medication.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Follow-up report received on 30 Apr 2012: Updated final diagnosis and hospitalization dates.

Follow-up report received on 15 May 2012: Updated the new suspected co-medications (study medication, etanercept and MTX), start date of study medication, concomitant medication (prednisolone), status of ibuprofen, seriousness criteria of the event "rash" (previously reported as serious) and suspectedness of the event "rash" (previously reported as not suspected).

Follow-up report received on 21 May 2012 (prior the circulation of previous report): Updated the causality of the events "DRESS syndrome, whole body pain/flare and rash" with the study medication as not related, last dose of the study medication (21 Mar 2012) and therapy dates of "ibuprofen, omeprazole, MTX and pantozol.

Follow-up report received on 20 Jun 2012: Updated the laboratory reports dated 13 Jun 2012, outcome of the event "rash" as complete recovery and seriousness criteria for the event "rash" as hospitalization (previously reported as non serious).

Follow-up report received on 11 Sep 2012: Updated the second occurrence of the event "worsening of systemic juvenile idiopathic arthritis" and it's clinical course.

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

llaris: Agree with PI assessment

ACZ: Flare or aggravation of the underlying SJIA is not uncommon to occur.

Reporter's comment:

NOVARTIS, 27.09.2012 DE-SZ09-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in subjects with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included prednisolone, omeprazole, pantozol, ibuprofen, etanercept and MTX. Study medication (canakinumab) was commenced on 03 Aug 2010 at a dose of 262 mg. The subject took omeprazole from 15 Jun 2010 to 12 Apr 2012 and then switched to pantozol from 12 Apr 2012. Ibuprofen was taken as on demand from 15 Jun 2010 to 12 Apr 2012. MTX was started on 15 Jun 2009 and currently ongoing. On 14 Mar 2012, the subject presented with rash. Last dose of study medication was received by the subject on 23 Mar 2012. On 05 Apr 2012, the subject was diagnosed with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare. The subject was hospitalized on 08 Apr 2012. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l (normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l), CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. Treatment with ibuprofen and omeprazole were permanently discontinued on 12 Apr 2012. Treatment with MTX and pantozol were not altered due to these events. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flare and was discharged on the same day. On 18 Apr 2012, the subject received most recent dose of study medication. On 13 Jun 2012, the subject's laboratory tests included ALT of 36 u/l, AST of 31 u/l, CRP of 1.3 mg/l, leukocytes of 3.95/ml, platelets of 223/ml and hemoglobin 12.3 g/dl. The subject also completely recovered from rash on 13 Jun 2012. Again on 11 Sep 2012, the subject was hospitalized due to arthritis left ankle nad right wrist . The subject was diagnosed with worsening of systemic juvenile idiopathic arthritis (second occurrence). Intraarticular injection left foot was planned on 12 Sep 2012. On 13 Sep 2012 the CRP was 0.6 mg/l, white blood count was 4.84/nl, hemoglobin was 12.7 and thrombocytes were 187/nl. At the time of reporting, outcome of the event "worsening of systemic juvenile idiopathic arthritis (second occurrence)" was not reported. The investigator assessed these events were serious (hospitalization).

The investigator did not suspect a relationship between the events "DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare" with the study medication. The investigator assessed the events "DRESS syndrome and rash" were related to ibuprofen. The investigator did not suspect any relationship between the event "worsening of systemic juvenile idiopathic arthritis (second occurrence)" with the study medication.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Follow-up report received on 30 Apr 2012: Updated final diagnosis and hospitalization dates.

Follow-up report received on 15 May 2012: Updated the new suspected co-medications (study medication, etanercept and MTX), start date of study medication, concomitant medication (prednisolone), status of ibuprofen, seriousness criteria of the event "rash" (previously reported as serious) and suspectedness of the event "rash" (previously reported as not suspected).

Follow-up report received on 21 May 2012 (prior the circulation of previous report): Updated the causality of the events "DRESS syndrome, whole body pain/flare and rash" with the study medication as not related, last dose of the study medication (21 Mar 2012) and therapy dates of "ibuprofen,

omeprazole, MTX and pantozol.

Follow-up report received on 20 Jun 2012: Updated the laboratory reports dated 13 Jun 2012, outcome of the event "rash" as complete recovery and seriousness criteria for the event "rash" as hospitalization (previously reported as non serious).

Follow-up report received on 11 Sep 2012: Updated the second occurrence of the event "worsening of systemic juvenile idiopathic arthritis" and its clinical course.

Follow up received on 19 Sep 2012: The symptoms, lab data, confirmed diagnosis and causality of SJIA were updated.

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

llaris: Agree with PI assessment

ACZ: Flare or aggravation of the underlying SJIA is not uncommon to occur.

Reporter's comment:

NOVARTIS, 13.12.2012 DE-SZ09-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in subjects with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included prednisolone, omeprazole, pantozol, ibuprofen, etanercept and MTX. Study medication (canakinumab) was commenced on 03 Aug 2010 at a dose of 262 mg. The subject took omeprazole from 15 Jun 2010 to 12 Apr 2012 and then switched to pantozol from 12 Apr 2012. Ibuprofen was taken as on demand from 15 Jun 2010 to 12 Apr 2012. MTX was started on 15 Jun 2009 and currently ongoing. On 14 Mar 2012, the subject presented with rash. Last dose of study medication was received by the subject on 23 Mar 2012. On 05 Apr 2012, the subject was diagnosed with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare. The subject was hospitalized on 08 Apr 2012. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l (normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l), CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. Treatment with ibuprofen and omeprazole were permanently discontinued on 12 Apr 2012. Treatment with MTX and pantozol were not altered due to these events. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flare and was discharged on the same day. On 18 Apr 2012, the subject received most recent dose of study medication. On 13 Jun 2012, the subject's laboratory tests included ALT of 36 u/l, AST of 31 u/l, CRP of 1.3 mg/l, leukocytes of 3.95/ml, platelets of 223/ml and hemoglobin 12.3 g/dl. The subject also completely recovered from rash on 13 Jun 2012. Again on 27 Aug 2012, the subject experienced arthritis left ankle nad right wrist which resulted in hospitalization on 11 Sep 2012. The subject was diagnosed for these events with worsening of systemic juvenile idiopathic arthritis (second occurrence). Intraarticular injection left foot was planned on 12 Sep 2012. On 13 Sep 2012 the CRP was 0.6 mg/l, white blood count was 4.84/nl, hemoglobin was 12.7 and thrombocytes were 187/nl. At the time of reporting, outcome of the event "worsening of systemic juvenile idiopathic arthritis (second occurrence)" was not reported. The investigator assessed these events were serious (hospitalization).

The investigator did not suspect a relationship between the events "DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare" with the study medication. The investigator assessed the events "DRESS syndrome and rash" were related to ibuprofen. The investigator did not suspect any relationship between the event "worsening of systemic juvenile idiopathic arthritis (second occurrence)" with the study medication.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Follow-up report received on 30 Apr 2012: Updated final diagnosis and hospitalization dates.

Follow-up report received on 15 May 2012: Updated the new suspected co-medications (study medication, etanercept and MTX), start date of study medication, concomitant medication (prednisolone), status of ibuprofen, seriousness criteria of the event "rash" (previously reported as serious) and suspectedness of the event "rash" (previously reported as not suspected).

Follow-up report received on 21 May 2012 (prior the circulation of previous report): Updated the causality of the events "DRESS syndrome, whole body pain/flare and rash" with the study medication as not related, last dose of the study medication (21 Mar 2012) and therapy dates of "ibuprofen, omeprazole, MTX and pantozol.

Follow-up report received on 20 Jun 2012: Updated the laboratory reports dated 13 Jun 2012, outcome of the event "rash" as complete recovery and seriousness criteria for the event "rash" as hospitalization (previously reported as non serious).

Follow-up report received on 11 Sep 2012: Updated the second occurrence of the event "worsening of systemic juvenile idiopathic arthritis" and it's clinical course.

Follow up received on 19 Sep 2012: The symptoms, lab data, confirmed diagnosis and causality of SJIA were updated.

Follow-up DR received on 06 Dec 2012: Updated the onset date of the event "worsening of systemic juvenile idiopathic arthritis (second occurrence)".

Sender's comment:

25 Apr 2012 (Sharad G): [PROD_67A] - accepted value.

04 May 2012 (Jhansi M): [PROD_067A]: Accepted value.

23 May 2012 (Ramachandran S): Follow-up report received on 22 May 2012: No new information was received.

15 Jun 2012 (Ramachandran S): Follow-up resent process was completed, hence the tickbox was unticked in the general screen for this case.

01 Aug 2012 (Ramachandran S): Follow-up DR received on 20 Jun 2012: No new information was received.

17 Aug 2012 (Kalyani Kunduru):case requires follow up check box was unticked (information received)

25 Sep 2012(Tahera Banu): Follow up received prior to the circulation of previous report on 24 Sep 2012. No new information was provided

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-12088754

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 15 Year	weight (kg): 62.2	BMI_int: 25.89	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Rash	20120314	20120613	2 Month	639 Day	recovered/resolved
2 Condition aggravated	20120405	20120416	12 Day	661 Day	recovered/resolved
3 General body pain	20120405	20120416	12 Day	661 Day	recovered/resolved
4 DRESS syndrome	20120405	20120416	12 Day	661 Day	recovered/resolved
5 Juvenile idiopathic arthritis	20120405			661 Day	Unknown
6 Condition aggravated	20120405	20120416	12 Day	661 Day	recovered/resolved
7 DRESS syndrome	20120405	20120416	12 Day	661 Day	recovered/resolved
8 Rash	20120314	20120613	2 Month	639 Day	recovered/resolved
9 General body pain	20120405	20120416	12 Day	661 Day	recovered/resolved
10 Juvenile idiopathic arthritis	20120405			661 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ACZ885 canakinumab		249 Mg milligram(s)/ 194	20100803- -	Juvenile idiopathic arthritis
2 ETANERCEPT etanercept		/	20050104-20060509 16 Month	
3 IBUPROFEN ibuprofen	Oral	800 Mg milligram(s)/	20100615-20120412 21 Month	
4 MTX methotrexate sodium	Oral	15 Mg milligram(s)/Week	20100615- -	Juvenile idiopathic arthritis

Drugs (concomitant)

5 OMEPRAZOLE omeprazole	Oral	40 Mg milligram(s)/Day	20100615-20120412 21 Month	Gastritis
6 PANTOZOL pantoprazole sodium	Oral	20 Mg milligram(s)/Day	20120412- -	Gastritis
7 prednisolone prednisolone		/	- -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Gastritis			Unknown	

Primary source

Qualification:	Physician	observestudytype:	Clinical trials
Literature:			

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 30.04.2012 DE-002147023-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in patients with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included Omeprazol, pantoprazol (pantoprazole) and MTX. Study medication (canakinumab) was commenced on 03 Aug 2010 at a dose of 249 mg. From 31 Jan 2012 to 01 Feb 2012, the subject took ibuprofen. On 14 Mar 2012, the subject presented with rash. On 05 Apr 2012, the subject presented with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput), whole body pain and flair. The subject was hospitalized on an unspecified date. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l(normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l),CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flair. On 18 Apr 2012, the subject received most recent dose of study medication. The subject's condition was improving from rash as of 18 Apr 2012. The investigator assessed the events as serious (hospitalization).

The investigator did not suspect a relationship between these events and the study medication but the event DRESS syndrome was assessed as suspected to ibuprofen.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

llaris: Agree with PI assessment

Reporter's comment:

NOVARTIS, 08.05.2012 DE-002147023-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in patients with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included Omeprazol, pantoprazole and MTX. Study medication (canakinumab) was commenced on 03 Aug 2010 at a dose of 249 mg. From 31 Jan 2012 to 01 Feb 2012, the subject took ibuprofen. On 14 Mar 2012, the subject presented with rash. On 05 Apr 2012, the subject was diagnosed with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare. The subject was hospitalized on 08 Apr 2012. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l(normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l),CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flare and was discharged on the same day. On 18 Apr 2012, the subject received most recent dose of study medication. The subject's condition was improving from rash as of 18 Apr 2012. The investigator assessed the events as serious (hospitalization).

The investigator did not suspect a relationship between these events and the study medication but the event DRESS syndrome was assessed as suspected to ibuprofen.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Follow-up report received on 30 Apr 2012: Updated final diagnosis and hospitalization dates.

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

Ilaris: Agree with PI assessment

Reporter's comment:

NOVARTIS, 22.05.2012 DE-002147023-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in patients with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included prednisolone, omeprazole, pantozol, ibuprofen, etanercept and MTX. Study medication (canakinumab) was commenced on 02 Aug 2010 at a dose of 249 mg. The subject took omeprazole from 15 June 2010 to 12 Apr 2012 and then switched to pantozol from 12 Apr 2012. Ibuprofen was taken as on demand from 15 Jun 2010 to 12 Apr 2012. MTX was started on 15 Jun 2009 and currently ongoing. On 14 Mar 2012, the subject presented with rash. Last dose of study medication was received by the subject on 23 Mar 2012. On 05 Apr 2012, the subject was diagnosed with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare. The subject was hospitalized on 08 Apr 2012. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l(normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l),CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. Treatment with ibuprofen and omeprazole were permanently discontinued on 12 Apr 2012. Treatment with MTX and pantozol were not altered due to these events. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flare and was discharged on the same day. On 18 Apr 2012, the subject received most recent dose of study medication. The subject's condition was improving from rash as of 18 Apr 2012. The investigator assessed these events (except rash) were serious (hospitalization). The investigator assessed the event "rash" as non-serious.

The investigator did not suspect a relationship between these events and the study medication. The investigator assessed the events "DRESS syndrome and rash" were related to ibuprofen.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Follow-up report received on 30 Apr 2012: Updated final diagnosis and hospitalization dates.

Follow-up report received on 15 May 2012: Updated the new suspected co-medications (study medication, etanercept and MTX), start date of study medication, concomitant medication (prednisolone), status of ibuprofen, seriousness criteria of the event "rash" (previously reported as serious) and suspectedness of the event "rash" (previously reported as not suspected).

Follow-up report received on 21 May 2012 (prior the circulation of previous report): Updated the causality of the events "DRESS syndrome, whole body pain/flare and rash" with the study medication as not related, last dose of the study medication (21 Mar 2012) and therapy dates of "ibuprofen,

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

Ilaris: Agree with PI assessment

Reporter's comment:

NOVARTIS, 26.06.2012 DE-002147023-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in patients with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included prednisolone, omeprazole, pantozol, ibuprofen, etanercept and MTX. Study medication (canakinumab) was commenced on 03 Aug 2010 at a dose of 262 mg. The subject took omeprazole from 15 Jun 2010 to 12 Apr 2012 and then switched to pantozol from 12 Apr 2012. Ibuprofen was taken as on demand from 15 Jun 2010 to 12 Apr 2012. MTX was started on 15 Jun 2009 and currently ongoing. On 14 Mar 2012, the subject presented with rash. Last dose of study medication was received by the subject on 23 Mar 2012. On 05 Apr 2012, the subject was diagnosed with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare. The subject was hospitalized on 08 Apr 2012. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l (normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l), CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. Treatment with ibuprofen and omeprazole were permanently discontinued on 12 Apr 2012. Treatment with MTX and pantozol were not altered due to these events. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flare and was discharged on the same day. On 18 Apr 2012, the subject received most recent dose of study medication. On 13 Jun 2012, the subject's laboratory tests included ALT of 36 u/l, AST of 31 u/l, CRP of 1.3 mg/l, leukocytes of 3.95/ml, platelets of 223/ml and hemoglobin 12.3 g/dl. The subject also completely recovered from rash on 13 Jun 2012. The investigator assessed these events were serious (hospitalization).

The investigator did not suspect a relationship between these events and the study medication. The investigator assessed the events "DRESS syndrome and rash" were related to ibuprofen.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Follow-up report received on 30 Apr 2012: Updated final diagnosis and hospitalization dates.

Follow-up report received on 15 May 2012: Updated the new suspected co-medications (study medication, etanercept and MTX), start date of study medication, concomitant medication (prednisolone), status of ibuprofen, seriousness criteria of the event "rash" (previously reported as serious) and suspectedness of the event "rash" (previously reported as not suspected).

Follow-up report received on 21 May 2012 (prior the circulation of previous report): Updated the causality of the events "DRESS syndrome, whole body pain/flare and rash" with the study medication as not related, last dose of the study medication (21 Mar 2012) and therapy dates of "ibuprofen, omeprazole, MTX and pantozol.

Follow-up report received on 20 Jun 2012: Updated the laboratory reports dated 13 Jun 2012, outcome of the event "rash" as complete recovery and seriousness criteria for the event "rash" as hospitalization (previously reported as non serious).

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator

causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

Ilaris: Agree with PI assessment

Reporter's comment:

NOVARTIS, 20.09.2012 DE-002147023-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in subjects with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included prednisolone, omeprazole, pantozol, ibuprofen, etanercept and MTX. Study medication (canakinumab) was commenced on 03 Aug 2010 at a dose of 262 mg. The subject took omeprazole from 15 Jun 2010 to 12 Apr 2012 and then switched to pantozol from 12 Apr 2012. Ibuprofen was taken as on demand from 15 Jun 2010 to 12 Apr 2012. MTX was started on 15 Jun 2009 and currently ongoing. On 14 Mar 2012, the subject presented with rash. Last dose of study medication was received by the subject on 23 Mar 2012. On 05 Apr 2012, the subject was diagnosed with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare. The subject was hospitalized on 08 Apr 2012. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l (normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l), CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. Treatment with ibuprofen and omeprazole were permanently discontinued on 12 Apr 2012. Treatment with MTX and pantozol were not altered due to these events. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flare and was discharged on the same day. On 18 Apr 2012, the subject received most recent dose of study medication. On 13 Jun 2012, the subject's laboratory tests included ALT of 36 u/l, AST of 31 u/l, CRP of 1.3 mg/l, leukocytes of 3.95/ml, platelets of 223/ml and hemoglobin 12.3 g/dl. The subject also completely recovered from rash on 13 Jun 2012. Again on 11 Sep 2012, the subject was hospitalized for worsening of systemic juvenile idiopathic arthritis (second occurrence). Intraarticular injection left foot was planned on 12 Sep 2012. At the time of reporting, outcome of the event "worsening of systemic juvenile idiopathic arthritis (second occurrence)" was not reported. The investigator assessed these events were serious (hospitalization).

The investigator did not suspect a relationship between the events "DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare" with the study medication. The investigator assessed the events "DRESS syndrome and rash" were related to ibuprofen. The investigator did not assess the event "worsening of systemic juvenile idiopathic arthritis (second occurrence)" with the study medication.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Follow-up report received on 30 Apr 2012: Updated final diagnosis and hospitalization dates.

Follow-up report received on 15 May 2012: Updated the new suspected co-medications (study medication, etanercept and MTX), start date of study medication, concomitant medication (prednisolone), status of ibuprofen, seriousness criteria of the event "rash" (previously reported as serious) and suspectedness of the event "rash" (previously reported as not suspected).

Follow-up report received on 21 May 2012 (prior the circulation of previous report): Updated the causality of the events "DRESS syndrome, whole body pain/flare and rash" with the study medication as not related, last dose of the study medication (21 Mar 2012) and therapy dates of "ibuprofen, omeprazole, MTX and pantozol.

Follow-up report received on 20 Jun 2012: Updated the laboratory reports dated 13 Jun 2012, outcome of the event "rash" as complete recovery and seriousness criteria for the event "rash" as hospitalization (previously reported as non serious).

Follow-up report received on 11 Sep 2012: Updated the second occurrence of the event "worsening of systemic juvenile idiopathic arthritis" and it's clinical course.

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

llaris: Agree with PI assessment

ACZ: Flare or aggravation of the underlying SJIA is not uncommon to occur.

Reporter's comment:

NOVARTIS, 27.09.2012 DE-002147023-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in subjects with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included prednisolone, omeprazole, pantozol, ibuprofen, etanercept and MTX. Study medication (canakinumab) was commenced on 03 Aug 2010 at a dose of 262 mg. The subject took omeprazole from 15 Jun 2010 to 12 Apr 2012 and then switched to pantozol from 12 Apr 2012. Ibuprofen was taken as on demand from 15 Jun 2010 to 12 Apr 2012. MTX was started on 15 Jun 2009 and currently ongoing. On 14 Mar 2012, the subject presented with rash. Last dose of study medication was received by the subject on 23 Mar 2012. On 05 Apr 2012, the subject was diagnosed with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare. The subject was hospitalized on 08 Apr 2012. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l (normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l), CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. Treatment with ibuprofen and omeprazole were permanently discontinued on 12 Apr 2012. Treatment with MTX and pantozol were not altered due to these events. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flare and was discharged on the same day. On 18 Apr 2012, the subject received most recent dose of study medication. On 13 Jun 2012, the subject's laboratory tests included ALT of 36 u/l, AST of 31 u/l, CRP of 1.3 mg/l, leukocytes of 3.95/ml, platelets of 223/ml and hemoglobin 12.3 g/dl. The subject also completely recovered from rash on 13 Jun 2012. Again on 11 Sep 2012, the subject was hospitalized due to arthritis left ankle nad right wrist . The subject was diagnosed with worsening of systemic juvenile idiopathic arthritis (second occurrence). Intraarticular injection left foot was planned on 12 Sep 2012. On 13 Sep 2012 the CRP was 0.6 mg/l, white blood count was 4.84/nl, hemoglobin was 12.7 and thrombocytes were 187/nl. At the time of reporting, outcome of the event "worsening of systemic juvenile idiopathic arthritis (second occurrence)" was not reported. The investigator assessed these events were serious (hospitalization).

The investigator did not suspect a relationship between the events "DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare" with the study medication. The investigator assessed the events "DRESS syndrome and rash" were related to ibuprofen. The investigator did not suspect any relationship between the event "worsening of systemic juvenile idiopathic arthritis (second occurrence)" with the study medication.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Follow-up report received on 30 Apr 2012: Updated final diagnosis and hospitalization dates.

Follow-up report received on 15 May 2012: Updated the new suspected co-medications (study medication, etanercept and MTX), start date of study medication, concomitant medication (prednisolone), status of ibuprofen, seriousness criteria of the event "rash" (previously reported as serious) and suspectedness of the event "rash" (previously reported as not suspected).

Follow-up report received on 21 May 2012 (prior the circulation of previous report): Updated the causality of the events "DRESS syndrome, whole body pain/flare and rash" with the study medication as not related, last dose of the study medication (21 Mar 2012) and therapy dates of "ibuprofen,

omeprazole, MTX and pantozol.

Follow-up report received on 20 Jun 2012: Updated the laboratory reports dated 13 Jun 2012, outcome of the event "rash" as complete recovery and seriousness criteria for the event "rash" as hospitalization (previously reported as non serious).

Follow-up report received on 11 Sep 2012: Updated the second occurrence of the event "worsening of systemic juvenile idiopathic arthritis" and its clinical course.

Follow up received on 19 Sep 2012: The symptoms, lab data, confirmed diagnosis and causality of SJIA were updated.

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

llaris: Agree with PI assessment

ACZ: Flare or aggravation of the underlying SJIA is not uncommon to occur.

Reporter's comment:

NOVARTIS, 13.12.2012 DE-002147023-PHHO2012DE005854

Sender diagnosis: not reported

Narrative

Case number PHHO2012DE005854, is an initial report received on 11 Apr 2012, from clinical study CACZ885G2301E1. This 15-year-old male subject (centre number 60, subject number 202) was enrolled in, an open-label extension study of canakinumab (ACZ885) in subjects with Systemic Juvenile Idiopathic Arthritis (SJIA) and active systemic manifestations.

The subject's concurrent condition included gastritis. Concomitant medications included prednisolone, omeprazole, pantozol, ibuprofen, etanercept and MTX. Study medication (canakinumab) was commenced on 03 Aug 2010 at a dose of 262 mg. The subject took omeprazole from 15 Jun 2010 to 12 Apr 2012 and then switched to pantozol from 12 Apr 2012. Ibuprofen was taken as on demand from 15 Jun 2010 to 12 Apr 2012. MTX was started on 15 Jun 2009 and currently ongoing. On 14 Mar 2012, the subject presented with rash. Last dose of study medication was received by the subject on 23 Mar 2012. On 05 Apr 2012, the subject was diagnosed with DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare. The subject was hospitalized on 08 Apr 2012. Laboratory tests performed on 08 Apr 2012, showed ferritin 3162mg/l, ALT (Alanine aminotransferase) 44 u/l (normal: less than 35u/l), AST (Aspartate aminotransferase) 121 u/l (normal: 16-46 u/l), CPK (Blood creatine phosphokinase) 594 u/l (normal: less than 123 u/l), LDH (Blood lactate dehydrogenase) 910 u/l (normal: 135-214 u/l), CRP (C-reactive protein) 35.50 mg/l (normal: 0-5 mg/l), leukocytes 2.33/ml (normal: 4.5-11.0/ml), platelets 78/ml (normal: 150-400/ml) and hemoglobin 13.1 g/dl (normal: 11.7-14.7g/dl). On 14 Apr 2012, the subject's laboratory tests included ferritin 643mg/l, ALT 70u/l, AST 89 u/l, CRP 1.80 mg/l, leukocytes 8.43/ml, platelets 177/ml and hemoglobin 12.5 g/dl. No action was taken with study medication. Treatment with ibuprofen and omeprazole were permanently discontinued on 12 Apr 2012. Treatment with MTX and pantozol were not altered due to these events. On 16 Apr 2012, the subject completely recovered from DRESS syndrome, whole body pain and flare and was discharged on the same day. On 18 Apr 2012, the subject received most recent dose of study medication. On 13 Jun 2012, the subject's laboratory tests included ALT of 36 u/l, AST of 31 u/l, CRP of 1.3 mg/l, leukocytes of 3.95/ml, platelets of 223/ml and hemoglobin 12.3 g/dl. The subject also completely recovered from rash on 13 Jun 2012. Again on 27 Aug 2012, the subject experienced arthritis left ankle nad right wrist which resulted in hospitalization on 11 Sep 2012. The subject was diagnosed for these events with worsening of systemic juvenile idiopathic arthritis (second occurrence). Intraarticular injection left foot was planned on 12 Sep 2012. On 13 Sep 2012 the CRP was 0.6 mg/l, white blood count was 4.84/nl, hemoglobin was 12.7 and thrombocytes were 187/nl. At the time of reporting, outcome of the event "worsening of systemic juvenile idiopathic arthritis (second occurrence)" was not reported. The investigator assessed these events were serious (hospitalization).

The investigator did not suspect a relationship between the events "DRESS syndrome (Drug Reaction with Eosinophilia and Systemic Symptoms, in disput) which was a reaction of ibuprofen intake, whole body pain and flare" with the study medication. The investigator assessed the events "DRESS syndrome and rash" were related to ibuprofen. The investigator did not suspect any relationship between the event "worsening of systemic juvenile idiopathic arthritis (second occurrence)" with the study medication.

Follow up received on 19 Apr 2012: Updated dose of study medication, added concomitant medication, new event (DRESS syndrome), lab tests, action taken with study medication, outcomes, recent dose of study medication and causality (suspected to ibuprofen).

Follow-up report received on 30 Apr 2012: Updated final diagnosis and hospitalization dates.

Follow-up report received on 15 May 2012: Updated the new suspected co-medications (study medication, etanercept and MTX), start date of study medication, concomitant medication (prednisolone), status of ibuprofen, seriousness criteria of the event "rash" (previously reported as serious) and suspectedness of the event "rash" (previously reported as not suspected).

Follow-up report received on 21 May 2012 (prior the circulation of previous report): Updated the causality of the events "DRESS syndrome, whole body pain/flare and rash" with the study medication as not related, last dose of the study medication (21 Mar 2012) and therapy dates of "ibuprofen, omeprazole, MTX and pantozol.

Follow-up report received on 20 Jun 2012: Updated the laboratory reports dated 13 Jun 2012, outcome of the event "rash" as complete recovery and seriousness criteria for the event "rash" as hospitalization (previously reported as non serious).

Follow-up report received on 11 Sep 2012: Updated the second occurrence of the event "worsening of systemic juvenile idiopathic arthritis" and it's clinical course.

Follow up received on 19 Sep 2012: The symptoms, lab data, confirmed diagnosis and causality of SJIA were updated.

Follow-up DR received on 06 Dec 2012: Updated the onset date of the event "worsening of systemic juvenile idiopathic arthritis (second occurrence)".

Sender's comment:

For ibuprofen:

Serious adverse drug reaction report DRESS syndrome, (hospitalization), assessed as unlisted according to the Core Data Sheet. Investigator causality is suspected.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Comment for Ibuprofen: The event of DRESS syndrome is confounded by multiple therapies.

Ilaris: Agree with PI assessment

ACZ: Flare or aggravation of the underlying SJIA is not uncommon to occur.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12090163

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 77 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gastrointestinal bleeding					recovered/resolved
2 Intestinal ischemia					recovered/resolved
3 Gastrointestinal ulcer					recovered/resolved
4 Intestinal ischaemia					recovered/resolved
5 Haemorrhage of jejunum					recovered/resolved
6 Ulcer of jejunum					recovered/resolved
7 Mucosal erosion					recovered/resolved
8 Jejunitis					recovered/resolved
9 Mesenteric artery thrombosis					recovered/resolved
10 Haematochezia					recovered/resolved
11 Small intestinal necrosis					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 IBUPROFEN	Oral	/	-	Headache
ibuprofen			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension			Unknown	
Atherosclerosis			Unknown	diffuse atherosclerosis
Headache			Unknown	
Diverticulum intestinal			Unknown	colon diverticula

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Monkemuller K, Fry LC. Ischemic jejunopathy. Gastrointest-Endosc 2012; 75(3) 672	

Narratives (nur bei elektronischem Eingang)

STADA Arzneimittel AG, 30.04.2012 DE-STADA-046490

Sender diagnosis: not reported

Narrative

The following information was published in the scientific literature.

Monkemuller K, Fry LC. Ischemic jejunopathy. Gastrointest-Endosc 2012; 75(3) 672

Summary:

A 77-year-old woman developed ischaemic jejunopathy after ingesting ibuprofen [dosage and time to onset not stated].

The woman was transferred to a hospital due to haematochezia that was characteristic of gastrointestinal bleeding, after ingesting large amounts of ibuprofen to treat her headaches. Oesophagogastroduodenoscopy (EGD) and colonoscopy showed ongoing bleeding and colon diverticulosis. She had pallor, a soft abdomen with increased bowel sounds, and a haemoglobin level of 8.9 g/dL. Repeat EGD and colonoscopy showed diverticula and blood clots in the colon, and blood coming from the ileocecal valve. An enteroscopy revealed folds in the mid-jejunum appearing as thick cords, along with bleeding and nonbleeding linear erosions and ulcers. Histological examinations showed haemorrhagic necrosis, mucosal ulcerations, and multiple arterial thrombi affecting small and large arteries. Diffuse atherosclerosis was evident on CT angiography. Her ischaemic jejunopathy had likely resulted from nonsteroidal anti-inflammatory drug ingestion.

The woman received crystalloids, blood transfusions and oxygen. No further bleeding episodes were noted during 1-year follow-up.<dbtext

To_Char_NARRATIVE_2</dbtext>

Sender's comment:

This case was classified as serious.

IBUPROFEN

The causal relationship between the suspected drug and the suspected adverse reactions was assessed as possibly related based on the temporal association and the known safety profile of ibuprofen.

The suspected adverse reactions are not explicitly stated according to the current Reference Safety Information (RSI). Gastrointestinal effects are listed such as gastrointestinal ulcers, with bleeding and perforation, and gastritis. Jejunitis was assessed as listed, intestinal ischaemia was assessed as unlisted.

This case does not change the overall benefit-risk balance of the medicinal product.

Reporter's comment:

Generics [UK] Ltd, 01.05.2012 DE-MYLANLABS-2012S1008426

Sender diagnosis: not reported

Narrative

Monkemuller K, Fry LC. Ischemic jejunopathy. Gastrointest-Endosc 2012; 75(3) 672

A 77-year-old woman developed ischaemic jejunopathy after ingesting ibuprofen [dosage and time to onset not stated]. The woman was transferred to a hospital due to haematochezia that was characteristic of gastrointestinal bleeding, after ingesting large amounts of ibuprofen to treat her headaches. Oesophagogastroduodenoscopy (EGD) and colonoscopy showed ongoing bleeding and colon diverticulosis. She had pallor, a soft abdomen with increased bowel sounds, and a haemoglobin level of 8.9 g/dL. Repeat EGD and colonoscopy showed diverticula and blood clots in the colon, and blood coming from the ileocecal valve. An enteroscopy revealed folds in the mid-jejunum appearing as thick cords, along with bleeding and nonbleeding linear erosions and ulcers. Histological examinations showed haemorrhagic necrosis, mucosal ulcerations, and multiple arterial thrombi affecting small and large arteries. Diffuse atherosclerosis was evident on CT angiography. Her ischaemic jejunopathy had likely resulted from nonsteroidal anti-inflammatory drug ingestion. The woman received crystalloids, blood transfusions and oxygen. No further bleeding episodes were noted during 1-year follow-up.

Sender's comment:

Gastrointestinal disorders events including haemorrhage are listed undesirable effects; ulceration or ischaemic jejunopathy is not listed. Causality is assessed as possible.

Reporter's comment:

Temmler Pharma GmbH & Co. KG, 07.05.2012 DE-TEMMLER06-005043

Sender diagnosis: not reported

Narrative

This literature case was received by Temmler Pharma on 03 May 2012 from Adis Reaction via literature research:

A 77-year-old woman developed ischaemic jejunopathy after ingesting ibuprofen (dosage and time to onset not stated). The woman was transferred to a hospital due to haematochezia that was characteristic of gastrointestinal bleeding, after ingesting large amounts of ibuprofen to treat her headaches. Oesophagogastroduodenoscopy (EGD) and colonoscopy showed ongoing bleeding and colon diverticulosis. She had pallor, a soft abdomen with

increased bowel sounds, and a haemoglobin level of 8.9 g/dL. Repeat EGD and colonoscopy showed diverticula and blood clots in the colon, and blood coming from the ileocecal valve. An enteroscopy revealed folds in the mid-jejunum appearing as thick cords, along with bleeding and nonbleeding linear erosions and ulcers. Histological examinations showed haemorrhagic necrosis, mucosal ulcerations, and multiple arterial thrombi affecting small and large arteries. Diffuse atherosclerosis was evident on CT angiography. Her ischaemic jejunoopathy had likely resulted from nonsteroidal anti-inflammatory drug ingestion. The woman received crystalloids, blood transfusions and oxygen. No further bleeding episodes were noted during 1-year follow-up.

Temmler ID: IBU-2012-025<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Company comment Temmler Pharma:

Gastrointestinal bleeding, ischemic jejunoopathy and mesenteric artery thrombosis occurred after ingestion of large amounts of ibuprofen.

This case is classified serious due to the patient's hospitalisation. Intestinal disorders like ulcers, bleeding and intestinal strictures are listed; but as ischemic jejunoopathy and mesenteric artery thrombosis are not verbally mentioned, it is considered to be unlisted.

The reaction occurred in a plausible temporal relationship with ibuprofen intake, no additional medication and other possible causes are stated and the patient recovered after ibuprofen was discontinued and the events were treated. Therefore the reactions are assessed as probably related to ibuprofen treatment.

Reporter's comment:

Dr. R. Pflieger Chemische Fabrik GmbH, 14.05.2012 DE-PFLEGER-001767

Sender diagnosis: not reported

Narrative

A 77-year-old woman developed ischaemic jejunoopathy after ingesting ibuprofen [dosage and time to onset not stated].

The woman was transferred to a hospital due to haematochezia that was characteristic of gastrointestinal bleeding, after ingesting large amounts of ibuprofen to treat her headaches. Oesophagogastroduodenoscopy (EGD) and colonoscopy showed colon diverticulosis and ongoing bleeding. She had pallor, a soft abdomen with increased bowel sounds, and a haemoglobin level of 8.9 g/dL. Repeat EGD and colonoscopy showed diverticula and blood clots in the colon, and blood coming from the ileocecal valve. An enteroscopy revealed folds in the mid-jejunum appearing as thick cords, along with bleeding and nonbleeding linear erosions and ulcers. Histological examinations showed haemorrhagic necrosis, mucosal ulcerations, and multiple arterial thrombi affecting small and large arteries. Diffuse atherosclerosis was evident on CT angiography. Her ischaemic jejunoopathy had likely resulted from nonsteroidal anti-inflammatory drug ingestion. The woman received crystalloids, blood transfusions and oxygen. No further bleeding episodes were noted during 1-year follow-up.<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Gastrointestinal ulcer is described in the Reference Safety Information (RSI) of the ibuprofen containing drugs of Dr. Pflieger as ADR. This information is considered as sufficient regarding the reported reaction.

Changes of RSI are not indicated.

Reporter's comment:

Recordati Pharma GmbH, 14.05.2012 DE-RECORDATI PHARMA GMBH-2012RC001587

Sender diagnosis: not reported

Narrative

A 77-year-old woman developed ischaemic jejunopathy after ingesting ibuprofen [dosage and time to onset not stated]. The woman was transferred to a hospital due to haematochezia that was characteristic of gastrointestinal bleeding, after ingesting large amounts of ibuprofen to treat her headaches. Her medical history was positive only for hypertension. Oesophagogastroduodenoscopy (EGD) and colonoscopy showed colon diverticulosis and ongoing bleeding. She had pallor, a soft abdomen with increased bowel sounds, and a haemoglobin level of 8.9 g/dL. Repeat EGD and colonoscopy showed diverticula and blood clots in the colon, and blood coming from the ileocecal valve. An enteroscopy revealed folds in the mid-jejunum appearing as thick cords, along with bleeding and nonbleeding linear erosions and ulcers. Histological examinations showed haemorrhagic necrosis, mucosal ulcerations, and multiple arterial thrombi affecting small and large arteries. Diffuse atherosclerosis was evident on CT angiography. Her ischaemic jejunopathy had likely resulted from nonsteroidal anti-inflammatory drug ingestion. The woman received crystalloids, blood transfusions and oxygen. No further bleeding episodes were noted during 1-year follow-up.

Evaluation and Comment by Recordati Pharma GmbH:

Ischaemic jejunopathy (GASTROINTESTINAL NECROSIS):

Labelling reference document: unlisted

Company causality assessment: possible

Temporal relationship: plausible

Alternative explanation: unknown

Plausibility of adverse drug event: yes

Dechallenge information: n.a.

Rechallenge information: n.a.

Comment: The term "ischaemic jejunopathy" was assessed as unlisted in the reference SmPC of Ibuprofen, however, it is to mention that the terms gastrointestinal bleeding, gastrointestinal ulcers, perforations or melaena are already mentioned in the SmPC especially in the context of treatment concerning elderly patients, which are representing substantial leading events of the ischaemic jejunopathy. Since the reported events gastrointestinal bleeding and haematochezia were assessed as symptoms of the ischaemic jejunopathy, they were not evaluated separately. It was reported that the patient's medical history obviously comprised only hypertension, however, no further information was given if there were any medications concomitantly applied. Nevertheless, in consideration of a plausible temporal relationship Recordati Pharma GmbH assessed causality of the occurrence of ischaemic jejunopathy as possibly related, since an association of the reported event to ibuprofen cannot be excluded. No corrective action is considered necessary.

Overall documentation quality: fair

Risk / Benefit assessment: unchanged

Action to be taken by MAH: not necessary

Sender's comment:

Evaluation see company narrative.

Reporter's comment:

FAIRMED, 21.05.2012 DE-FAIRMED-IBU-0011-12

Sender diagnosis: not reported

Narrative

A 77-year-old woman developed ischaemic jejunopathy after ingesting ibuprofen [dosage and time to onset not stated].

The woman was transferred to a hospital due to haematochezia that was characteristic of gastrointestinal bleeding, after ingesting large amounts of ibuprofen to treat her headaches. Oesophagogastroduodenoscopy (EGD) and colonoscopy showed colon diverticulosis and ongoing bleeding. She had pallor, a soft abdomen with increased bowel sounds, and a haemoglobin level of 8.9 g/dL. Repeat EGD and colonoscopy showed diverticula and blood clots in the colon, and blood coming from the ileocecal valve. An enteroscopy revealed folds in the mid-jejunum appearing as thick cords, along with bleeding and nonbleeding linear erosions and ulcers. Histological examinations showed haemorrhagic necrosis, mucosal ulcerations, and multiple arterial thrombi affecting small and large arteries. Diffuse atherosclerosis was evident on CT angiography. Her ischaemic jejunopathy had likely resulted from nonsteroidal anti-inflammatory drug ingestion.

The woman received crystalloids, blood transfusions and oxygen. No further bleeding episodes were noted during 1-year follow-up.

Sender's comment:

This is a case of a 77-year-old woman who developed ischaemic jejunopathy after ingesting ibuprofen

Reporter's comment:

Bayer Vital GmbH, 21.05.2012 DE-BAYER-2012-048449

Sender diagnosis: not reported

Narrative

serious, related, unlisted

This case report from Germany was derived from the medical literature on 15-May-2012, and describes the occurrence of ISCHEMIC JEJUNOPATHY with MUCOSAL ULCERATIONS in the mid-jejunum, in a 77-year-old female patient who had taken 'large amounts' of IBUPROFEN (Trade name not reported) for the indication of headaches.

=====

A 77-year-old woman was transferred to our hospital because of obscure overt GI bleeding characterized by hematochezia. Her medical history was positive only for hypertension.

She admitted to the ingestion of large amounts of IBUPROFEN to treat headaches. (Drug dosing and duration of drug use was not reported in the article).

EGD and colonoscopy performed at the referring hospital were unrevealing except for ongoing bleeding and colon diverticulosis.

Physical examination on admission revealed pallor, a pulse of 100 beats per minute, and a blood pressure of 90/60 mm Hg. Her abdomen was soft, with increased bowel sounds. The hemoglobin level was 8.9 g/dL (normal 11-14 g/dL), and the prothrombin time was 12 seconds (12-14 seconds).

Repeat EGD and colonoscopy disclosed colon diverticula, multiple blood clots in the colon, and fresh blood emanating from the ileocecal valve. Oral double-balloon enteroscopy disclosed a normal-appearing proximal jejunum. In the mid-jejunum, the folds appeared as thick cords, with multiple bleeding and nonbleeding linear erosions and ulcers. Histology showed mucosal ulcerations, hemorrhagic necrosis, with multiple arterial thrombi involving small and large arteries. There were no changes consistent with vasculitis. Laboratory test data were negative for vasculitic markers. CT angiography showed diffuse atherosclerosis but a patent aorta as well as mesenteric and celiac trunk arteries.

The patient improved after treatment with intravenous crystalloids, blood transfusions, and oxygen. She has not had further bleeding episodes during a 1-year follow-up period.

Authors' comment:

The ISCHEMIC JEJUNOPATHY likely resulted from nonsteroidal anti-inflammatory drug ingestion.

Sender's comment:

This case report was derived from medical literature and concerns a 77-year-old female patient who had taken Ibuprofen and experienced ISCHEMIC JEJUNOPATHY (hemorrhagic necrosis mid-jejunum and multiple arterial thrombi mid-jejunum) and MUCOSAL ULCERATIONS IN THE MID JEJUNUM (GI bleeding). The reported events were considered serious as required hospitalization. ISCHEMIC JEJUNOPATHY is unlisted and MUCOSAL ULCERATIONS IN THE MID JEJUNUM is listed for Ibuprofen according to the International Product Information. Since there is an implied temporal relationship between the events and Ibuprofen intake and the lack of an alternative explanation, causality was assessed as related.

Reporter's comment:

Dolorgiet GmbH, 01.06.2012 DE-DOLORGIET-2012ibu03735

Sender diagnosis: not reported

Narrative

A 77-year-old woman developed ischaemic jejunopathy after ingesting ibuprofen [dosage and time to onset not stated].

The woman was transferred to a hospital due to haematochezia that was characteristic of gastrointestinal bleeding, after ingesting large amounts of ibuprofen to treat her headaches. Oesophagogastroduodenoscopy (EGD) and colonoscopy showed colon diverticulosis and ongoing bleeding. She had pallor, a soft abdomen with increased bowel sounds, and a haemoglobin level of 8.9 g/dL. Repeat EGD and colonoscopy showed diverticula and blood clots in the colon, and blood coming from the ileocecal valve. An enteroscopy revealed folds in the mid-jejunum appearing as thick cords, along with bleeding and nonbleeding linear erosions and ulcers. Histological examinations showed haemorrhagic necrosis, mucosal ulcerations, and multiple arterial thrombi affecting small and large arteries. Diffuse atherosclerosis was evident on CT angiography. Her ischaemic jejunopathy had likely resulted from nonsteroidal anti-inflammatory drug ingestion.

The woman received crystalloids, blood transfusions and oxygen. No further bleeding episodes were noted during 1-year follow-up.

Sender's comment:

Reporter's comment:

onkovis, 07.12.2012 DE-KSK-PHARMA-2012-0065

Sender diagnosis: not reported

Narrative

Report from literature:

A 77-year-old woman developed ischaemic jejunopathy after ingesting ibuprofen [dosage and time to onset not stated].

The woman was transferred to a hospital due to haematochezia that was characteristic of gastrointestinal bleeding, after ingesting large amounts of ibuprofen to treat her headaches. Oesophagogastroduodenoscopy (EGD) and colonoscopy showed colon diverticulosis and ongoing bleeding. She had

pallor, a soft abdomen with increased bowel sounds, and a haemoglobin level of 8.9 g/dL. Repeat EGD and colonoscopy showed diverticula and blood clots in the colon, and blood coming from the ileocecal valve. An enteroscopy revealed folds in the mid-jejunum appearing as thick cords, along with bleeding and nonbleeding linear erosions and ulcers. Histological examinations showed haemorrhagic necrosis, mucosal ulcerations, and multiple arterial thrombi affecting small and large arteries. Diffuse atherosclerosis was evident on CT angiography. Her ischaemic jejunopathy had likely resulted from nonsteroidal anti-inflammatory drug ingestion.

The woman received crystalloids, blood transfusions and oxygen. No further bleeding episodes were noted during 1-year follow-up.

Monkemuller K, Fry LC. Ischemic jejunopathy. Gastrointestinal Endoscopy 75: 672, No. 3, Mar 2012.

Sender's comment:

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-12100613

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 15 Year	weight (kg): 62.4	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 DRESS syndrome	20120405	20120416	12 Day	2649 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 canakinumab canakinumab		/Month	20100802-	
2 etanercept etanercept		/	20050104-20060509	
3 ibuprofen ibuprofen		600 Mg milligram(s)/	-20120412	
4 methotrexate methotrexate sodium		15 Mg milligram(s)/Week	-	

Drugs (concomitant)

5 omeprazole omeprazole	40 Mg milligram(s)/Day	20120321-20120412
6 PANTOZOL pantoprazole sodium	40 Mg milligram(s)/Day	20120412-
7 prednisolone prednisolone	10 Mg milligram(s)/Day	-

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 15.05.2012 DE-PFIZER INC-2012110808

Sender diagnosis: not reported

Narrative

GERMAN BIOLOGICS IN PAEDIATRIC RHEUMATOLOGY REGISTRY (BIKER)

This is a report from Non-Interventional Study source for Protocol #B1801310, Center ID/Subject IDE060524. A contactable physician reported that a 15 year-old female subject received etanercept (Etanercept, Unknown) from 04Jan2005 to 09May2006 (unknown dose, frequency, and indication). Concomitant medications included methotrexate sodium (MTX HEXAL) at 15 mg, once weekly via an unspecified route of administration, ibuprofen, canakinumab, pantoprazole sodium (PANTOZOL), omeprazole, and prednisolone. The subject received ibuprofen (manufacturer unknown) from an unspecified date at 600mg as needed (2x1 tablet) until 12Apr2012, methotrexate sodium from an unspecified date at 15mg once weekly and ongoing, pantoprazole from 12Apr2012 at 40mg once per day, omeprazole (manufacturer unknown) from 21Mar2012 to 12Apr2012 at 40mg once per day, prednisolone (manufacturer unknown) from an unspecified date at 10mg once per day and ongoing, and canakinumab (manufacturer unknown) from 02Aug2010 at 4mg/kg once per month and ongoing).

On 14Mar2012, the subject experienced "rash (symptom of DRESS syndrome)", considered to be non-serious.

On [05Apr2012](#), the subject experienced flare, whole body pain, blood count changes (DRESS syndrome), for which she was hospitalized.

Relevant laboratory tests included: alanine aminotransferase 44 IU/l on 08Apr2012, aspartate aminotransferase 121 IU/l on 08Apr2012, blood creatine phosphokinase 594 IU/l on 08Apr2012, blood lactate dehydrogenase 910 IU/l on 08Apr2012, haemoglobin 13.1 g/dl on 08Apr2012, platelet count 78 x10⁹/l on 08Apr2012, white blood cell count 2.33 x10⁹/l on 08Apr2012, C-reactive protein 35.5 mg/l on 08Apr2012, and ferritin 3162 µg/l on 08Apr2012.

On 12Apr2012, ibuprofen was permanently discontinued. Methotrexate sodium and canakinumab doses were not changed. Treatment details were not provided. On 16Apr2012, the subject recovered from flare, whole body pain, blood count changes (DRESS syndrome) and was discharged from the hospital on the same day. The investigator considered that the relationship of the events, "rash (symptom of DRESS syndrome)" to treatment with

etanercept was unrelated and unlikely related to methotrexate sodium, pantoprazole sodium, omeprazole, prednisolone, canakinumab, or the underlying disease. However, the physician reported the events it was likely related to ibuprofen. The investigator considered the serious event, "flare, whole body pain, blood count changes (DRESS syndrome)" possibly related to methotrexate, canakinumab and ibuprofen and not related to prednisolone or etanercept.

Pfizer is the Marketing Authorization Holder of ibuprofen and methotrexate in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of ibuprofen/methotrexate has submitted the same report to the regulatory authorities.

Sender's comment:

Based on the information provided the company agrees with investigator's causality assessment: the event was not related to etanercept (patient stopped Enbrel about six year before the event onset) and possible related to methotrexate and ibuprofen. The impact of this report on the benefit/risk profile of the Pfizer product is evaluated as part of Pfizer procedures for safety evaluation, including the review and analysis of aggregate data for adverse events. Any safety concern identified as part of this review, as well as any appropriate action in response, will be promptly notified to Regulatory Authorities, Ethics Committees and Investigators, as appropriate. DRESS-Syndrom ist in der Produktinformation als mögliche Nebenwirkung von ENBREL (Etanercept), LANTAREL (Methotrexat) und SPALT (Ibuprofen) nicht beschrieben. Wir sehen zum gegenwärtigen Zeitpunkt keinen Anlass für eine geänderte Bewertung des Nutzen-/Risikoverhältnisses.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-12111434

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 15 Year	weight (kg): 62.4	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blood count abnormal					recovered/resolved
2 Rash					recovered/resolved
3 Drug rash with eosinophilia and systemic symptoms	20120405	20120416	12 Day		recovered/resolved
4 General body pain					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 CANAKINUMAB canakinumab	Unknown	4 mg/kg milligram(s)/kilogr	20100802-	
2 ibuprofen ibuprofen		600 Mg milligram(s)/	-20120412	
3 MTX HEXAL methotrexate	Unknown	15 Mg milligram(s)/Week	-	

Drugs (concomitant)

4 OMEPRAZOLE omeprazole		40 Mg milligram(s)/Day	20120321-20120412	
5 PANTOZOL pantoprazole sodium	Oral	40 Mg milligram(s)/Day	20120412-	
6 prednisolone prednisolone		10 Mg milligram(s)/Day	-	

Primary source

Qualification:	Other Health Professional	observestudytype:	Other studies
Literature:			
Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 22.05.2012 DE-NOVNCHHQ-GXKR2012DE001733

Sender diagnosis: not reported

Narrative

Case number GXKR2012DE001733, is an initial Non-Novartis registry report (German Biologics in Paediatric Rheumatology Registry (BiKeR)) received via another pharmaceutical company (DE-PFIZER-INC-2012110808) on 15May2012: This 15-year-old female subject (Patient ID E060524) was enrolled in an investigator initiated registry (study ID B1801310). The subject's medical history included treatment with Etanercept (manufacturer unknown) at an unreported dose from 04Jan2005 to 09May2006. Concomitant medications included Pantozol (pantoprazole), Omeprazole and prednisolone. The subject received MTX HEXAL (methotrexate) for an unreported indication at a dose of 15 mg weekly from an unspecified date ongoing and Ibuprofen (manufacturer unknown) for unreported indication at a dose of 600 mg as needed from an unspecified date until 12Apr2012. Canakinumab (MAH unknown) was taken at a dose of 4mg/kg once per month from 02Aug2010 ongoing. On 05Apr2012, the subject experienced flare, whole body pain and blood count changes (DRESS syndrome). The patient was hospitalised. Relevant laboartory tests of 08Apr2012 included: Platelet count 78x10 9/l, white blood cell count 2.33x10 9/l, haemoglobin 13.1 g/dl, C-reactive proteine 35.5 mg/l, alanine aminotransferase 44 IU/l, aspartate aminotransferase 121 IU/l, blood creatine phosphokinase 594 IU/l and blood lactate dehydrogenase 910 IU/l. Treatment with ibuprofen was stopped due to adverse events on 12Apr2012, MTX HEXAL and Canakinumab were ongoing at the time of last observation. On 16Apr2012 the patient was discharged from hospital. Outcome of adverse events was reported as recovered. The coordinating investigator considered (flare, whole body pain, blood count changes) DRESS syndrome possibly-related to MTX HEXAL , ibuprofen and canakinumab.

Sender's comment:

Methotrexate, Ibuprofen:

Serious adverse drug reaction report, Drug rash with eosinophilia and systemic symptoms (hospitalisation), assessed as unlisted according to the Core Data Sheet due to greater specificity (DRESS syndrome). The following terms (For ibuprofen: severe general hypersensitivity reactions, hypersensitivity reactions with skin rashes..; for MTX: e.g. allergic reaction, exanthema, eosinophilia..) are already included in the Core Data Sheet Information. The available information was considered inadequate to fully assess the case. The information provided in this individual case does not warrant a change to Core Data Sheet text. The topic will be monitored closely. Reporting healthcare professional causality is suspected for methotrexate and ibuprofen.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13057791

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age:	weight (kg): 86	BMI_int: 29.07	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Angioedema					not recovered/not resolved
2 Exanthematic drug eruption					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Oral	/	- 5 Week	Back pain
2 novalgin metamizole sodium	Oral	/	- -	Paranasal sinus mucocoele

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol use			Unknown	
Sinusitis			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 26.03.2013 DE-002147023-CHPA2013DE005139

Sender diagnosis: not reported

Narrative

Initial spontaneous report from a physician, via the German Health Authority (BfArM homepage, case no. DE-BFARM-13057791), received on 21 Mar 2013: This report refers to a male patient of unknown age. The patient's medical history included alcohol use. No concomitant medication was reported. The patient received ibuprofen (MAH unknown) for the treatment of back pain for 5 weeks and Novalgin (metamizole sodium) for the treatment of sinusitis. On an unknown onset date after starting the suspect drugs the patient experienced drug exanthema and angioedema of face and tongue. The patient was hospitalised due to the events. These events were considered serious by the sender. It was not reported whether or when treatment with suspect drugs were stopped. The event outcome was not recovered/not resolved. The physician assessed the causality as probable/likely to ibuprofen and Novalgin. No additional information was provided.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (hospitalization) not confirmed by a health care professional. The adverse event(s), Rash and Angioedema was/were listed in the Reference Safety Information. The available information was considered inadequate to fully assess the case. While a spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

SANDOZ, 26.03.2013 DE-SZ09-CHPA2013DE005139

Sender diagnosis: not reported

Narrative

Initial spontaneous report from a physician, via the German Health Authority (BfArM homepage, case no. DE-BFARM-13057791), received on 21 Mar 2013: This report refers to a male patient of unknown age. The patient's medical history included alcohol use. No concomitant medication was reported. The patient received ibuprofen (MAH unknown) for the treatment of back pain for 5 weeks and Novalgin (metamizole sodium) for the treatment of sinusitis. On an unknown onset date after starting the suspect drugs the patient experienced drug exanthema and angioedema of face and tongue. The patient was hospitalised due to the events. These events were considered serious by the sender. It was not reported whether or when treatment with suspect drugs were stopped. The event outcome was not recovered/not resolved. The physician assessed the causality as probable/likely to ibuprofen and Novalgin. No additional information was provided.

Sender's comment:

22 Mar 2013 (Samantha Pepe): PI labeling assessment provided by Sandoz.

Reporter's comment:sanofi-aventis, 26.03.2013 DE-SANOFI-AVENTIS-2013SA031184

Sender diagnosis: not reported

Narrative

Initial serious report (inpatient hospitalisation involved or prolonged) extracted from the web site of the German health authority (identification number DE-BFARM-13057791) on 22-Mar-2013. A physician reported on a male patient of unspecified age with a medical history of alcohol use. The patient was treated with ibuprofen (trade name not provided) for five weeks (indication: back pain) and with metamizole sodium (Novalgin) for an unspecified period of time (indication: sinusitis).

Under treatment with these drugs, the patient developed a drug-induced exanthema and an angioedema of his face and tongue.

No information was provided on action taken concerning ibuprofen and metamizole and on corrective treatment.

Outcome of events was reported as "not recovered/not resolved".

Causality determined by the attending physician (method: WHO Global Introspection): "probable/likely" for ibuprofen and metamizole.

Further information, including concomitant medication, dose and therapy dates of ibuprofen and metamizole, and date of onset of events, was not provided.

Sender's comment:

25-Mar-2013 Sanofi Company Comment:

Based on the available information a causal relationship to ibuprofen and metamizole cannot be excluded. However this case lacks further significant medical information, exact therapy dates, further relevant history, possible risk factors and corrective treatment.

Reporter's comment:McNeil Germany, Local Safety Officers, 28.03.2013 DE-JNJFOC-20130315470

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician, via a regulatory authority (BfArM (Germany) - DE-BFARM-13057791) from the website of the Bundesinstitut für Arzneimittel und Medizinprodukte and concerns a male patient of unspecified age from Germany: Local case ID: 00516/13.

The patient's height was 172 Centimeters and weight was 86 kilograms. The patient's medical history and concurrent conditions included: alcohol use. The patient was treated with ibuprofen (unspecified formulation, oral) at an unspecified dose and frequency for 5 weeks, initiated on an unspecified

date for back pain. The brand was not reported and it is assumed to be the J&J brand for reporting purposes. Non-company suspect drugs included: Novalgin (metamizole sodium, unspecified formulation, oral) at an unspecified dose and frequency, initiated on an unspecified date for sinusitis. Concomitant medications were not reported. On an unspecified date, the patient experienced angioedema and exanthema. Action taken with ibuprofen and metamizole sodium was not reported. The patient had not recovered from angioedema and exanthema. The physician had assessed the causality between ibuprofen, metamizole sodium and the events (angioedema and exanthema) as probable. The Company causality between ibuprofen and the events (angioedema and exanthema) was assessed as possible. This report was serious (hospitalization).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 29.03.2013 DE-Boehringer Ingelheim GmbH, Germany-2013-DE-00957DE

Sender diagnosis: not reported

Narrative

Initial information was received from the German Health Authority concerning a male patient (age not reported), who was on Ibuprofen, orally, for 5 weeks, for the treatment of back pain and Novalgin, orally, for the treatment of sinusitis.

On an unknown date, the patient experienced Exanthema and Angioedema. The events were considered to be serious due to hospitalization or prolonged hospitalization.

The patient has not yet recovered.

The status of the Ibuprofen and Novalgin was not reported.

There were no information about concomitant medication reported. Further concomitant diseases included alcohol use.

It is the opinion of the reporter that the events are probable/Likely related to Ibuprofen and Novalgin therapy.

Tracking of Changes:

22Mar2013 - initial information received

Sender's comment:

Reporter's comment:

001307602, 02.04.2013 DE-ABBOTT-13X-062-1067193-00

Sender diagnosis: not reported

Narrative

Spontaneous report from GERMANY by a physician of a male with events of DRUG EXANTHEMA and ANGIOEDEMA OF FACE AND TONGUE with IBUPROFEN.

Case received on 21 Mar 2013 from Health Authority; reference number DE-BfArM-13057791.

On unknown dates, the patient experienced DRUG EXANTHEMA and ANGIOEDEMA OF FACE AND TONGUE. NOVALGIN (NOVO-PETRIN) was also considered suspect.

Sender's comment:

Reporter's comment:

McNeil Germany, Local Safety Officers, 04.04.2013 DE-JNJFOC-20130315470

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician, via a regulatory authority (BfArM (Germany) - DE-BFARM-13057791) from the website of the Bundesinstitut für Arzneimittel und Medizinprodukte and concerns a male patient of unspecified age from Germany: Local case ID: 00516/13. The patient's height was 172 Centimeters and weight was 86 kilograms. The patient's medical history and concurrent conditions included: alcohol use. The patient was treated with ibuprofen (unspecified formulation, oral) at an unspecified dose and frequency for 5 weeks, initiated on an unspecified date for back pain. The brand was not reported and it is assumed to be the J&J brand for reporting purposes. Non-company suspect drugs included: Novalgin (metamizole sodium, unspecified formulation, oral) at an unspecified dose and frequency, initiated on an unspecified date for sinusitis. Concomitant medications were not reported. On an unspecified date, the patient experienced angioedema and exanthema. Action taken with ibuprofen and metamizole sodium was not reported. The patient had not recovered from angioedema and exanthema. The physician had assessed the causality between ibuprofen, metamizole sodium and the events (angioedema and exanthema) as probable. The Company causality between ibuprofen and the events (angioedema and exanthema) was assessed as possible. This report was serious (hospitalization).

Additional information was received from a contactable physician via business partner Pfizer (Pfizer: DE-PFIZER INC-2013095428) via a regulatory authority (BfArM (Germany) - DE-BFARM-13057791) from the website of the Bundesinstitut für Arzneimittel und Medizinprodukte on 22-MAR-2013. The patient's concurrent conditions included sinusitis. On an unspecified date, the patient experienced drug exanthema. The patient had not recovered from drug exanthema at the time of this report. The physician had assessed the causality between ibuprofen, metamizole sodium and the event drug exanthema as probable. The Company causality between ibuprofen and the event drug exanthema was assessed as possible.<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Reporter's comment:

PFIZERINC, 05.04.2013 DE-PFIZER INC-2013095428

Sender diagnosis: not reported

Narrative

This is a spontaneous report received from a contactable physician via German Health Authority (Bundesinstitut fuer Arzneimittel und Medizinprodukte, BfArM = contactable), Regulatory Authority report number DE-BFARM-13057791. A male patient of unknown age and ethnicity started to receive ibuprofen orally for back pain, metamizole sodium (NOVALGIN) orally for sinusitis. Patient history included ongoing sinusitis, alcohol use but unknown if ongoing. On an unknown date the patient suffered from drug exanthema and angioedema of the face and the tongue and was hospitalized on unknown date. The action taken with both drugs was unknown. The outcome of the events was reported as not recovered. Causal relationship (based on Global Introspection (WHO GI)) between the adverse event(s) and the intake of ibuprofen and metamizole sodium was assessed as probable/likely by the reporter.

Pfizer is the Marketing Authorization Holder of ibuprofen in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of ibuprofen has submitted the same report to the regulatory authorities.

Sender's comment:

Based on temporal association and known safety profile of the product, a contributory role of suspect drug to the reported events drug exanthema and angioedema cannot be excluded. The impact of this report on the benefit/risk profile of the Pfizer product is evaluated as part of Pfizer procedures for safety evaluation, including the review and analysis of aggregate data for adverse events. Any safety concern identified as part of this review, as well as any appropriate action in response, will be promptly notified to Regulatory Authorities, Ethics Committees and Investigators, as appropriate.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13118872

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 48 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Exanthematic drug eruption	20130205				not recovered/not resolved
2 Dyspnoea	20130205				not recovered/not resolved
3 Itching					not recovered/not resolved
4 Allergic exanthema	20130205				not recovered/not resolved
5 Shortness of breath					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 abilify aripiprazole	Oral	15 Mg milligram(s)/Day	20130201-20130205 5 Day	
2 abilify aripiprazole	Oral	10 Mg milligram(s)/Day	20130124-20130131 8 Day	
3 abilify aripiprazole	Oral	5 Mg milligram(s)/Day	20130122-20130123 2 Day	
4 dipiperon pipamperone	Oral	40 Mg milligram(s)/Day	20130204-20130205 2 Day	
5 ibuprofen ibuprofen	Oral	/	20130203- -	
6 ibuprofen ibuprofen	Oral	/	20130203-20130205 3 Day	
7 pantoprazol pantoprazole	Oral	20 Mg milligram(s)/Day	20130205-20130207 3 Day	
8 tavor lorazepam	Oral	1 Mg milligram(s)/Day	20130215- -	

Drugs (concomitant)

9 ATOSIL isopromethazine hydrochloride	Oral	25 Mg milligram(s)/Day	20130123-20130124 2 Day	Pruritus
10 l-thyroxin levothyroxine sodium	Oral	75 µg microgram(s)/	20130119- -	
11 promethazin promethazine	Oral	25 Mg milligram(s)/Day	20130123-20130124 2 Day	Product used for unknown indication
12 promethazin promethazine	Oral	50 Mg milligram(s)/Day	20130128-20130204 8 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hospitalisation	20130115		Yes	
Hypothyroidism			Unknown	
Schizoaffective disorder			Unknown	depressive type
Itching	201301	201301	No	
Pruritis	20130122		Yes	
Drug allergy			Unknown	

Primary source

Qualification:	Physician	observestudytype:
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Literature:

Narratives (nur bei elektronischem Eingang)European Electronic Reporting Group, 26.04.2013 DE-JNJFOC-20130408589

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician and concerns a 48-year-old female patient from Germany. The patient's height and weight were not reported. The patient's medical history and concurrent conditions included: schizoaffective disorder (depressive type) and active hypothyreosis. The patient was hospitalized due to unspecified event on 15-JAN-2013. On 22-JAN-2013, the patient was treated with non company suspect drug aripiprazole (unspecified formulation, oral) 5 mg daily, for an unknown indication. On 23-JAN-2013, the patient received isopromethazine hydrochloride 25 mg daily. On 24-JAN-2013, the dose of aripiprazole was increased to 10 mg. On 24-JAN-2013, the patient experienced pruritus, but later she recovered from pruritus on an unspecified day. On 01-FEB-2013, the dose of aripiprazole was increased to 15 mg per day. The treatment with isopromethazine hydrochloride was temporarily stopped and started again with 50 mg daily. On 03-FEB-2013, the patient was treated with Ibuprofen (unspecified formulation, oral) unspecified dose, for an unknown indication. On 04-FEB-2013, the patient's treatment was switched from isopromethazine hydrochloride to pipamperone (unspecified formulation, oral) 40 mg daily for an unknown indication. Concomitant medications included promethazine hydrochloride and levothyroxine sodium for an unspecified indication. On 05-FEB-2013, the patient experienced pruritus, urticaria and dyspnoea and was transferred from psychotic ward to dermatology department on the same day. The patient was diagnosed partly urticarial, partly erythema exsudativum multiforme generalised drug induced exanthema with globus sensation. The treatment with aripiprazole, ibuprofen and pipamperone (permanent discontinuation) was withdrawn in 05-FEB-2013. On 05-FEB-2013, the patient took non company suspect drug pantoprazole (unspecified, oral) 20 mg and later she experienced worsening of exanthema and consulted a dermatologist. On 06-FEB-2013, the patient was transferred back to the psychiatric ward. On 07-FEB-2013, the treatment with the pantoprazole was discontinued. On 08-FEB-2013 (one day later), the patient developed distinct worsening of exanthema with renewed dyspnoea and the patient was again transferred to dermatology department. On 08-FEB-2013, the patient's C-reactive protein (CRP) level showed 118.79 mg/l, leucocytes count was 10.5 /nl, neutrophils count was 9.2 /nl. The patient was treated with prednisolone 250 mg, clemastin 2 mg, ranitidin 50 mg, loratidine per day 40 mg and triamcinolonacetone 0.1% 4 to 5 times a day. On 15-FEB-2013, the patient was treated with lorazepam (unspecified formulation, oral) 1 mg, for an unspecified indication and on the same day, prednisolone dose was reduced, but the event urticaria was renewed at the whole body. The patient was discharged from the hospital on 19-FEB-2013. Action taken with lorazepam was not reported. The patient had not recovered from the event drug induced exanthema at the time of this report. The patient's outcome for the event dyspnoea was not reported. The reporter assessed the causality between the suspect drugs pipamperone, aripiprazole, ibuprofen, pantoprazole and lorazepam and the event generalised drug induced exanthema as possible. This report was serious (hospitalization).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:Reporter's comment:sanofi-aventis, 26.04.2013 DE-SANOFI-AVENTIS-2013SA041105

Sender diagnosis: not reported

Narrative

Case extracted from the website of the German health authority, reported by the institute for drug safety in psychiatry, Hannover, on 18-Apr-2013 in the course of the AMSP project (prospective multicentre program for active and continuous assessment of adverse drug reactions of marketed psychotropic drugs in psychiatric inpatients under naturalistic conditions of routine clinical treatment), AMSP case number D-26-001. This spontaneous case information involves a 48-year-old female patient. Medical history: schizoaffective disorder, hypothyroidism. On 22-Jan-2013 intake of aripiprazole 5mg/day. On 23-Jan-2013, additionally intake of promethazine 25 mg/day. On 24-Jan-2013, patient reported that she had been suffering from mild itching of whole body for the previous two days. Itching resolved completely during the following days. Laboratory values from 08-Feb-2013: white blood cell analysis 10.5 nl; neutrophil count 9.2 nl; C-reactive protein 118.79 mg/l. On 22-Jan-2013 the patient started treatment with 5mg aripiprazole with successive increase to 15 mg/day (10mg/day from 24-Jan-2013 to 31-Jan-2013, 15 mg/day from 01-Feb-2013). Promethazine was temporarily discontinued from 25-Jan-2013 to 27-Jan-2013, on 28-Jan-2013 re-initiated with 50 mg/day. On 03-Feb-2013, the patient took ibuprofen (trade name not provided) by herself (dose unknown). On 04-Feb-2013, therapy change from promethazine 50 mg/day to pipamperone 40 mg/day. On 05-Feb-2013, renewed occurrence of itching, this time with urticaria. Transfer to dermatologic clinic, diagnosis by dermatologist: as partly urticarial, partly erythema exsudativum multiforme-like generalised drug-induced exanthema with sensation of globus. Withdrawal of aripiprazole (last intake on 05-Feb-2012), pipamperone (date of last intake not reported) and ibuprofen (date of last intake not reported).

Corrective treatment: initially 250 mg prednisolone, 2mg clemastine fumarate, 50mg ranitidine intravenously, in further course following treatment was given: prednisolone 50mg/day, pantoprazole (trade name not provided) 20mg/day (start date on 05-Feb-2013), loratidine 40mg/day, and 0.1% triamcinolone acetonide in special milk four to five times daily.

On 06-Feb-2013, transfer back to psychiatric ward under continuation of corrective therapy.

On 07-Feb-2013, ambulatory transfer to dermatologic clinic due to renewed aggravation of exanthema after intake of pantoprazole. Pantoprazole was withdrawn (last intake on 07-Feb-2013) because of histrionic accentuation of dyspnoea.

On 08-Feb-2013, pronounced aggravation of exanthema with renewed dyspnoea, admission to dermatologic clinic.

On 15-Feb-2013, one-time administration of 1mg lorazepam and reduction of prednisolone, leading to renewed urticarisation on whole body.

Prior use: no

Re-exposition: no

Outcome reported as not recovered.

Causality assessment by AMSP group (WHO Global Introspection) concerning allergic exanthema: "possible" for ibuprofen, pantoprazole, aripiprazole, pipamperone, and lorazepam.

Comment on causality assessment by AMSP group: Uncommon the allergic reaction to several structurally different substances, maybe additive?

Allergologic clarification planned and remains to be seen.

Sender's comment:

22-Apr-2013 Sanofi Company Comment:

Based on the available information and taking into consideration the temporal relationship and safety profile a causal relationship to ibuprofen and pantoprazole, especially a histrionic accentuation, cannot be completely excluded.

However in this case the other suspected drugs aripiprazole and pipamperone provide alternative explanations for developing of urticaria and itching. In addition the patient reacted with urticarisation after intake of lorazepam.

Reporter's comment:

McNeil Germany, Local Safety Officers, 26.04.2013 DE-JNJFOC-20130411337

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician, via a regulatory authority (BfArM (Germany) - DE-BFARM-13118872), and concerns a 48 year old female patient from Germany: Local case ID: 00690/13.

The patient's height and weight were not reported. The patient's medical history and concurrent conditions included hypothyroidism and schizoaffective disorder. The patient was treated with ibuprofen (unspecified formulation, oral, dose and frequency unspecified) initiated on 03-FEB-2013 to 03-FEB-2013 for an unknown indication and pipamperone (unspecified formulation, oral) 40 mg once a day, initiated on 04-FEB-2013 to 05-FEB-2013 for an unknown indication. Non-company suspect drugs included: aripiprazole (unspecified formulation, oral) 5 mg once day initiated on 22-JAN-2013 to 23-JAN-2013, 10 mg once a day from 24-JAN-2013 to 31-JAN-2013 and 15 mg once a day 01-FEB-2013 to 05-FEB-2013 for an unknown indication, pantoprazole (unspecified formulation, oral) 20 mg once a day initiated on 05-FEB-2013 to 07-FEB-2013 and lorazepam (unspecified formulation, oral) 1 Milligram once a day initiated on 15-FEB-2013 to 15-FEB-2013 for an unknown indication. Concomitant medications included levothyroxine sodium (unspecified formulation, oral) 75 microgram once a day initiated on 19-JAN-2013 and isopromethazine hydrochloride (unspecified formulation, oral) 25 mg once a day initiated on 23-JAN-2013. On 22-JAN-2013. The patient received Abilify (aripiprazole, 5 mg a day) and the day later she received Atosil (isopromethazine hydrochloride, 25 mg a day). On 24-JAN-2013, she reported slight itching on the whole body since 2 days. In the following days, the patient's slight itching abated completely. The dosage of Abilify was gradually increased to 15 mg/day. In the further course of events Atosil was paused and later started again with 50 mg / day. On 03-FEB-2013 the patient took ibuprofen on her own account (dose and frequency unspecified) at home, without a doctor's prescription. On 04-FEB-2013, Atosil 50 mg / day was withdrawn and Dipiperon (pipamperone, 40 mg / day) was started. On the following day the patient developed pruritus again with urticaria also this time. The patient was diagnosed with part urticarial, part EEM (Erythema exsudativum multiforme) like generalized drug eruption with globus sensation. Treatment with Abilify, Dipiperon and ibuprofen were withdrawn due to histrionic accentuation of the shortness of breath and after measurement of sufficient oxygen saturation: Ativan 1 mg expidet. The patient was treated with prednisolone 50 mg / day, pantoprazole 20 mg / day, loratidine 40 mg / day and triamcinoloneacetonide 0.1% in the lanette cold milk 4 to 5times a day. A day later the patient was transferred back for psychiatric treatment without any psychiatric medication. The patient took only somatic medications which included prednisolone (250 mg), (clemastine 2 mg), ranitidine (intravenous, 50 mg) received from the department for dermatology. One day later, the patient visited the admission of the department for dermatology again due to recurring worsening of the exanthema after the patient took the prescribed (by the department for dermatology) pantoprazole at home. This time the patient was not admitted in the department for dermatology and pantoprazole was withdrawn. On the following day there was a marked deterioration of exanthema with renewed shortness of breath, resulting in an emergency admission (fully stationary admitted) in the department for dermatology (on 08-FEB-2013), where the patient remained until 19-FEB-2013. In the department for dermatology the patient received Tavor (lorazepam, 1 mg once) on 15-FEB-2013, recommended by the psychiatric council, and simultaneous reduction of prednisolone. The patient again

developed hives on the whole body. No further psychopharmacological treatment was applied in department for dermatology. The patient was advised for treatment with tavor (lorazepam, 1 mg / day) and zopi<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Reporter's comment:

001307602, 30.04.2013 DE-ABBOTT-13X-062-1080102-00

Sender diagnosis: not reported

Narrative

Case received on 22 Apr 2013 from Nycomed; reference number 0238002.

This medically confirmed case was reported by BFARM, with reference number DE-BFARM-13118872 via Nycomed Germany with reference number DE-0968.

It concerned a 48-year-old female patient who started therapy with pantoprazole 20 mg once daily due to an unspecified indication. The patient's medical history included schizoaffective disorder and hypothyroidism. The patient was also receiving co-suspect drug oral aripiprazole 5 mg daily from 22-JAN-2013 due to an unspecified indication. On 22-JAN-2013, the patient experienced mild pruritus generalised. One day later, the patient started therapy with promethazine. On 23-JAN-2013, therapy with aripiprazole 5mg daily was discontinued. Mild pruritus generalised persisted for 2 days. On 24-JAN-2013, dose of aripiprazole was increased to 10mg/day. It was reported that, over the next couple of days pruritus abated completely. On 1-FEB-2013, dose of aripiprazole was increase to 15mg daily. On 03-FEB-2013, the patient started therapy with co-suspect drug oral ibuprofen on her own without prescription (dose, frequency: unknown for an unspecified indication. On 04 -FEB-2013, therapy with promethazine 50 mg daily was switched to another co- suspect drug, pipamperone 40 mg daily. On the following day, the patient developed pruritus and urticaria which was diagnosed by dermatology department of university hospital Dusseldorf as partially urticarial, partially EEM (erythema exudative multiforme)-like generalized drug-induced exanthema with globus feeling. Therapy with piparnperone, aripiprazole and ibuprofen were discontinued. On the next day, the patient went to the psychiatric care, but psychotropic drugs were not prescribed any more. The patient only administered somatic medication. On 05-FEB-2013, the patient administered pantoprazole at home and she presented to dermatology department with deterioration of the exanthema. The patient was not admitted in the hospital for the same. On 07-FEB-2013, therapy with pantoprazole was discontinued. On 08-FEB-2013, the patient was hospitalized due to marked deterioration of exanthema with shortness of breath and after measuring sufficient oxygen saturation. On the same day, the patient's white blood cell count was 10.5 nl, neutrophil count was 9.2 nl and C-reactive protein was 118.79 mg/L (reference ranges not provided). The patient received treatment with prednisolone 50 mg daily, loratadine, pantoprazole 20mg daily, 0.1% triamcinolonacetone and a cooling ointment for 4-5 days. On 15-FEB-2013, the patient administered co-suspect drug, lorazepam 1 mg and therapy with prednisolone was reduced and the patient developed wheals on the entire body. She was hospitalized till 19-FEB-2013.

Action taken therapy with ibuprofen, pipamperone and lorazepam was not reported. The patient had not recovered from allergic exanthema at the time of reporting.

No further information was provided.

Nycomed's Company Remarks;

Allergic exanthema is assessed as listed as per the reference safety information of pantoprazole.

It appears that the patient is allergic to multiple medications. Additionally, exanthema is an adverse reaction commonly reported in association with ibuprofen. Moreover, other suspect medications including, piparnperone and aripiprazole may have contributed to the reaction in this patient. However, it was reported that the condition worsened after the patient started treatment with pantoprazole. Therefore, a role of pantoprazole in the reported reaction cannot be completely ruled out.

This single case report is not considered to change the current knowledge of the safety profile of pantoprazole.

Sender's comment:

Reporter's comment:

PFIZERINC, 30.04.2013 DE-PFIZER INC-2013132241

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a contactable physician based on information Pfizer received from Johnson and Johnson (License partner number DE-JNJFOC-20130408589), License partner for Ibuprofen.

This spontaneous report was received from a physician and concerns a 48-year-old female patient from Germany. The patient's height and weight were not reported. The patient's medical history and concurrent conditions included: schizoaffective disorder (depressive type) and active hypothyreosis. The patient was hospitalized due to unspecified event on 15-JAN-2013. On 22-JAN-2013, the patient was treated with non company suspect drug aripiprazole (unspecified formulation, oral) 5 mg daily, for an unknown indication. On 23-JAN-2013, the patient received isopromethazine hydrochloride 25 mg daily. On 24-JAN-2013, the dose of aripiprazole was increased to 10 mg. On 24-JAN-2013, the patient experienced pruritus, but later she recovered from pruritus on an unspecified day. On 01-FEB-2013, the dose of aripiprazole was increased to 15 mg per day. The treatment with isopromethazine hydrochloride was temporarily stopped and started again with 50 mg daily. On 03-FEB-2013, the patient was treated with Ibuprofen (unspecified formulation, oral) unspecified dose, for an unknown indication. On 04-FEB-2013, the patient's treatment was switched from isopromethazine hydrochloride to pipamperone (unspecified formulation, oral) 40 mg daily for an unknown indication. Concomitant medications included isopromethazine hydrochloride and levothyroxine sodium for an unspecified indication. On 05-FEB-2013, the patient experienced pruritus, urticaria and dyspnoea and was transferred from psychotic ward to dermatology department on the same day. The patient was diagnosed partly urticarial, partly erythema exsudativum multiforme generalized drug induced exanthema with globus sensation. The treatment with aripiprazole, ibuprofen and pipamperone (permanent discontinuation) was withdrawn in 05-FEB-2013. On 05-FEB-2013, the patient took non company suspect drug pantoprazole (unspecified, oral) 20 mg and later she experienced worsening of exanthema and consulted a dermatologist. On 06-FEB-2013, the patient was transferred back to the psychiatric ward. On 07-FEB-2013, the treatment with the pantoprazole was discontinued. On 08-FEB-2013 (one day later), the patient developed distinct worsening of exanthema with renewed dyspnoea and the patient was again transferred to dermatology department. On 08-FEB-2013, the patient's C-reactive protein (CRP) level showed 118.79 mg/l, leucocytes count was 10.5 /nl, neutrophils count was 9.2 /nl. The patient was treated with prednisolone 250 mg, clemastin 2 mg, ranitidin 50 mg, loratidine per day 40 mg and triamcinolonacetone 0.1% 4 to 5 times a day. On 15-FEB-2013, the patient was treated with lorazepam (unspecified formulation, oral) 1 mg, for an unspecified indication and on the same day, prednisolone dose was reduced, but the event urticaria was renewed at the whole body. The patient was discharged from the hospital on 19-FEB-2013. Action taken with lorazepam was not reported. The patient had not recovered from the event drug induced exanthema at the time of this report. The patient's outcome for the event dyspnoea was not reported. The reporter assessed the causality between the suspect drugs pipamperone, aripiprazole, ibuprofen, pantoprazole and lorazepam and the event generalized drug induced exanthema as possible. This report was serious (hospitalization).

Sender's comment:

Based on the information provided, a possible contributory role of the suspect drugs to the reported events drug eruption and dyspnea cannot be excluded. The impact of this report on the benefit/risk profile of the Pfizer product is evaluated as part of Pfizer procedures for safety evaluation, including the review and analysis of aggregate data for adverse events. Any safety concern identified as part of this review, as well as any appropriate action in response, will be promptly notified to Regulatory Authorities, Ethics Committees and Investigators, as appropriate.

Reporter's comment:

European Electronic Reporting Group, 10.05.2013 DE-JNJFOC-20130408589

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician and concerns a 48-year-old female patient from Germany. The patient's height and weight were not reported. The patient's medical history and concurrent conditions included: schizoaffective disorder (depressive type) and active hypothyreosis. The patient was hospitalized due to unspecified event on 15-JAN-2013. On 22-JAN-2013, the patient was treated with non company suspect drug aripiprazole (unspecified formulation, oral) 5 mg daily, for an unknown indication. On 23-JAN-2013, the patient

received isopromethazine hydrochloride 25 mg daily. On 24-JAN-2013, the dose of aripiprazole was increased to 10 mg. On 24-JAN-2013, the patient experienced pruritus, but later she recovered from pruritus on an unspecified day. On 01-FEB-2013, the dose of aripiprazole was increased to 15 mg per day. The treatment with isopromethazine hydrochloride was temporarily stopped and started again with 50 mg daily. On 03-FEB-2013, the patient was treated with ibuprofen (unspecified formulation, oral) unspecified dose, for an unknown indication. On 04-FEB-2013, the patient's treatment was switched from isopromethazine hydrochloride to pipamperone (unspecified formulation, oral) 40 mg daily for an unknown indication. Concomitant medications included promethazine hydrochloride and levothyroxine sodium for an unspecified indication. On 05-FEB-2013, the patient experienced pruritus, urticaria and dyspnoea and was transferred from psychotic ward to dermatology department on the same day. The patient was diagnosed partly urticarial, partly erythema exsudativum multiforme generalised drug induced exanthema with globus sensation. The treatment with aripiprazole, ibuprofen and pipamperone (permanent discontinuation) was withdrawn in 05-FEB-2013. On 05-FEB-2013, the patient took non company suspect drug pantoprazole (unspecified, oral) 20 mg and later she experienced worsening of exanthema and consulted a dermatologist. On 06-FEB-2013, the patient was transferred back to the psychiatric ward. On 07-FEB-2013, the treatment with the pantoprazole was discontinued. On 08-FEB-2013 (one day later), the patient developed distinct worsening of exanthema with renewed dyspnoea and the patient was again transferred to dermatology department. On 08-FEB-2013, the patient's C-reactive protein (CRP) level showed 118.79 mg/l, leucocytes count was 10.5 /nl, neutrophils count was 9.2 /nl. The patient was treated with prednisolone 250 mg, clemastin 2 mg, ranitidin 50 mg, loratidine per day 40 mg and triamcinolonacetone 0.1% 4 to 5 times a day. On 15-FEB-2013, the patient was treated with lorazepam (unspecified formulation, oral) 1 mg, for an unspecified indication and on the same day, prednisolone dose was reduced, but the event urticaria was renewed at the whole body. The patient was discharged from the hospital on 19-FEB-2013. Action taken with lorazepam was not reported. The patient had not recovered from the event drug induced exanthema at the time of this report. The patient's outcome for the event dyspnoea was not reported. The reporter assessed the causality between the suspect drugs pipamperone, aripiprazole, ibuprofen, pantoprazole and lorazepam and the event generalised drug induced exanthema as possible. This report was serious (hospitalization).

Additional information was received from a physician via regulatory authority (JACGER - Janssen-Cilag, Germany - DE-BFARM-13118872) via Pfizer (DE-PFIZER INC-2013122986) via Nycomed (0238002) on 30-APR-2013.

The patient was allergic to multiple medications. On 22-JAN-2013, the patient experienced mild pruritis generalized. Concomitant medications included isopromethazine hydrochloride for pruritis (previously reported as drug history). It was stated that the patient's condition worsened after starting treatment with pantoprazole. Therefore the role of pantoprazole in association with allergic exanthema could not be ruled out and thus the causality between the e<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Reporter's comment:

PFIZERINC, 14.05.2013 DE-PFIZER INC-2013132241

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a contactable physician based on information Pfizer received from Johnson and Johnson (License partner number DE-JNJFOC-20130408589), License partner for Ibuprofen.

This spontaneous report was received from a physician and concerns a 48-year-old female patient from Germany. The patient's height and weight were not reported. The patient's medical history and concurrent conditions included: schizoaffective disorder (depressive type) and active hypothyreosis. The patient was hospitalized due to unspecified event on 15-JAN-2013. On 22-JAN-2013, the patient was treated with non company suspect drug aripiprazole (unspecified formulation, oral) 5 mg daily, for an unknown indication. On 23-JAN-2013, the patient received isopromethazine hydrochloride 25 mg daily. On 24-JAN-2013, the dose of aripiprazole was increased to 10 mg. On 24-JAN-2013, the patient experienced pruritus, but later she recovered from pruritus on an unspecified day. On 01-FEB-2013, the dose of aripiprazole was increased to 15 mg per day. The treatment with isopromethazine hydrochloride was temporarily stopped and started again with 50 mg daily. On 03-FEB-2013, the patient was treated with ibuprofen (unspecified formulation, oral) unspecified dose, for an unknown indication. On 04-FEB-2013, the patient's treatment was switched from isopromethazine hydrochloride to pipamperone (unspecified formulation, oral) 40 mg daily for an unknown indication. Concomitant medications included isopromethazine hydrochloride and levothyroxine sodium for an unspecified indication. On 05-FEB-2013, the patient experienced pruritus, urticaria and dyspnoea and was transferred from psychotic ward to dermatology department on the same day. The patient was diagnosed partly urticarial, partly erythema exsudativum multiforme generalized drug induced exanthema with globus sensation. The treatment with aripiprazole, ibuprofen and pipamperone (permanent discontinuation) was withdrawn in 05-FEB-2013. On 05-FEB-2013, the patient took non company suspect drug pantoprazole (unspecified, oral) 20 mg and later she experienced worsening of exanthema and consulted a dermatologist. On 06-FEB-2013, the patient was transferred back to the psychiatric ward. On 07-FEB-2013, the treatment with the pantoprazole was discontinued. On 08-FEB-2013 (one day later), the patient developed distinct worsening of exanthema with renewed dyspnoea and the patient was again transferred to dermatology

department. On 08-FEB-2013, the patient's C-reactive protein (CRP) level showed 118.79 mg/l, leucocytes count was 10.5 /nl, neutrophils count was 9.2 /nl. The patient was treated with prednisolone 250 mg, clemastin 2 mg, ranitidin 50 mg, loratidine per day 40 mg and triamcinolonacetonide 0.1% 4 to 5 times a day. On 15-FEB-2013, the patient was treated with lorazepam (unspecified formulation. oral) 1 mg, for an unspecified indication and on the same day, prednisolone dose was reduced, but the event urticaria was renewed at the whole body. The patient was discharged from the hospital on 19-FEB-2013. Action taken with lorazepam was not reported. The patient had not recovered from the event drug induced exanthema at the time of this report. The patient's outcome for the event dyspnoea was not reported. The reporter assessed the causality between the suspect drugs pipamperone, aripiprazole, ibuprofen, pantoprazole and lorazepam and the event generalized drug induced exanthema as possible. This report was serious (hospitalization).

Follow-up (30Apr2013): New information received by Pfizer from Johnson and Johnson (manufacturer control number DE-JNJFOC-20130408589), license party for ibuprofen, includes: Additional information was received from a physician via regulatory authority (JACGER - Janssen-Cilag, Germany - DE-BFARM-13118872) via Pfizer (DE-PFIZER INC-2013122986) via Nycomed (0238002) on 30-APR-2013. The patient was allergic to multiple medications. On 22-JAN-2013, the patient experienced mild pruritis generalized. Concomitant medications included isopromethazine hydrochloride for pruritis (previously reported as drug history). It was stated that the patient's condition worsened after starting treatment with pantoprazole. Therefore the role of pantoprazole in association with allergic exanthema could not be ruled out and thus the causality between the event exanthema and pantoprazole was assessed as related. This case is a duplicate of 20130414884.

Both the reporter and Johnson and Johnson assessed the events to be possibly related to ibuprofen.

Pfizer is Marketing Authorization Holder of ibuprofen in the reporters country. This may be a duplicate report in situations where another Marketing Authorization Holder of ibuprofen has submitted the same report to the regulatory authorities.

Sender's comment:

Based on the information provided, a possible contributory role of the suspect drugs to the reported events drug eruption and dyspnea cannot be excluded. The impact of this report on the benefit/risk profile of the Pfizer product is evaluated as part of Pfizer procedures for safety evaluation, including the review and analysis of aggregate data for adverse events. Any safety concern identified as part of this review, as well as any appropriate action in response, will be promptly notified to Regulatory Authorities, Ethics Committees and Investigators, as appropriate.

Reporter's comment:

PFIZERINC, 21.05.2013 DE-PFIZER INC-2013132241

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a contactable physician based on information Pfizer received from Johnson and Johnson (License partner number DE-JNJFOC-20130408589), License partner for Ibuprofen.

This spontaneous report was received from a physician and concerns a 48-year-old female patient from Germany. The patient's height and weight were not reported. The patient's medical history and concurrent conditions included: schizoaffective disorder (depressive type) and active hypothyreosis. The patient was hospitalized due to unspecified event on 15-JAN-2013. On 22-JAN-2013, the patient was treated with non company suspect drug aripiprazole (unspecified formulation, oral) 5 mg daily, for an unknown indication. On 23-JAN-2013, the patient received isopromethazine hydrochloride 25 mg daily. On 24-JAN-2013, the dose of aripiprazole was increased to 10 mg. On 24-JAN-2013, the patient experienced pruritus, but later she recovered from pruritus on an unspecified day. On 01-FEB-2013, the dose of aripiprazole was increased to 15 mg per day. The treatment with isopromethazine hydrochloride was temporarily stopped and started again with 50 mg daily. On 03-FEB-2013, the patient was treated with Ibuprofen (unspecified formulation, oral) unspecified dose, for an unknown indication. On 04-FEB-2013, the patient's treatment was switched from isopromethazine hydrochloride to pipamperone (unspecified formulation, oral) 40 mg daily for an unknown indication. Concomitant medications included isopromethazine hydrochloride and levothyroxine sodium for an unspecified indication. On 05-FEB-2013, the patient experienced pruritus, urticaria and dyspnoea and was transferred from psychotic ward to dermatology department on the same day. The patient was diagnosed partly urticarial, partly erythema exsudativum multiforme generalized drug induced exanthema with globus sensation. The treatment with aripiprazole, ibuprofen and pipamperone (permanent discontinuation) was withdrawn in 05-FEB-2013. On 05-FEB-2013, the patient took non company suspect drug pantoprazole (unspecified, oral) 20 mg and later she experienced worsening of exanthema and consulted a dermatologist. On 06-FEB-2013, the patient was transferred back to the psychiatric ward. On 07-FEB-2013, the treatment with the pantoprazole was discontinued. On 08-FEB-2013 (one day later), the patient developed distinct worsening of exanthema with renewed dyspnoea and the patient was again transferred to dermatology department. On 08-FEB-2013, the patient's C-reactive protein (CRP) level showed 118.79 mg/l, leucocytes count was 10.5 /nl, neutrophils count was 9.2 /nl. The patient was treated with prednisolone 250 mg, clemastin 2 mg, ranitidin 50 mg, loratidine per day 40 mg and triamcinolonacetonide 0.1% 4 to 5 times a day. On 15-FEB-2013, the patient was treated with lorazepam (unspecified formulation. oral) 1 mg, for an unspecified indication and on

the same day, prednisolone dose was reduced, but the event urticaria was renewed at the whole body. The patient was discharged from the hospital on 19-FEB-2013. Action taken with lorazepam was not reported. The patient had not recovered from the event drug induced exanthema at the time of this report. The patient's outcome for the event dyspnoea was not reported. The reporter assessed the causality between the suspect drugs pipamperone, aripiprazole, ibuprofen, pantoprazole and lorazepam and the event generalized drug induced exanthema as possible. This report was serious (hospitalization).

Follow-up (30Apr2013): New information received by Pfizer from Johnson and Johnson (manufacturer control number DE-JNJFOC-20130408589), license party for ibuprofen, includes: Additional information was received from a physician via regulatory authority (JACGER - Janssen-Cilag, Germany - DE-BFARM-13118872) via Pfizer (DE-PFIZER INC-2013122986) via Nycomed (0238002) on 30-APR-2013. The patient was allergic to multiple medications. On 22-JAN-2013, the patient experienced mild pruritis generalized. Concomitant medications included isopromethazine hydrochloride for pruritis (previously reported as drug history). It was stated that the patient's condition worsened after starting treatment with pantoprazole. Therefore the role of pantoprazole in association with allergic exanthema could not be ruled out and thus the causality between the event exanthema and pantoprazole was assessed as related. This case is a duplicate of 20130414884.

Both the reporter and Johnson and Johnson assessed the events to be possibly related to ibuprofen.

Pfizer is Marketing Authorization Holder of ibuprofen in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of ibuprofen has submitted the same report to the regulatory authorities.

Follow-up (09May2013): New information received by Pfizer from Johnson and Johnson (manufacturer control number DE-JNJFOC-20130408589), license party for ibuprofen, includes: This version was created on 09-MAY-2013, for the purpose of case quality improvement. Upon review, following correction was made: The therapy with concomitant medication isopromethazine hydrochloride was initiated for an unknown indication (previously reported in narrative as pruritis).

Sender's comment:

Based on the information provided, a possible contributory role of the suspect drugs to the reported events drug eruption and dyspnea cannot be excluded. The follow-up information received does not alter the previous company clinical evaluation. The impact of this report on the benefit/risk profile of the Pfizer product is evaluated as part of Pfizer procedures for safety evaluation, including the review and analysis of aggregate data for adverse events. Any safety concern identified as part of this review, as well as any appropriate action in response, will be promptly notified to Regulatory Authorities, Ethics Committees and Investigators, as appropriate.

Reporter's comment:

European Electronic Reporting Group, 22.05.2013 DE-JNJFOC-20130408589

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician and concerns a 48-year-old female patient from Germany. The patient's height and weight were not reported. The patient's medical history and concurrent conditions included: schizoaffective disorder (depressive type) and active hypothyreosis. The patient was hospitalized due to unspecified event on 15-JAN-2013. On 22-JAN-2013, the patient was treated with non company suspect drug aripiprazole (unspecified formulation, oral) 5 mg daily, for an unknown indication. On 23-JAN-2013, the patient received isopromethazine hydrochloride 25 mg daily. On 24-JAN-2013, the dose of aripiprazole was increased to 10 mg. On 24-JAN-2013, the patient experienced pruritus, but later she recovered from pruritus on an unspecified day. On 01-FEB-2013, the dose of aripiprazole was increased to 15 mg per day. The treatment with isopromethazine hydrochloride was temporarily stopped and started again with 50 mg daily. On 03-FEB-2013, the patient was treated with Ibuprofen (unspecified formulation, oral) unspecified dose, for an unknown indication. On 04-FEB-2013, the patient's treatment was switched from isopromethazine hydrochloride to pipamperone (unspecified formulation, oral) 40 mg daily for an unknown indication. Concomitant medications included promethazine hydrochloride and levothyroxine sodium for an unspecified indication. On 05-FEB-2013, the patient experienced pruritus, urticaria and dyspnoea and was transferred from psychotic ward to dermatology department on the same day. The patient was diagnosed partly urticarial, partly erythema exsudativum multiforme generalised drug induced exanthema with globus sensation. The treatment with aripiprazole, ibuprofen and pipamperone (permanent discontinuation) was withdrawn in 05-FEB-2013. On 05-FEB-2013, the patient took non company suspect drug pantoprazole (unspecified, oral) 20 mg and later she experienced worsening of exanthema and consulted a dermatologist. On 06-FEB-2013, the patient was transferred back to the psychiatric ward. On 07-FEB-2013, the treatment with the pantoprazole was discontinued. On 08-FEB-2013 (one day later), the patient developed distinct worsening of exanthema with renewed dyspnoea and the patient was again transferred to dermatology department. On 08-FEB-2013, the patient's C-reactive protein (CRP) level showed 118.79 mg/l, leucocytes count was 10.5 /nl, neutrophils count was 9.2 /nl. The patient was treated with prednisolone 250 mg, clemastin 2 mg, ranitidin 50 mg, loratidine per day 40 mg and triamcinolonacetone 0.1% 4 to 5 times a day. On 15-FEB-2013, the patient was treated with lorazepam (unspecified formulation, oral) 1 mg, for an unspecified indication and on

the same day, prednisolone dose was reduced, but the event urticaria was renewed at the whole body. The patient was discharged from the hospital on 19-FEB-2013. Action taken with lorazepam was not reported. The patient had not recovered from the event drug induced exanthema at the time of this report. The patient's outcome for the event dyspnoea was not reported. The reporter assessed the causality between the suspect drugs pipamperone, aripiprazole, ibuprofen, pantoprazole and lorazepam and the event generalised drug induced exanthema as possible. This report was serious (hospitalization).

Additional information was received from a physician via regulatory authority (JACGER - Janssen-Cilag, Germany - DE-BFARM-13118872) via Pfizer (DE-PFIZER INC-2013122986) via Nycomed (0238002) on 30-APR-2013.

The patient was allergic to multiple medications. On 22-JAN-2013, the patient experienced mild pruritis generalized. Concomitant medications included isopromethazine hydrochloride for pruritis (previously reported as drug history). It was stated that the patient's condition worsened after starting treatment with pantoprazole. Therefore the role of pantoprazole in association with allergic exanthema could not be ruled out and thus the causality between the e<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Reporter's comment:

McNeil Germany, Local Safety Officers, 21.06.2013 DE-JNJFOC-20130408589

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician and concerns a 48-year-old female patient from Germany.

The patient's height and weight were not reported. The patient's medical history and concurrent conditions included: schizoaffective disorder (depressive type) and active hypothyreosis. The patient was hospitalized due to unspecified event on 15-JAN-2013. On 22-JAN-2013, the patient was treated with non company suspect drug aripiprazole (unspecified formulation, oral) 5 mg daily, for an unknown indication. On 23-JAN-2013, the patient received isopromethazine hydrochloride 25 mg daily. On 24-JAN-2013, the dose of aripiprazole was increased to 10 mg. On 24-JAN-2013, the patient experienced pruritus, but later she recovered from pruritus on an unspecified day. On 01-FEB-2013, the dose of aripiprazole was increased to 15 mg per day. The treatment with isopromethazine hydrochloride was temporarily stopped and started again with 50 mg daily. On 03-FEB-2013, the patient was treated with Ibuprofen (unspecified formulation, oral) unspecified dose, for an unknown indication. On 04-FEB-2013, the patient's treatment was switched from isopromethazine hydrochloride to pipamperone (unspecified formulation, oral) 40 mg daily for an unknown indication. Concomitant medications included promethazine hydrochloride and levothyroxine sodium for an unspecified indication. On 05-FEB-2013, the patient experienced pruritus, urticaria and dyspnoea and was transferred from psychotic ward to dermatology department on the same day. The patient was diagnosed partly urticarial, partly erythema exsudativum multiforme generalised drug induced exanthema with globus sensation. The treatment with aripiprazole, ibuprofen and pipamperone (permanent discontinuation) was withdrawn in 05-FEB-2013. On 05-FEB-2013, the patient took non company suspect drug pantoprazole (unspecified, oral) 20 mg and later she experienced worsening of exanthema and consulted a dermatologist. On 06-FEB-2013, the patient was transferred back to the psychiatric ward. On 07-FEB-2013, the treatment with the pantoprazole was discontinued. On 08-FEB-2013 (one day later), the patient developed distinct worsening of exanthema with renewed dyspnoea and the patient was again transferred to dermatology department. On 08-FEB-2013, the patient's C-reactive protein (CRP) level showed 118.79 mg/l, leucocytes count was 10.5 /nl, neutrophils count was 9.2 /nl. The patient was treated with prednisolone 250 mg, clemastin 2 mg, ranitidin 50 mg, loratidine per day 40 mg and triamcinolonacetone 0.1% 4 to 5 times a day. On 15-FEB-2013, the patient was treated with lorazepam (unspecified formulation, oral) 1 mg, for an unspecified indication and on the same day, prednisolone dose was reduced, but the event urticaria was renewed at the whole body. The patient was discharged from the hospital on 19-FEB-2013. Action taken with lorazepam was not reported. The patient had not recovered from the event drug induced exanthema at the time of this report. The patient's outcome for the event dyspnoea was not reported. The reporter assessed the causality between the suspect drugs pipamperone, aripiprazole, ibuprofen, pantoprazole and lorazepam and the event generalised drug induced exanthema as possible. This report was serious (hospitalization).

Additional information was received from a physician via regulatory authority (JACGER - Janssen-Cilag, Germany - DE-BFARM-13118872) via Pfizer (DE-PFIZER INC-2013122986) via Nycomed (0238002) on 30-APR-2013.

The patient was allergic to multiple medications. On 22-JAN-2013, the patient experienced mild pruritis generalized. Concomitant medications included isopromethazine hydrochloride for pruritis (previously reported as drug history). It was stated that the patient's condition worsened after starting treatment with pantoprazole. Therefore the role of pantoprazole in association with allergic exanthema could not be ruled out and thus the causality between the e<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13295108

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No			No

Patient

Initials:	Sex: Female	Birth: privacy	Age: 77 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Allergy	201308			3 Week	recovering/resolving
2 Scar pain					recovering/resolving
3 Vesiculobullous rash					recovering/resolving
4 Rash both legs					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 IBUFLAM	Oral	400 Mg milligram(s)/82	201308-20130827	Drug use for unknown indication
ibuprofen lysinate			-	

Primary source

Qualification: Consumer or other non health professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)[sanofi-aventis, 18.09.2013 DE-SANOFI-AVENTIS-2013SA086526](#)

Sender diagnosis: not reported

Narrative

Initial Report received on 28-aug-2013 from a consumer.

This spontaneous case information involves a 77-year-old female patient. Medical history not provided.

The patient started treatment with of ibuprofen lysinate (Ibuflam Lysin 400mg, tabfil) several days prior to this report. Dosage and indication not provided.

She then developed rash. After consultation with family doctor withdrawal of ibuprofen lysinate on 27-Aug-2013 was initiated. Considerate worsening of rash during the following night occurred.

No information provided on corrective treatment.

Additional information was received from the patient by letter on 16-Sep-2013:

The patient reported in her signed "Patients"Consent to Release Confidential and Privileged Medical Information" form that she had been hospitalized at Skin Hospital of University of Mainz, Germany from 30-Aug-2013 to 09-Sep-2013.

Further information was not reported.

Follow-up is in progress.

Sender's comment:

Sanofi Company Comment dated 16-SEP-2013: Based on the compatible temporal relationship and known safety profile of the suspected drug the causal role of ibuprofen in occurrence of rash cannot be excluded. However, indication for drug intake, precise therapy data, detailed patient's medical history, complete list of concomitant medications, specific circumstances surrounding the event, corrective treatment, outcome, exclusion of other possible causes, etc. would be necessary to make a proper medical assessment of the case.

Reporter's comment:

sanofi-aventis, 08.11.2013 DE-SANOFI-AVENTIS-2013SA086526

Sender diagnosis: not reported

Narrative

Initial Report received on 28-aug-2013 from a consumer.

This spontaneous case information involves a 77-year-old female patient. Medical history not provided.

The patient started treatment with of ibuprofen lysinate (Ibuflam Lysin 400mg, tabfil) several days prior to this report. Dosage and indication not provided.

Three weeks after treatment start a severe allergy was developed with red spots on legs up to knees. Twenty four hours later, her legs became totally red with small to large blister. The patients' husband stated that ibuprofen tablets had caused the severe allergy. After consultation with family doctor withdrawal of ibuprofen lysinate on 27-Aug-2013 was initiated. Considerate worsening of rash during the following night occurred.

The patient's primary care physician referred the patient to a dermatology hospital on 30-Aug-2013, where she was treated for 11 days, with severe pain. She was discharged from the hospital with open wounds and subsequently had been treated by a dermatologist until now for the last 8 weeks. The wounds healed very slowly and the wound dressings had to be changed every 3 days. The reporter required for compensation.

Additional information was received from the patient by letter on 16-Sep-2013:

The patient reported in her signed "Patients"Consent to Release Confidential and Privileged Medical Information" form that she had been hospitalized at Skin Hospital of University of Mainz, Germany from 30-Aug-2013 to 09-Sep-2013.

Further information was not reported.

Additional information received on 30-Oct-2013 from patients' husband.

First dose to event; main event changed to allergy, symptoms added, corrective treatment

Sender's comment:

31-Oct-2013 Sanofi Company Comment:

Based on the compatible temporal relationship and known safety profile of the suspected drug the causal role of ibuprofen in occurrence of allergy with rash cannot be excluded. However, indication for drug intake, precise therapy data, detailed patient's medical history, complete list of concomitant medications, specific circumstances surrounding the event, more information on corrective treatment, further outcome, exclusion of other possible causes, etc. would be necessary to make a proper medical assessment of the case.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13308620

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Female Birth: privacy Age: 73 Year weight (kg): 64 BMI_int: 28.44 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Generalized pruritus	20130917		10 Day	2 Day	recovered/resolved
2 Generalized urticaria	20130917		10 Day	2 Day	recovered/resolved
3 Vesicles	20130917		10 Day	2 Day	recovered/resolved
4 Petechiae	20130917		10 Day	2 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuhexal akut 400mg ibuprofen		/	20130916-20130917 2 Day	Rib fracture

Drugs (concomitant)

2 fosavance 70mg/5600IE alendronate sodium; colecalciferol	1 Df dosage form/Week 245	-	Osteoporosis
3 l-thyroxin 100 levothyroxine sodium	/ 245	-	Hypothyroidism

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Drug allergy			Unknown	Codein, Ketoprofen
Penicillin allergy			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13404411

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 83 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Exanthematic drug eruption	20131025	20131030	6 Day	1 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 IBUPROFEN ibuprofen	Oral	400 Mg milligram(s)/Day 245	20131025-20131026 2 Day	Pain

Drugs (concomitant)

2 AMOXICILLIN amoxicillin	Oral	3000 Mg milligram(s)/Day 245	20131017-20131021 5 Day	Tooth extraction
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Tooth extraction			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)NOVARTIS, 02.01.2014 DE-002147023-GXKR2013DE003903

Sender diagnosis: not reported

Narrative

Case no. GXKR2013DE003903 is an initial spontaneous report from a physician, via the German Health Authority (BfArM homepage, case no DE-BFARM-13404411), received on 27Dec2013: This report refers to a 83-year-old female patient with a medical history of tooth extraction. Concomitant medication included amoxicillin. For pain, the patient received ibuprofen (MAH unknown) from 25Oct2013 at a daily oral dose of 1x400 mg. From the same day, the patient experienced exanthematic drug eruption (markedly truncal and mostly spotted). The event was reported serious. Treatment with suspect medication was stopped on 26Oct2013. The outcome of the event was reported as not resolved. Causality was assessed as probable/likely by the reporter.

Sender's comment:

Based on review of available reported data there is a reasonable possibility of causal relationship for the event(s).

Reporter's comment:Boehringer Ingelheim GmbH, Germany, 03.01.2014 DE-Boehringer Ingelheim GmbH, Germany-2013-BI-44165DE

Sender diagnosis: not reported

Narrative

Initial information was received from a physician via health authority (DE-BFARM-13404411) in Germany concerning an 83 year old female patient, who was on Ibuprofen (formulation not reported) 400mg once a day orally for the treatment of pain. The lot number was not reported. The patient

initiated treatment on 25Oct2013. Duration of Ibuprofen was 2 days.

On 25Oct2013, the patient had exanthematic drug eruption. The event was considered serious due to hospitalization and prolonged hospitalization.

Duration of the event was reported to be 6 days and first time of the event was 1day. Last time of the event was 2 day. Time interval between last dose of drug and start of reaction/event was 2 days.

Therapy for event was not reported.

The patient had not yet recovered from the event, however end date was reported as 30Oct2013.

Ibuprofen was discontinued on 26Oct2013.

Concomitant medication included Amoxicillin for tooth extraction from 17Oct2013 to 21Oct2013. Further concomitant diseases were not reported.

It was the opinion of the reporter that the event was related to Ibuprofen therapy.

Tracking of Changes:

30Dec2013 initial information received via the German Health Authority.

Sender's comment:

Reporter's comment:

McNeil Germany, Local Safety Officers, 03.01.2014 DE-JNJFOC-20131217230

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician, via a regulatory authority (McNeil Germany - DE-BFARM-13404411), and concerned an 83-year-old female patient from Germany: 02392/13.

The patient's height and weight was not reported.

The patient's medical history was not reported.

The patient was treated with ibuprofen (unspecified formulation, batch, oral) 400 mg once a day, initiated on 25-OCT-2013 for pain.

Concomitant medications included amoxicillin.

On 25-OCT-2013, the patient experienced an exanthematic drug eruption.

On 26-OCT-2013, the treatment with ibuprofen was withdrawn.

The patient had not recovered from exanthematic drug eruption.

This report was serious (hospitalization).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Reporter's comment:

Bayer Vital GmbH, 06.01.2014 DE-BAYER-2014-000499

Sender diagnosis: not reported

Narrative

Case status: serious, related and listed.

This is a spontaneous case report received from a physician via regulatory authority (case# DE-BFARM-13404411) in GERMANY on 27-DEC-2013 which refers to an 83-year-old female patient who received IBUPROFEN (ibuprofen).

No information given on patient's history, past drugs and concurrent conditions.

The patient received the following concomitant medication: Amoxicillin (amoxicillin) for tooth extraction.

On 25-OCT-2013 the patient started IBUPROFEN at 400 mg, QD (every day) orally for pain. It was unknown whether IBUPROFEN was used previously.

On 25-OCT-2013 the patient experienced EXANTHEMATIC DRUG ERUPTION and therefore the patient was hospitalized. The outcome of the event was not recovered / not resolved. IBUPROFEN was discontinued on 26-OCT-2013.

Reporter causality:

The relationship between EXANTHEMATIC DRUG ERUPTION and IBUPROFEN was assessed as probable/likely [global introspection (WHO GI) method].

Sender's comment:

Reporter's comment:

McNeil Germany, Local Safety Officers, 07.02.2014 DE-JNJFOC-20131217230

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician, via a regulatory authority (McNeil Germany - DE-BFARM-13404411), and concerned an 83-year-old female patient from Germany: 02392/13.

The patient's height and weight was not reported.

The patient's medical history was not reported.

The patient was treated with ibuprofen (unspecified formulation, batch, oral) 400 mg once a day, initiated on 25-OCT-2013 for pain.

Concomitant medications included amoxicillin.

On 25-OCT-2013, the patient experienced an exanthematic drug eruption.

On 26-OCT-2013, the treatment with ibuprofen was withdrawn.

The patient had not recovered from exanthematic drug eruption.

This report was serious (hospitalization).

Follow-up information was received on 29-JAN-2014:

It was determined that nullification and deletion were required for Manufacturer Report Number 20140100747 as it is a duplicate of AER 20131217230. Business partner (Pfizer Inc., DE-PFIZER INC-2013373433) and secondary reporter was added to the case.

The reporter's causality between exanthematic drug eruption and ibuprofen was assessed as probable/likely with the method Global Introspection (WHO GI).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Reporter's comment:

McNeil Germany, Local Safety Officers, 13.02.2014 DE-JNJFOC-20131217230

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician, via a regulatory authority (McNeil Germany - DE-BFARM-13404411), and concerned an 83-year-old female patient from Germany: 02392/13.

The patient's height and weight was not reported.

The patient's medical history was not reported.

The patient was treated with ibuprofen (unspecified formulation, batch, oral) 400 mg once a day, initiated on 25-OCT-2013 for pain.

Concomitant medications included amoxicillin.

On 25-OCT-2013, the patient experienced an exanthematic drug eruption.

On 26-OCT-2013, the treatment with ibuprofen was withdrawn.

The patient had not recovered from exanthematic drug eruption.

This report was serious (hospitalization).

Follow-up information was received on 29-JAN-2014:

It was determined that nullification and deletion were required for Manufacturer Report Number 20140100747 as it is a duplicate of AER 20131217230. Business partner (Pfizer Inc., DE-PFIZER INC-2013373433) and secondary reporter was added to the case.

The reporter's causality between exanthematic drug eruption and ibuprofen was assessed as probable/likely with the method Global Introspection (WHO GI).

Additional information was received on 06-FEB-2014.

Additional reporter was added.<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14196939

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Female Birth: privacy Age: 8 Year weight (kg): 50 BMI_int: 20.81 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gingivostomatitis	20110331	20110416	17 Day	3 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Oral	600 Mg milligram(s)/	20110329-20110330	Head pain
			2 Day	

Primary source

Qualification: Consumer or other non health professional observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

Drug Commission of the German Medical Association, 01.07.2014 DE-DCGMA-14163665

Sender diagnosis: not reported

Narrative

Sender's comment:

Reporter's comment:

Maßnahme: Umstellung auf das Schmerzmittel Paracetamol. Nach Gabe von Ibuprofen treten bei Stress sofort wieder Bläschen im Mund auf.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14230888

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Male	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Swelling					not recovered/not resolved
2 Ankle ulcer					not recovered/not resolved
3 Blisters					not recovered/not resolved
4 Redness					not recovered/not resolved
5 Stevens-Johnson syndrome					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 IBUFLAM	Oral	/	-	Pain NOS
ibuprofen		82	-	

Primary source

Qualification: Consumer or other non health professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

sanofi-aventis, 01.08.2014 DE-SA-2014SA102466

Sender diagnosis: not reported

Narrative

Initial information regarding this unsolicited case from Germany was reported by a consumer and transmitted to sanofi 29-Jul-2014.

This case concerns an unknown age of male patient who experienced redness, blisters, ankle ulcer and swelling after starting ibuprofen (Ibuflam).

Patient's past medical history and concomitant medications were unknown.

On an unknown date, the patient started treatment with oral film coated ibuprofen tablet (lot # and expiration date unknown) (Dosing details were unknown) for pain NOS (analgesic therapy). On an unknown date, the patient developed redness, blisters (Stevens-Johnson syndrome), a hole/opening in the ankle region (5 cm in diameter) and swelling under the medication. The patient has received treatments with fatty gauze, dressings, etc. from a dermatologist for 5 months without significant success. Corrective treatment included fatty gauze and dressings.

Sanofi causality: Possible.

Seriousness criteria included important medical event.

Therapy status with ibuprofen was unknown. At the time of report, event were not resolved.

No further relevant information was reported.

Sender's comment:

Sanofi Company Comment dated 31-Jul-2014: Based on information provided, the relation is difficult to determine because limited information is provided. Needed more information regarding details of medical history or concurrent conditions, family history and causality assessment received directly from a treating health care provider to make complete assessment do case.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14242856

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 21 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen		/	-	Dysmenorrhea
ibuprofen			-	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Balint B. Blood Transfus. 2014 Jul;12(3):438-9. doi: 10.2450/2014.0065-14. Ibuprofen-induced extensive toxic epidermal necrolysis - a multidisciplinary therapeutic approach in a single case. Balint B , Stepic N , Todorovic M , Zolotarevski L , Ostojic G , Vucetic D , Pavlovic M , Novakovic M .	

Narratives (nur bei elektronischem Eingang)

Temmler Pharma GmbH & Co. KG, 11.08.2014 DE-TEMLER06-007043

Sender diagnosis: not reported

Narrative

Temmler Pharma received this case on 07-Aug-2014 via literature research on the homepage of NBCI (National Center for Biotechnology Information):

A case of a 21-year old female with extensive erythema, necrosis, and exfoliating bullous detachment of the epidermis and mucous membranes (conjunctival, oral and genital; affected skin area=80%; Initially, she had an influenza-like prodrome after taking ibuprofen to treat a headache and dysmenorrhea.

She was admitted to the Clinic for Plastic Surgery of MMA (Belgrade, Serbia) and on presentation she was febrile (39.3 °C) with a characteristic positive Nikolsky's sign.

Laboratory analyses were as follows: haemoglobin, 121 g/L; white blood cell count, 6.77×10⁹/L (neutrophils 80%, lymphocytes 25%, eosinophils 4%, monocytes 1%), platelet count, 466×10⁹/L; elevated levels of C-reactive protein, aspartate aminotransferase and alanine aminotransferase (330 mg/L, 80 IU/L and 145 IU/L, respectively), and low concentrations of total proteins (50 g/L) and albumin (22 g/L). The results of coagulation studies and tests for viral infections were normal. Skin biopsy demonstrated prominent cell death with basal vacuolar changes and lymphocyte infiltrates, obscuring the dermo-epidermal junction which confirmed the clinical diagnosis of TEN.

The first-line treatment was immediate withdrawal of the culprit drug, elimination of the drug and its metabolites, and fluid resuscitation with crystalloid infusions (1 mL/kg of body weight per hour) via a central venous catheter, adjusted on the bases of the arterial blood pressure (>65 mmHg), central venous pressure (<10 mmHg) and urine output (diuresis rate). The patient was isolated (using aseptic techniques and state) and local dermatological and ocular topical Treatment were applied continuously. Nutrition was provided enterally via a nasogastric tube.

In the early treatment of this patient, physicians performed their originally designed multimodal therapeutic apheresis -plasma exchange (PE) combined with leucapheresis- using COBE®-Spectra apheresis-sets (Terumo BCT, Lakewood, CO, USA) and a sterile connected multibag system. Multimodal therapeutic apheresis simultaneously provides rapid improvements in more than one blood abnormality and aids the patient's recovery from a life-threatening emergency to a clinical condition with a potentially positive outcome. The rationale for initial plasma exchange in this case was to eliminate/decrease the level of residual ibuprofen and its metabolites, critical cytokines (such as tumour necrosis factor- α , interferon- γ), and drug-induced inflammatory mediators (perforin, granzyme B released from cytotoxic T lymphocytes and granulysin secreted by cytotoxic T lymphocytes and natural killer cells) from the circulation (urgent plasma depuration). Plasma exchange was performed on three consecutive days by processing an average of 5900±952 mL of the patient's whole blood. A total of 5.4-fold the plasma volume was exchanged and replaced by albumin in normal Saline. The basic goal of the leucapheresis-treatment was to reduce the circulating lymphocyte count in the patient's blood to obtain an immunomodulatory effect.

The patient's subsequent systemic treatment included broad-spectrum antibiotics (chosen on the basis of the skin microbial findings), intravenous immunoglobulins (dose 1.0 g/kg of body weight per day for 3 consecutive days; infused over 6 hours) and corticosteroids (dose 0.5 mg/kg of body weight)^{1,2}. This young female patient recovered completely after 1 month of intensive systemic and topical treatment.

Temmler Pharma: DIC-2014-042<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Company comment Temmler Pharma:

Toxic epidermal necrolysis is listed in the SmPC of ibuprofen (IBU). The reaction occurred in a plausible temporal relationship with IBU intake. The reaction abated after IBU was discontinued and appropriate treatment was applied. No other potential cause of the reaction was reported. A causal relationship with IBU was assessed as probable.

This case is medically confirmed and the overall benefit-risk-ratio is not changed.

Reporter's comment:

Temmler Pharma GmbH & Co. KG, 13.08.2014 DE-TEMMLER06-007043

Sender diagnosis: not reported

Narrative

Temmler Pharma received this case on 07-Aug-2014 via literature research on the homepage of NBCI (National Center for Biotechnology Information):

A case of a 21-year old female with extensive erythema, necrosis, and exfoliating bullous detachment of the epidermis and mucous membranes (conjunctival, oral and genital; affected skin area=80%; Initially, she had an influenza-like prodrome after taking ibuprofen to treat a headache and dysmenorrhea.

She was admitted to the Clinic for Plastic Surgery of MMA (Belgrade, Serbia) and on presentation she was febrile (39.3 °C) with a characteristic positive Nikolsky's sign.

Laboratory analyses were as follows: haemoglobin, 121 g/L; white blood cell count, 6.77×10⁹/L (neutrophils 80%, lymphocytes 25%, eosinophils 4%, monocytes 1%), platelet count, 466×10⁹/L; elevated levels of C-reactive protein, aspartate aminotransferase and alanine aminotransferase (330 mg/L, 80 IU/L and 145 IU/L, respectively), and low concentrations of total proteins (50 g/L) and albumin (22 g/L). The results of coagulation studies and tests for viral infections were normal. Skin biopsy demonstrated prominent cell death with basal vacuolar changes and lymphocyte infiltrates, obscuring the dermo-epidermal junction which confirmed the clinical diagnosis of TEN.

The first-line treatment was immediate withdrawal of the culprit drug, elimination of the drug and its metabolites, and fluid resuscitation with crystalloid infusions (1 mL/kg of body weight per hour) via a central venous catheter, adjusted on the bases of the arterial blood pressure (>65 mmHg), central venous pressure (<10 mmHg) and urine output (diuresis rate). The patient was isolated (using aseptic techniques and state) and local dermatological and ocular topical Treatment were applied continuously. Nutrition was provided enterally via a nasogastric tube.

In the early treatment of this patient, physicians performed their originally designed multimodal therapeutic apheresis -plasma exchange (PE) combined with leucapheresis- using COBE®-Spectra apheresis-sets (Terumo BCT, Lakewood, CO, USA) and a sterile connected multibag system. Multimodal therapeutic apheresis simultaneously provides rapid improvements in more than one blood abnormality and aids the patient's recovery from a life-threatening emergency to a clinical condition with a potentially positive outcome. The rationale for initial plasma exchange in this case was to eliminate/decrease the level of residual ibuprofen and its metabolites, critical cytokines (such as tumour necrosis factor- α , interferon- γ), and drug-induced inflammatory mediators (perforin, granzyme B released from cytotoxic T lymphocytes and granulysin secreted by cytotoxic T lymphocytes and natural killer cells) from the circulation (urgent plasma depuration). Plasma exchange was performed on three consecutive days by processing an average of 5900±952 mL of the patient's whole blood. A total of 5.4-fold the plasma volume was exchanged and replaced by albumin in normal Saline. The basic goal of the leucapheresis-treatment was to reduce the circulating lymphocyte count in the patient's blood to obtain an immunomodulatory effect.

The patient's subsequent systemic treatment included broad-spectrum antibiotics (chosen on the basis of the skin microbial findings), intravenous immunoglobulins (dose 1.0 g/kg of body weight per day for 3 consecutive days; infused over 6 hours) and corticosteroids (dose 0.5 mg/kg of body weight)^{1,2}. This young female patient recovered completely after 1 month of intensive systemic and topical treatment.

FU 1 Information 12-Aug-2014:

Temmler Pharma ID has been corrected from DIC-2014-042 to IBU-2014-042.

The Country of Reporter has been corrected from Germany to Serbia.

These incorrect information had been previously entered by mistake.

The Symogen Limited number has been added (SYM-2014-24185).

The Country code of Temmler Pharma's database case number will remain DE due to technical reason<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Company comment Temmler Pharma:

Toxic epidermal necrolysis is listed in the SmPC of ibuprofen (IBU). The reaction occurred in a plausible temporal relationship with IBU intake. The reaction abated after IBU was discontinued and appropriate treatment was applied. No other potential cause of the reaction was reported. A causal relationship with IBU was assessed as probable.

FU1: 12-Aug-2014:

No Change of initial assessment.

This case is medically confirmed and the overall benefit-risk-ratio is not changed.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14386518

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 7 Year	weight (kg): 34	BMI_int: 17.35	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Red face	20141209				Unknown
2 Redness of external ear	20140409				recovered/resolved
3 Exanthema generalized	20141209				not recovered/not resolved
4 Acute generalized exanthematous pustulosis	20140409				recovered/resolved
5 Mouth swelling	20141209				Unknown
6 Allergic reaction	20140409				recovered/resolved
7 Blood pressure fluctuation	20141209				Unknown
8 Facial flushing	20141209				Unknown
9 Instability cardiovascular	20141209				recovered/resolved
10 Itching	20141209				recovered/resolved
11 Nose oedema	20141209				Unknown
12 Perianal itching	20141209				recovered/resolved
13 Stomach pain	20140409				recovered/resolved
14 Syncope	20140409				recovered/resolved
15 Tachycardia	20141209				recovered/resolved
16 Urticaria	20141209				Unknown
17 Vomiting	20141209				recovered/resolved
18 Vomiting	20140409				recovered/resolved
19 Erythema ear	20141209				recovered/resolved
20 Periumbilical pain	20141209	20141209	1 Day		recovered/resolved
21 Quincke's oedema	20140409				Unknown
22 Pustule	20141209				Unknown
23 Pustule	20141209				Unknown
24 Nose oedema	20141209				Unknown
25 Generalised illness	20141209	20141209	1 Day	50 Minute	recovered/resolved
26 Blood pressure decreased	20141209				recovered/resolved
27 Anaphylactic reaction	20141209		3 Hour	1 Day	recovered/resolved
28 Exanthema generalized	201404				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 nurofen junior zaepfchen 125 mg ibuprofen ibuprofen	Rectal	250 Mg milligram(s)/ 242	20141209-20141209	Drug provocation test
2 Nurofen Junior 125 mg Zäpfchen	Rectal	1 Df dosage form/ 242	20141209- -	Drug provocation test

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Accelerated hair loss			Unknown	In early summer of 2014
Pollinosis			Unknown	Appeared for the first time in 2014
Anti-thyroid antibody positive			Unknown	In early summer of 2014
Vaccination			No	
Tiredness			No	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)Drug Commission of the German Medical Association, 18.12.2014 DE-DCGMA-14165275

Sender diagnosis: not reported

NarrativeSender's comment:

Anaphylaktische Reaktion im Rahmen einer stationären Provokationstestung.

Reporter's comment:RBHC, 23.12.2014 DE-RB-071314-14

Sender diagnosis: not reported

Narrative

Report No 1 received from RB Germany, Date 09-DEC-2014, Reference No: RBD-46697590.

Suspect Drug: Nurofen Junior 125 mg Zäpfchen (Ibuprofen)

Dosing: 1 suppositories.

Case reference number DE-RB-071314-14 is a spontaneous case report sent by a physician which refers to a female aged 8 years.

INTRODUCING COMMENT

This ICSR refers to an enquiry of a hospital regarding the composition of "Nurofen Junior 125 mg Zäpfchen". Additional case information was obtained during a telephone conversation with the hospital on 09-DEC-2014. This case will be followed up further.

Please note the suspected drug "Nurofen Junior 125 mg Zäpfchen" (suppositories containing 125 mg Ibuprofen per unit) is referred to as "Nurofen suppositories" in this ICSR.

REPORT TERMS AND DESCRIBE REACTIONS (including relevant test/lab data)

1. Quincke's oedema (occurred in APR-2014)
2. Condition of collapse (occurred in APR-2014)
3. Generalized exanthema (occurred in APR-2014)
4. Formation of pustules (occurred in APR-2014)
5. Bright red ears (occurred in APR-2014)
6. Anaphylactic reaction (occurred on 09-DEC-2014)
7. Decrease of blood pressure (occurred on 09-DEC-2014)
8. Generalized macular exanthema (occurred on 09-DEC-2014)
9. Formation of pustules on the tragi of the ears (occurred on 09-DEC-2014)
10. Swelling of nose (occurred on 09-DEC-2014)
11. Swelling of the mouth angles (occurred on 09-DEC-2014)
12. Bright red face (occurred on 09-DEC-2014)
13. Bright red ears (occurred on 09-DEC-2014)
14. Abdominal pain (occurred on 09-DEC-2014)
15. Indisposition (occurred on 09-DEC-2014)

NARRATIVE

An 8 year old girl (height: 140 cm, weight: 34 kg) experienced Quincke's oedema, generalized exanthema, formation of pustules, bright red ears and condition of collapse 20-25 minutes after the administration of one suppository Nurofen in ??-APR-2014. A treatment including Epinephrine was required (NOS). This reaction first was considered as caused by a viral infection and later as an ADR. In this context a provocation test was performed under emergency preparedness, i.e. monitoring of general condition, blood pressure, heart rate and oxygen saturation in a hospital on 09-DEC-2014.

The test was divided in three successive phases each over two hours of monitoring. At first Nurofen suspension 2% (NOS) was applied to the lips, then 1 ml of the suspension was administered orally and in a final step one Nurofen suppository (125 mg Ibuprofen) was inserted rectally. 50 minutes after the administration of the suppository the girl complained about indisposition and abdominal pain over 2-3 minutes. Then she got bright red ears and pustules on the tragi of the ears, about 1 mm in diameter. Thereafter a macular exanthema developed starting from the décolleté, spreading over arms, abdomen and legs, later leading to confluence. Furthermore swelling of nose and mouth angles and a bright red face were observed 8-9 minutes after the initial symptoms. From a base line of 120/60 mm Hg the girl's blood pressure had fallen to 60/40 mm Hg one hour after the administration of the suppository. Due to the clinical picture an anaphylactic reaction was diagnosed. The girl was administered Dimetindene (Fenistil), Ranitidine (Ranitic), Prednisolone (Solu-Decortin) and Epinephrine (Suprarenin) intravenously. By this treatment circulatory stabilization was achieved, however the generalized macular exanthema was still present two hours after the administration of the suppository. At that time the exanthema showed confluent macules. The girl was hospitalized for further treatment.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, was classed as medically significant due to anaphylactic reaction, blood pressure decreased, circulatory collapse and angioedema, required hospitalization. The patient was hospitalized.

The reporter's causality for the case is Possible. The reporting physician's clinical assessment for drug NUROFEN JUNIOR 125 MG ZÄPFCHEN [Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's assessment for the case is serious, possible and unlisted.

No further information was available at the time of report.

Outcome: Not Recovered / Not Resolved.

CAUSALITY ASSESSMENT ON EVENT-LEVEL

EVENT SOURCE METHOD RESULT

1. Quincke's oedema-Reckitt Benckiser Germany-WHO-UMC-Probable
2. Condition of collapse-Reckitt Benckiser-Germany WHO-UMC-Probable
3. Generalized exanthema-Reckitt Benckiser-Germany WHO-UMC-Probable
4. Formation of pustules-Reckitt Benckiser Germany-WHO-UMC-Probable
5. Bright red ears Reckitt-Benckiser Germany-WHO-UMC-Probable
6. Anaphylactic reaction-Reckitt Benckiser Germany-WHO-UMC-Probable
7. Decrease of blood pressure-Reckitt Benckiser Germany-WHO-UMC-Probable
8. Generalized macular exanthema-Reckitt Benckiser Germany-WHO-UMC-Probable
9. Formation of pustules on the tragi of the ears-Reckitt Benckiser Germany-WHO-UMC-Probable
10. Swelling of nose Reckitt-Benckiser Germany-WHO-UMC-Probable
11. Swelling of the mouth angles-Reckitt Benckiser Germany-WHO-UMC-Probable
12. Bright red face-Reckitt Benckiser Germany-WHO-UMC-Probable
13. Bright red ears-Reckitt Benckiser Germany-WHO-UMC-Probable
14. Abdominal pain-Reckitt Benckiser Germany-WHO-UMC-Probable
15. Indisposition-Reckitt Benckiser Germany-WHO-UMC-Probable

SENDER'S COMMENTS:

ASSESSMENT ON CASE-LEVEL

SERIOUSNESS EXPECTEDNESS CAUSALITY

Serious, Unexpected, Probable.

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

EVENT SOURCE METHOD RESULT

1. Quincke's oedema-Reckitt Benckiser Germany DE-SmPC-Unexpected
2. Condition of collapse-Reckitt Benckiser Germany DE-SmPC-Unexpected
3. Generalized exanthema-Reckitt Benckiser Germany DE-SmPC-Expected*
4. Formation of pustules-Reckitt Benckiser Germany DE-SmPC-Unexpected
5. Bright red ears-Reckitt Benckiser Germany DE-SmPC-Unexpected
6. Anaphylactic reaction-Reckitt Benckiser Germany DE-SmPC-Expected
7. Decrease of blood pressure-Reckitt Benckiser Germany DE-SmPC-Expected
8. Generalized macular exanthema-Reckitt Benckiser Germany DE-SmPC-Expected*
9. Formation of pustules on the tragi of the ears-Reckitt Benckiser Germany DE-SmPC-Unexpected
10. Swelling of nose-Reckitt Benckiser Germany DE-SmPC-Expected**
11. Swelling of the mouth angles-Reckitt Benckiser Germany DE-SmPC-Expected**
12. Bright red face-Reckitt Benckiser Germany DE-SmPC-Unexpected
13. Bright red ears-Reckitt Benckiser Germany DE-SmPC-Unexpected
14. Abdominal pain-Reckitt Benckiser Germany DE-SmPC-Expected
15. Indisposition-Reckitt Benckiser Germany DE-SmPC-Unexpected

* Verbatim unexpected however skin rashes are expected; this labelling is assessed to cover the reported reaction sufficiently.

** Verbatim unexpected however facial oedema is expected; this labelling is assessed to cover the reported reaction sufficiently.

REFERENCE DOCUMENT

German SmPC of "Nurofen Junior 125 mg Zäpfchen" (suppositories containing 125 mg Ibuprofen per unit, MA no.: 55141.00.00, procedure no.: DE/H/400/001) of JUN 2009.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

The case information obtained from the hospital is assessed as sufficient for medical case evaluation.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior 125 mg Zäpfchen" cannot be deduced from the assessment of this individual case. Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

REMARKS

Nurofen suppositories only contain Ibuprofen and hard fat as suppository base. The course of the provocation test suggests a causal involvement of Nurofen, however the present data do not allow to identify the causative agent.

Sender's comment:Reporter's comment:

Reporter causality: Possible. Diagnosis: Anaphylactic reaction

RBHC, 07.01.2015 DE-RB-071314-14

Sender diagnosis: not reported

Narrative

Report No 1 received from RB Germany, Date 09-DEC-2014, Reference No: RBD-46697590.

Suspect Drug: Nurofen Junior 125 mg Zäpfchen (Ibuprofen)

Dosing: 1 suppositories.

Case reference number DE-RB-071314-14 is a spontaneous case report sent by a Physician which refers to a female aged 8 years.

INTRODUCING COMMENT

This ICSR refers to an enquiry of a hospital regarding the composition of "Nurofen Junior 125 mg Zäpfchen". Additional case information was obtained during a telephone conversation with the hospital on 09-DEC-2014. This case will be followed up further.

Please note the suspected drug "Nurofen Junior 125 mg Zäpfchen" (suppositories containing 125 mg Ibuprofen per unit) is referred to as "Nurofen suppositories" in this ICSR.

REPORT TERMS AND DESCRIBE REACTIONS (including relevant test/lab data)

1. Quincke's oedema (occurred in APR-2014)
2. Condition of collapse (occurred in APR-2014)
3. Generalized exanthema (occurred in APR-2014)
4. Formation of pustules (occurred in APR-2014)
5. Bright red ears (occurred in APR-2014)
6. Anaphylactic reaction (occurred on 09-DEC-2014)
7. Decrease of blood pressure (occurred on 09-DEC-2014)
8. Generalized macular exanthema (occurred on 09-DEC-2014)
9. Formation of pustules on the tragi of the ears (occurred on 09-DEC-2014)
10. Swelling of nose (occurred on 09-DEC-2014)
11. Swelling of the mouth angles (occurred on 09-DEC-2014)
12. Bright red face (occurred on 09-DEC-2014)
13. Bright red ears (occurred on 09-DEC-2014)
14. Abdominal pain (occurred on 09-DEC-2014)
15. Indisposition (occurred on 09-DEC-2014)

NARRATIVE

An 8 year old girl (height: 140 cm, weight: 34 kg) experienced Quincke's oedema, generalized exanthema, formation of pustules, bright red ears and condition of collapse 20-25 minutes after the administration of one suppository Nurofen in ??-APR-2014. A treatment including Epinephrine was required (NOS). This reaction first was considered as caused by a viral infection and later as an ADR. In this context a provocation test was performed under emergency preparedness, i.e. monitoring of general condition, blood pressure, heart rate and oxygen saturation in a hospital on 09-DEC-2014. The test was divided in three successive phases each over two hours of monitoring. At first Nurofen suspension 2% (NOS) was applied to the lips, then 1 ml of the suspension was administered orally and in a final step one Nurofen suppository (125 mg Ibuprofen) was inserted rectally. 50 minutes after the administration of the suppository the girl complained about indisposition and abdominal pain over 2-3 minutes. Then she got bright red ears and pustules on the tragi of the ears, about 1 mm in diameter. Thereafter a macular exanthema developed starting from the décolleté, spreading over arms, abdomen and legs, later leading to confluence. Furthermore swelling of nose and mouth angles and a bright red face were observed 8-9 minutes after the initial symptoms. From a base line of 120/60 mm Hg the girl's blood pressure had fallen to 60/40 mm Hg one hour after the administration of the suppository. Due to the clinical picture an anaphylactic reaction was diagnosed. The girl was administered Dimetindene (Fenistil), Ranitidine (Ranitic), Prednisolone (Solu-Decortin) and Epinephrine (Suprarenin) intravenously. By this treatment circulatory stabilization was achieved, however the generalized macular exanthema was still present two hours after the administration of the suppository. At that time the exanthema showed confluent macules. The girl was hospitalized for further treatment.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, was classed as medically significant due to anaphylactic reaction, blood pressure decreased, circulatory collapse and angioedema, required hospitalization. The patient was hospitalized.

The reporter's causality for the case is Possible. The reporting Physician's clinical assessment for drug NUROFEN JUNIOR 125 MG ZÄPFCHEN [Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's assessment for the case is serious, possible and unlisted.

No further information was available at the time of report.

Outcome: Not Recovered / Not Resolved.

CAUSALITY ASSESSMENT ON EVENT-LEVEL

EVENT SOURCE METHOD RESULT

1. Quincke's oedema-Reckitt Benckiser Germany-WHO-UMC-Probable
2. Condition of collapse-Reckitt Benckiser-Germany WHO-UMC-Probable
3. Generalized exanthema-Reckitt Benckiser-Germany WHO-UMC-Probable
4. Formation of pustules-Reckitt Benckiser Germany-WHO-UMC-Probable
5. Bright red ears Reckitt-Benckiser Germany-WHO-UMC-Probable
6. Anaphylactic reaction-Reckitt Benckiser Germany-WHO-UMC-Probable
7. Decrease of blood pressure-Reckitt Benckiser Germany-WHO-UMC-Probable
8. Generalized macular exanthema-Reckitt Benckiser Germany-WHO-UMC-Probable
9. Formation of pustules on the tragi of the ears-Reckitt Benckiser Germany-WHO-UMC-Probable
10. Swelling of nose Reckitt-Benckiser Germany-WHO-UMC-Probable
11. Swelling of the mouth angles-Reckitt Benckiser Germany-WHO-UMC-Probable
12. Bright red face-Reckitt Benckiser Germany-WHO-UMC-Probable
13. Bright red ears-Reckitt Benckiser Germany-WHO-UMC-Probable
14. Abdominal pain-Reckitt Benckiser Germany-WHO-UMC-Probable
15. Indisposition-Reckitt Benckiser Germany-WHO-UMC-Probable

SENDER'S COMMENTS:

ASSESSMENT ON CASE-LEVEL

SERIOUSNESS EXPECTEDNESS CAUSALITY

Serious, Unexpected, Probable.

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

EVENT SOURCE METHOD RESULT

1. Quincke's oedema-Reckitt Benckiser Germany DE-SmPC-Unexpected
2. Condition of collapse-Reckitt Benckiser Germany DE-SmPC-Unexpected
3. Generalized exanthema-Reckitt Benckiser Germany DE-SmPC-Expected*
4. Formation of pustules-Reckitt Benckiser Germany DE-SmPC-Unexpected
5. Bright red ears-Reckitt Benckiser Germany DE-SmPC-Unexpected
6. Anaphylactic reaction-Reckitt Benckiser Germany DE-SmPC-Expected
7. Decrease of blood pressure-Reckitt Benckiser Germany DE-SmPC-Expected
8. Generalized macular exanthema-Reckitt Benckiser Germany DE-SmPC-Expected*
9. Formation of pustules on the tragi of the ears-Reckitt Benckiser Germany DE-SmPC-Unexpected
10. Swelling of nose-Reckitt Benckiser Germany DE-SmPC-Expected**
11. Swelling of the mouth angles-Reckitt Benckiser Germany DE-SmPC-Expected**

- 12. Bright red face-Reckitt Benckiser Germany DE-SmPC-Unexpected
- 13. Bright red ears-Reckitt Benckiser Germany DE-SmPC-Unexpected
- 14. Abdominal pain-Reckitt Benckiser Germany DE-SmPC-Expected
- 15. Indisposition-Reckitt Benckiser Germany DE-SmPC-Unexpected

* Verbatim unexpected however skin rashes are expected; this labelling is assessed to cover the reported reaction sufficiently.

** Verbatim unexpected however facial oedema is expected; this labelling is assessed to cover the reported reaction sufficiently.

REFERENCE DOCUMENT

German SmPC of "Nurofen Junior 125 mg Zäpfchen" (suppositories containing 125 mg Ibuprofen per unit, MA no.: 55141.00.00, procedure no.: DE/H/400/001) of JUN 2009.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

The case information obtained from the hospital is assessed as sufficient for medical case evaluation.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior 125 mg Zäpfchen" cannot be deduced from the assessment of this individual case. Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

REMARKS

Nurofen suppositories only contain Ibuprofen and hard fat as suppository base. The course of the provocation test suggests a causal involvement of Nurofen, however the present data do not allow to identify the causative agent.

Report No 2 received from RB Germany through German Regulatory Health Authority-BFARM, Date 23-DEC-2014, Reference No: RBD-46697590, DE-BFARM-14386518.

Information added/updated:

Adverse event "Anaphylactic reaction" updated to "Anaphylactic reaction grade III-IV".

Adverse event "Anaphylactic reaction" outcome updated to Recovered/Resolved.

Reporter causality updated to "certain".

Reporter causality for "Anaphylactic reaction" updated from "possible" to "certain".

Action taken updated from "Unknown" to "Drug withdrawn"

Dechallenge ticked "No" in suspect drug.

In dosage tab, unit dose and daily dose 250mg added.

Reporter health authority updated.

INTRODUCING COMMENT

This follow-up information refers to an ICSR identified on the BfArM web page on 22-DEC-2014 published by BfArM on 19-DEC-2014 (day 0). The follow-up of this case will be continued. Please note the suspected drug "Nurofen Junior 125 mg Zäpfchen" (suppositories containing 125 mg Ibuprofen per unit) is referred to as "Nurofen suppositories" in this ICSR.

REPORT TERMS AND DESCRIBE REACTIONS (including relevant test/lab data)

Anaphylactic reaction grade III-IV (occurred on 09-DEC-2014).

FOLLOW-UP (1) INFORMATION

Report terms and describe reactions

Substitute "Anaphylactic reaction" by "Anaphylactic reaction, grade III-IV" (adopted from the BfArM CIOMS)

Remarks to the dosage of Nurofen suppositories used for provocation

The ICSR originating from BfArM indicates "250 mg" (NOS) as daily dose corresponding to 2 Nurofen suppositories. During a telephone conversation on 09-DEC-2014 the reporting Physician stated that 1 Nurofen suppository was used for provocation testing.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, was classed as medically significant due to anaphylactic reaction, blood pressure decreased, circulatory collapse and angioedema, required hospitalization. The patient was hospitalized.

The reporter's causality for the case is Certain. The reporting Physician's clinical assessment for drug NUROFEN JUNIOR 125 MG ZÄPFCHEN [Ibuprofen] is as follows: serious; with a relatedness of Certain.

The company's assessment for the case is serious, possible and unlisted.

No further information was available at the time of report.

Outcome: Not Recovered / Not Resolved.

Sender's comment:

Reporter's comment:

Reporter causality: Certain. Diagnosis: Anaphylactic reaction

RBHC, 27.01.2015 DE-RB-071314-14

Sender diagnosis: not reported

Narrative

Report No 1 received from RB Germany, Date 09-DEC-2014, Reference No: RBD-46697590.

Suspect Drug: Nurofen Junior 125 mg Zäpfchen (Ibuprofen)

Dosing: 1 suppositories.

Case reference number DE-RB-071314-14 is a spontaneous case report sent by a Physician which refers to a female aged 8 years.

INTRODUCING COMMENT

This ICSR refers to an enquiry of a hospital regarding the composition of "Nurofen Junior 125 mg Zäpfchen". Additional case information was obtained during a telephone conversation with the hospital on 09-DEC-2014. This case will be followed up further.

Please note the suspected drug "Nurofen Junior 125 mg Zäpfchen" (suppositories containing 125 mg Ibuprofen per unit) is referred to as "Nurofen suppositories" in this ICSR.

REPORT TERMS AND DESCRIBE REACTIONS (including relevant test/lab data)

1. Quincke's oedema (occurred in APR-2014)
2. Condition of collapse (occurred in APR-2014)
3. Generalized exanthema (occurred in APR-2014)
4. Formation of pustules (occurred in APR-2014)
5. Bright red ears (occurred in APR-2014)
6. Anaphylactic reaction (occurred on 09-DEC-2014)
7. Decrease of blood pressure (occurred on 09-DEC-2014)
8. Generalized macular exanthema (occurred on 09-DEC-2014)
9. Formation of pustules on the tragi of the ears (occurred on 09-DEC-2014)
10. Swelling of nose (occurred on 09-DEC-2014)
11. Swelling of the mouth angles (occurred on 09-DEC-2014)
12. Bright red face (occurred on 09-DEC-2014)
13. Bright red ears (occurred on 09-DEC-2014)
14. Abdominal pain (occurred on 09-DEC-2014)
15. Indisposition (occurred on 09-DEC-2014)

NARRATIVE

An 8 year old girl (height: 140 cm, weight: 34 kg) experienced Quincke's oedema, generalized exanthema, formation of pustules, bright red ears and condition of collapse 20-25 minutes after the administration of one suppository Nurofen in ??-APR-2014. A treatment including Epinephrine was required (NOS). This reaction first was considered as caused by a viral infection and later as an ADR. In this context a provocation test was performed under emergency preparedness, i.e. monitoring of general condition, blood pressure, heart rate and oxygen saturation in a hospital on 09-DEC-2014. The test was divided in three successive phases each over two hours of monitoring. At first Nurofen suspension 2% (NOS) was applied to the lips, then 1 ml of the suspension was administered orally and in a final step one Nurofen suppository (125 mg Ibuprofen) was inserted rectally. 50 minutes after the administration of the suppository the girl complained about indisposition and abdominal pain over 2-3 minutes. Then she got bright red ears and pustules on the tragi of the ears, about 1 mm in diameter. Thereafter a macular exanthema developed starting from the décolleté, spreading over

arms, abdomen and legs, later leading to confluence. Furthermore swelling of nose and mouth angles and a bright red face were observed 8-9 minutes after the initial symptoms. From a base line of 120/60 mm Hg the girl's blood pressure had fallen to 60/40 mm Hg one hour after the administration of the suppository. Due to the clinical picture an anaphylactic reaction was diagnosed. The girl was administered Dimetindene (Fenistil), Ranitidine (Ranitic), Prednisolone (Solu-Decortin) and Epinephrine (Suprarenin) intravenously. By this treatment circulatory stabilization was achieved, however the generalized macular exanthema was still present two hours after the administration of the suppository. At that time the exanthema showed confluent macules. The girl was hospitalized for further treatment.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, was classed as medically significant due to anaphylactic reaction, blood pressure decreased, circulatory collapse and angioedema, required hospitalization. The patient was hospitalized.

The reporter's causality for the case is Possible. The reporting Physician's clinical assessment for drug NUROFEN JUNIOR 125 MG ZÄPFCHEN [Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's assessment for the case is serious, possible and unlisted.

No further information was available at the time of report.

Outcome: Not Recovered / Not Resolved.

Report No 2 received from RB Germany through German Regulatory Health Authority-BFARM, Date 23-DEC-2014, Reference No: RBD-46697590, DE-BFARM-14386518.

Information added/updated:

Adverse event "Anaphylactic reaction" updated to "Anaphylactic reaction grade III-IV".

Adverse event "Anaphylactic reaction" outcome updated to Recovered/Resolved.

Reporter causality updated to "certain".

Reporter causality for "Anaphylactic reaction" updated from "possible" to "certain".

Action taken updated from "Unknown" to "Drug withdrawn"

Dechallenge ticked "No" in suspect drug.

In dosage tab, unit dose and daily dose 250mg added.

Reporter health authority updated.

INTRODUCING COMMENT

This follow-up information refers to an ICSR identified on the BfArM web page on 22-DEC-2014 published by BfArM on 19-DEC-2014 (day 0). The follow-up of this case will be continued. Please note the suspected drug "Nurofen Junior 125 mg Zäpfchen" (suppositories containing 125 mg Ibuprofen per unit) is referred to as "Nurofen suppositories" in this ICSR.

REPORT TERMS AND DESCRIBE REACTIONS (including relevant test/lab data)

Anaphylactic reaction grade III-IV (occurred on 09-DEC-2014).

FOLLOW-UP (1) INFORMATION

Report terms and describe reactions

Substitute "Anaphylactic reaction" by "Anaphylactic reaction, grade III-IV" (adopted from the BfArM CIOMS)

Remarks to the dosage of Nurofen suppositories used for provocation

The ICSR originating from BfArM indicates "250 mg" (NOS) as daily dose corresponding to 2 Nurofen suppositories. During a telephone conversation on 09-DEC-2014 the reporting Physician stated that 1 Nurofen suppository was used for provocation testing.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, was classed as medically significant due to anaphylactic reaction, blood pressure decreased, circulatory collapse and angioedema, required hospitalization. The patient was hospitalized.

The reporter's causality for the case is Certain. The reporting Physician's clinical assessment for drug NUROFEN JUNIOR 125 MG ZÄPFCHEN [Ibuprofen] is as follows: serious; with a relatedness of Certain.

The company's assessment for the case is serious, possible and unlisted.

No further information was available at the time of report.

Outcome: Not Recovered / Not Resolved.

Report No 3 received from RB Germany, Date 12-JAN-2015, Reference No: DE-BFARM-14386518, RBD-46697590.

Information added and updated:

Introducing Comment

This ICSR/follow-up information refers to a hospital's record on discharge made by a Physician obtained by Reckitt Benckiser Germany on 12-JAN-2015.

Please note the suspected drug "Nurofen Junior 125 mg Zäpfchen" (suppositories containing 125mg Ibuprofen per unit) is referred to as "Nurofen suppositories" in this follow-up (2) information.

Report Terms and Describe Reactions (including relevant test/lab data)

1. Allergic reaction*
2. Syncope*
3. Quincke's oedema*
4. Generalized pustular erythematous exanthema*
5. Bright red ears*
6. Vomiting*
7. Stomach pain*
8. Anaphylactic reaction grade III-IV to Nurofen suppositories **
9. Decrease of blood pressure**
10. Tachycardia**
11. Instability of blood pressure**
12. Instability of the cardiovascular system**
13. Vomiting**
14. Periumbilical abdominal pain**
15. Urticaria**
16. Generalized macular exanthema**
17. Itching
18. Bright red face**
19. Facial flushing**
20. Reddening of the left pinna**
21. Bright red ears**
22. Formation of pustules on the tragus of the ears**
23. Swelling of nose**
24. Swelling of the mouth angles**
25. Indisposition**
26. Anal itching**

* The reactions are related to the adverse event occurred on 09-APR-2014.

** The reactions are related to the provocation test performed on 09-DEC-2014.

Follow-Up (2) Information

Report terms and describe reactions

Reactions occurred on 09-APR-2014:

Add "Allergic reaction"

Substitute "Condition of collapse" by "Syncope"

Substitute "Generalized exanthema" by "Generalized pustular erythematous exanthema"

Delete "Formation of pustules" (included in the term "Generalized pustular erythematous exanthema")

Add "Vomiting"

Add "Stomach pain"

Reactions occurred on 09-DEC-2014:

Substitute "Anaphylactic reaction grade III-IV" by "Anaphylactic reaction grade III-IV to Nurofen suppositories"

Add "Tachycardia"

Add "Instability of blood pressure"

Add "Instability of the cardiovascular system"

Add "Vomiting"

Substitute "Abdominal pain" by "Periumbilical abdominal pain"

Add "Urticaria"

Substitute "Macular exanthema" by "Generalized macular exanthema"

Add "Itching"

Add "Facial flushing"

Add "Reddening of the left pinna"

Add "Anal itching"

Addendum to the narrative

The clinical course of the adverse event already described in the initial and the follow-up (1) report has to be supplemented by the following information originating from the hospitals record on discharge, dated 15-DEC-2014. In so far this follow-up information is of complementary nature and does not describe the whole course of the case as standalone report.

The 8-year-old girl was hospitalized for the first time on 09-APR-2014 due to an allergic reaction after the administration of one Nurofen suppository for treatment of a feverish infection with body temperatures up to 38.5°C.

The condition of collapse was specified as syncope. The reported skin reaction (generalized exanthema) was specified as generalized pustular erythematous exanthema starting in the face and spreading out over the whole body afterwards.

The second elective admission to hospital from 09- to 10-DEC-2014 was made for carrying out a provocation test with Nurofen. From the girl's medical history the following information became known: vaccination according to the STIKO recommendations (German commission for vaccination); clinical signs of pollinosis which appeared for the first time in 2014; increased level of an anti-thyroid autoantibody (NOS) found in early summer of 2014; episode of tiredness and increased hair loss occurred in early summer of 2014.

The examination on admission revealed that the girl was in a good general and nutritional condition without pathological findings. She had no infections prior to admission. The results of the laboratory investigations on admission are summarized below.

Regarding the course of provocation testing at first some drops of the Nurofen suspension (containing 20mg Ibuprofen/ml) were applied to the lips. Thereafter 20 mg Ibuprofen (1 ml of the Nurofen suspension) was administered orally. The monitoring of vital signs performed after each dosage over 2 hours showed no allergic reaction. Immediately after the rectal administration of one Nurofen suppository the girl complained about anal itching for a few minutes. 45minutes after the administration the anaphylactic reaction started including periumbilical abdominal pain, reddening of the left pinna, violent vomiting and strong itching. These reactions occurred within a few minutes. Pulmonary symptoms did not occur. For treatment crystalloid fluid infusion (NOS), Epinephrine inhalation ("Infectokrupp") and Salbutamol inhalation (NOS) was used. Due to cardiovascular instability including drop in blood pressure and tachycardia the therapy was intensified by i.v. administration of Epinephrine. After the initial reactions had completely resolved the anaphylaxis recurred delayed in time, showing facial flush, pronounced urticaria and instability of blood pressure. Therefore a drug therapy using again fluid supply (bolus of 40 ml/kg, NOS), Dimetindene ("Fenistil"), Prednisolone ("Solu-Decortin") and Epinephrine was provided. Thereafter the anaphylactic reaction resolved completely without recurrence.

According to the hospitals record on discharge the suspicion of an anaphylactic reaction to Nurofen suppositories could be confirmed by the provocation test, however it could not be established whether the allergic reaction was caused by Ibuprofen or the fatty suppository base. Therefore the hospital recommended strictly avoiding Ibuprofen administration as well as the administration of drugs in suppository form. The girl received a first aid kit including emergency medicines and an allergy pass.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, was classed as medically significant due to 'anaphylactic reaction', 'syncope', 'angioedema', required hospitalization. The patient was hospitalized.

The reporter's causality for the case is Certain. The reporting physician's clinical assessment for drug NUROFEN JUNIOR 125 MG ZÄPFCHEN [Ibuprofen] is as follows: serious; with a relatedness of Certain.

The company's assessment for the case is serious, possible and unlisted.

No further information was available at the time of report.

Outcome: Not Recovered / Not Resolved.

Causality Assessment on Event-Level

Event Source Method Result

1. Allergic reaction Reckitt Benckiser Germany WHO-UMC Possible
2. Syncope Reckitt Benckiser Germany WHO-UMC Possible
3. Quincke's oedema Reckitt Benckiser Germany WHO-UMC Possible
4. Generalized pustular erythematous exanthema Reckitt Benckiser Germany WHO-UMC Possible
5. Bright red ears Reckitt Benckiser Germany WHO-UMC Possible
6. Vomiting Reckitt Benckiser Germany WHO-UMC Possible
7. Stomach pain Reckitt Benckiser Germany WHO-UMC Possible
8. Anaphylactic reaction grade III-IV to Nurofen suppositories
Reckitt Benckiser Germany WHO-UMC Possible
9. Decrease of blood pressure Reckitt Benckiser Germany WHO-UMC Possible
10. Tachycardia Reckitt Benckiser Germany WHO-UMC Possible
11. Instability of blood pressure Reckitt Benckiser Germany WHO-UMC Possible
12. Instability of the cardiovascular system Reckitt Benckiser Germany WHO-UMC Possible
13. Vomiting Reckitt Benckiser Germany WHO-UMC Possible
14. Periumbilical abdominal pain Reckitt Benckiser Germany WHO-UMC Possible
15. Urticaria Reckitt Benckiser Germany WHO-UMC Possible
16. Generalized macular exanthema Reckitt Benckiser Germany WHO-UMC Possible
17. Itching Reckitt Benckiser Germany WHO-UMC Possible
18. Bright red face Reckitt Benckiser Germany WHO-UMC Possible
19. Facial flushing Reckitt Benckiser Germany WHO-UMC Possible
20. Reddening of the left pinna Reckitt Benckiser Germany WHO-UMC Possible

21. Bright red ears Reckitt Benckiser Germany WHO-UMC Possible
22. Formation of pustules on the tragus of the ears Reckitt Benckiser Germany WHO-UMC Possible
23. Swelling of nose Reckitt Benckiser Germany WHO-UMC Possible
24. Swelling of the mouth angles Reckitt Benckiser Germany WHO-UMC Possible
25. Indisposition Reckitt Benckiser Germany WHO-UMC Possible
26. Anal itching Reckitt Benckiser Germany WHO-UMC Possible

Sender's Comments:

Assessment on Case-Level
 Seriousness Expectedness Causality
 Serious Unexpected Possible

Expectedness Assessment on Event-Level

Event Source Method Result

1. Allergic reaction Reckitt Benckiser Germany DE-SmPC Expected
2. Syncope Reckitt Benckiser Germany DE-SmPC Unexpected
3. Quincke's oedema Reckitt Benckiser Germany DE-SmPC Unexpected
4. Generalized pustular erythematous exanthema Reckitt Benckiser Germany DE-SmPC Expected*
5. Bright red ears Reckitt Benckiser Germany DE-SmPC Unexpecte
6. Vomiting Reckitt Benckiser Germany DE-SmPC Expected
7. Stomach pain Reckitt Benckiser Germany DE-SmPC Expected**
8. Anaphylactic reaction grade III-IV to Nurofen suppositories Reckitt Benckiser Germany DE-SmPC Expected
9. Decrease of blood pressure Reckitt Benckiser Germany DE-SmPC Expected
10. Tachycardia Reckitt Benckiser Germany DE-SmPC Expected
11. Instability of blood pressure Reckitt Benckiser Germany DE-SmPC Unexpected
12. Instability of the cardiovascular system Reckitt Benckiser Germany DE-SmPC Unexpected
13. Vomiting Reckitt Benckiser Germany DE-SmPC Expected
14. Periumbilical abdominal pain Reckitt Benckiser Germany DE-SmPC Expected**
15. Urticaria Reckitt Benckiser Germany DE-SmPC Unexpected
16. Generalized macular exanthema Reckitt Benckiser Germany DE-SmPC Expected*
17. Itching Reckitt Benckiser Germany DE-SmPC Expected
18. Bright red face Reckitt Benckiser Germany DE-SmPC Unexpected
19. Facial flushing Reckitt Benckiser Germany DE-SmPC Unexpected
20. Reddening of the left pinal Reckitt Benckiser Germany DE-SmPC Unexpected
21. Bright red ears Reckitt Benckiser Germany DE-SmPC Unexpected
22. Formation of pustules on the tragus of the ears Reckitt Benckiser Germany DE-SmPC Unexpected
23. Swelling of nose Reckitt Benckiser Germany DE-SmPC Expected***
24. Swelling of the mouth angles Reckitt Benckiser Germany DE-SmPC Expected***
25. Indisposition Reckitt Benckiser Germany DE-SmPC Unexpected
26. Anal itching Reckitt Benckiser Germany DE-SmPC Unexpected

* Verbatim unexpected however skin rashes are expected; this labelling is assessed to cover the reported reaction sufficiently.

** Verbatim unexpected however abdominal pain is expected; this labelling is assessed to cover the reported reaction sufficiently.

*** Verbatim unexpected however facial oedema is expected; this labelling is assessed to cover the reported reaction sufficiently.

Reference Document

German SmPC of "Nurofen Junior 125 mg Zäpfchen" (suppositories containing 125 mg Ibuprofen per unit, MA no.: 55141.00.00, procedure no.: DE/H/400/001) of JUN 2009

Extend And Quality Of Reported Case Information

The case information provided by the hospitals record on discharge is assessed as sufficient for medical case evaluation.

Conclusion

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior 125 mg Zäpfchen" cannot be deduced from the assessment of this individual case.

Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

Remarks

Nurofen suppositories only contain the active agent Ibuprofen and hard fat as suppository base. The course of the provocation test proves the causal involvement of Nurofen suppositories; however the present data do not allow to identify the causative agent.

Sender's comment:

Reporter's comment:

Reporter causality: Certain.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-15198398

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
No	No	No	No	No	No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 56 Year	weight (kg): 80	BMI_int: 28.34	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Oral mucosa redness	20150614			11 Day	not recovered/not resolved
2 Oral mucosal exfoliation	20150614			11 Day	not recovered/not resolved
3 Coating in mouth	20150614			11 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuflam 600 ibuprofen	Oral	1200 Mg milligram(s)/Day	20150604-20150614 11 Day	Shoulder pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Shoulder dislocation			Unknown	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-15219602

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 10 Year	weight (kg): 50	BMI_int: 21.36	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Abdominal pain	20150705			1 Day	not recovered/not resolved
2 Aseptic meningitis	20150705			1 Day	not recovered/not resolved
3 Blisters	20150705			1 Day	not recovered/not resolved
4 Feeling sick					not recovered/not resolved
5 Neck stiffness					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuflam ibuprofen	Oral	300 Mg milligram(s)/Day 82	20150704-20150704	Headache

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

sanofi-aventis, 14.07.2015 DE-SA-2015SA098948

Sender diagnosis: not reported

Narrative

Initial Information regarding unsolicited case was received from a pharmacist and transmitted to sanofi on 06-Jul-2015.

This case involves a 10-year-old female patient who experienced aseptic meningitis, neck stiffness, feeling sick, blisters and abdominal pain after starting ibuprofen (Ibuflam).

Relevant medical history and concomitant medications were unknown.

On 04-Jul-2015, the patient was started on therapy with ibuprofen 400 mg film coated tablet (Lot number and expiration date unknown) once (dose unknown) via route for headache. On 05-Jul-2015, i.e. 1 day after the initiation of ibuprofen therapy, the patient experienced aseptic meningitis, neck stiffness, blisters at hairline, abdominal pain and general feeling of sickness. As a corrective treatment betamethasone valerate/fusidic acid (Fucicort) was applied at the blisters. Headache was still present. Headache and neck stiffness still ongoing on 06-Jul-2015. Additionally occurrence of temperature up to 37.8 degree Celcius. The pharmacist would like to exclude aseptic meningitis and she will go to a pediatrician/hospital with the patient.

Seriousness criteria: medically significant

Action taken: drug withdrawn nos

Therapy with ibuprofen was discontinued on 04-Jul-2015.

As a corrective treatment betamethasone valerate/fusidic acid (Fucicort) was applied at the blisters. Corrective treatment was unknown for all other events.

At the time of reporting, outcome of the events was not resolved.

Sanofi causal assessment: Possible

No further relevant information was reported.

Sender's comment:

Sanofi Company Comment dated 10-Jul-2015: In this case patient experienced events aseptic meningitis, neck stiffness, general feeling of sickness, blisters at hairline and abdominal pain after using suspect drugs. Limited information regarding medical history and other factors precludes medical assessment of this case.

Reporter's comment:

sanofi-aventis, 22.07.2015 DE-SA-2015SA098948

Sender diagnosis: not reported

Narrative

Initial Information regarding unsolicited case was received from a pharmacist and transmitted to sanofi on 06-Jul-2015.

This case involves a 10-year-old female patient who experienced aseptic meningitis, neck stiffness, feeling sick, blisters and abdominal pain after starting ibuprofen (Ibuflam).

Relevant medical history and concomitant medications were unknown.

On 04-Jul-2015, the patient was started on therapy with ibuprofen 400 mg film coated tablet (Lot number and expiration date unknown) at a dose of 300 mg daily (3/4 tablet in the evening) via route for headache. On 05-Jul-2015, i.e. 1 day after the initiation of ibuprofen therapy, the patient experienced aseptic meningitis, neck stiffness, blisters at hairline, abdominal pain and general feeling of sickness. As a corrective treatment betamethasone valerate/fusidic acid (Fucicort) was applied at the blisters. Headache was still present. Headache and neck stiffness still ongoing on 06-Jul-2015. Additionally occurrence of temperature up to 37.8 degree Celcius. The pharmacist would like to exclude aseptic meningitis and she will go to a pediatrician/hospital with the patient.

Seriousness criteria: medically significant

Action taken: drug withdrawn nos

Therapy with ibuprofen was discontinued on 04-Jul-2015.

As a corrective treatment betamethasone valerate/fusidic acid (Fucicort) was applied at the blisters. Corrective treatment was unknown for all other events.

At the time of reporting, outcome of the event (neck stiffness) was resolving. The outcome of the other events was not resolved.

Reporter causality description: Probable

Sanofi causal assessment: Possible

No further relevant information was reported.

Additional information received from the pharmacist via adverse drug reaction report form on 15-Jul-2015:

Dosing details and prior use of suspect drug was updated. The outcome of the event neck stiffness changed to resolving. Reporter causality was added. Patient's weight and height were added.

Sender's comment:

Sanofi Company Comment follow-up dated 15-Jul-2015:- Reporter's causality updated as probable. New information received does not change the previous assessment of case.

Sanofi Company Comment dated 10-Jul-2015: In this case patient experienced events aseptic meningitis, neck stiffness, general feeling of sickness, blisters at hairline and abdominal pain after using suspect drugs. Limited information regarding medical history and other factors precludes medical assessment of this case.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-15400784

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 45 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythroderma	20151201	20151210	10 Day	15 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 BKM120 buparlisib	Oral	100 Mg milligram(s)/Day 5	20151117-20151201 15 Day	Metastatic melanoma
2 IBUPROFEN SANDOZ ibuprofen	Unknown	500 Mg milligram(s)/	20151128-201511 -	Common cold

Drugs (concomitant)

3 dexamethason dexamethasone	Unknown	2 Mg milligram(s)/Day	201504-20151201 -	Brain metastases
4 KEPPRA levetiracetam	Unknown	3000 Mg milligram(s)/Day	201504-20151201 -	Brain metastases
5 PANTOPRAZOL pantoprazole sodium sesquihydrate	Unknown	20 Mg milligram(s)/Day	201504-20151201 -	Prophylaxis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Common cold			Yes	
Malignant melanoma	200709		Yes	

Primary source

Qualification:	Physician	observestudytype:	Clinical trials
Literature:			
Qualification:	Other Health Professional	observestudytype:	Clinical trials
Literature:			

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 08.12.2015 DE-002147023-PHHO2015DE020197

Sender diagnosis: not reported

Narrative

Case number# PHHO2015DE020197, is an initial report received on 02 Dec 2015 from a clinical study CBKM120ZDE01T. This report refers to a 45 year old male subject (center ID: 0002; subject ID: 001) enrolled in A mono centric, open label, uncontrolled, single arm phase II trial of buparlisib in subjects with metastatic melanoma with brain metastases not eligible for surgery or radiosurgery.

Medical history included common cold. Current condition included malignant melanoma. Concomitant medications included Keppra (levetiracetam), Dexamethason (dexamethasone) and Pantoprazol (pantoprazole sodium sesquihydrate). From an unknown date in Nov 2015, the subject received ibuprofen for the treatment of common cold at an unknown dose (route: unknown) and was received most recent dose of ibuprofen an unknown date in Nov 2015. The subject started study medication BKM120 on 17 Nov 2015 at a dose of 100 mg QD (oral). On 01 Dec 2015, the subject received most recent dose of study medication. The subject presented in emergency department room with generalized itchy rash (grade3). On 01 Dec 2015, the subject was diagnosed with "preerythrodermia" (dermatitis exfoliative). The subject was hospitalized on an unknown date for the event of dermatitis exfoliative. The subject's treatment included Prednisolon (prednisolone). Treatment with BKM120 was temporarily interrupted on 02 Dec 2015. The outcome of the event dermatitis exfoliative was reported as condition unchanged. The event dermatitis exfoliative was considered serious (hospitalization) by the investigator.

The investigator assessed dermatitis exfoliative as suspected to be related to treatment with BKM120 and concomitant medication Ibuprofen.

Sender's comment:

Based on review of data available at this time and no other contributory factor Novartis agrees with the investigator's suspected assessment. Investigator also considered it to be suspected to ibuprofen.

Reporter's comment:

SANDOZ, 08.12.2015 DE-SZ09-PHHO2015DE020197

Sender diagnosis: not reported

Narrative

Case number# PHHO2015DE020197, is an initial report received on 02 Dec 2015 from a clinical study CBKM120ZDE01T. This report refers to a 45 year old male subject (center ID: 0002; subject ID: 001) enrolled in A mono centric, open label, uncontrolled, single arm phase II trial of buparlisib in subjects with metastatic melanoma with brain metastases not eligible for surgery or radiosurgery.

Medical history included common cold. Current condition included malignant melanoma. Concomitant medications included Keppra (levetiracetam), Dexamethason (dexamethasone) and Pantoprazol (pantoprazole sodium sesquihydrate). From an unknown date in Nov 2015, the subject received ibuprofen for the treatment of common cold at an unknown dose (route: unknown) and was received most recent dose of ibuprofen an unknown date in Nov 2015. The subject started study medication BKM120 on 17 Nov 2015 at a dose of 100 mg QD (oral). On 01 Dec 2015, the subject received most recent dose of study medication. The subject presented in emergency department room with generalized itchy rash (grade3). On 01 Dec 2015, the subject was diagnosed with "preerythrodermia" (dermatitis exfoliative). The subject was hospitalized on an unknown date for the event of dermatitis exfoliative. The subject's treatment included Prednisolon (prednisolone). Treatment with BKM120 was temporarily interrupted on 02 Dec 2015. The outcome of the event dermatitis exfoliative was reported as condition unchanged. The event dermatitis exfoliative was considered serious (hospitalization) by the investigator.

The investigator assessed dermatitis exfoliative as suspected to be related to treatment with BKM120 and concomitant medication Ibuprofen.

Sender's comment:

Based on review of data available at this time and no other contributory factor Novartis agrees with the investigator's suspected assessment. Investigator also considered it to be suspected to ibuprofen.

Reporter's comment:

NOVARTIS, 02.02.2016 DE-002147023-PHHO2015DE020197

Sender diagnosis: not reported

Narrative

Case number# PHHO2015DE020197, is an initial report received on 02 Dec 2015 from a clinical study CBKM120ZDE01T. This report refers to a 45 year old male subject (center ID: 0002; subject ID: 001) enrolled in a mono centric, open label, uncontrolled, single arm phase II trial of buparlisib in subjects with metastatic melanoma with brain metastases not eligible for surgery or radiosurgery.

Current condition included malignant melanoma and common cold. He was diagnosed with brain metastases in Sep 2014. Concomitant medications included Keppra (levetiracetam), Dexamethason (dexamethasone) and Pantoprazol (pantoprazole sodium sesquihydrate). From an unknown date in Nov 2015, the subject received ibuprofen for the treatment of common cold at an unknown dose (route: unknown) and received most recent dose of ibuprofen an unknown date in Nov 2015. The subject started study medication BKM120 on 17 Nov 2015 at a dose of 100 mg QD (oral). On 01 Dec 2015, the subject received most recent dose of study medication. The subject was presented in emergency department with generalized itchy rash (grade3). On 01 Dec 2015, the subject was diagnosed with "preerythrodermia" (dermatitis exfoliative). The subject was hospitalized on an unknown date for the event of dermatitis exfoliative. The subject's treatment included Prednisolon (prednisolone). Treatment with BKM120 was temporarily interrupted on 02 Dec 2015. The outcome of the event dermatitis exfoliative was reported as condition unchanged. The event dermatitis exfoliative

was considered serious (hospitalization) by the investigator.

The investigator assessed dermatitis exfoliative as suspected to be related to treatment with BKM120 and concomitant medication Ibuprofen.

Following an internal review on 28 Jan 2016, of the reported information of 02 Dec 2015 the following changes were made: Diagnosis of brain metastases was added in the narrative, listedness of the event "preerythrodermia" (dermatitis exfoliative) was amended to unlisted (previously captured as listed).

Sender's comment:

Based on the information available, plausible temporal relationship and absence of alternative reasons reported, Novartis cannot exclude the causality of the BKM120 and concomitant ibuprofen with exfoliative dermatitis reported. Follow-up is ongoing.

Reporter's comment:

SANDOZ, 02.02.2016 DE-SZ09-PHHO2015DE020197

Sender diagnosis: not reported

Narrative

Case number# PHHO2015DE020197, is an initial report received on 02 Dec 2015 from a clinical study CBKM120ZDE01T. This report refers to a 45 year old male subject (center ID: 0002; subject ID: 001) enrolled in a mono centric, open label, uncontrolled, single arm phase II trial of buparlisib in subjects with metastatic melanoma with brain metastases not eligible for surgery or radiosurgery.

Current condition included malignant melanoma and common cold. He was diagnosed with brain metastases in Sep 2014. Concomitant medications included Keppra (levetiracetam), Dexamethason (dexamethasone) and Pantoprazol (pantoprazole sodium sesquihydrate). From an unknown date in Nov 2015, the subject received ibuprofen for the treatment of common cold at an unknown dose (route: unknown) and received most recent dose of ibuprofen an unknown date in Nov 2015. The subject started study medication BKM120 on 17 Nov 2015 at a dose of 100 mg QD (oral). On 01 Dec 2015, the subject received most recent dose of study medication. The subject was presented in emergency department with generalized itchy rash (grade3). On 01 Dec 2015, the subject was diagnosed with "preerythrodermia" (dermatitis exfoliative). The subject was hospitalized on an unknown date for the event of dermatitis exfoliative. The subject's treatment included Prednisolon (prednisolone). Treatment with BKM120 was temporarily interrupted on 02 Dec 2015. The outcome of the event dermatitis exfoliative was reported as condition unchanged. The event dermatitis exfoliative was considered serious (hospitalization) by the investigator.

The investigator assessed dermatitis exfoliative as suspected to be related to treatment with BKM120 and concomitant medication Ibuprofen.

Following an internal review on 28 Jan 2016, of the reported information of 02 Dec 2015 the following changes were made: Diagnosis of brain metastases was added in the narrative, listedness of the event "preerythrodermia" (dermatitis exfoliative) was amended to unlisted (previously captured as listed).

Sender's comment:

Based on the information available, plausible temporal relationship and absence of alternative reasons reported, Novartis cannot exclude the causality of the BKM120 and concomitant ibuprofen with exfoliative dermatitis reported. Follow-up is ongoing.

Reporter's comment:

SANDOZ, 11.02.2016 DE-SZ09-PHHO2015DE020197

Sender diagnosis: not reported

Narrative

Case number# PHHO2015DE020197, is an initial report received on 02 Dec 2015 from a clinical study CBKM120ZDE01T. This report refers to a 45

year-old male subject (center ID: 0002; subject ID: 001) enrolled in a mono centric, open label, uncontrolled, single arm phase II trial of buparlisib in subjects with metastatic melanoma with brain metastases not eligible for surgery or radiosurgery.

Current condition included malignant melanoma and common cold. He was diagnosed with brain metastases in Sep 2014. The subject did not have any history of skin conditions, allergies and concurrent illness during the period of event. Concomitant medications included Keppra (levetiracetam), Dexamethason (dexamethasone) and Pantoprazol (pantoprazole sodium sesquihydrate). The subject started study medication BKM120 on 17 Nov 2015 at a dose of 100 mg QD (oral). On 28 Nov 2015, the subject started ibuprofen for the treatment of common cold at a dose of 500 mg PRN, and received most recent dose of ibuprofen an unknown date in Nov 2015. On 01 Dec 2015, the subject received most recent dose of study medication BKM120, and on the same day presented to emergency department with generalized itchy rash (grade 3). The subject was diagnosed with "preerythrodermia" (dermatitis exfoliative). The subject was consequently hospitalized on 02 Dec 2015, and treatment with BKM120 was temporarily interrupted on the same day. Ibuprofen was continued unchanged. On 10 Dec 2015, the event dermatitis exfoliative was considered completely recovered, and the subject was discharged from the hospital on the same day. The event dermatitis exfoliative was considered serious (hospitalization) by the investigator.

The investigator assessed dermatitis exfoliative as suspected to be related to treatment with BKM120 and concomitant medication Ibuprofen.

Following an internal review on 28 Jan 2016, of the reported information of 02 Dec 2015 the following changes were made: Diagnosis of brain metastases was added in the narrative, listedness of the event "preerythrodermia" (dermatitis exfoliative) was amended to unlisted (previously captured as listed).

Follow up report received on 03 Feb 2016: Updated co-suspect ibuprofen dosage details, hospitalization start and end dates, outcome and recovery date

Sender's comment:

Based on the information available, plausible temporal relationship and absence of alternative reasons reported, Novartis cannot exclude the causality of the BKM120 and concomitant ibuprofen with exfoliative dermatitis reported.

Reporter's comment:

NOVARTIS, 11.02.2016 DE-002147023-PHHO2015DE020197

Sender diagnosis: not reported

Narrative

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Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-15424627

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 68 Year	weight (kg): 65	BMI_int: 23.59	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin edema	201501				not recovered/not resolved
2 Haematoma	201501				not recovered/not resolved
3 Skin tear	201501				not recovered/not resolved
4 Skin desquamation	201501				not recovered/not resolved
5 Impaired healing	201501				not recovered/not resolved
6 Increased skin sensitivity					Unknown
7 Skin thinness	201501				not recovered/not resolved
8 Skin reaction					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen 400mg ibuprofen	Oral	400 Mg milligram(s)/ 245	- -	Migraine
2 IMODIUM AKUT LINGUAL loperamide hydrochloride	Oral	1 Df dosage form/ 245	- -	Product used for unknown indication
3 Triam 40 triamcinolone	Intramuscular	1 Df dosage form/Month	201411-201503 4 Month	Back pain

Drugs (concomitant)

4 BISOPROLOL RATIOPHARM 2,5mg bisoprolol fumarate	Oral	2.5 Mg milligram(s)/Day	- -	Cardiac arrhythmia
5 CETIRIZIN 5mg cetirizine hydrochloride	Oral	5 Mg milligram(s)/Day 245	201502-201505 3 Month	Pollen allergy
6 FAUSTAN 5mg diazepam	Oral	5 Mg milligram(s)/ 245	- -	Sleep disorder

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Fructose intolerance			Yes	
Back pain			Yes	
Sleep disorder			Yes	due to migraine attack
Latex allergy			Yes	
Pollen allergy			Yes	birch, alder, hazel
Multiple allergies			Yes	to alder, hazel.
Prolapsed disc NOS			Unknown	SUSPECTED
Cardiac arrhythmia			Yes	
Lactose intolerance			Yes	
Migraine			Yes	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Sender diagnosis: not reported

Narrative

This spontaneous report from a report was received from a pharmacist via the AMK (Drug Commission of German Pharmacists, AMK# DE-199160) and concerns a 68 year old female patient from Germany: local case identification number: 02750.15.

The patient's weight was 65 kilograms and height was 166 centimeters. The patient's medical history included suspected disc prolapse and concurrent conditions included: allergy (latex, pollen of birch, alder, hazel), back pain, cardiac arrhythmia, lactose-fructose-intolerance and migraine. The patient was treated with loperamide hydrochloride (tablets, oral) 1 dose(s) as needed, approximately 3 to 4 per week initiated on an unspecified date for an unspecified indication and ibuprofen (tablet, oral) 400 mg initiated on an unspecified date for migraine along with non-company suspect drug triamcinolone (ampoules, intramuscular) 1 dose(s) monthly initiated in NOV-2014 for back pain. Concomitant medications included bisoprolol fumarate for cardiac arrhythmia, diazepam for sleep disorder due to migraine attack, and cetirizine hydrochloride for pollen allergy. In JAN-2015 the patient experienced large skin alteration at the upper side of her forearms for the first time: epidermis got thinner (skin thinner), subcutis was swollen (subcutaneous edema), immediate hematoma after slight touch (hematoma), and skin rupture with 10 centimeters length (skin tear), skin detachment, poorly healing wounds. Treatment with triamcinolone was withdrawn and action taken with loperamide hydrochloride and ibuprofen was not reported. Since MAR-2015, the patient did not get triamcinolone injections but the skin did not get better. Then all "as needed" medication was withdrawn. Treatment with cetirizine was withdrawn. It was reported that when the patient occasionally ingested loperamide hydrochloride (1/month) she immediately experienced skin reaction and abrasive wound to only slight touch (increase skin sensitivity) on an unspecified date. The wound needed at least 2 to 3 weeks for healing. The patient had not recovered from subcutaneous edema, hematoma, skin tear, skin detachment, poor healing of wounds and skin thinner and outcome for increase skin sensitivity and skin reaction was not reported. The pharmacist suspected a causality between the drugs (loperamide hydrochloride and triamcinolone) and the reported adverse events.

This report was serious (medically significant).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-16053571

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Female Birth: privacy Age: 48 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Eyelid oedema	20151230		2 Week	2 Day	recovered/resolved
2 Ulceration of mouth	20151230		2 Week	2 Day	recovered/resolved
3 Hand rash	20151230		2 Week	2 Day	recovered/resolved
4 Giddiness	20151230		2 Week	2 Day	recovered/resolved
5 Exanthema generalised	20151230		2 Week	2 Day	recovered/resolved
6 Gastric spasm	20151230		2 Week	2 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibuprofen 600 mg Filmtabletten ibuprofen sodium	Oral	300 Mg milligram(s)/Day	20151229-20151230 2 Day	Disorder tendon

Drugs (concomitant)

2 antistax esculoside; vitis vinifera	/	-
3 biotin biotin	/	-
4 magnesium magnesium	/	-
5 vitamin b komplex calcium pantothenate; nicotinamide; pyridoxine hydrochloride; riboflavin; thiamine hydrochloride	/	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Disorder tendon			Unknown	Bänderdehnung Zeigefinger

Primary source

Qualification: Pharmacist observestudytype:
Literature:

number of reports: 144